

# Max Healthcare (MAXHEALTH)

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## Call: May 2023

Max Healthcare Institute Limited  
(CIN: L 72200MH2001PLC322854)  
Regd. Office: 401, 4th Floor, Man Excellenza, S. V. Road, Vile Parle (West),  
Mumbai, Maharashtra -400056  
Phone: +91 - 22-26101035,  
E-mail: [secretarial@maxhealthcare.com](mailto:secretarial@maxhealthcare.com),  
[investors@maxhealthcare.com](mailto:investors@maxhealthcare.com)  
Corporate Office: 2nd Floor, Capital Cyberscape, Sector -59, Gurugram, Haryana 122011  
[www.maxhealthcare.in](http://www.maxhealthcare.in)  
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National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,  
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Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on May 17, 2023

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements)  
Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings call , organized on May 17, 2023, on financial results of the Company for the quarter and financial year ended March 31, 2023.

The said transcript is also available on the website of the Company at  
[www.maxhealthcare.in/investors/investor-resources](http://www.maxhealthcare.in/investors/investor-resources) .

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Arora  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited  
Q4 & FY23 Earnings Conference Call Transcript  
May 17 , 2023

Moderator Ladies and gentlemen, good day and welcome to the Max Healthcare Institute Limited Earnings Conference Call.  
Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj from CDR India. Thank you, and over to you.

Suraj : Good morning , everyone and thank you for joining us on Max Healthcare Q 4 FY'23 Earnings Conference Call. We have with us today Mr. Abhay Soi – Chairman and Managing Director; and Mr. Yogesh Sareen – Senior Director and Chief Financial Officer of the Company . We will begin the call with opening remarks from the Management , following which we will have the forum open for an interactive question and answer session.

Before we start , I would like to point out that some statements made in today's call may be forward -looking in nature and a disclaimer to this effect has been included in the Earnings Presentation shared with you already.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi : A very good morning to everyone. I am pleased to welcome you to Max Healthcare 's 4th Quarter Earnings Call.

At the outset, I would like to state that this has been a seminal year for Max Healthcare on many accounts and an important one of them being the overall translation of Rs. 1,636 crore of EBITDA to Rs. 1,281 crore of free cash flows. Consequently, in spite of a decadal opportunity for investment in the hospital sector, we have been encouraged to declare our maiden dividend .

Q4 was a robust quarter for us and exhibited the results of commendable execution of our strategy by teams on the ground . This quarter once again reflected our best ever performance across nearly all the financial and operating parameters recording significant growth year -on-year.

We commissioned and operationalized a 92-bed oncology block at Max Shalimar Bagh from 1st March . This contributed positively to both revenue and EBITDA in its very first month of launch. EBITDA margin on the incremental revenue was in the range of 35 % to 40% due to operating leverage and overall occupancy at the hospital

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was 83% in Q4. We expect this to contribute to improvement in EBITDA , both in absolute and margin terms in the ensuing quarter.

Key Highlights of Q4 Performance:

Occupancy for the quarter improved to 77% from 68% in Q4 last year and remained at the same level as in the previous quarter. However, it is pertinent to note that operating capacity moved up by 100 beds in March 2023 compared to December 2022 .

Institutional bed share fell to 29% compared to 33% in Q4 last year and remained flat compared to the previous quarter. Again , there was higher capacity in the current quarter as well.

PSU tariffs for room rent and consults have been revised in mid -April. Further discussions are ongoing to increase tariffs for packages , diagnostics , etc. In view of these developments, we have not taken a hard call yet for some of the major accounts. At the same time, all our hospitals have been tasked with improving their operating occupancy thresholds to accommodate growth in the preferred channels. Network gross revenue was Rs. 1,637 crore compared to Rs. 1,298 crore in Q4 last year and Rs. 1,559 crore in the previous quarter. This reflects a growth of 26% year-on-year and 5% quarter -on-quarter . The year-on-year increase was driven by growth in AR POB and occupied bed days.

Revenue from international patients grew by 43% year-on-year and 10% quarter -on-quarter . This now accounts for around 9.1% of the revenue from hospitals and amounts to 120% of pre -COVID levels.

Digital revenue grew to Rs. 292 crore and accounted for 18% of overall revenue .

Led by improvement in channel mix and specialty mix , ARPOB for the quarter rose to approximately Rs. 70,700, reflecting a growth of 11% year-on-year and 6% quarter -on-quarter .

We reported our highest -ever network operating EBITDA of Rs. 437 crore compared to Rs. 304 crore in Q4 last year and Rs. 419 crore in the previous quarter, reflecting a growth of 44% year-on-year and 4% quarter -on-quarter .

The Network operating EBITDA margin stood at 28.2% versus 24.8% in Q4 last year and 28.3% in the previous quarter . Annualized EBITDA per bed, most importantly , rose to Rs. 70.3 lakhs, yet again our highest ever, clocking a growth of 25% year-on-year and 5% quarter -on-quarter .

Profit after tax was Rs. 320 crore versus Rs. 172 crore in Q4 last year and Rs. 269 crore in the previous quarter . The year-on-year growth of 85% was primarily attributable to improvement in operating metrics of all the hospitals and lower finance costs.

Free cash flow from operations stood at Rs. 425 crore , of which Rs. 65 crore was

deployed towards ongoing capacity expansion projects. The Net Cash position improved to Rs. 733 crore at the end of March 2023 compared to Net Debt of Rs. 441 crore last year.

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Continuing our efforts to give back to the community, we treated approximately 36,600 OPD and 1200 IPD patients from economically weaker sections of society free of charge.

Both our strategic business units continued to maintain their growth momentum.

1) Max@Home reported a top line of Rs. 37 crore, reflecting a growth of 26% year-on-year and 2% quarter-on-quarter.

2) Max Lab reported a gross revenue of Rs. 31 crore, reflecting a like-to-like growth of 57% year-on-year and 10% quarter-on-quarter.

Performance for the Full Year:

Network gross revenue stood at Rs. 6,234 crore, reflecting a growth of 18% on a like-to-like basis.

Network operating EBITDA stood at Rs. 1,636 crore, registering a growth of 25% on a like-to-like basis while ARPOB improved by 15% due to price and improvements in payor mix and case mix leading to margin expansion by 152 basis points.

EBITDA per bed grew by 22% year-on-year and touched a new high of Rs. 65.9 lakh.

Status of Expansion projects coming on stream by FY 2025 is as follows:

We have recently signed an ATS (Agreement to Sell) for purchase of land to enable

expansion of Max Vashi, which is consistently operating at more than 80% occupancy. If and when the deal is consummated, it has the potential to add 100 Brownfield beds to our network.

For 300 beds at Dwarka, interior work is in progress, lifts are under installation and external development has started. Medical equipment has been ordered. And as communicated earlier we expect to commission the hospital by end of Q2 FY24, subject to the developer obtaining occupation certificate by that time, of which we are quite certain.

For 329 beds at Nanavati the work is in full swing at the site. Foundation and column work have already begun. As you may be aware, Larsen & Toubro (L&T) is handling the project and we expect to commission the facility by the end of FY25.

For 300 beds at Sector 56 Gurgaon in Phase -I, D-wall work is complete and excavation is underway, while the civil contractor mobilizes for starting construction by end of June.

In order to obviate the delays at Max Smart caused due to tree transplantation issues, we are fast tracking the construction at Vikrant site, which is part of the same complex.

We have received in-principle environmental clearance (EC) approval and are expecting other approvals over the next 6 to 8 weeks.

Lastly, we continue to actively, but prudently, evaluate inorganic growth opportunities for strategic deployment of our cash surplus from operations. With this, we open the floor for Q&A.

Moderator: Thank you very much. We have our first question from the line of Ashwin Agarwal from Akash Ganga Investments.

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Ashwin Agarwal: Abhay, could you highlight what is your vision going beyond three to five years beyond what have you announced in terms of organic opportunities? And whether you would be looking at the South and the other regions in the country?

Secondly, you own around 24% stake now so would you like to increase your stake so that you can use the stock as an option in terms of acquisition, else you get diluted?

Abhay Soi: So, let me start with the last one first. Firstly, thank you for the compliment. The consequence of this performance has been that three years back when we were listing the company, one of the biggest critiques of the Indian Healthcare or the Hospital sector was that there are no free cash flows and you have to keep redeploying. I think with Rs. 1,281 crore of free cash flows translating from Rs. 1,636 crore of EBITDA which shows ~80% translation of EBITDA to free cash flows, I think we have answered that question. So, that is, as far as I am concerned, right up there as far as our achievements for the current year.

Now with respect to my vision over the next three to five years, I think there is a multi-decadal opportunity in the hospital sector. The kind of infrastructure, which is required to be created simply because you need to address that demand and we are seeing this every day. So all the cash flows that we are generating, we would be

redeploying it into the sector and we have been looking at pretty much every part of India.

We have been very clear about two things that we will only look at places where at least a few of our competitors have proven viability. We will go there and we will do it better, like we do in each one of the micro-markets that we currently operate in. We have not been the first anywhere, but we are the top performer wherever we operate. So, I think that is the strategy that we want to employ. So, that doesn't preclude any place, but of course we like a 'cluster approach'. If I have to just do one hospital in Kerala or just one hospital in Chennai or something I would not do it, we prefer clusters so we will look at chains and we will look at clusters.

Ashwin Agarwal: Anything on your stake in terms of using the stock as an opportunity in terms of M&A?

Abhay

Soi: So, look I think my stock, one of the consequences of our success is that it reflects on the market cap and the stock price. To buy anything meaningful, I would effectively have to leverage my current stock, which I am not interested in doing because I don't leverage my stock at all. You know 1% of the stock will cost me Rs. 500 crore, so pre-tax that money is about Rs. 800 crore. As my only form of employment is Max Healthcare, my salary does not permit me to buy more stock. But I would not be afraid of diluting if there is a great opportunity, let me just put it this way. When I say diluting, if there is an opportunity for a merger or an acquisition for the growth of the company, I would not shy away from the opportunity.

Ashwin Agarwal: Yes, that is what I was wanting to know.

Abhay Soi: There is no, and there has never been an issue as far as that is concerned.

Ashwin Agarwal: Would you be only city-centric in next three to five years in terms of opportunities or you would also look into Tier-II opportunities? And do you have opportunities like Nanavati which give you a gateway to any big city?

Abhay Soi: No, absolutely. I think, firstly, we are very happy doing Tier-II and Tier-III cities also. Our highest ROCE business is Mohali, second highest is Dehradun in Uttarakhand,

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it is not Delhi and Bombay. Like I said, we will go to any city where two or three of our competitors have proven viability. We have a list of 21 such cities.

Moderator: Thank you. We have our next question from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: Abhay, my first question is on your payor mix change plan. So, you mentioned we have seen upward revision in PSU tariffs etc., because of that is there any rethinking from your side to bring the institutional bed share to 15%-16%, which you highlighted earlier from 29% currently?

Abhay Soi: So, look, I think the trajectory is changing on account of two or three things. One is that we are finding marginally more capacity, we are operating at higher levels than what we were thinking of previously, so we are able to keep that business and be able to do more and you are seeing that in your margins month-on-month. The best thing is that if you can, we were pushing down that business because we wanted to accommodate our preferred channel of CTI. Now necessarily you are always going to find some elasticity towards the end, so you see the operating levels becoming higher.

Secondly, we have added 100 more beds, which is about 3% more capacity that has been added. Yet you see that the PSU number has still come down marginally.

The third thing is that there has been an increase in rates as far as PSU is concerned, an increase in tariffs, which will work well for us in the current year. That is about 20% or 30% of the total tariffs that they have revised. They have revised it quite significantly by about 70%. So, only four line items, but that amounts to about 20% of the total, 20%? --

Yogesh Sareen: The overall increase would be around 4% to 5% on the billing to the PSUs.

Abhay Soi: But the rest of the tariffs are looking to be revised in, they are saying by July. So, I think we have a little bit of a wait and watch here. Because if we have the same sort of increase by July, it has been proposed that rest of the packages and everything else is being revised. This is what we have been informed.

Damayanti Kerai: Just to clarify, this 4% to 5% tariff hike is for the PSU contracts so far and you are expecting ...

Yogesh Sareen: So, what I am saying is that for the line items that they have raised prices on, the average price increase is around 70%, but in a way the contribution of those line items is around 10% to 12%, so that means the overall increase would be 4% to 5%.

Damayanti Kerai: Okay, and in July, another round of price hike?

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Yogesh Sareen : Yes, we are expecting another round of price hike.

Abhay Soi : For the rest of the items.

Yogesh Sareen : They have not touched the diagnostics, they have not touched the packages, they have not touched the other elements of the blood bank etc. They have not touched that. They just touched four items : one is room rent for ICU beds, room rent for single room, and IP consultations and OP consultations. Only these four elements have been changed.

Abhay Soi : And whatever they have touched has gone up by 70% on average.

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So, let us say there are 100 things and the revenue is 100, only 12% of the revenue has been touched by them. They have increased that 12% by about 70%, and for the balance 88%, we are expecting something to be done by July.

Damayanti Kerai : So, my second question is on your average revenue per occupied bed. A gain, I think you have surprised positively quarter-after-quarter. In the last two years, we have seen around 15% increase. And I understand specialty mix change is the big driver of it. So, can you explain what has changed so significantly in last two-three years that you continue to see better and better specialty mix and how should we see this part moving ahead?

Abhay Soi : I think your ARPOB is being generated by payor mix and specialty mix. It is not only purely payor mix. We have also seen a massive increase in international business, which plays on the ARPOB. It is a combination of factors. During the last three years, you are looking at COVID in the middle, right. So I think your current ARPOB vis-à-vis a COVID year is going to be higher, and you need to compare it to pre-COVID years. Then you take up, perhaps, the cost increases over this latent period of COVID, because the disease burden has come back after COVID. In the middle, you are looking at COVID business, which was high occupancy but lower ARPOB. Now you have business, which is essentially high ARPOB, but your occupancy has moved up because there was not any capacity creation during the COVID years. So, I think you are going to see ARPOB move up, but obviously you can't do a comparison with the COVID year.

Damayanti Serai : Not even with COVID year, so if I look at the period of FY'19 before COVID, you were somewhere at Rs.46,000 to Rs.47,000 ARPOB, and now it has moved up to say, Rs.70,000 plus. So, I am asking from that perspective.

Abhay Soi : I will just let you know what the FY19 ARPOB was. But as you move up the occupancy curve, you are going to distill your payor mix. Now you are talking about a time when my PSU payor mix was 40% plus and now it's 29%, right. I think as you go up the curve, you start getting that operating leverage when you start getting that better quality of business. We still have hospitals in the portfolio that are doing Rs. 90,000 + ARPOB.

Yogesh Sareen : So, our FY'20 ARPOB was Rs. 50,300.

Abhay Soi : So Rs. 50,300 has gone up to Rs. 70,700 in three years now.

Yogesh Sareen : The average is Rs. 67,400 for FY'23. So, if you take year-to-year average, it will be 10% to 11% higher.

Abhay Soi : So, it's 10% to 11% increase right, from Rs. 50,300 to Rs. 67,400 you are saying.

Yogesh Sareen : Yes

Damayanti Serai : So, going ahead also say since we continue to see better mix both in payor and specialty part --

Abhay Soi : I think from distillation of payor mix itself, you should see an increase in ARPOB. For our clinical mix, of course like I said, there is always innovation going on. It's not only at Max Healthcare, but we have plenty of single hospitals in Mumbai, for example, which have been there for 20 odd years, which have no payor mix lever as it has sort

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of saturated. They are not adding an extra bed, not a square inch that they have been able to add, yet the clinical mix sort of drives ARPOB up. I mean, there are lovely exam

ples

of that -- Hinduja Hospital, Breach Candy Hospital and so on and so forth.

Damayanti Kerai : So, comfortably, we should be seeing say high single digit to say double-digit growth in the ARPOB going ahead also?

Abhay Soi : I avoid giving any forward-looking guidance.

Damayanti Serai : And my last question is on your view on the competitive landscape in Delhi NCR market given we have seen many of your competitors trying to step up their presence. So, is there any possibility of bed oversupply in say foreseeable future, if not now?

Abhay Soi : I have not seen any new beds come up in Delhi NCR or even under construction.

Damayanti Serai : Actually , I think Apollo has a facility coming up in Gurugram so –

Yogesh Sareen : This facility was already there by the way; the first floor is already constructed. I think Apollo has just done an agreement ; I mean it was not operating but I think this building has been there for quite some time. So, let us see when it starts to operate, it is not yet operational, right?

Damayanti Serai : Yes.

Moderator : Thank you. We have a next question from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur : My first question is a kind of a clarification , when I look at the audited cash flow statement , the CAPEX incurred under the line item of purchase of property is around Rs. 335 crore whereas in the proforma numbers that we talked about in the executive summary, the company has given a CAPEX number of Rs. 208 crore. I guess there might be some technicality here. Can you please explain me why this difference in audited and executive summary?

Yogesh Sareen : So, just to clarify that point. When we report the Rs. 208 crore spend, that spend is on the capacity expansions, on the projects. So, whatever is the routine CAPEX in the cash flow , it comes in the property line, but eventually when we report numbers to investors we take it out from the free cash flows. When Abhay says that we have Rs. 1,281 crore of free cash flows against Rs. 1,636 crore of EBITDA , in that Rs. 1,281 crore whatever amount that is spent , the total network spend is around Rs. 211 crore on the routine CAPEX and that amount is taken out of the free cash flows . We only report the number of Rs. 208 crore , which is on the capacity expansion, basically on our ongoing projects. There are two type of CAPEX spends, one is the routine CAPEX , which is replacements happening in the running hospitals . And the other one is CAPEX for the capacity expansions . Rs. 208 crore is the number for the capacity expansion and whatever is the number for the routine replacement in the running hospitals , it is taken out from the operating cash flows.

When you say cash from operations , this number is already netted there, but in the audited financials this number , by virtue of the fact that if cash flow is as per a particular Ind -AS requirement, both numbers flow into that number.

Nikhil Mathur : So, Rs. 335 crore is the right number to look at, when we are looking at on a proforma basis?

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Yogesh Sareen : No, that number will be around Rs. 419 crore vis-à-vis what you see is only consolidated financials, right. These consolidated financials don't have the partner healthcare facilities (PHFs) number. The overall number , if I take the network cash flow, will be Rs. 419 crores of which Rs. 211 crore would be routine CAPEX and Rs. 208 crore will be capacity expansion.

Nikhil Mathur : In the cash balance, there is a difference of Rs. 100 crore again in the audited and the proforma balance sheet that you have given , some technicality here as well --?

Yogesh Sareen : So, if you read the very first page that we have put out there in our investor or earnings update , you will find what the difference is for . We have the PHFs where we control the medical operations through the Hospital Management Committee; we

have a 3 :2 ratio so we control the operations. Since it is a medical services agreement , we are not able to consolidate the PHF financials with the Group financials. So, what we do is we do a memorandum consolidation for the investors, get it certified and that's how we report the numbers to the investors. So, if you read the very first slide that would clarify this doubt that you have.

Nikhil Mathur : On the bed expansion plans, I think in the last investor presentation there was the page that you share regularly now that Shalimar Bagh has come on stream, we should refer to that bed expansion plan in the previous investor presentation , that still holds or how should we look at the expansion plan ?

Abhay Soi : That is right. Except for the Max Saket (Smart) site – as we said there is a little bit of delay there , everything else is going to be in line .

Nikhil Mathur : And what's the CAPEX plan for FY24 both routine as well as ongoing projects?

Abhay Soi: Routine would be Rs. 170 crore and ongoing projects would be Rs. 900 crore.

Nikhil Mathur : So, Rs. 900 crore plus Rs. 170 crore ?

Yogesh Sareen : Rs. 900 crore would be for the capacity expansion , which is on the ongoing projects , and around Rs. 170 crore will be on the routine.

Nikhil Mathur : On the international foot falls, how should we look at growth in this business in the coming 2 to 3 years? Now FY 23 has been pretty strong at 43% kind of growth. Do we expect to grow in FY24 as well on this particular base? And not just short term, what are your thoughts on the 2 -to-3-year horizon as well in the growth of international business ?

Abhay Soi : I think I have said on numerous occasions before that there is an exponential opportunity, and what we are seeing is only incremental . And now the Government has put its weight behind it. Hopefully , we will be able to tap it in the medium run, but in the short run we will still see incremental gain. I am going to avoid giving you any sort of guidance on the number , like I avoid on every other financial parameter as well. But we believe this is a space where we have not even hit the tip of the iceberg.

Nikhil Mathur : I also read somewhere , I don't know how true this is, that Delhi NCR accounts for almost 70 % to 80% of international foot fall that come into the country.

Abhay Soi : 40%.

Nikhil Mathur : And one final question, the tariff hikes whether on the PSU front or whatever you are contemplating on the packages side, is it a reflection of supply demand balance

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being in favor of private hospitals at this point of time in Delhi NCR and do you think we foresee that this is likely to be an industry phenomenon not just specific to Max?

Abhay Soi : No, this is an industry phenomenon . CGHS rates are for the whole country.

Nikhil Mathur : This is in response to the news that had come in that government has allowed some price hikes on the CGHS side .

Yogesh Sareen : Yes, and by the way, the last price was fixed in 2014 . It is a revision after 9 years, in a way .

Nikhil Mathur : But you are also considering revising prices for the packages, I mean, which is --?

Abhay Soi : No, that is also government.

Nikhil Mathur : That was on the government side.

Yogesh Sareen : Yes, so they just revised the prices for four elements . Other billing items for example, pathology, radiology, blood bank, packages, surgeon charges etc. all those things are under consideration –

Abhay Soi: Lot of things we do under package right? It is not itemized billing , say in case of CABG or C-section . They have not increased the C-section or the CABG package rates . They have increased the room rents, the doctor visit charges, etc.

Moderator : Thank you. We have our next question from the line of Lavanya Tottala from UBS.

Lavanya Tottala : I just wanted to get a clarification on these packages , so is the tariff hike only related

to CGHS packages or is it something related to diagnostic business of Max also?

Abhay Soi : No, so right now they have not increased the diagnostics . Like Yogesh mentioned, out of only 12% of the revenues or line items that account for 12% of the revenues , price increase has happened over there and it is up to 70% increase . For balance 88%, which includes diagnostics , includes packages , and includes a lot of other things , has not happened . It is under consideration by the government and we have been told by July there may be some visibility on this.

Yogesh Sareen : Ma'am, do not mix the CGHS price increase with the other price increase . There is 29% bed share by PSU where the tariff is given to us by the state , Central Government actually. So, we are talking of that element of the business. For the other business, we do increase prices. We have increased pricing from 1st April. We also increase the prices with insurance companies as and when their contracts become due . So, just do not mix the two .

Lavanya Tottala : So, that's the reason I am just asking for clarification that this is everything the discussion is related to CGHS and nothing related to the Max --?

Yogesh Sareen : That's right.

Lavanya Tottala : So, Max Diagnostics are we planning to put any price increases for just diagnostic business -- in general Max diagnostic business ?

Yogesh Sareen : There are two elements to the diagnostic business: one is in-hospital diagnostics and the other is outside retail business . For the in-hospital diagnostics , we do increase prices every April and we have done some bit of it this year as well . For the non -

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captive pathology business or the retail business , we have not increased any prices there.

Lavanya Tottala : And on the ARPOB I understand that increase in ARPOB is supported by higher international contribution this quarter. So, do you expect an increase in international business in the coming year FY24 or do you think that we have reached the optimal level in Q4?

Abhay Soi : So, I think I have answered this question earlier. Like I said , there is an immense opportunity , exponential opportunity in the medium run . In the short run, it is still incremental. Nevertheless, I think it is clearly going to be incremental. It is nowhere near the peak . I mean with the comparative advantage that we have , there is no reason for it to slow down . It should only snowball and become much larger.



Lavanya Tottala : So, I just wanted to check because we have already crossed the pre -COVID level, so it should be incremental from here, but not a jump or significant jump that we have seen over last few quarters. So, I just wanted to check that.

Abhay Soi : It is a 10% growth quarter -on-quarter in international . Even pre-COVID you had a very good clip of growth. If you look at even pre -COVID levels , the growth rate was not slow. I mean actually 2 years later to go up 20% over pre -COVID levels means you still haven't caught up to the rate of growth.

Moderator : Thank you. We have our next question from the line of Andrey Purushottam from Cogito Advisors .

Andrey Purushottam : I just had a question on the 87% increase in PAT on Q4 '23 compared to '22. I can make out that there has been an increase in international business , I can make out that there has been a decrease in institutional business , I can make out that there is an increase in occupancy. Could you give some more granular insights as to what are the elements of the channel or payor mix that have contributed to this 87% increase ?

And if you think that many of these profit drivers will remain in the future.

Abhay Soi : You want me to describe this 87% increase impact?

Andrey Purushottam : Yes.

Abhay Soi : Every line item before that?

Andrey Purushottam : No, not every line. I am just saying I have identified , as I said , 3 of those factors.

Could you just give us a broad sense of which are the other specific factors when you channel mix and payor mix that have additionally contributed to this 87 %?

Abhay Soi : It is payor mix, it is clinical mix , it is better cost management, lower consumption ratios , lower finance costs. I mean it is pretty much everything, but cost management other than payor mix and channel mix .

Andrey Purushottam : Anything specific in payor mix, apart from what I have outlined --?

Abhay Soi : In payor mix, there is a reduction in PSUs , while the cash, insurance and international business has grown . Effectively , it is not about reducing PSU, but it is about increasing these three other elements.

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Andrey Purushottam : And you think that the operating margins of 28.2% or so are maintainable in the future ?

Abhay Soi : I focus more on EBITDA per bed, not operating margin , because you would rather do a \$10,000 surgery and have a 20% margin than do a \$2 ,000 surgery and have a 50% margin, right. I think the question really is that the EBITDA per bed, even if it means lower margins tomorrow , going to increase tomorrow or not, because that is really what matters from EBITDA overall growth standpoint as well as ROCE standpoint . That, in my mind, will grow. We will continue growing .

Yogesh Sareen : We have the numbers Quarter 1 was 26.6%, Quarter 2 was 27.7% , Quarter 3 was 28.3%, and this quarter it is 28.2%. So, obviously that means it has been consistent , right?

Abhay Soi : Over last quarter, there has been an increase in EBITDA per bed, but if you see the margin , it is marginally lower . But we are happier about the fact that EBITDA per bed has increased. That is why your overall EBITDA has increased .

Moderator : Thank you. We have our next question from the line of Prakash Agarwal from Axis Capital.

Prakash Agarwal : Just trying to understand this 4% to 5% price increase is limited to the 17 % to 20% of the total mix, is that right?

Yogesh Sareen : 29% of the beds are occupied by the PSUs , which constitute around 17 .5% of the revenue right . Now out of this 17.5 % --

Prakash Agarwal : 17.5% in value terms , which is about 4% to 5% in terms of pricing .

Yogesh Sareen : Yes, so I would say that if nothing else happens , then there is a 4 % to 5% increase in the price. Now , we do expect price increases in the other elements because you cannot increase the room rent only for open surgery . You have to increase the packages as well . The moment you admit that the room rent has to go up that means it has to go up for the room rent component under packages as well .

Prakash Agarwal : So, the current increase is 4.5% on 17.5%?

Yogesh Sareen : Yes, that is what I am saying.

Prakash Agarwal : And you also mentioned that every year you take some price hike around April. So, that is more standard across the board in the hospital business ?

Abhay Soi : That is right, yes .

Prakash Agarwal : So, that is 2 % to 3% or it's like a high single digit kind of increase ?

Abhay Soi : No, it is 2% to 2.5%.

Prakash Agarwal : So, what I am trying to understand, we had fairly good ARPOB increase last year

and couple of years , just in terms of headroom of growth we do have international lever , we do have case mix , payor mix lever s. So, then 10% to 12% is not way to off to model for this year as well , would that be right?

Abhay Soi : I am going to avoid giving you guidance on that Prakash , like I always have. But there is no reason for it to not climb.

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Prakash Agarwal : And secondly on the volume side , occupancy side on a blended basis is 77% and we would be having few hospitals , which are 80 % to 85%. So, I am just trying to understand the headroom in terms of , you mentioned that you have focus on growth in the preferred channels A, which is more on the pricing, but immediately how can

we improve the occupancy or we are already operating at an optimal level of occupancy?

Abhay Soi : No, look I think that at certain places where you have absolutely hit the capacity , over there you will automatically start distilling. The levers that we have are the following : one is that at the very sort of end when you start pushing the envelope, there is some elasticity in terms of operations . We were earlier at 75% to 76% and now at 77% to 78%, but we also operate at 80% to 81%. So, each one of these facilities will have more efficient system of discharging and so on and so forth when it starts coming to the edge.

Second is, we have mentioned that about close to 100 odd beds internally that we are unlocking : 10 beds here, 20 beds here, 30 beds here, so on and so forth. 100 beds is about 3% of the total capacity. A couple of percentage points through occupancy and that gives you about 4 % to 5%. On top of that, we also have the payor mix distillation that we can do as and where necessary.

Third , finally – but what you want to do is first you want to maximize on the first two, then you want to move to the payor mix distillation and – then finally you have another 300 beds coming in. Then by next year, nearly every hospital will have its own beds coming.

Prakash Agarwal : And this CARE Hyderabad one is finally not happening right?

Abhay Soi : Well, I will not say that because we have sued for specific performance and the matter is in arbitration , it is sub-judice so I will not delve into it . But , we have asked for specific Performance over there .

Prakash Agarwal : Okay, because I read Blackstone is getting into that so I was just checking on that, okay. Lastly, on the regulatory side off late there has been news flows on having reservations for the government as well as the poor and the needy. I mean there is too much unverified information. So, is it happening somewhere ? Is it already happened? It had already happened, you know, in the past and it's just a rehash or if you could just --?

Abhay Soi : I think you rightly put it . Firstly, I think there is way too much being made out of something way too little. First and foremost, please understand that there is already a Right of Health across the country, through a Supreme Court direction . If a patient comes dying or a trauma patient comes into hospital you can not say show me your wallet first. You have to take him in and stabilize him . That is Right to Health even as per Supreme Court . No hospital would ever even think of not doing that . I mean, really, if somebody was to do that you will read about it in the papers . That is how rare it is. And you have seen those situations before in the papers, etc. No proper hospital will you know deny a patient.

Let us say somebody comes to a hospital , it is an accident patient and a number of these patients come year on year . If we were to actually reject that patient – firstly, it is against our own policy . It is against the Supreme Court strictures . More importantly , if you do something like that , the media will string you, it will hang you. It is inconceivable to do any of these things.

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Secondly, most states like Delhi NCR, Delhi , particularly, already for the last 10 years has the Delhi Arogya Kosh (DAK), which basically means that any accident victim comes over to your ER , DAK will pay you some money for it. Now the fact is in any case we were doing it free.

Now in Rajasthan, they pretty much rehash ed the same thing and they said no, no Right to Health. Right to Health in emergency , which already exists.

Prakash Agarwal : So, this is limited to emergency and not routine surgery etc.?

Abhay Soi : No, only emergency . Even in Rajasthan, it is purely emergency. In fact , they have diluted it down by saying it is not applicable to the private sector, it is only applicable to hosp

itals , which have been funded or subsidized by the state government. That is the clarification given by the Rajasthan Government , which itself doesn't make sense because it is not as if at a private hospital when somebody goes , then you are going to reject the patient . You would still do it free.

Prakash Agarwal : Just one last one on the trust hospitals we also do a set reservation for the poor and the needy. Is that right?

Abhay Soi : That is right. Every quarter when you see our results , you will see those numbers being announced .

Yogesh Sareen: So, not only trust hospitals but also some of the hospitals where we have bought this land in auction, in the listed company , there we have some obligation to have 10% beds free and we do provide for them.

Prakash Agarwal : Lastly, congratulations for Shalimar Bagh – turning it EBITDA positive in month one itself in March .

Abhay Soi : Yes, EBITDA positive in the first month and giving a 35 % to 40% EBITDA margin on the incremental revenue . This is something that we have always spoken about – Brownfields do not suppress your EBITDA , in fact, they are accretive . We have always said that they will break even in the first quarter or two and obviously, we want to give you a more conservative estimate . Now you have seen the operating leverage come in.

Moderator : Thank you. We have our next question from the line of Tushar Manudhane from Motilal Oswal.

Tushar Manudhane : On PSU tariff revision , how much of a spread would get reduced in terms of EBITDA per bed with this price hike compared to say non -PSU or non -institutional patient EBITDA per bed?

Abhay Soi : It will actually rise per bed, because of the price hike.

Tushar Manudhane : No, the perspective here was that in the hospitals where we are already running up at a very high occupancy. That way we have a choice in terms of selecting the patient or distilling the patient as you pointed in the earlier commentary. So, just trying to understand is this sufficient enough to reduce that spread for EBITDA per bed through the patient payor mix?

Abhay Soi : No, it will not. But what happens is that if tomorrow I want to switch off 100 beds, yesterday I only needed to fill 40 out of those 100 PSU beds to breakeven and to make back the money that I am losing by switching off those 100 beds . So, I would

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get 60 more beds to work with . Now that number from 40 has gone up to say 55, tomorrow that number will go up to 70 or 80. So what happens is that it becomes marginal , but does it get to the same amount? No. So yes, I think what you are going to see is that your trajectory sort of changes, but directionally you are going in the same direction . Albeit, you are doing it without impacting EBITDA , and well it's actually a more preferred approach since you are taking a fiscal call on the basis that it is going to be more accretive with these new numbers rather than less.

Tushar Manudhane : And if you could also clarify this tariff, the proportion of PSU patient pool is higher in which hospital or rather which cluster of hospitals of Max healthcare?

Yogesh Sareen : We do not give hospital wise data. You have to understand that the hospitals , which are fuller , there the proportion would be lower and where it is less full, they will be having more --

Abhay Soi : For higher occupancy , we have obviously moved away from that right. The lower occupancy places is, like Yogesh said, where you have higher amount of PSU. So, anywhere where your occupancy is getting higher , say to 80% to 85%, you start moving away from this business, you start distilling it over there . You do the business where you have idle beds.

Tushar Manudhane: Lastly on this Gurgaon thing is no more land coming up for auction.

Abhay Soi: No.

Tushar Manudhane: Because we have seen other hospitals also having some land parcel in this area.

So, is there more area available for hospital expansion as such or this is more or less done from the government side ?

Abhay Soi : Look, I think it is like Mumbai , right . I think you can have Central Bombay and you can have some place in Mulund . If you look at Gurgaon , the question is where in Gurgaon , Gurgaon extension , northern extension , etc., but where exactly. On the main Golf Course Road , there is no land available.

Tushar Manudhane : So, the intention to understand was within that particular area where the hospital space is supposed to be sort of premium so in that space, there is no land parcel available?

Yogesh Sareen : Not that we know of , yes .

Moderator : Thank you. We have a next question from the line of Raj Rishi from DCPL.

Raj Rishi : Just wanted to find out if presently obviously the situation is great for you guys. How long do you think this supply won't come in looking at the present fantastic scenario? Like, right now the demand supply gap is huge, which is why your occupancy is what it is. So, whenever the situation is so robust, obviously new supply is going to come in – in various geographies.

Abhay Soi : Not necessarily , I mean no new hospital has come up in Mumbai in 20 years, because there is no land available. No new hospitals have come up in Delhi for 12 years, maybe one, because no land is available. How do you get land in these metros and land at viable cost, right ? If you want to buy land, you have to understand there is a height limit of 45 meters when you set up a hospital. If you want to set up a 400 - bed hospital, you need 4 to 5 acres of land, contiguous land. Where do you get it in Delhi and Mumbai ?

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Raj Rishi : That's right.

Abhay Soi : One of the biggest plus points that we have is that we have this land in our network, in our system.

Raj Rishi : And Abhay you had mentioned medical tourism going up many times by 2030. So, how do you see it, like what is the size presently and where do you see it in 2030 for the sector I am asking?

Abhay Soi : You know, if you are saying 2030, there is such a massive comparative advantage that we have in India. I mean we are less than 5% of the cost of the U.S. We are less than 30% of Singapore. We are less than half of Thailand. I mean, look, it is not like rich people from the U.S. will start coming to India. But in my case that's the largest part of the world pyramid , where the bottom is all self-paid. And for them, affordability is an issue. This has the potential of surpassing the IT sector. That is the amount of comparative advantage that we have and no other country has such advantage . I think because the comparative advantage that you hold in this space , no other country even comes close. While in IT other countries may come close, here there is nothing. The sheer volumes and complexities our doctors handle , what they do in a year, the Western counterparts do not do in a decade. Our skill sets are right up there and our cost is fraction of the global cost . Aging population, rising costs, most government treasuries going bankrupt , this is going to be something else. This is not even the tip of the iceberg right now.

Raj Rishi : And Abhay another thing, just a thought , like generally when private equity guys are so heavy on sector it seems that it's sort of topping out when smart guys like Emami or Manipal dilute and private equity guys buy. It's just a thought, I mean what are your comments on that?

Abhay Soi : No, I do not disagree , I think you had hospitals and hospital chains change hands many times. For Manipal, I think this is the 6th or 7th private equity round that they have done in Manipal Hospitals over the last 10 -12 years. If I look at, it is not as if Max was not up for sale, we bought Max. It is not as if other hospitals were not up for sale. Those transactions also happened in the

past.

I do not think it is a question of tapping out. I think there is very clearly a multi -decadal opportunity to invest in this sector in India, which is much more true today , but it's not something which was lost in the past either. The only difference was the biggest critique and I remember when we were listing the company in 2020 , now most of you will bear this out, right. The biggest critique of this sector was that the Indian Healthcare sector is a great investment but it is a mirage because there are no free cash flows. How do you invest? You have to keep raising money and keep reinvesting.

We have Rs. 1,281 crore free cash flows coming with Rs. 1,636 crore EBITDA. That is almost 80% throughput of the free cash flows. And you are seeing this , we are not raising any capital. My entire 2,900 beds is at a cost of Rs. 4,500 crore and that should not even be 50% of my free cash flows over next 4-5 years. And that's on a debt free balance sheet. Today this is the compounding. There is a massive multi -decadal opportunity to invest , I think, and to create infrastructure, so PE and all sorts of funds are going to be coming in.

Raj Rishi : Just one last question , how do you see this asset -light model? How heavy can it be for you guys ?

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Abhay Soi : Sorry, just understand one thing . A PE puts in money at valuations and they need to underwrite 20% to 25% IRR in dollar terms. So, only because they are seeing this opportunity and I believe Manipal for example was probably done at 29 to 30 times

2024 EBITDA. Those guys are expecting to do 2 5% CAGR from there on dollar. On the contrary, I think when PE's come in they see opportunity.

Raj Rishi : One last question about the asset-light model, how heavy can it be for you guys? Like are you thinking of increasing that part of the business?

Abhay Soi: Well, I think quite heavy from that standpoint. I think over the next couple of quarters you will be coming across some announcements from us on this front. This is really the way for us to grow particularly where we are looking at any form of Greenfields. We do not like to build on our own. We like to collaborate because it decreases the construction and development risk, particularly in geographies that we are not currently present in. So, I think you are going to see some of those come through now, because that's going to be a big focus area for us. We have a 36% cash-on-cash ROCE, so we can afford 8% yield to developers.

Moderator : Thank you. We have a next question from the line of Neha Manpuria from Bank of America.

Neha Manpuria : Abhay, you mentioned improvement in specialty mix quite a few times. And if I were to look at the chart maybe through the profile obviously there is improvement in oncology, a little bit in Neuro, Cardiac. So, are the beds that we are adding essentially focusing on doing more higher ARPOB work like adding more Onco beds etc. or are we seeing the kind of work that we do within Cardiac, within Neuro itself increase that is improving this specialty mix. So, how do I differentiate between the two?

Abhay Soi : I think it is a combination of the two. First and foremost, if you look at capacity spread and you look at overall ARPOB, the ARPOB that you see of Rs. 70,700 is a summation of ARPOBs. When you see our occupancy is 77%, it is a weighted average. It means there may be some hospital, which is operating at 70% occupancy. There may be another one, which is operating at 90% occupancy. Similarly, in ARPOB you will have certain departments that are low ARPOB. You will have a certain payor mix that is lower ARPOB. Yet, you are hitting the capacity thresholds. Firstly, if you are hitting capacity thresholds, then you are going to look at expanding over there. What you would seek is that if today I am creating more capacity and I was to take my lowest payor mix and my lowest ARPOB specialty

specialty

there, that is really what I am creating it for. Any expansion that you are doing is serving the lowest common factor because that is what you are doing it for. Otherwise, if you were not to do that brownfield expansion, what you are going to do is you are going to dilute your payor mix and you are going to dilute your clinical mix.

Neha Manpuria : But it could also be I am asking this one is let's say I am adding more Onco beds or I am adding more OT or ICU beds because of the improvement in the clinical --

Abhay Soi : What is an Onco bed? You know a lot of our beds are fungible, except for some daycare beds etc., but the beds are all similar. I mean in Cardiac, for example, there may be a Cath lab, but the OTs are similar. It's sort of quite fungible within the hospital, you have to understand that. We may segregate it and we may do some clinical programs there. But the way I will value it, please understand this, is that say I have 100 beds, of which say 50% beds are very high ARPOB and 50% beds are low ARPOB, and they are fully occupied. Then when I look at creating another 50 beds, the way I will calculate it is to take my lowest ARPOB of 50 beds and to put it

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over there. Alternatively, my opportunity is to just play the lower ARPOB out and occupy my current facility.

Neha Manpuria : So, essentially that would mean you either add a new specialty or add doctors, which will allow you to remove that lower ARPOB work, right?

Abhay Soi : No, even if you don't do that and a lot of that stuff could be aspirational, right. Today when you have a waiting list of two or three days in your ER for patients, you know there is unsatiated demand at your doorstep. I mean take Shalimar Bagh's case in-point. We put up a new Onco block over there, so what happens immediately is that the oncology patients from our current facility or the pre-existing facility move into the new Onco block. So, what happens to the old block? Our high ARPOB business may have gone out but the lower ARPOB business has started climbing up over there. When I evaluate setting up 100 beds in the new block, I have to evaluate vis-à-vis the low ARPOB business and not the high ARPOB business, even though I may be doing it for the high ARPOB business. Yet what you see is that even without adding much doctors, etc., within the first month we broke even and hit a 40% EBITDA margin. We have overall an occupancy, now this is what matters, of this incremental facility as well as the existing facility, of the entire hospital of 83%. I mean, I am not saying it is 83% of the new 100 beds, I am saying it is 83% of 300 beds.

Neha Manpuria : The reason why I am asking this is for example we added the new Onco block, so that's not necessarily increasing my entire Onco contribution quarter -on-quarter. It could just be moving patients to the new block to unlock sort of free capacity to move on.

Abhay Soi : There is some increase, but really it is not a jump increase that happens because you have opened a new block. Of course, the new Onco block will have a disproportionate increase in Oncology, but it is not as if other services are not increasing.

Let me put it this way, let's say a hospital of mine has 50% PSU business and 50% other business and is running at full capacity. When the question comes about expanding over there, when a note is put up to expand, my first question is whom are you expanding for? Are you expanding for PSU? Because alternatively you could be distilling the PSU. So, for that expansion, whatever specialty that goes there is irrelevant. You obviously put up the specialty with the highest ARPOB, which will be more accretive etc., but it has to be evaluated looking at the lowest opportunity.

Yogesh Sareen: Neha, the hospitals are passed on both fronts: one is that within this specialty they have to work on

increasing the quantum of procedures, which are high teen procedures. Then they are also supposed to allocate more beds and fill up more beds for high ARPOB specialty. So both things play into it and that's what the hospital's job is.

Neha Manpuria : And is there any way for us to sort of segregate as to how much of our ARPOB increases come, let's say from an improvement in specialty --?

Yogesh Sareen : So, I can tell you that say the Quarter 4 increase is 5%. Around 2% of this is because of the procedures within the specialty, which means that with the same number of beds for each specialty, the specialty ARPOB has gone up. Around 2% of this is because of relative change in the Occupied Bed Days ( OBDs ) to the specialty. The Occupied Bed Days of Oncology and Cardiology, etc. have gone up, and they are high ARPOB specialty. The mix has changed of the beds - that is 2% impact and 2%

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impact is that within the specialty the ARPOB has gone up. I am talking about overall increase.

Neha Manpuria : The entire quarter -on-quarter increase.

Abhay Soi : Yes, I am saying 5% increase, 2% is from here, 2% from there.

Moderator : Thank you. We have a next question from the line of Amit Thawani, an individual investor.

Amit Thawani : My first question is can you tell me what the price increase that we have got in by the PSUs?

Yogesh Sareen : So, Amit there are four elements where the prices have increased : one is on the ICU beds charge ; second is the normal ward bed charge ; third is the I P consult s and fourth is the OPD consult s. OPD consult has gone up from Rs.150 to Rs.350, IPD from Rs.300 to Rs.350. These are the prices change s. So, overall average if I take , it's a 70% price hike we have got. This is after nine years; the last one was in 2014 . Now the point is, this is only a small quantum of the line items that get billed to the patients in the PSU. So, we do expect that there will be further price increases by the government because if the room rent prices increase, that means that the room rent, which is part of packages will also go up, right . Let us say cardiac surgery, there is 12 days stay and there is a package charge that we charge to the PSU patient, that should also go up. The CGHS authorities have put up a note to Finance Ministry, etc. so we expect that there should be a decision on that by July.

Amit Thawani : So, just trying to understand the institutional business is 20% of our sales. So, how much of that could ....

Abhay Soi : In bed share, understand that bed share is 29% and revenue is 17% . Now for 17% of revenue, 12% of that 17% will go up by 70% . That is a net impact of 4% to 5% on the institutional part.

Amit Thawani : My second question is the digital revenue is about 18% of our overall sales. Just trying to get a sense of what the margins on that digital revenue are. Is it higher than the overall margin of the company or is it lower?

Abhay Soi : No, it is just a channel. People used to call up earlier. People would come to hospital as walk-ins. Then they used to call up call centers. Now they will do it through digital platform . They are booking appointments, so your margin does not change. You can go into customer acquisition costs, advertising, etc. but that is meaningless here.

Amit Thawani : So, the press release said digital revenue from online marketing activities.

Abhay Soi : That is right, digital revenues means it is digital booking. If you book a liver transplant through our website, then that is digital revenue.

Yogesh Sareen : If you come on the website , we track the patient footfalls on the website and then we see conversion from there. So, if there is conversion from the website that is accounted as digital revenue.

Amit Thawani : My last question is on the contribution of ALOS to ARPOB, we saw the ALOS go up this quarter.

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Abhay Soi : Look,

ALOS is meaningless, right . A hospital that does more complex surgeries will have a longer ALOS. If I do a bone marrow transplant, the ALOS is 20 days. If I have a liver transplant , and this is the business you want actually , the ALOS is longer than doing non-surgical work. When you do more surgeries, you have more ALOS, but all of it plays out in ARPOB. Eventually what is it that you have , you have inventory and you want to earn the maximum in a day from there. So, it is all based on the ARPOB . No point having a very short ALOS and having a lower ARPOB . You would rather have a high ARPOB. You have beds into days in a year , so what you want is higher amount of revenue from each of the days.

Moderator: Thank you. We have a next question from the line of Alankar Garude from Kotak.

Alankar Garude : Abhay assuming the CARE acquisition does not go through, can you help us understand whether there are any meaningful acquisition opportunities of hospital chains left in India which fit your criteria?

Abhay Soi : Yes, plenty.

Alankar Garude : And we do not have any preference on geography, right. It is just the criteria , which you have of two players being successful and you preferring a cluster approach.

Abhay Soi : That is right. I mean our approach is a little more less casual than just that. We have worked out a list over 20 -21 cities , but that is an important criteria. What we find is that at the umbrella level , these are the places that typically will have the demand, supply, doctors , etc. and so we will be comfortable over there. I am putting it very simplistically by saying that at least two or three of my competitors have proven viability . In each one of these cases , this is what I find common. You read the profile s that we want to take . We are not pioneers; we do not like to go to uncharted territories.

Alankar Garude: And the other thing is, would it be fair to assume that most of these would be standalone assets now or if at all just two, three hospitals per chain or there are still say chains operating five to 10 facilities across India, which are on the block right now.

Abhay Soi : I think there are plenty of conversations happening and you are going to see consolidation. Firstly, consolidation happens because there is a requirement for those conversations , right. I mean in the case of Max Healthcare, you have seen Rs. 1,636 crore of EBITDA translating into Rs. 1,281 crore of free cash flows. Now typically for some companies and most mid -sized players that you will see , will have a Rs. 500 crore to Rs. 600 crore EBITDA and their EBITDA translates to a Rs. 200 crore to Rs. 300 crore free cash flow at best. This is pre-Ind AS, post maintenance CAPEX, post any increase in working capital and taxes. Now this is a capital -intensive sector , so if you have Rs . 200 crore to Rs. 250 crore, you can build one 500-bed hospital every four years . That is all you can do, one hospital every four years. If you are a listed company or unlisted, where do you go from there?

Alankar Garude: So, my question was more from some of our peers especially the unlisted peer s being quite aggressive on the M&A front and also private equity interest in the space continues to remain quite high.

Abhay Soi : Look , I will tell you something that there always be some people trying to outbid you . What is it that we look at? Our criteria is very simple , I want to look at a 20% ROCE three or four years down the line or four or five years down the line. If we have a clear line of sight and visibility of that ROCE, we will come in. Now a PE fund . Typically , will be getting excited at the multiples that they are paying . But our ability

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to underwrite an EBITDA or a business case, four -five years down the line is much more pronounced than a PE. I mean, for us to underwrite , let us say, in situations where we believe where 2x o

r 3x or 3.5x of EBITDA, is very possible three or four years down the line. For a PE to underwrite that gets very difficult . So with the amount we are willing to pay at the outset is a derivative of that , of where I believe I can take the business four years down the line. Do keep in mind, our EBITDA per bed is the highest in the industry , you know that , by 50% to 55%. A PE cannot underwrite my

case. He has to underwrite the industry case.

Alankar Garude: And the second one is more of a quick clarification. So, is it now fair to assume that we may not reach that 15% institutional volume mix number by March '24? But given that we are operating at high occupancies, the institutional volume mix will still gradually keep on coming down.

Abhay Soi : No, it will come down. We have done a bit of a pause , like I said , because we are expecting some increment, although it is still coming down. We have also found more capacity. We have more elasticity in operations. The price is moving up. It is not going to reach 15% immediately but your trend is going to keep reducing over there because eventually, it is finite, right . I can suffer that higher price for some time, but it is still not the same as my CTI business.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to management for closing comments. Over to you, Sir.

Abhay Soi : Thank you so much for being on the call. I think this has been a seminal year for us on many counts and we are happy to have most of you as our investors and the rest of you, we look forward to them joining our cap table. So, thank you very much.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.



## Call: Aug 2023

Max Healthcare Institute Limited  
(CIN: L72200MH2001PLC322854)  
Regd. Office: 401, 4th Floor, Man Excellenza, S. V. Road, Vile Parle (West),  
Mumbai, Maharashtra -400056  
Phone: +91 - 22-26101035,  
E-mail: [secretarial@maxhealthcare.com](mailto:secretarial@maxhealthcare.com),  
[investors@maxhealthcare.com](mailto:investors@maxhealthcare.com)  
Corporate Office: 2nd Floor, Capital Cyberscape, Sector -59, Gurugram, Haryana 122011  
[www.maxhealthcare.in](http://www.maxhealthcare.in)

August 14, 2023

Listing Department ,  
National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,  
BSE Limited  
Phiroze Jeejeebhoy Towers,  
Dalal Street,  
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on August 8, 2023

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements)  
Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings call , organized on August 8, 2023, on financial results of the Company for the quarter ended June 30 , 2023.

The said transcript is also available on the website of the Company at  
[www.maxhealthcare.in/investors/investor-resources](http://www.maxhealthcare.in/investors/investor-resources) .

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Aroraa  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited  
Q1 FY24 Earnings Conference Call Transcript  
August 08 , 2023

Moderator Ladies and gentlemen , good day and welcome to the Max Healthcare Institute Limited's Earnings Conference Call .

Please note that this conference is being recorded.

I now hand the conference over to Mr. Anoop Poojari from CDR India. Thank you, and over to you sir.

Anoop Poojari : Good morning everyone and thank you for joining us on Max Healthcare's Q1 FY'24 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director; Mr. Yogesh Sareen – Senior Director and Chief Financial Officer; and Mr. Keshav Gupta – Senior Director Growth, M&A and Business Planning of the Company.

We will begin the call with opening remarks from the Management , following which we will have the forum open for an interactive question and answer session.

Before we start, I would like to point out that some statements made in today's call may be forward-looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi : A very good morning to everyone. I am pleased to welcome you again to Max Healthcare 's First Quarter Earnings Call for fiscal 2024.

Before we delve into the Quarter 1 highlights , I am happy to share that this is our 11th consecutive quarter of year -on-year growth in revenue and operating EBITDA.

Second, we have added 44 beds to our capacity through internal reconfiguration during this quarter. This is in addition to the 92 -bed oncology block we commissioned at Shalimar Bagh in March this year. Notably, Max Shalimar Bagh hospital has reported an average occupancy of 77% in Q1, with a year-on-year growth in revenue and EBITDA of 37% and 43% , respectively.

Third, we have released the in-patient module on our proprietary app, 'Max MyHealth', which allows IPD patients to track the entire process right from the admission to their discharge. It also provides them access to the reports and up-to-

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date billing data , with the ability to make payments online. This will not only enhance patient satisfaction on one hand , but also reduce administrative burden on the other.

Highlights of our Q1 performance

1) Occupied Bed Days (OBDs ) went up by 3% year -on-year and the average occupancy for the quarter stood at 74% , do keep in mind this is on a higher capacity, remaining flat compared to Q1 last year. This is on expected lines coming off from seasonally the best quarter to the worst quarter. With increase in occupied bed days (OBDs) and marginal drop in ALOS, the admissions were up by 4%.

2) Institutional bed share fell to 29.7% compared to 30.3% in Q1 last year.

However, it is pertinent to note that excluding Max Shalimar Bagh , where we strategically decided not to optimize pay or mix at the cost of occupancy in light of the newly added 122 beds on a base of 280 beds , the share dropped by 270 basis points year -on-year to 27.4% of institutional business on overall basis. Moreover, as you may be aware , CGHS has revised its tariff in April and June this year for certain segments after a gap of nine years. And we expect further revisions for the balance segments in the coming quarters. Therefore, we have not taken any aggressive calls on disempanelments .

3) The average revenue per occupied bed (ARPOB) for the quarter touched a new high of Rs. 74,800 , reflecting a growth of 13% year -on-year and 6% quarter -on-quarter. This was mainly led by improvement in specialty mix, particularly Oncology, Orthopedics, Cardiac Sciences, and doubling of robotic procedure volumes. This was further aided by price revisions across almost all channels, which usually happens on the 1st of April.

4) Network gross revenue was Rs. 1,719 crore compared to Rs. 1,473 crore in Q1 last year and Rs. 1,637 crore in the previous quarter . This reflects a growth of 17% year -on-year and 5% quarter -on-quarter. The year -on-year increase was largely driven by growth in ARPOB and occupied bed days (OBDs) .

5) Revenue from international patients grew by 31% year -on-year and 3% quarter -on-quarter. This now accounts for around 9% of the revenues from our hospitals.

6) Digital revenue grew to Rs. 356 crore and accounted for 21% of our overall revenue.

7) Direct costs were up year -on-year, due to growth in surgical mix, medical oncology and doubling of robotic procedure volumes. On the indirect costs side,

while the overall percentage is lower, there is an increase in absolute costs due to commissioning of 4% additional capacity, creation of around 10 new operation theatres (OTs) and 80 ICU beds by cannibalization of existing ward beds over the last 12 months. In addition, we have strengthened our projects, digital and homecare teams, while also increasing efforts on ESG activities and marketing for the international channel.

8) Network operating EBITDA stood at Rs. 436 crore, reflecting a growth of 18% year-on-year, while remaining relatively flat quarter-on-quarter, in spite of the fact that we are coming off perhaps Quarter 4, which is the best quarter, onto Quarter 1, which is usually the worst quarter.

9) The operating margin stood at 26.8% versus 26.5% in Q1 last year and 28.2% in the previous quarter. This is largely because we have been focusing on higher

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payor mix, which is international patients, higher clinical mix which is Oncology, Cardiac Sciences and robotics, etc., which in percentage terms may provide lower margins, but in absolute terms provides higher value.

10) Most importantly, annualized EBITDA per bed, rose to Rs. 70.4 lakhs, yet again our highest ever, clocking a growth of 14% year-on-year and a nominal growth quarter-on-quarter.

11) Profit after Tax was Rs. 291 crore versus Rs. 229 crore in Q1 last year and Rs. 320 crore in the previous quarter. The year-on-year growth

of 27% was primarily attributable to the flow through of improved EBITDA.

12) Free cash flow from operations stood at Rs. 261 crore, of which Rs. 38 crore was deployed towards the ongoing capacity expansion projects. Net cash position improved to Rs. 957 crore at the end of June 2023 compared to net debt of Rs. 217 crore same time last year. Expediting our routine CAPEX, coupled with buildup of account receivables, led to higher working capital during the quarter.

13) Continuing our efforts to give back to the community, we treated approximately 37,500 OPD and 1,260 in-patients from economically weaker sections free of charge.

14) Both our strategic business units (SBUs) continue to report robust performance:

■ Max@Home reported a top line of Rs. 40 crore, reflecting a growth of 24% year-on-year and 7% quarter-on-quarter, with 'Critical Care' and 'Medical Rooms' service lines being the major contributors to this growth.

■ MaxLab expanded its geographic footprint to 36 cities and reported a gross revenue of Rs. 34 crore. This reflected like-to-like growth of 39% year-on-year and 10% quarter-on-quarter.

15) During the months of June and July, there was a slowdown in the projects activity due to heavy rain in Mumbai, Punjab and Delhi NCR. The current status of our expansion projects coming on stream over the next three years is as follows:

■ For 300 beds at Dwarka – The majority of the MEP and interior work are complete, while application for power, water and sewage connections are in progress. The developer is expected to apply for occupancy certificate (OC) in the latter half of this quarter. We expect to commission the hospital in Q3, subject to the developer obtaining the OC.

■ For 329 beds at Nanavati Hospital in Phase I – Excavation and Raft work are nearly complete. Steel fabrication and wall casting for LINAC and Brachytherapy are in progress. The project is largely on time and we expect to cast the ground floor slab by the end of October.

■ For 300 beds at Sector 56 Gurgaon in Phase I – 50% of site excavation is complete and the EPC contract is in the final stages of execution. All statutory approvals have been received.

■ For 350 beds at Max Smart in Phase I – As stated during the Q4 update in

May this year, this project was facing delays due to forest approval for tree transplantation. Happy to report that the final forest approval has now been received and we are initiating the process of transplanting the trees shortly after subsiding of rains.

■ For 300 beds at Vikrant (Saket) – Environmental clearance has been approved and is under processing with the State-level Environment Impact Assessment Authority (SEIAA). DDA has reconstituted our file and we plan to formally upload the drawings for approval in September. So, this should be well on its way as well.

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■ For 190 beds at Mohali – Demolition of existing building and shifting of services have been done. D-wall is completed and excavation work is underway. EPC contract is in the final stage of negotiations. The project is largely on time.

16) Lastly, I would just like to state the three months already into this fiscal, we continue to strive to improve operational metrics across all our current facilities and businesses, keenly monitor the execution of all our projects underway, while also prudently evaluating inorganic opportunities in our existing geographies as well as new, promising geographies.

With this I would like to open the floor for Q&A.

Moderator: Thank you very much. The first question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: Just on the kind of growth what we have seen on ARPOBs and how sustainable that is let's say for next 12 to 24 months? And secondly, while the ARPOB growth is there, the EBITDA per bed has been pretty stable may

be on a quarter-on-quarter

basis. So, if you could share your comments on that?

Abhay Soi: We will continue to keep doing what we are doing – focusing on higher end specialities and better payor mix, etc., so your ARPOB growth should continue. And when it comes to EBITDA per bed being almost flat quarter-on-quarter, like I said Q4 is the most solid quarter in the year. It's the strongest quarter and Q1 is the weakest quarter. I am very encouraged by the fact that in spite of this our Q1 results are flat with our Q4 because if you look at it historically, and I am taking away that one COVID year, Q1 is usually lower than Q4 because on April 1 you have increase in costs, particularly your salary costs, for the whole year. Therefore, Quarter 1 normally is seasonally the weakest quarter with high indirect costs, and thus has lower absolute EBITDA and margins emanating from it. But that is not the case this year. It has been flat compared to Q4, so it's actually a very encouraging sign.

Tushar Manudhane: So, what kind of growth one can think of on a sustainable basis given the kind of bed additions that are happening over next 12 to 24 months, on EBITDA per bed?

Abhay Soi: I am not going to give you any guidance on the exact EBITDA per bed, but I think this is our 11th quarter of year-on-year growth like I mentioned. Also, the fact that recently when we opened the Max Shalimar Bagh, i.e. 122 beds on a base of 280 beds, you can see that there is 77% overall occupancy within the first month or two itself. And it is giving us a revenue and EBITDA growth of 37% and 43%, respectively. That should hopefully continue into the future as well and it should be positive from all of these standpoints.

Tushar Manudhane: And just while you have already highlighted hospital wise the kind of work underway in terms of bed expansion, but just on the CAPEX number if you could also sum up for FY'24 and for the quarter?

Abhay Soi: We are looking at around Rs. 900 crore.

Yogesh Sareen: Tushar, the cash outflow expected is around Rs. 900 crore. That's obviously subject to change and we are just trying to give you a block number. But the outflow could shift from one month to another.

Moderator: The next question is from the line of Damayanti Kerai from HSBC.

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Damayanti Kerai: My first question is on your Shalimar Bagh, clarity on your comment that you decided not to optimize payor mix. So, does it mean like you have taken up more institutional patients there?

Abhay Soi: Yes, basically we do not disempanel over there and we don't filter business because the extra capacity is, firstly, at a significantly lower cost due to operating leverage.

So, even at the institutional rates, we still generate EBITDA, as we have already seen. But the first thing to do is that you bring in the occupancy and then start filtering

down at a later stage.

Damayanti Kerai : Okay, so institutional bed share at around 29.7% for first quarter. So, earlier you guided like by end of '24 or so, you will bring it down to 15% to 16%, does it still hold or you have some different thoughts now?

Abhay Soi : Excluding Shalimar Bagh , it has come down to 27.4% and clearly we have had some increases in pricing for institutional businesses recently . Although, it doesn't have a big impact on ARPOB, it's about 0.6% to 0.7% so far. But we are expecting further revisions of balance rates in the next month or two as far as institutional is concerned. That makes us go slow on the churn , but again the gap is still too much between institutional and cash . It just pushes the can down by a quarter or two , not more than that. We are still going to face capacity constraints and going to have to distill because our CTI business on the preferred channels still need those beds.

Damayanti Kerai : So, you said like gap between CGHS state and CTI patient reduces so you might not

be chasing that 15% to 16% bed share very strictly, right?

Abhay Soi : We are not per se chasing a 15% to 16% bed share . Let me put it in perspective.

There is a certain rate at which our CTI business is growing. If the new capacity doesn't come in , we have no place to accommodate the growing CTI business. The best case for us is if we can keep the institutional business and we can accommodate the increased CTI business. As and when we can't , we will distill that capacity and bring institutional down. Today , let's say if I had a magic wand and we could put up these extra beds to accommodate the increase in CTI, we would not let the institutional business go either. We are having to do that so the trajectory that we have set, it will come down to about 15% by that particular time . It may go and up down a couple of quarters because of these changes, but that's about it. But that's still our trajectory . Let's put it this way, if I can today acquire big unoccupied capacities , theoretically , adjacent to my hospital s, then I will do both the businesses.

Damayanti : My second question is on EBITDA , so healthy ARPOB but occupancy down sequentially and then EBITDA down a round 260 basis point quarter -on-quarter. So, does it mean like you did more high-ticket surgeries where ARPOB is high but there you might be paying higher like higher payout to doctors , etc., like higher cost and that's why your margins declined sequentially, it should be read in that way?

Abhay Soi : You need to complete that story . A higher end surgery , let's say you do a LVA T surgery , it costs you Rs. 85 lakhs . The implant itself will cost you about Rs. 56 lakhs.

Now a patient stays in hospital for a few days , say 4-5 days , the hospital will make about Rs. 15 lakhs . The percentage margin is much lower. When you do robotics, higher end Oncology, Cardiac Sciences, Orthopedics , or any of the higher end surgeries , the percentage margins are lower. But the absolute bill is higher and in spite of the lower percentage margins, the absolute profitability is higher per patient and per bed.

When you do international business, the international people come for more severe diseases to India and so the billing is higher. ARPOB for the patient is higher. The

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percentage revenue is lower , but the absolute EBITDA per bed is higher. So, I have always said, I would rather do a \$10,000 surgery with a 20% margin than do a \$2,000 surgery with a 50% margin. As you move up the payor mix and you move up the clinical mix, you are going to see margins , in percentage terms , come down but in absolute terms ARPOB as well as absolute EBITDA per bed will move up. And that is what you have seen.

So, our overall EBITDA , in spite of a lower occupancy and cost increasing on 1st of April, has been flat over Q4. And in spite of Q4 being the strongest quarter and Q1 the weakest quarter . I mean that's a big positive isn't it.

Damayanti Kerai : My last question is on your utilized bed so as per your presentation in last quarter we have 2,523 utilized beds of the total operating beds and this quarter it came down to 2,474 . So, how should we read into it?

Yogesh Sareen : As we said that generally Quarter 1 is a weak quarter on occupancies . You will see the same trend even last year . Last year also the occupancy was 74% in Quarter 1, 77% in Quarter 4. This is the seasonality in the business . During the Quarter 1 when you have summers set in, doctors do go on leave . Generally, occupancy in the months of May and June come down.

Abhay Soi: Not just us, for any other hospital or hospital group , you will always see that Quarter 1 is always lower on occupancy, EBITDA and everything else compared to Quarter 4.

Yogesh Sareen : But, at the same time , even if the occupancy is same, the OBDs have gone up . If we were not to add beds in Shalimar Bagh and other places

, then the occupancy would

have been 2% higher. So, you have to also consider and that factor into account .

Abhay Soi : Our occupied bed days have moved up.

Moderator : The next question is from the line of Nikhil Mathur from HDFC Mutual Fund.

Nikhil Mathur : Just one question I have. Now you have reported pretty strong occupancy on the incremental beds in Shalimar Bagh. We are able to track numbers for one of your peer Fortis as well as in Shalimar Bagh they have reported decent numbers on a Q-o-Q basis, despite 1Q basically is generally weak. So, it kind of seems that in that particular micro market the demand is very strong. Would you say that this is reflective of many of your other markets as well where any concerns, I mean there is some bit of divided opinion whether the demand factor is likely to be weak or not in this particular year. Would you say that a similar trend is kind of playing out in other micro market as well especially in which you are present?

Abhay Soi : I don't see this playing out anywhere , in fact for anybody . I have read other people's numbers and our numbers, year-on-year we have an increase in occupied bed days .

If our total capacity goes up by let's say 4-5% and our occupied bed days go up by 3-4%, while our percentage occupancy remains the same, it is bound to happen .

When you increase capacity , in percentage terms your occupancy may be flat, but in the actual terms your occupancy is more.

Yogesh Sareen : Also, I think we have not seen any dwindling of demand in any of the hospitals. If that's what you are looking at, then there is no impact on demand . While in June quarter, the occupancy is generally lower , but we have seen that occupancy coming back in this quarter.

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Abhay Soi : I don't cater to this point at all . You have to look at the business year -on-year rather than quarter -on-quarter. Unless you have a fourth quarter , which is supposed to be a good quarter and is running below the third quarter or something like that , then I will be worried.

Nikhil Mathur : No, I am looking at it on a year-on-year basis only, but I mean in this particular micro market the numbers are looking strong on Q -on-Q basis as well, so that's why this question I had. But I mean I completely understand what your thoughts are on the demand and how its panning out.

Moderator : We have the next question from the line of Lavanya Tottala from UBS.

Lavanya Tottala : I just have one question, so I understood that there is an increase in employee cost , but if I look at Max consolidated level, there is also increase in raw material as percentage of sales. So, is there any increase in the pharma medicines that you are seeing ?

Abhay Soi : As far as the pharma cost is concerned , there is an increase because , like I mentioned, we are doing more higher -end work . Like I said earlier, we doubled your robotic procedures ; when you do all of these the consumables are more expensive in that and is higher as a percentage of the procedure. But the overall EBITDA per procedure, overall value that we get out of the billing is higher. So, in terms of percentage it is lower, but you will see that the ARPOB is higher . Cost of goods goes up, but your absolute EBITDA per bed and EBITDA also moves up. So, that is what we have seen .

Your second question was with respect to the employee cost. The employee cost moves up by usually about 6.5 -7% per year and it happens on April 1st. This year you will see a larger year -on-year increase because we also have 144 new beds that we put up, we have created 80 more ICU beds and we have created 10 more OTs. When we are doing that , the manning of all of these is more . Theoretically if you double capacity , then you will almost be doubling your employee strength, and therefore your employee cost. But then you have to have a resultant increase in revenue. So, if you look at i

t as percentage of revenue , there is not much of a change there.

When you look at it in absolute terms, it is bound to increase because we are putting up more capacity. If we put 4 -5% more capacity, that itself should increase our employee cost by about at least a few percentage points. And then you add a few percentage points of higher ICU beds . A regular ward , for example , requires 5 :1 nursing, while ICU requires 1:1 nursing. It's almost five times the amount of people that are required for ICU versus ward . But it generates you more revenue as well. So, you want to do all of those things.

Moderator : The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha : I think there is some confusion regarding the seasonality at least on my end. So, I think there is seasonality with specialty mix that plays out in different quarters and then there is what you cited as in terms of volume also kind of there is some seasonality and that impacts our revenue on a quarter -on-quarter, year -on-year

basis. So, could you help us explain how this seasonality because as far as -- Quarter 2 is more like internal medicine patient because of infectious season in the country, right. So, how that impacts our revenue and EBITDA on a quarter, let's say starting from Quarter 1 to Quarter 4, how you see specialty seasonality and then how it impacts revenue and EBITDA?

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Abhay Soi : Orthopedics happen less in winter, Cardiac Sciences happen s more in winter . More insured patients come in for annual checkups, etc. in Q4 because it is lapsing and so on . For a ny hospital or hospital chain over the last 10 years, whether it's a mature hospital or otherwise , when you speak to their management or yo u look at the ir history, Q4 is the strongest, while Q1 is the weakest as far as occupanc y is concerned . As far as Q2 is concerned, occupancy is higher , like you rightly said , because of dengue and more internal medicine admissions, etc. So, you have the highest occupancy in Q2 , but you have lower ARPOBs in Q2 . That's a very standard thing .

Yogesh Sareen : Our Q1 is also down because doctors were on leave.

Abhay Soi : They go on summer holidays; they postpone their surgeries , their children 's schools are on holidays. In fact, people postpone their own procedures because its examination time for kids , etc. All of these things matter and not only this year, but they have been a factor every year since I have been in healthcare or even much before .

Kunal Dhamesha : And let's say to that extent the trends in July and August how are those panning out, because we are anyway one and half months into the next quarter, right.

Abhay Soi : I am not going to give you any forward -looking guidance , but it's secular.

Kunal Dhamesha : And secondly on the 13% ARPOB growth I am not sure if I have missed your initial comment , because I joined a little late. But on the 13% year -on-year growth in ARPOB, if you can highlight the major moving pieces , because we have quite a few right CGHS rates revision, international --

Abhay Soi : Revision in CGHS rates ha ve not impacted ARPOB by more than 0.6 -0.7%. Let's look at year -on-year to start with. Our international business has grown by 31%, our robotics ha ve doubled , our Oncology business is growing at a much faster pace and so is our Cardiac Sciences, Orthopedic s. These are all high end specialties . When you have that, your ARPOB increases . There are less medical admissions during this time , so that also kind of helps you r overall ARPOB to increas e.

The best part is that at least I can't remember the last time Q1 was almost equal to Q4 as far as overall EBITDA is concerned. I won't comment on the COVID year . Now this is not due to revision CGHS rates . Every year we have an increase in pri cing, which impacts our revenue by 2 -2.5%. But in spite of the increase in sa

laries and

other indirect cost s on the 1st of April, we not only absorbed it, but our Q1 overall EBITDA is equal to our Q4 EBITDA .

Kunal Dhamesha : And then if I may just t wo very housekeeping kind of question, what was the international bed share, we have share d the payor mix , but bed share if you can share?

Yogesh Sareen : It's around 5 .5%.

Abhay Soi : And i t gives you 9% revenue.

Kunal Dhamesha : And just a clarification, so the way I understood is any procedure which uses more devices, which is where our EBITDA per bed would be higher in absolute term , but in terms of margin, it might not be accretive, is the correct way to underst and.

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Abhay Soi : Actually, any high-end procedure has lower percentage margins and high value margin s. If you look at Oncology, Orthopedics, Cardiac Sciences , etc. all of these have lower margins percentage wise, but higher ARPOBs, higher billing, higher EBITDA per patient and higher EBITDA per bed.

Yogesh Sareen : Typically , you will also pay a doctor more for a surgical procedure compare d to medical specialties , so there obviously would be a higher impact of that also on margins .

Abhay Soi : Actually, for medical specialt ies, we are only giving the patient s medicine , doctor visits , etc. Your margins maybe more , but your absolute billing is low er.

Moderator : We have the next question from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampi l: Just wanted to know earlier you used to give a schedule of new capacity coming online all the way up to FY'28, I haven't seen it for the last couple of quarters . So, just wondering if all those timelines given earlier two, three quarter back still stay or is there any change in that?

Abhay Soi : We just gave you an update so there is no necessary change in that. And I think other than the update that has been given in last couple of quarters.

Yogesh Sareen : It's there in our investor presentation.

Abhay Soi: It's there in our investor presentation, on our website.

Yogesh Sareen : But I think Abhay has given you the update . Firstly, everything else is on time except for Max Smart , where we are delayed because of the forest approvals and those approvals have also been received now, and we have to transplant the trees . That's the only project where we find some delays . And we said it last time , that because of those delays we are fast -tracking the Vikrant Foundation capacity addition .

Abhay Soi : Usually any data which is static , which doesn't change, we don't give out every quarter . But you can see it on our website , it is part of our investor presentation.

Moderator : The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha : So, to the fact that we have added this 144 beds in Shalimar Bagh and we have also said that we have added some of the doctors , etc., so would it be fair to say that the addition of let 's say manpower would be largely in line with what our average is at network level would be.

Abhay Soi : Are you talking about for the incremental b eds at Shalimar Bagh and others?

Kunal Dhamesha : Yes.

Abhay Soi : No, it's lower . Because you don't have to add the same amount of manpower, and especially not the same cost of manpower . I have mentioned earlier that there is operating leverage in it. But that's not all, we added 144 beds , 80 ICU beds, 10 OTs .

All of this generates a lot more than what it was previ ously being used for , both in terms of revenue and EBITDA , but has higher manpower requirement.

And then we have , over the last year, strengthen ed our projects team because of all the projects we are doing, the digital team for the app that we have come out with , increased efforts on ESG , etc., as well. All of these things play out, but I would

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encourage you to look at it as a percentage of sales rather than an increase  
necessa rily year -o

n-year. Our salary increase has been the same, 6.5-7%.

Kunal Dhamesha : And one of the strategy side our earlier comment was that we would be mainly focus ed on the metro cities like Delhi and Mumbai etc ., but some of the competitions have been now opening up into Tier II cities and they are seeing good kind of ramp up in terms of both top -line and the profitability . So, would we also look at these markets going forward and probably broaden our horizon as to where we are going to expand?

Abhay Soi : Any city , where at least one or two of our competitors have proven viability , we will enter into those cities. Presently the expansions we are doing, we are not doing because we only want to be in metros, we are doing because we have a business need . We have run out of capacities and there is demand which is s urning on my doorstep . I have waiting of a couple of days , if not more , sometimes in ICUs . We are doing brownfield capacity in order to tap that demand . First and foremost I need to alleviate the business need that I have. Thereafter we are looking at other cities we continue t o look at . As we have said in the past, there are 21 cities that we have identified that we are looking at. These are cities where at least one or two of our peers have proven viability. So, we think we can go there and do it better.

Kunal Dhamesha : And in terms of inorganic activity, obviously probably my sense is valuation could be a factor, but do you see that intensity, I mean in terms of the ask price versus what we are willing to pay is reducing, increasing over the last let's say six months?

Abhay Soi : For that you have to know how much we are willing to pay . We are usually guided by ROCE, we seek a 20 % to 25% ROCE in a business case . For us, i t's less important what we are paying today, of course nobody likes to pay more . But what is important to us is what is the business case we are willing to underwrite four or five years down the line. Do keep in mind that our EBITDA per bed is at least 50% to 55% better than the next best player in the industry. So, our ability to under write a stronger business case is more than others and that's what we are willing to back .

Kunal Dhamesha : So, that is why I ask about the spread and not if you are willing to pay right . So, spread whether that ha s increased, decreased you know, can give a sense --?

Abhay Soi : That is irrelevant . The point is this that it does not matter what somebody is willing to pay. I f we are shooting for a 20 % to 25% ROCE pre -tax, four or five years down the line , with a particular ability to underwrite a business case so that's the maximum number we are willing to pay today. Now somebody else will be willing to pay a higher number, but the fact is the other person th en has to under write a business case which is at least equivalent to mine .

Kunal Dhamesha : And the last again a housekeeping question, let's say the consumption of drugs if we



can quantify as maybe percentage of revenue in our hospital business as a percentage of revenue or as a percentage of COGs would be helpful, a broad range would also be fine not --?

Abhay Soi : That's not a static number . Let's say if we were to do more higher -end surgeries, this number would increase. What you need to do, we have beds and we have days in a year, right EBITDA per bed --

Kunal Dhamesha : Quarterly basis maybe two to three years of average would also be fine or a range would also be fine.

Abhay Soi : We don't give forward -looking statements and guidance .

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Kunal Dhamesha : I am asking about the historical what would be the drugs as a percentage of probably revenue.

Yogesh Sareen : That is in the range of 23% to 25% , depending on which month or quarter you are talking . That's the range I would say historically. And we don't call it material cost, we call it direct cost and this includes F&B (food &

beverages), drugs , and other direct costs.

Kunal Dhamesha : This includes what?

Abhay Soi : F&B and so on and so forth . We don't give a precise number as far as drugs or consumables are concerned . It includes F&B and some of the other costs as well.

Moderator : The next question is from the line of Dheeresh Pathak from WhiteOak Capital.

Dheeresh Pathak : So, you gave a commentary on the projects that are underway, but I am just trying to get a better understanding of the commissioning timelines. So, Dwarka, which is 300 beds Q3 of this fiscal year, FY'23 -- end of Q3, right. In FY'25 as per the earlier Investor presentation but there were four assets Smart is delayed so now there are three ; Nanavati is 329, Mohali 190 and Gurgaon Sector 56 , 300. So, of these three if you can individually call out their commissioning timelines, as per your current understanding based on the current project progress.

Yogesh Sareen : What we given is the timelines for the completion of the construction of these projects. You can presume that in the next quarter of that will be the commissioning . For example, if the slide says Q4 FY'25 that means Q1 FY'26 will be the commissioning . Once the beds are constructed , we need to obtain the OC, so there is a time lag of a 1.5-2 months, but as Abhay had mentioned already , we are largely on time for these projects.

Abhay Soi : Sometimes it comes sooner also . More or less it's the timeline .

Yogesh Sareen : Yes. All projects are on time except for the Max Smart , which is part of the Saket Complex . We are delayed by six to seven months there because of the forest approval and that's the only project where we feel there will be a slip in timeline. But we are fast tracking the Vikrant one which is also in Saket Complex.

Dheeresh Pathak : So, individually if you go, Nanavati 329 you expect in Q4 of '25?

Abhay Soi : Yes, as scheduled.

Yogesh Sareen : So, this will be finished in Q4 FY'25 . That means March '25 is when we finish and we will be starting this in the first quarter of FY'26, between February and April.

Dheeresh Pathak: Mohali?

Yogesh Sareen : Similar.

Abhay Soi : Similar.

Dheeresh Pathak: So, most of the assets Q4 of FY'25 commissioning and Q1 of FY'26 is where the commercial --

Abhay Soi : We had mentioned in the end of '25.

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Yogesh Sareen : Yes, end of '25.

Dheeresh Pathak : Now just to understand for these assets whatever is the estimated CAPEX outlay, I don't know if that number is handy or not, how much have you already spent?

Yogesh Sareen : So, I think as Abhay has given you the projects update, for example , in Nanavati we would have spent probably around 26% of the total spent. But other than Nanavati, Mohali we just finished the excavation, we have done the D wall . The contracts have been given to the EPC vendors now . So, I would say a large amount of CAPEX is yet to be spent. On an overall basis, it will be probably at 12 -13% spent, but Nanavati is the only one where we have more than 25% spent.

Yogesh Sareen : Also, the CAPEX spent , because it also includes equipment, is largely back ended.

Abhay Soi : Yes, the super structure doesn't cost too much, around Rs. 2,000 square foot . So, if you are building 1 million square feet you will spend Rs. 200 crore for the super structure. That's not the expensive part of the project , which we do upfront.

Moderator : We have the next question from the line of Harith Ahamed from Avendus Spark.

Harith Ahamed : Couple of questions on the CAPEX numbers that you talked about. Firstly, the operating cash flows you mentioned is lower because there was higher routine CAPEX spends this quarter. Will you be able to quantify the same, the routine CAPEX that you spent this quarter and the budgeted spend for the year? Just trying to understand what is our ongoing regular maintenance CAPEX.

Yogesh Sareen : Yes,

so typically we spend around Rs. 170 -180 crore in a year . This quarter alone we have spent Rs. 70 crore. That means we advanced and fast tracked the spend to get the benefit of it during the course of the year .

Harith Ahamed : And on the project CAPEX, the Rs. 38 crore spend I am assuming it was lower because of the weather situation that we had in the Delhi region and then we should expect it to pick up. So, for the year is there a number that you can share?

Yogesh Sareen : Yes, we shared earlier that it's around Rs. 900 crore spend for this year.

Harith Ahamed : So, there will be a significant pick up as we go into the next year?

Yogesh Sareen : Yes.

Harith Ahamed: So, on the M&A front are there any opportunities that we are exploring currently, our name keeps coming up in, as per various media articles in various M&A situations.

And on the litigation s that we had initiated against Care is there any update and what are the next steps on that particular situation?

Abhay Soi : CARE matter is sub-judice , so we can't really speak about it, but we have filed an appeal in the Bombay High Court. As far as inorganic expansions are concerned, yes our name is everywhere , whether that is true or not is beside the point. But a lot of places where we are looking at opportunities, our name doesn't come up or those matters are not in the public space . But, coming to your point , we are looking at it quite actively . We have fortified our teams as well . Keshav has been a new addition to head M&A from our standpoint. So, we are quite focused on this.

Moderator : The next question is from the line of Pallavi Deshpande from Sameeksha Capital.

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Pallavi Deshpande : Just wanted to understand how was the operating cash flow and if you could also highlight on the debtor days. And secondly what would be the debtor days separately for the government mix?

Yogesh Sareen : DSO at the end of the quarter was 66 days . It has gone up compared to 55 days at the end of March. There is a build up of accounts receivables ( AR) in the PSU and insurance segments. But, we are seeing that in July this has started to unwind a bit.

Pallavi Deshpande : So, just continuing on this I didn't follow the first part. There you said there were two parts to the increase, so the first part was due to ...

Yogesh Sareen : Yes, I said that the overall DSO went up by 11 days , from 55 days at the end of March to 66 days at the end of June. This is basically because of the build-up of AR, for the PSU segment (CGHS and ECHS) and also some build up in the insurance segment . The insurance segment had a 24 -days DSO at the end of March , which has gone up to 34 days at the end of June, so there is some build up there. But as we speak today , there is some unwinding which has happened in July . We have got some of those over dues in July and things are getting normalized.

Pallavi Deshpande : And overall I, on the strength of the PSUs and the CGHS are we going to see better years than last year in terms of government making the payments faster?

Yogesh Sareen : Yes, I mean so far the story looks bright, but we don't know what's going to happen. When the ir budget gets finalized, how much time they take for new allocation , etc., will determine how fast they will pay.

Moderator : The next question is from the line of Mayur Patel from 360 ONE Asset Management.

Mayur Patel : Just in your business planning about the new assets which you mentioned would be commissioned by end of FY25, what would be the peak occupancy which you are targeting in the new assets, is it possible to share some thoughts around that?

Abhay Soi : Peak capacity would be similar to our Network levels , could go up to about 80%.

Mayur Patel : In line with whatever are your matured assets currently 77% to 80% is fair to assume?

Abhay Soi : That's right.

Moderator : The next question is from the line

of Amit Kadam from Canara Robeco Mutual Fund .

Amit Kadam : So, just on the timelines that you provided for the commissioning of those biggest beds, so from that what I can understand is that from here on 300 beds of Dwarka is expected to get commissioned in start of the Quarter 4 of this year and then we have something coming in the quarter one was FY26.

Abhay Soi : Yes, last quarter FY'25, first quarter FY'26.

Amit Kadam : But then I am just focusing on the commissioning or revenue generation --

Abhay Soi : It's a little early to project exactly in which month it will happen . I mean when we said end of FY'25, we meant February/March . Now whether it continues to be February/March or its March/April , is a little difficult to project at this point . But assume it comes on stream next year for calculation purposes.

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Amit Kadam : So, the thing which I want to understand so during this particular period, so I understand we have a 300 bed to ramp it up , what are other levers in that particular year which could still make sure that our growth momentum continues?

Abhay Soi : We fall back on pay or mix, that's one. We have another 20 to 30 beds on the margins , which will come on stream through the year. This is in terms of occupancy, the quality of occupancy and occupied bed days. And of course, we have our case mix , which has increased as you have seen . That continues to hold strong. If you look at Q4 or Q1 last year versus Q1 this year , other than occupancy , and take out the 2 -2.5% increase in revenues, we have seen an increase in ARPO B because of increase in clinical mix largely , and in payor mix more international.

Amit Kadam : Just wanted to understand that when that Dwarka comes on stream what are the changes this thing will have on our overall number, because I think this will be a kind of a new complete standalone asset even though in the same region, but it would have its own individual standalone case where you have to do all the recruitments start from top to bottom.

Abhay Soi : Right, yes so typically there is breakeven period , but it shouldn't be consuming more than Rs. 30-40 crore .

Yogesh Sareen : There is some time taken to breakeven so there may be some loss in the initial phase. But that is the story with any green field .

Abhay Soi : But the loss should not exceed Rs. 30 -40 crore .

Yogesh Sareen : Yes, it's not a big sum of money that we are going to lose . Dwarka is a green field so we are targeting a breakeven in the 10th month of operations.

Abhay Soi: We intend to do better but our projection is about 10th month odd breakeven, but it should not consume more than Rs. 30 -40 crore .

Amit Kadam : And the third and final thing is on the international thing, so we have been doing it quite well in that particular segment, how do I look at this particular thing at least on an annualized basis how this piece should further do for us like moving towards double digit is like a percentage away, but then how do I look at this particular piece beyond that particular percentage point increase?

Abhay Soi : It's going well . I have always maintained that this is a big area of growth for us . Again I will stay away from making forward looking statements, but Afghanistan , which was 12% of our business , is still down to zero . Recently they have started some flights from Afghanistan , so that's a very big positive. Hopefully , in the next few quarters that should begin and should give us something as well. This business is growing, and I am going to avoid giving any forward -looking statements . But do keep in mind that we have had a 13% year -on-year growth , of which 3% has been through occupied bed days (OBDs) or occupancies as you may call it. Out of the 10% , if you take out a couple of percentage points for price increase , I think the balance has really been through clinical mix, a little bit has been through

rough international and so on.

Amit Kadam : Why I was asking, so one thing is that the volume or the inflow or the patient count may go up because there would be certain like some geographies would open up which were restricted or maybe there was some embargos. And second is that some efforts what you guys have taken by going into various geographies setting up front -end offices there that also all led to your inflow. And second is that this inflow could also have some kind of delayed surgeries because they were not able to have that particular --

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Abhay Soi : Well not so far. I don't think we have got business from any place , which had those embargos , but hopefully in the future some of it may be through that. We don't see any sporadic demand right now . It has been fairly kind of secular and should continue, I mean that's the plan. We have not seen any one-time business , if that is what you are saying , like bottled up demand which has suddenly come in and the rest is going to come later. We have seen this secular growth through the year.

Moderator : The next question is from the line of Alankar Garude from Kotak.

Alankar Garude : Firstly, Abhay when are we planning to commission the 100 Brownfield beds at Vaishali?

Abhay Soi : Vaishali was commissioned two years ago.

Alankar Garude : Brownfield, we had signed an agreement right for 100 beds?

Yogesh Sareen : We have signed an agreement to sell (agreement to buy for us) , so there are some conditions precedent (CPs) for that . Basically there are some ongoing litigation s at the High Court and district courts, and we have to settle those litigations first before we can take the handover of that piece of land.

Abhay Soi : Right now it's early stage . I mean right now firstly we have to procure the land . That's why I was wondering which Vaishali you are talking about. One, the agreement to purchase the land has to be sorted and then we will purchase the land and then will be building over there so that's –

Alankar Garude : And the same question on Gurgaon the seco nd piece of land I think there as well there is a litigation ongoing right?

Abhay Soi : Yes.

Yogesh Sareen : We have received a s tatus quo order from the Punjab High Court . There is a litigation ongoing since they cancelled the lease deed after one year of giving us the possession .

Abhay Soi : And taking full money.

Yogesh Sareen : And taking the full money . I think we have a good case there . The next date is somewhere in December.

Abhay Soi : But that w as in any case post 2028 plan , it was never part of the plan till 2028. Even if you see the CAPEX plans in the i nvestor presentation, it was something which was never even kind of accounted , neither the f unds nor commissioning nor start date for it was ever accounted for.

Moderator : The n ext question is from the line of Senthilkumar Natarajan from Joindre Capital Services Ltd.

Senthilkumar N.: Can you share the revenue and EBITDA growth for exclusively March, Shalimar in Q1FY24 on a year -on-year basis?

Yogesh Sareen : So, we already said that the revenue has grown by 3 7% YoY in Shalimar Ba gh and 43% EBITDA growth.

Senthilkumar N.: So, what is the contribution in topline from Shalimar --?

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Yogesh Sareen : So, I don't have the ready number .

Senthilkumar N.: And my second question is what is the gross debt as on June 2023?

Abhay Soi : There is no net debt, there is net cash.

Yogesh Sareen : On a gross basis , it will be Rs. 64 1 crore , which includes the CC limits .

Abhay Soi : But we have got cash against it of Rs. 1,681 crore .

Yogesh Sareen : Yes so there is a net surplus of Rs. 957 crore .

Moderator : The next question is from the line of Amit Thawani, an individual investor.

Amit Thawani : I saw the presentation and I saw the performance in the Oncology segment and even some of the peer hospitals have reported some r

eally good numbers in the Oncology

segment. So, I was just wondering what's happening, is there some kind of change in the health insurance policies that are allowing more coverage of Oncology or can you elaborate little on what's happening there?

Abhay Soi : Nothing in particular . The incidence of cancer is increasing, awareness is increasing , more people are acquiring health insurance and are able to afford to come to private sector hos pitals, which obvious ly for more high -end diseases are preferred compared to perhaps smaller nursing homes. So, we have seen a drive towards that.

Yogesh Sareen : Also, we have seen that in Oncology , there is more usage of ORC drugs, which improves the revenue / ARPOB because these ORC drugs are generally expensive .

Amit Thawani : And how does Oncology growth impact our in -patient, out -patient revenue, because how is really radiation and chemotherapy really accounted in terms of bed occupancy?

Abhay Soi : It's not , its day care. Any day care procedure is not part of occupancy or IPD. It comes through into the ARPOB because you take all the revenues and you divide it by in -patient beds . But any day care procedure is not part of occupancy.

Amit Thawani : So, the ARPO Bs will go up?

Abhay Soi : It's not a part of the ~3,500 beds since these are no t census beds. When you compute occupancy , you take a denominator o f total number of beds . We have ~3500 beds , which does not include the non -census beds, which is a chemo beds, dialysis beds, day care beds, ER beds , etc.

Moderator : The next question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai : I just have one question so Abhay you mentioned you added 44 beds though internal restructuring , etc., during the 1stst Quarter. How many more such beds can come say in 24?

Abhay Soi : We expect another 20 to 30 beds.

Damayanti Kerai : For this fiscal right, 20 to 30 beds?

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Yogesh Sareen : So, plan is to add another 40 beds . Some of them are getting added in July and some will happen in October .

Damayanti Kerai : 20 to 30 beds?

Yogesh Sareen : 20 to 40 beds.

Damayanti Kerai : And it will be an ongoing exercise right in coming years also you can choose to add such bed?

Abhay Soi : No, we can't . We are now pressing and at the final stages of capacity , we require that capacity so somehow you will find it. There is always some elasticity at the end. We move the laboratories out, we move the offices out, we move the kitchen out , etc. So, that's what we are doing to add these beds.

Abhay Soi : This is not annual; you can't do it every year let me just put it this way.

Moderator : The next question is from the line of Lavanya Tottala from UBS .

Lavanya Tottala : I just wanted a small confirmation, so the CGHS increase in rate is related to the radiology test that has not come in, in the Q1 any impact right. And what kind of impact that can have on our part?

Abhay Soi : No, some of it came in June and some of it came in May , but all of the impact so far has been about 0.6-0.7% on ARPOB.

Yogesh Sareen : For CGHS business , 60% of what we bill is material costs , which includes drugs, consumables, etc., and balance 40% are the items which are tariff items . 26% out of the 40% has been touched by the price revisions so far and on that 26% , we have around 50% price increase. In a nutshell , overall 5% price increase in the total billing for the PSU patients. And so that's the story today . The absolute amount for us would be around , if I take the Q4 volumes and the price increase which has happened till June, Rs. 49-50 crore number on an annualized basis.

Lavanya Tottala : All of that is still not there in Q1 number right then?

Yogesh Sareen : Yes, it won't be there fully. Q1 would be probably Rs. 10 -11 crore out of the Rs. 50 crore full year annualized number that we see in terms of price revisions.

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Moderator : The next question for the line of Alankar Garude from Kotak .

Alankar Garude : Just one follow up, historically in the run up the general elections has Max seen any increase on the receivables front given the higher institutional mix?

Abhay Soi : Not really .

Yogesh Sareen : No. There was an increase in the receivables by end of June, but in July we have seen some of it unwinding. So, we don't see any impact of that.

Alankar Garude : Yes but even in the prior years be it 2014, 2019 no impact per se?

Abhay Soi : No.

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Moderator : Thank you. Ladies and gentlemen that was our last question. I would now like to hand the conference over to the management for closing comments. Over to you sir.

Abhay Soi : So, I would like to thank each one of you for taking time the time out and being on the call . Appreciate your time, thank you and see you next quarter.

Moderator : Thank you. On behalf of Max Healthcare Institute Limited that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy .

## Call: Nov 2023

Max Healthcare Institute Limited  
(CIN: L72200MH2001PLC322854)  
Regd. Office: 401, 4th Floor, Man Excellenza, S. V. Road, Vile Parle (West),  
Mumbai, Maharashtra -400056  
Phone: +91 - 22-26101035,  
E-mail: [secretarial@maxhealthcare.com](mailto:secretarial@maxhealthcare.com),  
[investors@maxhealthcare.com](mailto:investors@maxhealthcare.com)  
Corporate Office: 2nd Floor, Capital Cyberscape, Sector -59, Gurugram, Haryana 122011  
[www.maxhealthcare.in](http://www.maxhealthcare.in)

November 13, 2023

Listing Department ,  
National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,  
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Dalal Street,  
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on November 7, 2023  
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations,  
2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organized on November 7, 2023,  
on financial results of the Company for the quarter and half year ended September 30, 2023.

The said transcript is also available on the website of the Company at  
[www.maxhealthcare.in/investors/investor-resources](http://www.maxhealthcare.in/investors/investor-resources).

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Arora  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited  
Q2 & H1 FY24 Earnings Conference Call  
November 07, 2023

Moderator: Good day and welcome to the Max Healthcare Institute Limited Earnings  
Conference Call. Please note that this conference is being recorded.  
I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank  
you and over to you, sir.

Suraj Diga walekar: Thank you, Aman . Good morning, everyone, and thank you for joining us on Max Health care Q2 and H1 FY24 Earnings Conference Call. We have with us Mr. Abhay Soi, Chairman and Managing Director, Mr. Yogesh Sareen, Senior Director and Chief Financial Officer, and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning. We will begin the call with opening remarks from the management, following which we will have the forum open for an interactive Q&A session. Before we start, I would like to point out that some statements made in today's call may be forward -looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make the opening remarks.

Abhay Soi: A very good morning to everyone. We are pleased to welcome you to Max Health care's earnings call for the second quarter and first half of fiscal year 2024. Let me start by stating that our performance in the first half of this fiscal year has set a commendable precedent for us to follow in the latter half. We recorded a year -on-year increase of 17% in network revenue and 20% in EBITDA in H1, while Q2 turned out to be the 12th consecutive quarter of year -on-year growth. Our Q2 performance this year largely mirrored our quarter -on-quarter performance last year ,

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alluding to the steady state of our operations as well as secular demand for quality healthcare services.

Further, with a granular focus on execution and capital allocation, as is evident from our pre -tax ROC E of 38.3% in Q2, we are well poised for the next leg of growth that is set to come from planned capacity expansion as well as inorganic opportunities. On that note, we are happy to share that the developer of an upcoming hospital in Dwarka has applied for the occupancy certificate, which is a significant milestone. It is the final milestone. We expect to commission the same in the fourth quarter of the current year.

Moreover, our most recent brownfield expansion, Max Shalimar Bagh, has reported an overall average occupancy of 78% and a year-on-year revenue and EBITDA growth of 41% and 48% , respectively in the second quarter.

On the clinical front, we have signed a memorandum of understanding with Intuitive Surgical, the US -based pioneers of robotic surgical systems, to establish Southeast Asia's first Total Program Observation Centre located at our Max Saket facility. This centre is expected to have a positive impact on both India and Southeast Asia's surgical healthcare ecosystem by enabling healthcare professionals to drive advancements in patient care using robotic -assisted surgery and elevate surgical healthcare standards in the region.

Moving on to the highlights of our Q2 performance :

- 1) Occupied bed days grew by 3% year -on-year and 5% quarter -on-quarter, reflecting an average occupancy of 77% for the quarter. 93% of the year -on-year and 118% of the quarter -on-quarter increase in occupied bed days was driven by preferred channels, which is cash, insurance, TPA, and international (CTI) . With the increase in occupied bed days and marginal drop in A LOS , the inpatient discharges were up by 7% year -on-year.
- 2) Even OP volumes exhibited a strong growth of 14% year -on-year and 4% quarter -on-quarter.
- 3) Institutional bed shares fell to 27.3% compared to 27.9% last year and 29.7% in Q1 this year. However, after excluding Max Shalimar Bagh, our most recent

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expansion, overall institutional bed share stood at 25.4% during the second quarter.

- 4) Average Revenue Per Occupied Bed (ARPOB) for the quarter stood at INR 74,600, growing by 13% year -on-year, and remaining flat quarter -on-quarter due to seasonality. Year -on-year improvement was witnessed across all specialties, with oncology being the key driver.
- 5) Network gross revenue was INR 1,827 crore compared to INR 1,567 crore in the Q2 last year and INR 1,719 crore in the previous quarter. This reflects an increase of 17% year -on-year, led by growth in ARPOB and occupied bed days (OBDs) . Quarter -on-quarter growth of 6% was mainly driven by increase in OBDs .
- 6) Revenue from international business grew significantly by 25% year -on-year and 11% quarter -on-quarter, now accounting for around 9% of the total revenue from our hospitals. During the quarter, we have operationalized company -owned patient assistance centre (PAC) in Nepal, while all formalities for the Bangladesh centre have been completed. We expect to operationalize this centre shortly. This is despite the Afghanistan business, which was 12% of our total international

business, still down to zero.

7) Direct costs were lower quarter -on-quarter due to increase in medical patients , attributable to seasonal vector -borne diseases. On the indirect costs side, while the overall percentage was lower, there was an increase in absolute costs primarily due to marketing costs for international channels and seasonal increase in power consumption.

8) Network operating EBITDA stood at INR 497 crore, just below the magic mark of INR 500 crore, reflecting growth of 21% year -on-year and 14% quarter -on-quarter. Accordingly, the operating margin increased to 28.7% versus 27.7% in the Q2 I

ast year and 26.8% in the previous quarter.

9) Most importantly, annualized EBITDA per bed rose to INR 75 lakhs, yet again our highest ever, clocking a growth of 17% year -on-year and 7% quarter -on-quarter.

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10) Profit after tax was INR 338 crore versus INR 267 crore in Q2 last year and INR 291 crore in the previous quarter on a like -to-like basis. The year -on-year improvement of 26% was primarily attributable to flow through of improved EBITDA and lower finance costs.

11) Free cash flow from operations was significantly higher this quarter at INR 436 crore, of which INR 90 crore was deployed towards ongoing capacity expansion projects. Net cash position improved to INR 1,303 crore at the end of September 2023 compared to net cash of INR 42 crore same time last year.

12) Continuing efforts to support the local communities, we treated approximately 39,000 patients in OPD and 1,300 patients in IPD from economical weaker section s free of charge .

13) Both our strategic business units continued to tr ead strongly on the ir growth trajectory :

- Max@Home reported a top line of INR 42 crore , reflecting a growth of 23% year-on-year and 8% quarter -on-quarter . We continue to receive good feedback for our services and the same is reflected in the SBU 's revenue growth.

- Max Lab, the non -captive pathology vertical , offers its services in 36 cities and now has an operational network of over 1,000 collection centres and active partners. This SBU reported a gross revenue of INR 39 crore, reflecting a like -for-like growth of 32% year -on-year and 15% quarter -on-quarter.

Now , coming to the status on our upcoming expansion projects :

- As most of you know, 122 beds at Shalimar Bagh have been operationalized at the start of this financial year. And as mentioned earlier, the hospital reported an annual occupancy of 78% for the quarter.

- For 300 beds at Dwarka – Application for occupancy certificate (OC) has been submitted in October and majority of the interior works have been completed. And some of it is just being finished as we speak. We expect to

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commission the hospital in later half of Q4 , subject to receipt of OC by the developer.

- For 329 beds at Nanavati – Excavation and raft work is complete. Steel fabrication up to the ground level and slab work have also been completed. Ground level structure is expected to be completed in the current quarter and the project continues to be on schedule.

- For 300 beds at Sector 56, Gurgaon in Phase I – D-wall has been completed and the site excavation is almost done. EPC contractor is already on board and design development is under process. TDR approval for additional 0.5 FAR has been received and the project is on schedule.

- For 190 beds at Mohali – D-wall is complete and excavation work is underway . All statutory approvals to start the construction have been received and the project is almost entirely on time. EPC contractor has been mobilized and the design development is in progress.

- 350 beds at Max Smart at Saket Complex , which had seen some delays initially – We have now initiated the process of transplanting the trees as permissions had taken some time to come . This has been on the critical part, but now the project is back on schedule and work should start by December 2023 .



- For 300 beds at Vikrant at Saket complex – Environmental clearance (EC) has been received and the submission of drawings in the Municipal Corporation of Delhi is in process .
  - For 250 beds at Patparganj – Drawings have been submitted to the Municipal Corporation of Delhi and the application for environment clearance has been submitted.
- So, all the other projects are on schedule and there is no delay as such. And finally, coming to the overview of the company's performance in the first half of the financial year:

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#### 1) Network gross revenue stood at

INR 3,546 crore, reflecting a growth of 17% year-on-year.

2) Network operating EBITDA grew by 20% year -on-year to INR 933 crore . Increased ARPOB , improved case mix and augmentation of network bed capacity by 130 beds resulted in margin expansion to 27.8%, while EBITDA per bed grew by 15% to INR 72.8 lakhs per bed.

3) In the first half, we generated INR 697 crore of free cash flow from operations after interest, tax, working capital changes and routine capex, of which INR 128 crore was deployed towards ongoing expansion projects.

With this, we open the floor for Q&A.

Moderator: Thank you very much. The first question is from the line of Damayanti Kerai from HSBC.

Damayanti Kerai: My question is you continue to see progress in reducing bed share to institutional patients. So , a few quarters back, you had given an indication that we would like to bring it down to the industry level. But with Shalimar Bagh, I guess, you have taken on more institutional bed to ramp up occupancy, etc. So, do you still target to bring it down to, say, industry average and when it will likely happen?

Abhay Soi: There is no classified industry level , so to say . I think it highly sort of changes between metros and non -metros. You have more PSUs, headquarters, etc., out of Delhi NCR, so you have a larger share of institutional businesses coming through. Now , two-three things have happened.

One is apples -to-apples from 29.7%, we have come down to about 25% in the current quarter. Second is that it's on increased capacity, including Shalimar Bagh . Thirdly , certain rates have moved up in CGHS and we're expecting certain other rates for institutional business to move up in the current quarter , because of which we have taken the foot o ff the accelerator a little bit .

And finally, even within the institutional business, there's been a churn in the specialties we are catering to and the ones we are not catering to. And all of it largely comes down and plays out in your higher EBITDA. So , what we're seeing is, although because of the overall capacity constraints that we have, our occupancy has

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moved up by only 3% year -on-year, but there's also been a churn of about 3% to 4% year-on-year within the payor mix. And all of that tends to translate into a higher percentage margin as well as EBITDA per bed.

I mean, apples for apples, the same inventory going forward will be coming down as far as institutional business is concerned, but to your point, as you rightly mentioned, as and when we have new capacit ies coming in, those capacities initially will see a n increase in institutional business. So , in percentage terms, it will move up, or remain stagnant just when those capacit ies are coming. But I think overall, it still translates to better EBITDA margins.

Damayanti Kerai: Okay, so you mentioned like...

Abhay Soi: Just to complete that point, you may have seen because of new capacity coming in Shalimar Bagh, the institutional business has moved up. But if you see the EBITDA coming from those incremental beds, basis this increase in institutional business , it is yet at a 40% margin.

Damayanti Kerai: Okay, so that means obviously you are getting much better realization from these set of patients also. As you mentioned, rate hike in CG HS could be one of the reasons which might be contributing and then obviously specialty mix?

Abhay Soi: Not that, this is largely because of the operational efficiencies. We have a huge operating leverage for the new beds. We don't have the fixed costs, etc. As a result, what happens is that even the lower payor mix becomes more viable and even more viable on the new beds. Because our operating cost is very low on the incremental beds.

Damayanti Kerai: Okay, so it's primarily driven by efficiency as you mentioned. Like you have better absorbed overheads there that is resulting in these kinds of numbers .

Abhay Soi: That's right.

Damayanti Kerai: Okay, and just a clarification, you mentioned there has been a bit of hike on the CGHS patient s also. So right now, like what is the difference between that price channel and then the normal cash and others, like very broadly .

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Yogesh Sareen: If you take the ARPOB of these two channels, the CTI channel and the preferred channel , CTI is 85% higher than the CGHS / PSU channel.

Damayanti Kerai: So the preferred channel is 85% better ARPOB number?

Abhay Soi: If your PSU ARPOB is 100, CTI ARPOB will be 185.

Damayanti Kerai: Okay, 100. Okay, my last question is your difference between gross revenues and net revenues which you go for pro forma financials, that's primarily driven by what you pay for EWS patients, right? That's right.

Yogesh Sareen : Largely that number, yes.

Damayanti Kerai: Yes, and I'm seeing that number has broadly remained somewhere like 5% of gross revenues. So, should we assume similar numbers to trend, even if like say we are commissioning new facilities ahead and then according to government rules, we have to allocate some beds for the EWS?

Yogesh Sareen: Dwarka does not have any EWS obligation .

Abhay Soi: Dwarka does not have, Mumbai will have, and others will have. I mean, Gurgaon also won't have.

Damayanti Kerai: Okay, so Dwarka and Gurgaon does not have.

Abhay Soi : Other than Gurgaon, Dwarka and Mohali , all others will have EWS obligations .

Damayanti Kerai: Okay, okay. Thank you.

Abhay Soi: Out of around 2,600 additional beds, I think approx. 1,000 beds will not have EWS obligations , balance will have.

Damayanti Kerai: Approximately 1,200 beds will be utilized and then others don't have such requirement ?

Abhay Soi: Yes, so let's say we have approx. 2,600 beds further coming up, out of which around 1,000 beds will not have any EWS obligation s. Now the balance 1, 600 will have the 10% obligation , so let's say about 160 beds out of 2, 600 beds.

Damayanti Kerai: Okay, got it. Thank you. And all the best.

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Abhay Soi: Thank you.

Moderator: The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: Thank you for the opportunity, sir. First on the housekeeping question on the international patient bed share. What was that for the quarter?

Yogesh Sareen : Yes, that will be around 5%.

Kunal Dhamesha: 5% only?

Yogesh Sareen : Yes, the ARPOB is higher, so that's how the revenue share goes up. It's 5.5% to be exact .

Kunal Dhamesha: 5.5%, okay. Okay, so then probably the pricing, etc., kind of more or less has remained the same. Okay, thanks for that. And secondly, when I look at our specialty mix, oncology therapy, if I see, has been growing at almost 2x the overall revenue growth, at least for the last two quarters. And even if I loo k at a longer -term trend, for the last nine quarters in a row, it has grown faster than our overall revenue growth. So what are we doing differently there? And is it a market growth or?

Abhay Soi : Yes, I think the overall growth is 17%. And oncology growth is about 26% -28%.

Not exactly double. But there's been a focus on oncology, and we've also seen a larger disease burden playing out. We've also had a focus on robotics. And as we are moving up the value chain, what we are seeing is more people are choosing better technology.

As you're seeing higher percolation of insurance, then people tend to do less of window shopping for higher end procedures . They go to more established corporate hospitals and brands. We are seeing more -and-more people for oncology and some of the other specialities , whereas they would have gone to smaller places earlier and now coming to larger hospitals . So, insurance plays an important part.

Yogesh Sareen : Also, oncology tends to have higher entry barriers . There's a lot of investment required in equipment, bunkers, infrastructure, et

c. To that extent , the patients do navigate towards bigger players.

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Kunal Dhamesha: Sure. And is it possible to share the split of this 25% of revenue mix between, let's say, surgical and non-surgical? Because we have also have chemotherapy and radiotherapy included in this 25%, right?

Yogesh Sareen : Well, that's not a public number. But obviously, a large part of this will be chemotherapy . But I would say, it's a fair share of all three – radiation, surgical and medical. We haven't publicly disclosed that number , so we won't publicly disclose the exact split between the three.

Kunal Dhamesha: Sure. And is it fair to say that oncology would be highly accretive to profitability for us?

Yogesh Sareen : Yes. The oncology happens to have a higher ARPOB. To that extent, yes, it'll be higher profitability also.

Abhay Soi: It depends on how you're looking at it. I mean, it also occupies more space. So, there's also a return on capital and return on real estate over there.

Kunal Dhamesha: Okay. So return on capital is also higher in your view? Or does it require more space so it would more or less...

Abhay Soi: Return on capital is even with the rest.

Kunal Dhamesha: Okay. Great. Thanks for that clarity. And secondly, on d rivers of our strong ARPOB growth of around 13% year -on-year for the first half, if I kind of, do some back calculation using the bed share and revenue share, it seems that CGHS ARPOB or institutional ARPOB would have at least gone up by around 20%, 25%. Is that a fair number or I'm overestimating, underestimating?

Yogesh Sareen : No, that's right. The PSU ARPOB has gone up by 28%.

Kunal Dhamesha: 48% for the first half ?

Abhay Soi: 28% year-on-year. And that's not because of price increase. It's also because of the mix change, which I was mentioning earlier. It's not a change in price. The change in price will not even have INR 14 crore impact on your overall revenue.

Kunal Dhamesha: Okay. Perfect. Thank you. I have more questions. I'll get back in the queue.

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Moderator: The next question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil : Hi. Good morning and congrats on a great set of numbers. Just one question on the expansion plans. For all the facilities, the greenfield facilities that are coming up over the next couple of years, what's your internal target for EBITDA breakeven, in how many months or quarters?

Abhay Soi: Almost 90% of our expansion is brownfield. And normally, we see EBITDA breakeven in a quarter or two if not the first quarter itself. Our last brownfield experience was that we had EBITDA breakeven , and w e were hitting 40% margin s within 40 days.

Yogesh Sareen : On Greenfields , we do see breakeven within the first 12 months. That means 11th or 12th month should be the EBITDA breakeven month.

Abhay Soi: Greenfields are only 10% of the total expansion.

Bino Pathiparampil : Okay, understood. So, within a year for Greenfields and within a quarter for brownfields?

Abhay Soi : Yes.

Bino Pathiparampil : Great. Thank you.

Moderator: Thank you. The next question is from the line of Nitin Agarwal from Dam Capital.

Nitin Agarwal: Hi, can you give us some numbers on what has been the increase in discharges, outpatient discharges of Q -o-Q and Y -o-Y?

Abhay Soi: 7%.

Nitin Agarwal: This is Y -o-Y?

Abhay Soi: Yes. Q-o-Q doesn't matter because you have more medical patients in Q2 . So, when you have more medical patients, your ALOS is lower, and your discharges tend to be higher . Look at everything on a year -on-year basis for better comparison .

Nitin Agarwal: And secondly, on the seasonality part of it, typically, how should one think about seasonality in our business? Q2 obviously is bigger than Q1. And how should we think about the rest of the year with respect to Q2?

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Abhay Soi: See, Q4 is the best quarter

in a year. Q2 is the second -best quarter. Q1 and Q3 are weak quarters. And that seasonality in the business that happens every year.

Typically, your H2 is better than H1.

Nitin Agarwal: And I guess given the way things are, that's a trend we should follow even this year for us.

Abhay Soi: That's right unless something disruptive happens. Typically, the secular trend in

healthcare is that H2 is better than H1. And Q4 is peak because of the burden of the diseases, etc. Also, Q2 is the second best simply because you have seasonality due to dengue and vector season. Q1 is weak because you have just had an increase in salaries and fixed costs on the 1st of April. So, your margins are kind of squeezed. And Q3 is the festival season, which is Diwali, Christmas, etc.

Nitin Agarwal: And secondly, on your expansion plans, barring beyond what we've already outlined so far, how are we thinking about expansion? We've got enough cash reserves on books now in terms of the inorganic growth opportunities which are there. I mean, how do you see the landscape playing out? There's been a lot of private interest in the space, which probably, I don't know, would have had its own challenges for value buying. So, how are you looking at the inorganic growth opportunities outside of NCR?

Abhay Soi: I think there are quite a few. There are 21 cities that we're looking at. And we've been busy at it. And hopefully in the near future, very shortly, we should come up with some surprising stuff. But yes, we want to maintain some fiscal discipline. There are quite a few opportunities, both on the build side, partner side, on the asset-light model, as well as certain acquisitions as well. So, yes, we intend to deploy this capital.

Nitin Agarwal: And last one, on the Shalimar bag expansion that we did in Brownfield, what is the capacity utilization on that?

Abhay Soi: 78% on the overall, new and old beds combined.

Nitin Agarwal: And when you would have put the new capacities, the older one would have been closer to 80% plus.

Abhay Soi: 82%-83%. Yes.

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Nitin Agarwal: And just sort of reconfirming, on the incremental beds, we are making 40% incremental margins.

Abhay Soi: Yes, that's right. And within 40 days of opening those beds.

Nitin Agarwal: Right, right. And in your assessment, incremental Brownfields that we're going to be putting out, how should we think about -- We did talk about the first quarter break - even. But in terms of... Is Shalimar Bagh an exception in the way it's played out, or this is going to be a template that's broadly going to get replicated across the new Brownfields?

Abhay Soi: Look, honestly, our Shalimar Bagh experience was the same as our experience in Vaishali before that as well. Because, theoretically, you're tapping into untapped demand on your doorstep to start with. And then you have operating leverage -- you don't have any real fixed costs with those incremental decisions. Theoretically, this should be the template going forward as well. I don't see that changing.

Moderator: The next question is from the line of Ankur, an individual investor.

Ankur: Hi. I think my question is partly answered in one of the previous questions that was raised by one of the participants. Really, it was about the last two years. There's a bit of a concern that we haven't acquired any project and added anything on to our already announced development pipeline. And obviously, we've been running an underleveraged balance sheet for a while now. And then now, we've got all this cash accumulating.

And you've talked about acquisitions, M&A, and all of that. But I mean, it's two years since we added anything. And, on the Greenfield side of things, are we looking at any Greenfield projects that we want to add? And I know you keep saying imminently that there should be some announcements. But it also takes about, I think, if you add a new pr

ject, a Greenfield project, about three or four years before it's operationalized. So, if you can throw some light on all these things, please?

Abhay Soi: Yes, I think there's always a tug of war between the desire to expand and the fiscal discipline. And one has to maintain that. It's not as if we haven't been looking. And we are quite certain that shortly we should be able to deploy the cash. Do keep in mind that it's not a huge amount of cash. Because even to construct a 500-bed

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hospital, you require about INR 1,000 crores. To acquire a 500-bed hospital will cost you another maybe INR 1,000-1,500 crores.

So, one or two acquisitions and you're done. On one side, we are excited about the fact that we are accumulating cash. But we are also conscious of the fact that this amount of cash and even the ability to leverage the balance sheet is not going to take us very far. Today, transactions are available at 15-16X EV/EBITDA. What that means is, even at entry, if I was to go and spend INR 5,000 crores, we'll be able to

buy INR 300 crore EBITDA? That's about 15% of my total EBITDA.

So, I can increase my EBITDA by 15% by deploying INR 5,000 crore. And that would pretty much use up all my cash and my leverage ability. So, it's important to, while we notice that there's cash accumulating, do keep in mind that one, this is a capital-intensive sector. And secondly, there are a massive number of opportunities in the sector yet.

So, if we want to sort of participate in that, we need to accumulate the cash and spend it with the right amount of fiscal discipline at the right time. Because there is money, but it's not that much money also.

Ankur: Yes. And also, like, you know, like as where we've been going and it's almost, let's say, 20% sort of EBITDA growth, cash flow growth over the last couple of years.

And going forward also, it seems we're going to continue that trajectory over the next four, five years. So, then beyond that, to continue growing at the 20% sort of rate, we'll also need to keep adding the bed capacity at that sort of rate. Right?

So, we need to have like this continuous development pipeline, which keeps, you know, every year keeps adding projects year on year so that, you know, the growth continues for long duration. So, I'm sure you guys are working on it, but just -- and you mentioned, like, we're looking at 20 cities. So, but there has been no, like, actual project acquisition. So, that was my only question. But, yes.

Abhay Soi: No, you're absolutely right. Just keep in mind two aspects. Right? In the last two years, there's not been any significant capacity expansion. Yet, we're seeing a round 20% increase in EBITDA. So, in the next three to four years, we're going to have around 2,600 beds coming up, increasing your capacity, 85% of which is through

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brownfields. That is almost like doubling your capacity over the next three to four years.

Those are coming on stream. So, shouldn't that be giving you expansion for the next five or seven or ten years itself? And given the fact that your breakeven is so short in these brownfields, it'll add a bunch of more cash flows, all of which again gets deployed and gives you further returns. So, there are three streams of growth over here.

One is your current bed capacity, which has been growing in terms of EBITDA. Then all the expansions that are already announced, which are already underway, which we said are largely online. And the third is what you want to do with this cash. Right? It's an exponential 3X strategy. It's not a strategy of 15% -20% growth. If I was not to sort of deploy this cash, give it all back as dividends, yet you'll be doubling your capacity over the next three years to four years.

Ankur: Yes. I think that's it, from my side. I understand where you're coming from. And also, it's clear, like, as the cash flow keeps accruing

over the next three, four years

and they keep growing, you continue to add on to your development pipeline and continue this growth for long term. So, that's it from my side and thank you and all the best.

Moderator: The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services.

Tushar Manudhane: So, just on the organic basis, on EBITDA per bed, while we've already, I presume you've optimized in terms of efficiency at the overall level. So, how do you think about the levers for improving EBITDA per bed for the next two to three years?

Abhay Soi: I think EBITDA per bed, now the first half has already happened, right? And second half is usually marginally better than the first half. At least for the rest of the year, some sort of trajectory has been already articulated.

Tushar Manudhane: Beyond FY24, how do you think about it? In the sense, the case mix or the payor mix is also, we have already taken good price hike on account of institutional patients, the insurance penetration is...

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Abhay Soi: No, it's not the price hike. In fact, the price hike impact is only INR 14 crore. There's a 28% increase in PSU ARPOB and that's due to the clinical mix, because we're moving into higher end procedures. We're distilling the procedures.

Tushar Manudhane: Right. Okay.

Abhay Soi: Price hike has been negligible, in fact.

Yogesh Sareen: In fact, of the 28% increase in PSU ARPOB, only 5% is due the price hike in the PSU segment.

Abhay Soi: So, like Yogesh rightly pointed out, out of the 28% increase in PSU ARPOB, only 5% increase is due to price hike.

Tushar Manudhane: Right? No, I meant to ask that how much more can further be optimized, if not on price hike, but other levers so as to drive the EBITDA per bed, maybe in mid-teens or more or less growth over the next two to three years?

Abhay Soi: In a similar fashion, as our payor mix starts moving up on a particular trajectory and our clinical mix moves up on a particular trajectory, all of it goes to our EBITDA.

Our indirect cost is increasing by around 6-7% every year. The difference between our revenue increase, now you have to make an assumption of our revenue increase, and the indirect cost increase is all going down to the EBITDA effectively.

Tushar Manudhane: Got it. Understood. Thank you.

Moderator: Thank you. The next question is from the line of Banshi Desai from JP Morgan. Please go ahead.

Banshi Desai : Hi. Thanks for the opportunity. So, I have one question and this is on the advancement of robotics that we have seen in the overall healthcare space. We have seen more and more specialties using robotics and even the non-complex ones are making use of robots. In general, where are we today in terms of surgeries which are getting done on robots and what is the scope here, where it can go to? Also, I am assuming this will be lucrative enough. What is it in terms of ARPOB and margins, how different they are compared to our traditional surgery work?

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Abhay Soi: First and foremost, although robotics has been around for some time, over the last couple of years, there has been a sudden uptake of that and acceptability between both doctors and patients has been quite dramatic as far as robotics is concerned. To be honest, it surprised us also on the upside. As I mentioned earlier, our total number of robotic procedures have more than doubled in the last one year. And, it has been a pleasant surprise. For most specialties now, you get into this flywheel concept as their acceptance goes up. We are being pushed by many of our hospitals and many of the clinicians now, to set up robots because, frankly, it has become more and more viable.

What happens is, of course, it's at a higher cost compared to laparoscopic, or even for that matter, a general surgery for the same procedure. So, it leads to higher ARPOBs. But

the contribution levels from robotics are lower. And EBITDA in terms of margins is lower, in percentage terms, although in value terms it is higher. And this is something that we have previously mentioned with respect to both higher-end payor mix and clinical mix, that you get lower percentage margins but higher value in terms of absolute EBITDA coming from this.

Yogesh Sareen: It also helps us in terms of reduction in ALOS, right? EBITDA per bed is obviously better. But in terms of margins, that may probably be lower than the overall average.

Abhay Soi: Right now, it's a little difficult to present a trajectory of where do we see growth happening and will it continue to be 100% growth, or will it taper down to 70% or 50%? Or will it increase from here?

Banshi Desai : Got it. But the adoption has increased. And in general, it also improves your throughput, right, within a particular specialty?

Abhay Soi: That's right. And, I mean, overall, it's got all the positives with it. And, as public acceptability increases, as technology advances, there are entry barriers in this, smaller places can't adopt it increasingly and the awareness increases, then you see this move towards larger hospitals and more sophisticated healthcare systems. It also results in better medical outcomes for the patients.

Banshi Desai : Got it. Thank you. That's it from my side.

Yogesh Sareen: Thank you.

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Moderator: The next question is a follow-up question from the line of Kunal Dhamisha from Macquarie.

Kunal Dhamisha: So on Dwarka. So as we are kind of getting closer to the commissioning of the facility, would we like started hiring in terms of doctors or key specialties, nurses, paramedical staff, or would it be more closer to the commissioning?

Abhay Soi: We have started all of that. And at least all the head of the programs and functions are already in place and in the system. As we speak, they are working in some of our other facilities. So, as far as the soft power is concerned, the people are concerned, all of it is in line and as per schedule. There is no lull on that. And there's enough availability and excitement around this facility from a clinical standpoint.

Kunal Dhamisha: And would it be more like a staggered hiring in terms of specialties, or we would go with the full-fledged 300-bed operationalization on day one?

Abhay Soi: No. We will do about 164 beds, like Yogesh pointed out. And as we require more

beds, we will open up more and more.

Kunal D hamesha: And one follow-up on the robotic surgery, while you have alluded that it's good from the ARPOB perspective and the absolute EBITDA perspective, but in a longer term, if let's say even the other hospitals also kind of start affording it, does that bring the market to the equal level in terms of surgical outcome, etc., and then the importance of brand or surgeon's skill kind of get reduced? Do you see that happening?

Abhay Soi: Not particularly. I think the market is growing. There's only so many players that can adopt robotics. Your people also have to be trained on it. You have to have availability of the talent to be able to do this. All of that will take time. But before that, your market would have already expanded. And we've seen this, right? I mean, why robotics? As far as any technology is concerned, the larger players adopt it first. The smaller players adopt it thereafter. The market only continues to expand. But I've never seen the larger players' market share go down because the smaller players have adopted the technology thereafter. You actually end up increasing the size of the market. All the smaller players also have a hand in increasing awareness for that product.

Kunal D hamesha: And for us, is it more like a capex model or is it more a pay-per-use model?

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Abhay Soi: Actually, we started robotics as pay-per-use because

we were quite unsure. But we've bought back or bought more than 50% of the robots recently because it kind of surprised us in any case. So, yes, at this point in time, we have a hybrid model simply because we started off with pay-per-use. But now we've got into buying it back. So, we've bought more than 50% of the robots that we have.

Kunal D hamesha: Okay. And return on capital is higher on...

Abhay Soi: Of course. That's why we are buying.

Kunal D hamesha: Okay. Thank you, sir.

Abhay Soi: Thank you.

Moderator: Thank you. The next question is from the line of Naysar Parikh from Native Capital.

Naysar Parikh: Yes, hi. Thanks for the opportunity. My first question was on the overall industry trend. Are you seeing still there is a gap between supply and demand growth? And do we see for the next, couple of years, there will be leverage to continuously grow price and ARPOB? How are you seeing that?

Abhay Soi: I'm not seeing it over the next couple of years, but I'm seeing it over the next few decades. I think this is a multi-decadal opportunity. There is a huge amount of under penetration. There's a massive gap between supply of quality healthcare and demand for quality healthcare, which is only increasing as we go by.

So, that's the reason -- and India perhaps offers this opportunity which nowhere else, no other country, no other health system in the world does. I mean, at one side, you operate almost like a utility because it's inflation-free, it's an insulated business. But on the other hand, you also have this massive growth opportunity because of just to sheer lack of penetration or availability of quality facilities.

Naysar Parikh: Got it. And on the international patient side, you said around 5-5.5% of your beds. Is there scope for that to significantly go up to, say, 10% or higher? And how are we seeing the international traffic on the hospital side?

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Abhay Soi: Traffic has grown by 25% in revenue terms at least over last year. Even in terms of total number of beds, that is in terms of volume of patients, also it has grown by 25% over the same quarter last year. And 11% over the last quarter itself.

And do keep in mind, that this is without 12% of the total business which came from Afghanistan earlier, now being down to zero. If you assume that coming back to normalcy, it will mean an increase of around 30% or more over last year. So, I mean, where does this train stop? I think we haven't even scratched the tip of the iceberg. You know, this should continue, in my mind, at a significantly higher pace than the rest of the hospital growth.

Naysar Parikh: Got it. And the 2,600-bed expansion that we have, can you give us some idea in terms of how much can come in the next six months and how much in FY25?

Abhay Soi: 300 beds should come in by end of FY24, current fiscal year. FY25, towards the end, you will have another 329 beds of Nanavati coming. Then Mohali, another 190 beds again, same time next year. And 300 beds at Gurgaon, same time next year. So, you have about 819 beds coming in the next one year.

Naysar Parikh: So, 300 by the end of this year and another 800 to 900 by the end of next year.

Abhay Soi: Yes, it's available in the presentation on our website. You can see year-by-year capex and date of completion.

Naysar Parikh: Got it. And just one last data point. You said the institutional ARPOB obviously has improved significantly. So, now, where does the gap between institutional and non-institutional ARPOB lie? What would be the gap?

Abhay Soi: 85%. If ARPOB for institutional is INR 100, for non-institutional, it is INR 185.

Naysar Parikh: Okay. Thank you.

Moderator: The next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: So, you mentioned about expecting to make some announcements on the expansion bed shortly. So, just wanted to check whether

when it comes to different expansion models,

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like say between partnership, built-to-suit, O&M, and acquisitions, do we have any specific preference?

Abhay Soi: No. I mean, acquisitions are, at the right price for the existing, but otherwise built-to-suit, in the sense being asset-light, is very good. We don't particularly like Greenfields.

Alankar Garude: Understood. Okay. And on that point, on this CARE acquisition, you have been providing regular updates, including one yesterday night. Now, on one hand, the appeal is reserved for orders and on the other hand, Blackstone seems to have announced the acquisition, at least as per media articles. So, not sure what to make out of this. Can you please help elaborate on the current situation?

Abhay Soi: The situation is what it is. We have made an appeal to the High Court, now it's for the High Court to decide.

Alankar Garude: Understood. Okay. And one final question.

Abhay Soi: I can't give you any opinion on that. It's for the judge to decide.

Alankar Garude: True. And one final question. Now, when it comes to some of these allied services, we are into diagnostics, then at home, but we have seen some of the other hospital chains doing far more as far as some of these allied healthcare services is concerned, getting into pharmacies, then insurance, diagnostics in a maybe a bigger way. So, maybe in future, not immediately, but in future, are we open to being more aggressive on some of these allied services?

Abhay Soi: I am open to anything and everything in the healthcare business, which others have succeeded in. Philosophically, we don't like to do pioneering things. When they succeed, we will study, we will learn from their mistakes and we will gain confidence from what they got right, and then we will do it better, like we already do. Anybody who does it, I'm very open to doing those things, but let somebody else do it successfully first. There are more than enough examples in front of you where people have jumped into a situation and got it wrong. That's not a game we play. That's not what we're good at, to be honest.

Alankar Garude: Fair enough. Okay. That's it from my side. Thank you.

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Moderator: The next question is from the line of Amit Kavani, as an individual investor.

Amit Kavani: Hi, my first question is that I don't know if it's already been asked or have you answered it, but the revision impact on the institutional business, can you tell us what is expected to be in the December quarter and March quarter?

Abhay Soi: I have no idea what it is expected to be because they haven't taken us into confidence on that. So far, the impact has been 5% of the ARPOB of the PSU business, but we have absolutely no clue how the government is thinking about it.

Amit Kavani: So, no further institutional revision has been announced?

Abhay Soi: No. We were expecting it this quarter and now hopefully we are expecting it next quarter, but we don't know when it will come through and how much will it be.

Amit Kavani: Okay. The second question is actually, when I speak to other hospital companies who are not really in metros, they say that the institutional business does not have lower margins than the overall business. So, just trying to understand the kind of, the difference between them and us. Is it just because that we are in metros that our -- we are -- non-institutional business is higher paying? Is that the conclusion to reach?

Abhay Soi: No. The institutional business, let's say, has a n ARPOB of, let's say, INR 40,000. If the rest of your business, for whatever reasons, has a n ARPOB of INR 40,000 or lower, then it doesn't impact you much, does it? My ARPOB is the highest in the industry. Now, why is that? A lot of players have a n ARPOB of INR 40,000. Now, that should be a function of two or three things. One is that they're not doing high-end clinical pr



ograms like transplants, high -end oncology, etc. They have more medical patients. Their pay or mix is not very diverse . They don't have international patients. They don't have cash -paying patients, insurance patients to that extent, and whatever else it is. Substitution of institutional business isn't there. So, obviously, for them, there is no difference in the two.

Amit Kavani: But the question actually is that suppose someone has a hospital, let's say, in Ranchi, which is like a Tier 2 city. So, will the CGHS compensation to them be the same as another hospital in, Saket?

Yogesh Sareen : Almost the same. There are differences in the NCR price and non -NCR price, but I would say not a material difference.

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Abhay Soi: Negligible difference , basically the same. The only difference is, Ranchi, there won't be too many CGHS patients, right?

Amit Kavani: Got it. Got it. Got it. Thank you. Thank you, Abhay. Thank you.

Moderator: Thank you. The next question is a follow -up line from Kunal D hamesha from Macquarie.

Kunal Dhamesha: Thank you, sir. So, on the CGHS and self -pay ARPOB, we have said the difference of around 85%. Can you also quantify what would be the difference for the international patient s? So, let 's say CGHS is INR 100...

Yogesh Sareen : Well, international patients typically is 1.5x of the cash and insurance.

Kunal Dhamesha: So, could it be roughly around INR 250? Like, if CGHS is INR 100...

Abhay Soi: If CGHS is INR 100, cash and insurance is INR 185. This is 50% more than INR 185.

Kunal Dhamesha: Okay, okay. Perfect.. And these are the numbers for H -1, I would say? Or, like, more or less, this remains...

Abhay Soi : They're current numbers , running numbers .

Kunal Dhamesha: Current numbers. Okay, okay. And secondly, on CGHS, on institutional, you said that we are now taking higher complex procedures, etc. So, do we have that flexibility, you know, to choose on the specialty on CGHS or some of the institutional business?

Abhay Soi: Well, we inherently do, because some of our hospitals are now disengaged, and other hospitals which are engaged may not sometimes have those kinds of facilities .

Kunal Dhamesha: Okay.

Abhay Soi: There's a churn which happens.

Kunal Dhamesha: Okay, so, basically, some word of mouth, something, you know, more people...

Abhay Soi: Not word of mouth. Like for some of our hospitals, we 've stepped out of CGHS contracts . So, you start moving away from that.

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Kunal Dhamesha: So, we have the flexibility of, you know, saying no to other specialties, basically?

Abhay Soi: It's not a question of flexibility. It's a matter of contract. We've gone and told them that we can't treat patients, right? Our contract has been amended to that effect . It's not a flexibility that we have. If we have a flexibility for all of it, then you do all of it. You can't start anything. You can start for whatever arrangement you have.

Kunal Dhamesha: Okay. So, our contract is only for few specialties where we have, you know, strong base and more complex. Okay. Perfect. Thank you.

Moderator: Thank you. Ladies and gentlemen, that was the last question for the day. I would now like to hand the conference back to the management for the closing remarks.

Thank you, and over to you.

Abhay Soi: Thank you all for coming on to Max Network 's Q2 FY24 results. We look forward to seeing you for our next results as well. Thank you very much. Goodbye.

## Call: Feb 2024

Max Healthcare Institute Limited  
Corp Office: 2nd Floor, Capital Cyberscape,  
Sector - 59, Golf Course Extension Road,  
Gurugram - 122102, Haryana  
T: +91 -124-620 7777

www.maxhealthcare.in Max Healthcare Institute Limited  
Regd. Office: 40 1, 4th Floor, Man Excellenza, S. V. Road,  
Vile Parle (West), Mumbai, Maharashtra - 400 056  
T: +91 -22 2610 0461/62  
E: secretarial@maxhealthcare.com, investors@maxhealthcare.com

(CIN: L72200MH2001PLC322854)

February 7, 2024

Listing Department ,  
National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,  
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Phiroze Jeejeebhoy Towers,  
Dalal Street,  
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on February 1 , 202 4  
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organized on February 1 , 202 4, on financial results of the Company for the quarter and nine months ended December 31, 2023.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/investors/investor -resources](http://www.maxhealthcare.in/investors/investor-resources) .

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Arora  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited  
Q3 & 9M FY24 Earnings Conference Call  
February 01 , 2024

Moderator Ladies and gentlemen, good day and welcome to Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India . Thank you and over to you Suraj .

Suraj Digawalekar: Thank you, Michelle. Good morning , everyone and thank you for joining us on Max Healthcare 's Q3 and 9 M FY24 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director, Mr. Yogesh Sareen – Senior Director and Chief Financial Officer, and Mr. Keshav Gupta – Senior Director – Growth , M&A and Business Planning.

We will begin the call with opening remarks from the management , following which we will have the forum open for an interactive Q&A session.

Before we start, I would like to point out that some statement made i n today's call may be forward -looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi: A very good morning to everyone and a warm welcome to Max Healthcare 's Earnings Call for the 3rd Quarter .

This quarter has been exceptional for us , primarily for two reasons. First, it represents our 1 3th consecutive quarter of year -on-year growth. We recorde d a healthy increase in network revenue and EBITDA by 14% and 12% , respectively. And more importantly, we managed to grow ARPOB and EBITDA per bed sequentially, despite Q3 being a seasonally lean quarter.

Second, t his quarter marks our foray into central UP, which is one of the most populous and fast-growing states in the country , through the acquisition of 550 bed Sahara Hospital in Lucknow. This acquisition will fortify our presence in Northern India and is in line with our articulated strategy of entering new markets where peers have demonstrated success. We expect to consummate the transaction in Q4 and are ready with a plan of action to improve the infrastructure , augment the operational bed

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capacity , add to the existing medical tale nt as well as integrate the hospital with our IT systems.

On the clinical front , we have launched the CAR -T cell therapy at Max Vaishali in collaboration with ImmunoACT . Also known as a 'living drug ' , this groundbreaking therapy is expected to enhance the quality of life for many patients and reflects our continuous endeavor to provide our patients with the latest therapeutic options for cancer treatment.

Now, coming to the highlights of Q3 Performance :

1. Occupied bed days dipped marginally by around 1% year-on-year due to lower prevalence and spill -over of vector borne diseases in this quarter compared to last year. Average occupancy for the network was 73%.

2. Institutional bed share stood at 29 .5% compared to 29.4% last year, and 27.3% in Q2 this year. However, after excluding Max Shalimar Bagh , the overall institutional bed share stood at 27.4% during Q3 and occupied bed days were down by 8% for th is segment. Sequential increase institutional bed share was a conscious call taken owing to Q 3 being a seasonally weak quarter on occupancy.

3. Average revenue per occupied bed for the quarter improved to Rs.76,800 , growing by 15% year-on-year and 3% quarter -on-quarter . Year -on-year improvement was witnessed across all specialties with oncology, neurology and renal sciences being the key drivers.

4. Network gross revenue was around Rs. 1,779 crore compared to Rs. 1,559 crore in Q3 last year, and Rs. 1,827 crore in the previous quarter. This reflects an increase of 14% year-on-year, driven mainly by growth in ARPOB . Revenues declined by 3% quarter -on-quarter due to festive season in Q3 and vector -borne diseases led jump in admissio ns during Q2.

5. Revenue from international business grew significantly by 25% year-on-year. This pa yor channel now accounts for around 9.4% of the total revenue from our hospitals.

6. Network operating EBITDA stood at Rs. 471 crore reflecting a growth of 12% year-on-year and a decline of 5% quarter -on-quarter. Most importantly,

annualized EBITDA per bed rose to our highest ever of Rs.75.6 lakhs , clocking a growth of 13% year-on-year and 1% quarter -on-quarter . Operating EBITDA margin stood at 27.9% for the quarter .

7. Year-on-year growth in EBITDA was impacted due to additional charge of Rs.25 crore , driven by movement in provision for doubtful debts , reversal of provision for old phantom stock option plan in Q3 last year after launch of the new ESOP scheme in November 2022 , GST impact o f variable management fees and one - time litigation cost .

8. Max Shalimar Bagh, where we added 122 beds , recorded year-on-year growth of 36% and 42% in its revenue and EBITDA respectively , with an average occupancy of 74% .

9. Profit after tax was Rs.338 crore versus Rs. 269 crore in Q3 last year and at the same level as in the previous quarter . The year -on-year improvement o f 26% was primarily attributable to the flow through of improved EBITDA and lower finance costs.

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10. Free cash flow from operations generated this quarter amounted to Rs. 226 crore , after additional outlay of Rs. 40 crore for purchase of robotic systems at four of our hospitals . Of this , Rs. 137 crore was deployed towards the ongoing capacity expansion projects and Rs. 97 crore was distributed as dividend.

11. Net cash position improved to Rs. 1,295 crore at the end of December 2023 compared to Rs. 372 crore same time last year.

12. Continuing our effort to support the local communities , we treated approximately 36,700 patients in OPD and 1 ,250 patients in IPD from economically weaker sections of society absolutely and entirely free of charge.

13. Both our strategic business units continue to maintain growth momentum .

Max@Home reported a top line of Rs. 44 crore reflecting a strong growth of 24% year-on-year and 5% quarte

r -on-quarter. We continue to witness good demand for this SBU's services as d emonstrated by over 50% repeat transactions by patients in the last one year.

Max Lab, the non -captive pathology vertical , now offers its services in 41 cities and has a network of over 1 ,000 collection centers and active partners. This SBU reported a gross revenue of Rs. 34 crore , reflecting a growth of 20% year-on-year.

14. On the status of our expansion projects :

For 300 beds at Dwarka – As informed previously, application for occupancy certificate was submitted in October . Finishing is under progress . In the meanwhile, we already have the unit head and all other key functional heads in place. We have commenced hiring for middle -level staff as well.

For the 329 b eds at Nanavati – Basement and ground level structures have just been completed , while steel fabrication above ground floor has begun. The project continues to be largely on schedule .

For 300 beds at Sector -56 Gurgaon – Approval for structural drawing has been received in the first week of Jan and RCC works have already commenced.

For 190 beds at Mohali – Design development is under finalization . In the meanwhile , the base raft concrete activities have started.

Both of these are largely on schedule.

For 350 beds at Max Smart (Saket C omplex ) – Tree transplantation work is underway , with 159 trees transplanted and balance 316 trees in process for transplantation . As per plan , existing structures have been demolished and shoring work for sewage and water treatment plants is going on. Barring the initial delay of 6-7 months th e project is on schedule now.

For 300 beds at Max Vikrant (Saket Complex ) – The building plans have been resubmitted to the Municipal Corporation of Delhi post their initial review and the dues for water and sew er infrastructure charges have been paid. Tender documents are under review for floating to the contractors. Application for NOC

by Forest department has also been filed .

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For 250 beds at Patparganj – The fire department issued NOC for the building plans and municipal corporation approval is in progress.

All projects continue to be largely on schedule despite some disturbances linked to enforcement of GRAP (Graded Response Action Plan) in NCR to combat air quality issues.

Finally, moving on to the overview of the company performance for the nine months ended December 31, 2023 :

15. Network gross revenue stood at Rs. 5,325 crore , reflecting a growth of 16% year-on-year.

16. Network operating EBITDA grew by 17% year-on-year to Rs. 1,404 crore . Increased ARPOB , improved case mix and augmentation of network bed capacity by 146 beds translated into improvement in EBITDA per bed by 15% to Rs. 73.8 lakhs per bed .

17. During the nine months we generated Rs. 924 crore of free cash flow from operations , after interest , working capital changes and routine CAPEX , of which Rs. 265 crore has been deployed towards ongoing expansion projects and Rs. 97 crore was distributed as dividend.

With this will open the floor for questions and answers .

Moderator: Thank you. The first question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets .

Damayanti Kerai: My first question is on this institutional tariff revision , which we saw in the month of December. So, you mentioned there will be net loss of around Rs. 6 crore annually , is this like ongoing and just want to understand if this wouldn't have happened the net gain which you mentioned for 3rd Quarter Rs. 14 crore should we have added this Rs. 6 crore back, Rs. 14 + Rs. 6 if the prices were not declined?

Yogesh Sareen : No, Rs. 14 crore is for the quarter. That is the run rate , which also has some impact of this price revision that happened in the month of December. So, around one week 's impact is already reflecting there. Rs. 6 crore number is an annualized number , while

Rs. 14 crore is a quarterly number.

Abhay Soi: Coming to your question , if you didn't have the Rs. 6 crore then you would have added this to the EBITDA for this quarter, which would be around Rs. 1.5 crore .

Damayanti Kerai: Okay. And do you expect any further revision in tariffs for this channel or do you think like now government has broadly done and we might not see any additional ?

Abhay Soi: We are expecting revisions in tariff. We think that even the PET CT revision has been erroneously done and we are in talks with them to revise the same .

Damayanti Kerai: Okay, so further revisions can come right and then , that should be like maybe beneficial to our institutional channel.

Abhay Soi : We expect that even this PET CT tariff should get revised the other way. We believe it was an error.

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Damayanti Kerai: Okay, Rs. 10,000 went to some Rs. 11,500 or something, right?

Yogesh Sareen : No, the tariff for PET CT has come down by around 50%, from Rs. 22,500 to around Rs. 11,000. So, it is 50% reduction and that has a major impact because oncology happens to be a big share of revenue for us. There is Rs.10 crore downward impact for the PET CT , while the cardiac tariff has gone up by around Rs. 4 crore . The overall net impact is Rs. 6 crore and these are all annualized numbers .

Damayanti Kerai: Okay, understood . And Abhay , a few quarters back you talked about this Heal in India initiative, which should be giving good boost to your international business. So, any update on that front or any discussions ongoing around that initiative ?

Abhay Soi: What they have done is that there has been a lot of movement in the embassies , etc.

They're fast-tracking medical visas, they've taken off those policies on reporting to police stations, they have brought down the price of the medical visas and are generally promoting it globally . The government of India is participating in various medical tourism forums. And this is leading to a trickle effect right now . Trickle effect meaning that it's a growth of 25% Y-o-Y for us . In some manner , just given the size of the opportunity , it's not exponential yet but it's still incremental at 25% CAGR on this channel .

Damayanti Kerai: Okay. And from March perspective right now it's around 9.4% revenue contributed by international patients, how much it can go up in say next three years, even if we don't

hear any big changes by the government and these kinds of small regular improvements happen?

Abhay Soi: It's growing at 2.5% CAGR. And it's not just this quarter but last couple of quarters it has been growing at this pace, so I don't really see it abating.

Yogesh Sareen: If the overall revenue growth is 14% and this channel is doing 25%, then obviously this share should go up going forward.

Damayanti Kerai: Got it. And my last question is, as you mentioned, oncology is a big focus for you and you have been investing in this segment. So, which are, I will say, key growth opportunities as per Max focus, and then what kind of expectation you have from this segment, in terms of addition to your hospital revenues, over the next few years?

Abhay Soi: There are three streams of oncology – surgical, medical and radiation. There are various sort of innovations happening within these three. If you look at surgical, with more robotics happening, it translates to better ALOS and better outcomes. At the same time, while the percentage margins are lower, but absolute value to hospitals is more because in terms of the EBITDA per bed that you are able to derive from that is higher than what you do for normal oncology surgery. And robotics number has sort of doubled for us over last year and we are seeing major growth in this. Now, similarly if you look at radiation, from a typical LINAC you have gone into various kind of innovations. And there have

been other kind of innovations within medical oncology

as well. But now, we are also looking at immunotherapy, like we have mentioned that we started CAR-T cell therapy recently. This is another stream altogether, which is a sub segment of chemo therapy. Globally, it's a little expensive right now and we are seeing prices come down so adaptability of that is going to be a lot more going forward. But that's what the world is moving towards is oncology, its immunotherapy now.

Moderator: Thank you. The next question is from the line of Harith Ahamed from Avendus Spark.

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Harith Ahamed: My first question is on the recently acquired Lucknow hospital. So, currently, we have around 250 operational beds there and you talked about the capacity of 550 beds.

So, can you share your thought process in terms of making the operational bed count higher more towards the capacity's headcount?

Abhay Soi: The amount of area, which has already been constructed, is close to about 1 million square feet and the number of beds operational are 250 so, there's obviously an opportunity to ramp up to 550 as intended originally within this area. The facility has been ignored for a while, so we are going to be changing the equipment and redo some of the infrastructure. When I say redo the infrastructure, it is basically internal renovation that needs to be done. It's a brand-new building made by L & T and designed by Hafeez Contractor, started in 2008-2009. It's not an old structure but we need to renovate it to some extent. Within a couple of years, we should be at that 550-mark, perhaps a little before that. Please do keep in mind that we in the process of closing the transaction. We are not in the saddle as yet. Majority of the conditions precedent (CPs) as per schedule have been done and the rest are also on schedule. We will be able to share more details about this facility once all the CPs are completed.

Harith Ahamed: Okay. My second question is on the timelines of some of the upcoming hospitals. So, apart from Dwarka, you have guided for three other hospitals, getting commissioned in the next 12 to 15 months. Mohali, Nanavati and Gurgaon but when I look at the project CAPEX specifically, we are tracking a bit lower versus our guidance of Rs.900 crore for FY24, we are at around Rs.300 crore nine months of FY24 so, are we still expecting these three hospitals other than Dwarka to get commissioned in FY 25?

Abhay Soi: Absolutely. So, Dwarka which is getting commissioned, do keep in mind it is off our books, because this was the asset light model. The developer is investing the money, whereas we are bringing the medical equipment in place. The hospital is complete in all respects and we are right now in the phase where we are doing the finishes to the hospital. And, sort of whatever medical equipment that we brought in, we are implementing it. It's not going to be a major CAPEX for us going forward either as far as Dwarka is concerned. It was never meant to be because it is asset light, where we are essentially paying a lease amount for that at a yield of 6.5-7% p.a. It is a sixty-year contract for finished land and building of 300 beds. As far as Nanavati is concerned, we've already built our three basements and we've come to ground level. Now, this is where things start speeding up. Please do keep in mind that the first part of construction is the superstructure, which doesn't really cost that much money. If you were to build, it typically costs Rs.1500 to Rs.1800 per square foot. So, if you are building even a million square feet, then you would have spent like Rs.120 crore just

doing the construction of the superstructure. Real money is spent towards the latter part of it when you go into finishing , interiors, equipment , lifts, air conditioning, etc. So, we are on schedule and as far as the payments is co

ncerned, I would not look at

that since the payments are always lumpy towards the end . So, I would not look at that as a surrogate for completion of projects, at least at this stage.

Harith Ahamed: Okay, last one with your permission . Slightly broader question , when I look at the Saket C omplex currently. And if I am correct, we have around 800 beds there. And in the next three years we are looking to add 800 , 900 beds and we have plans to add maybe 500, 600 beds beyond FY 28. So, that's almost 2000 beds in almost a single location. So, it's kind of unheard of so can you share your thought process behind such a large number of beds at a single location . How are we going to differentiate and how do we plan to whatever strategy to ensure that there is no overcapacity situation in that ?

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Abhay Soi: Firstly, Saket C omplex is the main flagship as you are aware . We are pretty much tearing at the seams as far as Saket is concerned. We've added 40-50% more capacity in a place like Shalimar Bagh , and within the first month or so, our occupancy went up by about 80 -85%. These 800-1000 beds that you are speaking of is not coming all together, as you may appreciate. First, the 300 odd beds come into play. My belief is , when we add 40% capacity on top of the current capacity of 770 beds that we have, those will be more or less instantly taken up. We could have a very quick occupancy of the first 300 beds. And very soon thereafter we are going to be ready for additional capacity . Like in the case of Shalimar Bagh , we have already run out of capacity. So, even over here we expect to have the same situation and will require the next 300 beds. So, w hat we can't do is start constructing 300 beds then and take another three years since we will need those 300 beds once these first 300 beds are occu pied. And that really speaks for the first year. Now then comes the last phase of let's say another 300 beds thereafter. Now that only when these 300 beds , the second 300 beds get occupied, we will look at the last piece . We will make the superstructure, which I again mentioned is not a major cost . This is if things are going as per plan , but we can always stall . We can, at the last 300 beds, always say look, we are completing the superstructure and the external façade but we are not fitting it out if we see any sort of softness of demand . Our belief is that this is the trajectory that we have over the next five to six years, and we will be able to occupy all those beds . But it does not mean that after phase one, if, let's say for some reason, the first 300 beds don't get occupied, we are not going to fit out the next year 300 beds. We will never build the next 300 beds.

Harith Ahamed: Okay.

Abhay Soi: This is the base case . At various stages and at various places, these are the default options . It's a running hospital , so we can't have a disruptive process . The superstructure cost isn't a lot . Again, i f you're looking at a million square feet , it is only Rs. 150-180 crore for that matter .

Yogesh Sareen : Also, FY 28 onwards is a placeholder . That's the potential we have , but we are not investing anything for the FY 28 capacity now. We don't even have the approval s from the Board to spend on that. It only reflects that we can build those many more beds on those land parcels .

Abhay Soi: Theoretically , in Lucknow, it is 27 acres of land so you can build another 3500 beds . That's the potential. So, you see the potential post 2028 up to infinity , but it does not mean we will be building 3,500 beds there .

Moderator: Thank you. The next question is from the line of Neha Man puria from Bank of America .

Neha Manpuria: Abhay , on the Lucknow acquisition, how should we think about narrowing the operating performance gap that is there between , let's say , what Max is doing now . In Lucknow particularly since we wil

I continue to ramp up the beds, is it fair to assume that we need to get to that full 550 to narrow that gap meaningfully and it would happen over a period of time ?

Abhay Soi: We are quite confident that within a couple of years we will be able to get all of these beds in the condition or perhaps better than what we have in any other hospital and be able to catch up with our peers as well in terms of every financial parameter, if not exceed .

Neha Manpuria: Okay, but this would take a few years in your view ?

Abhay Soi: It should be done within two years.

Neha Manpuria : Got it. And second, just extending the Lucknow question to a broader strategy , now that we have this, we will have 500 beds, you said you have land for adding more capacity. But do you see that as a market where you can have let's say, multiple facilities and is that something that you're looking at this could be a third hub outside of let's say the current two hubs that you have?

Abhay Soi: Absolutely. So, we are looking at Lucknow just to find certainly many more healthcare facilities. UP's population is the size of Europe and literally it's got only one and a half corporate hospitals over there right now as we speak. Besides, hopefully, when we get in , we will become two and a half. But I don't know if you've been there recently, but the level of development, connectivity, roads , infrastructure , the kind of focus that city is getting , I think it's going to be a metro very soon . And just the scale of the infrastructure, the airport , connectivity with other areas through rail, air, road, every which means . On top of that, the government itself has a very strong program . The Chief Minister 's Relief Fund over there actually pays hospital tariffs for patients. So, you can imagine there is an entire population where the Chief Minister 's Relief Fund is paying hospital tariffs for patients at the hospitals . Not the CGHS rates or some other low rates. It certainly justifies more than one hospital on multiple fronts .

Neha Manpuria: And this would be through M&A, do you think to just make it happen faster or would you choose a more organic approach of expanding into that market?

Abhay Soi: Unfortunately, I don't think there are too many M&A opportunities or existing hospitals in Lucknow. They are far and few, if we are able to get lucky again like we have got in this case . Of course , we will look at an acquisition first because that's faster, but if not, then we will look at organic growth as well.

Neha Manpuria: Right. And my second question is on Dwarka, you mentioned that the unit head and key function heads have already been appointed , is that cost already sitting in the quarter because if I look at our direct , hasn't increased as much in the last three quarters ?

Abhay Soi: That's right, that cost is sitting on our books. Also, certain doctors have been on boarded and that cost may also be sitting in this quarter.

Neha Manpuria: Okay. So, as we commission this and phase it up, you should start seeing that contribute EBITDA pretty quickly ?

Abhay Soi: That's right , because that cost is already incurred. The biggest cost is always the non - medical personnel , like we have always mentioned , around 23% or 24% of the overall revenue . In fact, for a new hospital it's higher because revenues are lower. It's basically the significant functional heads that you have under that cost . And that comes in closer to the hospital commissioning, which is now . Very shortly we should have that hospital up and running.

Neha Manpuria: And for breakeven in Dwarka is 12 months, 18 months a good timeframe to assume ?

Abhay Soi: Well, that's the textbook. We are hoping to do it faster .

Moderator: Thank you. The next question is on the line of Kunal Dhamesha from Macquarie .

Kunal Dhamesha: So, first one on the institutional rates there has been

some revision upward, some

revision downward, but when we sum it up, let's say for the first nine months what

could have been the ARPOB impact in that channel . For the first half we had suggested the institutional ARPOB has grown at 28% year-on-year, if you could provide that number for the first nine months ?

Abhay Soi: In my knowledge , it's been revised upwards on everything except for one item, which went down and we believe it is a mistake , so we have made appropriate representation.

Yogesh Sareen : Kunal, you have the number with you . The price increase is Rs. 14 crore per quarter and there is a Rs. 6 crore annualized impact of the downward revision . So, that means the net impact is Rs.50 crore p.a. on the institutional business in terms of price . So, if you divide it with the numbers, you will get the increase in the ARPOB rate, which will be around 3 -4%.

Yogesh Sareen : In the overall ARPOB , the impact is less than 1% .

Kunal Dhamesha: No, but last quarter we had said that institutional ARPOB increased by 28% ?

Yogesh Sareen : We are talking about the overall ARPOB impact , which is less than 1%.

Kunal Dhamesha: No, but if our institutional revenue is around 18% and that revenue is growing at 30%, then the weighted average impact should be around 5% , right?

Yogesh Sareen : Let's not confuse ourselves. First of all, the total impact is Rs. 50 crore as things stand today. On the overall top line, it has an impact of less than 1% . Our top line last year was around Rs. 6,200 crore, so if we take Rs. 50 crore on this, it will be less than 1%.



That's on the overall ARPOB . Now, if we talk about the ARPOB of the institutional al channel itself – on around Rs.1200 crore of business in a year from institutional , there is a Rs. 50 crore impact , so it will be a round 4%.

Kunal Dhamesha: Yes, this is clear but last quarter ...

Yogesh Sareen : The last quarter growth in ARPOB is not only price , we never said that it is all because of price.

Abhay Soi: There has been a growth in institutional ARPOB , but we haven't said it's driven only by price .

Yogesh Sareen Even this quarter if you see , the price impact is only 4% and rest is the business mix. Also, you should understand that when onco logy goes up , the chemo therapy goes up, the ARPOB improves .

Abhay Soi: We have been focusing even with institutional on the higher end specialty . That's the outcome you see .

Kunal Dhamesha: Sure. And the 15% ARPOB growth that we have seen, first half was also 13%, 14%.

Going into next year what is , is that what your kind of would be thinking about in terms of ARPOB growth, because clearly these numbers are in my view at least is above the historical trend line. And we have been doing high end surgeries , robotic surgeries . Do you think that this can sustain this run rate of ARPOB growth for the next two to three years?

Abhay Soi: We keep asking the same question of whether there any one-time price change that has led to this ? In my mind , there are a few factors which have led to higher ARPOB .

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One is more higher end surgeries , second is international patients , third is proliferation of insurance, etc. These are things which are leading to higher ARPOB and if any of these is non-secular , then your guess is as good as mine. My belief has been that , they seem pretty secular and that should read into next year as well. None of this is based on one time price changes, etc.

Yogesh Sareen : It's not only us. You can see the competition , ARPOB has been improving for all other hospital chains also.

Abhay Soi: Unless there's a non -secular trend which has been propelling it. But , I'm at a loss of what it could be.

Kunal Dhamesha: And sir one logistic question on the international patient bed share this quarter , if you can share it?

Yogesh Sareen : So, it w

ill be around 5.8%.

Moderator: Thank you. The next question is from the line of Ankit Shah from Canara Robeco AMC .

Ankit Shah: So, my question pertains to the Dwarka additions. So, you mentioned that some of the cost s have started comin g, but the major portion of the non -medical cost s will come going forward. So, can you give some sens e of like, how much of that non -medical cost would be of the total cost . And also, you had mentioned earlier that the total losses initially in this should not exceed Rs. 30-40 crore so can you give some sense on how this, would this be completely front ended or cost come in a stepwise fashion so it will be more divided , some granularity on that front?

Abhay Soi: I feel it wil l grade down , so it will be in a decreasing fashion . Of course, the first month is more , the second month should be similar and then it sort of starts peter ing down till you hit breakeven. So, the textbook would say that this size of the hospital should have around Rs.40-50 crore loss, but we hope to perform better and maybe break even hopefully sooner. This is hard to project , because as and when the doctors come in and those particular programs kick in , certain doctors will be brough t in with certain minimum guarantees, and others with certain other minimum guarantees, etc. So, in order to give you that loss figure and what is the trajectory of the loss, we need to figure out what are the exact revenue figures going to be in that particular month.

Ankit Shah: So, on the hiring front, does the hiring happen in one shot for the entire capacity or does it happen in stepwise fashion ?

Abhay Soi: No, we are going to be coming up with around 160 b eds first, and of which , let's say, we want to operate around 100 beds , then in the first go we will be hiring at least for 100 beds . Unless we get enough of those kinds of clinical programs to kick in where we start off with 160 beds . Right now, we are only doing this much but let's say if there are 20 letters of intent (LOIs ) , which we have issued to 20 various doctors, and 12-15 of them have joined while others are in the process , it's in the works , right? So, as and when we have it in hand, we know this doctor is joining , we need to get him enough nurses and resident doctors to support him.

Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services .

Tushar Manudhane: Sir just on the international patient s would like to understand the growth maybe for the quarter and as well as for the nine months , if you could break down into volume and maybe the realization growth ?

Abhay Soi: The growth has been 25% both for nine months as well as the quarter .

Yogesh Sareen : Yes, so the ARPOB has grown by 9% in the quarter, but the bed share is 5.8% and it is flat overall Q -o-Q. Revenue from this channel was Rs. 158 crore last quarter and Rs. 157 crore this quarter.

Tushar Manudhane: Right. So, the 9% increase in realization is to do with case mix or it's like the similar treatment ?

Yogesh Sareen : There is no price element in this , so it is basically case mix . We haven't increased the prices . Typically , the price of international patients is linked to the self-pay patients , which gets revised only in the month of April. To that extent, there is no price element in this, it is only the case mix which is increas ing the ARPOB by 9%.

Tushar Manudhane: Understood. And secondly, just on the overall operational cost, in terms of let's say employee cost and other expenses for FY 25 in particular were considering the different new hospitals coming up. So, what kind of cost increase one should think about?

Yogesh Sareen : We can tell you about the cost increase on the existing hospitals, which is typically an increment of around 7%. And then if any additional bed is getting created

, there will

be manning required . So, typical manning would be six people to one bed. If we are adding 100 beds to the operational capacity , you should assume that the re will be six hundred more peop le. I can't give you exact number of the growth, etc., but I'm giving you the math to get to that number.

Tushar Manudhane: Got it. So, effectively trying to understand considering the scope of ARPOB growth as well, but at the same time addition of cost on account of , at the existing sites, as well as new hospital getting added. So, is there a scope for f urther margin improvement in FY 25, or we would be let's say having bed related revenue growth, but margin would be pretty stable, is that the way to understand or there is a scope for margi n improvement even in FY 25?

Abhay Soi: No, you are mixing two things. One is our current capacity , which is about 3,500 beds .

There is a certain cost growth and certain revenue growth. Assuming the revenue growth continues on the same trajectory , and then there is cost growth , which would be similar to what happened last year. So, EBITDA growth in that fashion should be currently what is there and plus, because you get some operating leverage as well. So, whatever EBITDA growth that you for existing hospitals , you will continue having that. Then comes the new hospital s. The new hospital will be, let's say 300 beds, which is about 7% to 8% further c apacity . Those 300 new beds on day one will start with the lower ARPOB and negative EBITDA . Then the re is the question of when do you break even – six months, eight months or 12 months ? If, say, you breakeven even in 12 months , then it will have a negative impact on the EBITDA for the year . If you breakeven in six months, then it may have a neutral impact on your EBITDA one year later. Even a neutral impac t will still pull the margin down because if it's zero, you are taking the positive EBITDA and you are taking the negligible EBITDA on an increased revenue base . So, you have got two conflating kinds of wins happening over here. But my belief is because it's only 8% bed addition on the overall bed base , it doesn't move the needle too much.

Moderator: Thank you. The next question is from the line of Karan Gupta from Varanium Capital .

Karan Gupta: So, basically my question is related to international business. So, what kinds of attraction we are seeing in the television for medical tourism and what systems we are taking to increase the traction , or maybe increase or improve the quality of our treatments . I am as king this question, because in India the treatment cost is 1/5 of what the US and Europe is. So, what's the traction coming from US and Europe , because basically you said in your early con-calls that the major traction is coming from Bangladesh . So, what's the position of these two US and Europe?

Abhay Soi: We don't get many patients from US and UK because even though India is much cheaper, but in US and UK, medical treatment is sponsored . For instance, NHS pays for it in UK . And in the US also , it's the government . So, it doesn't matter if it's expensive since somebody else is paying for it. It has never been our case that people from developed countries will come to India . You will only see people come to India from countries where you have self-paying patient s.

Karan Gupta: Okay. So, when you talk about medical tourism, where are the patients coming from ?

Abhay Soi: We're getting patients from around 145 countries.

Moderator: Thank you. The next question is from line of Amit Kadam from Canara Robeco Mutual Fund .

Amit Kadam: Just couple of things , the swing what we saw in the inpatient volumes this quarter, largely you attributed to the vector borne disease , just wanted to know because we have a very limited understanding like in last three years or so , we haven't see

n such

kind of large swing because the bed days for this quarter is -1%. So, can you just throw from your past experience we had seen such kind of large swing which could have a material impact on the inpatient volumes, and which could result in bed days ?

Abhay Soi: What we are seeing is that last year same quarter , we had a big influx of the dengue patients . We haven't had the same level of influx this year. Therefore, there is a reduction of 1% on occupied bed days (OBDs).

Yogesh Sareen : If you see the historical trends, you will find that from Q2 to Q3, there is always a decline . If you pick up any competitor 's previous reports of Q2 and Q3 of previous years, you 'll find that Q3 is always softer . And therefore -1% OBD is very normal. If you compare Apollo , Fortis, Narayana , and you pick up their last year results , you will find that there's a decline in Q3 over Q2. There is no exception here. This is the secular trend, and you will always find Q3 is down compared to Q2 because of the festivities – Diwali, Dussehra , etc. Now, in our case one peculiar feature is that in last year, especially in Delhi, we had an influx of dengue patients in October . Typically , dengue season finishes by September, but last year, we had patients even in October , which was a Diwali month . We had dengue patients and this Diwali month 's occupancy was 76%, which is unheard of. In a Diwali month , you will usually have an occupancy of less than 70%. So, last year October was exceptionally high on occupancy because of dengue , and that is the reason why you see a sharper drop in OBDs . Otherwise, it would have been probably flat , if we were to take out the drop in internal medicine and pediatrics , where we get these dengue patients admitted . The drop in these specialities is 12% , while you see that Y-o-Y the drop in OBDs is only by 1%. But these particular specialities, which is 22% of the overall bed share dropped by 12% . So, what we are trying to say is that there is nothing operationally wrong in this quarter. It's only when you compare Y-o-Y because of this October influx of patients , there looks to be a sharp drop, rather than what it should actually look like.

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So, if you adjusted for this influx, then you 'll find that the OBDs actually went up Y-o-Y.

Abhay Soi: It is a seasonal business . Q2 and Q4 are the strongest quarters in the hospital business, while Q1 and Q3 are weak quarters. Q1 because the cost structure moves up and Q3 because the revenue comes down.

Yogesh Sareen : Is that clear Amit?

Amit Kadam: Yes. No, I was just not contesting your that particular point. I just wanted to know because we have a very limited history because we got , so from the data point we did not have , I did not have the insight that whether such kind of some seasonality , when I'm talking seasonality , I'm trying to compare Y -o-Y because of some like vector borne disease, we have a material impact on the volume growth Y-o-Y. I can understand how Q2 to Q3 seasonality works. But I was just saying in the past history whether such kind of events like vector borne is not there, could have a material impact on the volume growth just wanted to.

Abhay Soi : 100% , it does . And it's not an anomaly .

Yogesh Sareen: And it depends on when the rains happen, when this epidemic type stuff happens .

Abhay Soi: You are not comparing 9-months to 9-months , you are only comparing this quarter to last quarter Y-o-Y. On an overall annual basis, it will be not a significant impact on the occupancy .

Amit Kadam: Right that I get it .

Abhay Soi: But yes, say if you don't have a dengue season , the drop can be much sharper than this, in fact much sharper than the 1%. Between 1% to 3% fluctuation on both sides, positive as well as negative.

Amit Kadam: Okay, got it sir. And this on the outpatient thing is it just a n outcome of the

similar

effect that because there were lesser patients visiting because of the there was no dengue disease ?

Abhay Soi: That's right.

Amit Kadam: So, nothing much to read in this particular thing based on this particular like because I've earlier seen this outpatient at least growing at mind -teens or kind of thing, but

now it showed 3% in this particular quarter there is nothing much to read .

Abhay Soi: One thing you are rightly doing is, you are looking at it Y-o-Y rather than Q-o-Q, because we have seasonality in this. So, Q3 in any case is weaker than Q2. But if you look at it on Y-o-Y basis , you can attribute it to the number of dengue patients .

Amit Kadam: Okay. And just one more final point is on the Lucknow thing. So, as per the current timelines we should be able to integrate that particular business financials to ours by quarter one next year FY25?

Yogesh Sareen: Yes. It depends on when does the transaction closing happens . Say, even if the closing happens in this month , then we'll have to consolidate it from the date of closing . We don't have a choice .

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Moderator: Thank you. Ladies and gentlemen, this will be the last question for today, which is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Sir on that question on ARPOB , we have reported a 15% ARPOB CAGR over the last three years. And you alluded various factors – case mix , international , institutional insurance, et cetera . Now, would it be fair to say that a larger chunk of this improvement can be attributed to an improved case mix?

Yogesh Sareen: So, Alankar you know the elements of ARPOB increase – one is the increase in the prices, which we've been saying that it can be up to 3%. Then there is an increase in OPDs – you know that OPD revenue increase will improve the ARPOB because you don't consume any beds for those OPD revenues. So, that typically contributes around 2%. So, this makes  $3 + 2 = 5\%$ . Then we have growth in oncology . In our case , you have seen that the oncology growth is outpacing the overall growth. Oncology happens to be a high-end specialty , so that typically adds another 2-3% to the ARPOB . And then comes this whole channel mix and the case mix . If you take oncology out of the case mix , then there is improvement in balance specialties as well and the channel mix . This typically has been contributing around 6-7% increase , and we don't see any reason why it should not continue going forward. From our perspective , if there's a 9-10% growth in ARPOB , that should be reasonable.

Abhay Soi: But, having said that , you have to keep in mind one thing that we are talking about a healthcare system where there is capacity constraint in some manner . You have got occupancy levels that are pretty much at the threshold. When you have this situation, then, typically, you will see a higher uptick on ARPOB vis-à-vis occupancy because you can't move up on occupancy as there aren't so many beds available . So, more and more patients get pushed to daycare, all smaller surgeries compared to larger surgeries. So, you would schedule the larger surgeries first . The smaller surgeries would be scheduled at or the appointments are given at later dates, etc., so some of the later date patients may evaporate. So, what you see is that at the quality of work starts moving up. But at the same time, if I had a magic wand in my hand and I could put up beds adjacent to each one of the hospitals, then what you would see is the higher uptick on occupancy and lower on ARPOB . There is always an interplay between the two.

Alankar Garude: That's helpful . Maybe a question linked to that is , as we double our bed count over the next four , five years would the case mix for the newer beds be as good as our flagship hospitals right from year one or the ramp up in our case mix would be ?

Abhay Soi :

You can look at expansion akin to Shalimar Bagh , where we did a brownfield, since 85-90% of the expansion is brownfield essentially. If you look at Shalimar Bagh where we added 40% or more beds, and you see our experience over there, the new beds got taken up more by the institutional business , which we are bringing down . So, therefore, if you see our institutional business , apples to apples, year-on-year, it is the same but if we look at it excl. Shalimar Bagh , then we are far better in the institutional, because overall institutional business has come down . But what it means is that even if your ARPOB is lower but your operating leverage is so significant , that your EBITDA per bed that comes out of from the incremental beds is significantly better. And in fact, the operating margin even from these beds is higher than the existing beds.

Alankar Garude: Understood. My second question is, you spoke about the potential of Lucknow and Uttar Pradesh for us in general earlier. Now, when it comes to expansion beyond our already announced plans, you said earlier that we are looking at some 20, 21 cities for potential expansion. Will we continue to aggressively look out for acquisition

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opportunities in these 20 odd cities now, or do our priorities get aligned more towards Uttar Pradesh now with the Lucknow entry ?

Abhay Soi: No. Our priorities don't get aligned towards UP from that standpoint, because it's not as if we are going to get two or three acquisitions in a place like Lucknow . Even if we have a strategy of looking at building it out, it's going to still take some time . It takes years , while we are looking at deploying our balance sheet today. And we have the wherewithal to do it, for us to add 3 to 4 hospitals in a space of one to two years is not a big deal because rest of the expansion that we are doing is essentially brownfield in any case . It doesn't tax us more . It takes us as much effort to do expansion on Shalimar Bagh as operating Shalimar Bagh , and similarly, wherever we are doing brownfields . Both from financial and human resources standpoint, we are quite comfortable with what we are doing and we are prepared to go to the other cities as well. We are actively looking at it, but not aggressively . We don't like to do things aggressively. But we are actively pursuing opportunities in other cities as well.

Moderator: Thank you. As that was the last question for today. I would now like to hand the conference over to the management for closing comments . Over to you.

Abhay Soi: Thank you all for being on the call , yet again for Q3 and we are looking forward to a robust Q4, as usually is the case because of the seasonality. Thank you so much .

## Call: May 2024

Max Healthcare Institute Limited  
Corp Office: 2nd Floor, Capital Cyberscape,  
Sector - 59, Golf Course Extension Road,  
Gurugram - 122102, Haryana  
T: +91 -124-620 7777

www.maxhealthcare.in Max Healthcare Institute Limited  
Regd. Office: 401, 4th Floor, Man Excellenza, S. V. Road,  
Vile Parle (West), Mumbai, Maharashtra - 400 056  
T: +91 -22 2610 0461/62  
E: secretarial@maxhealthcare.com, investors@maxhealthcare.com

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National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

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Sub.: Transcript of Earnings Call held on May 23 , 202 4  
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organized on May 23 , 202 4, on financial results of the Company for the quarter and financial year ended March 31, 202 4.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/investors/investor -resources](http://www.maxhealthcare.in/investors/investor-resources) .

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Aroraa  
SVP - Company Secretary and Compliance Officer

Encl.: As above

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded. I now hand the conference over to Mr. Anoop Poojari from CDR India. Thank you, and over to you.

Anoop Poojari: Thank you. Good evening, everyone, and thank you for joining us on Max Healthcare's Q4 and FY24 Earnings Conference Call. We have with us Mr. Abhay Soi, Chairman and Managing Director; Mr. Yogesh Sareen, Senior Director & Chief Financial Officer; and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning of the Company. We will begin the call with opening remarks from the management, following which we'll have the forum open for an interactive question-and-answer session. Before we start, I would like to point out that some statements made in today's call may be forward-looking in nature, and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi: A very good afternoon to everyone, and a warm welcome to Max Healthcare's earnings call for the last quarter of fiscal 2024. This has been yet another noteworthy year for us. We witnessed a robust growth in revenue and profitability, with both gross revenue and operating EBITDA registering 16% to 17% growth year-on-year. We touched all-time highs for all our key operating and financial metrics including ARPOB, margin and EBITDA per bed. This year also marked our foray into 2 new cities, namely Nagpur and Lucknow, adding ~750 beds to our existing bed capacity with further expansion potential. Both these cities represent key markets in the fast-growing and populous states of Maharashtra and Uttar Pradesh. We consummated the Nagpur transaction on February 9 for a net consideration of Rs. 395 crore, financed through internal accruals and QIP funds. While the Lucknow transaction was consummated on March 7 for a net consideration of Rs. 993 crore, with Rs. 600 crore being

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financed through external debt. We have already embarked upon the performance improvement journey for these hospitals, which include revamping of existing infrastructure, expansion of bed capacity, strengthening of clinical talent and establishing a robust sales & marketing programme.

These hospitals have been renamed as Max Super Specialty Hospital, Lucknow and Max Super Specialty Hospital, Nagpur. We expect them to be the key drivers for future growth in these regions.

In addition, we have acquired ~5.4 acres of prime land at Shaheed path in Lucknow with the potential to build ~550 beds. The plans for this hospital will be firmed up once we turn around the operations of MSSH Lucknow.

Now coming to the Q4 performance – this is our 14th consecutive quarter of year-on-year growth.

The new hospitals have added Rs. 42 crore of revenue, Rs. 3 crore of EBITDA and a net loss of Rs. 11 crore during Q4, including one-time transaction expenses. Consequently, overall Network gross revenue stood at Rs. 1,890 crore, registering a growth of 15% year-on-year and 6% quarter-on-quarter. While Network operating EBITDA was Rs. 503 crore, a growth of 15% year-on-year and 7% quarter-on-quarter. PAT dipped marginally to Rs. 311 crore compared to Rs. 320 crore in Q4 last year, due to increase in effective tax rate having an impact of Rs. 31 crore, net loss from the new hospitals of Rs. 11 crore and movement in non-cash item of fair value of contingent consideration by Rs. 25 crore, which reflects improved projected profitability for the managed hospitals.

Growth numbers from here on are being shared on a like-for-like basis, excluding new hospitals.

1) Average occupancy for the Network was 75%. Occupied Bed Days (OBDs) rose marginally by around 1% year-on-year and 3% quarter-on-quarter, driven largely by increased admissions under preferred channels (Cash, TPA / Insurance and International). Growth in occupancy and footfall was impacted due to capacity constraints.

2) Average revenue per occupied bed (ARPOB) for the quarter improved to Rs. 78,100, growing by 10% year-on-year and 2% quarter-on-quarter. Year-on-year growth was driven by increased share of super specialties (oncology, neurology and cardiac sciences) and increase in tariff, including that for the institutional segment.

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3) Institutional bed share was 29.1% compared to 29.2% last year and 29.5% in the previous quarter. However, after excluding Max Shalimar Bagh, the overall institutional bed share stood at 27.1% during Q4 and OBDs were down by 6% year-on-year for this segment.

4) Network gross revenue was Rs. 1,847 crore compared to Rs. 1,637 crore in Q4 last year and Rs. 1,779 crore in the previous quarter. This reflects an increase of 13% year-on-year and 4% quarter-on-quarter.

5) The international patient revenue grew by 14% year-on-year, slightly subdued due to

credit risk -related actions taken by us and visa -related issues in two markets.

6) Network operating EBITDA touched the magic mark of Rs. 500 crore , reflecting a growth of 14% year -on-year and 6% quarter -on-quarter. Most importantly, annualized EBITDA per bed rose to our highest ever of Rs. 78.5 lakhs , clocking a growth of 12% year-on-year and 4% quarter -on-quarter . While operating EBITDA margin stood at 28.4% for the quarter.

7) Year -on-year growth in EBITDA was impacted due to cost of people hired for Dwarka hospital , GST on variable management fees and higher provisioning due to build -up of accounts receivable.

8) Max Shalimar Bagh, where we added 122 beds , recorded year -on-year growth of 33% and 48% in its revenue and EBITDA, respectively, with an average occupancy of 78%.

9) Profit after tax was Rs. 322 crore versus similar level in Q4 last year and Rs. 338 crore in the previous quarter.

10) Free cash flow from operations generated this quart

er amounted to Rs. 412 crore . Of this,

Rs. 176 crore was deployed towards the ongoing capacity expansion projects and Rs. 1,509 crore was spent for recent acquisitions.

11) Consequently, Net Cash position stood at Rs. 22 crore at the end of March 2024 compared to Rs. 733 crore same time last year.

12) Continuing ou r efforts to support the local communities, we treated approximately 35,200 patients in OPD and 1,200 patients in IPD from economically weaker section s of society entirely free of cost.

13) Both our strategic business units (SBUs) continued to report significant growth in the revenue and profitability :

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- Max@Home reported a top line of Rs. 46 crore , reflecting a strong growth of 25% year-on-year and 3% quarter -on-quarter. This SBU continues to garner customer loyalty and has now expanded its footprint to over 10 cities.

- Max Lab, the non -captive pathology vertical , now offers its services in 41 cities and has a network of over 1,100 collection centres and active partne rs. This SBU reported a gross revenue of Rs. 39 crore , reflecting a growth of 26% year -on-year and 15% quarter -on-quarter.

14) On the status of expansion projects :

- For 303 beds at Dwarka – We are awaiting last few licenses and will be ready to launch the hospital by early June. We have already onboarded over 280 people, including senior docto rs.

- For 241 beds at Na navati (Phase I) – The hospital structure should be up by mid - July, and the proj ect is on schedule , with expected completion by Q4 FY25.

- For 375 beds at Max Smart (Saket Complex ) – Post the initial delay due to tree transplantation issues, this project has now been fast tracked and is expected to be completed by Q1 FY26 , 9 months ahead of the previously communicated timelines .

- For 155 beds at Mohali – Slab work for the 3 basements is underway and base raft has been completed in May. The number of beds has reduced due to change in configuration and requirement to build a fire ramp, which is a new requirement as per Punjab Fire Authorities. Project completion is expected by Q1 FY26.

- For 300 beds at Sector -56 Gurgaon (Phase I) – Slab work for the 3 basements is in progress. The pace of work at the project has slowed down due to restricted working hours in view of adjoining residential complexes. This will likel y impact the project timeline by a maximum of 6 months.

- For 250 beds at Patparganj (Phase I) – Fire and Water Departments have issued NOCs for the building plan, while the Environmental Clearance ( EC) and Municipal Corporation approvals are in process . Tendering work has been initiated.

- For 415 beds at Max Vikrant (Saket Complex ) (Phase I) – EC and Consent -to-Establish (CTE) have been received, while tendering work has been initiated.

- For Nagpur Hospital – Work has been initiated to add 25 beds through internal reconfiguration by Q3 FY25 , while we are simultaneously firming up plans to augment infrastructure by another 140 beds.

- For Lucknow Hospital – We have commenced work s for installing additio nal 140 beds and refurbishing existing 250 beds by December 2024. Further 50 beds will be added through internal configuration in FY26. In addition, we plan to put up a new tower with 300 beds in the first phase by Q1 FY27.

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Finally, moving on to overall p erformance, including new hospitals , for the 12 months ended March 31, 2024.



15) Network gross revenue stood at Rs. 7,215 crore , including new hospitals, reflecting a growth of 16% year -on-year.

16) Network operating EBITDA grew by 17% year -on-year to Rs. 1,907 crore , including Rs. 3 crore from new hospitals. Increased ARPOB, improved case mix and augmentation of network bed capacity translated into a 13% improvement in EBITDA per bed to Rs. 74 lakhs.

17) During the fiscal year, we generated Rs. 1,336 crore of free cash flow from operations after interest, tax,

working capital changes and routine capex , of which Rs. 441 crore has been deployed towards ongoing expansion projects, Rs. 97 crore was distributed as dividend and Rs. 1,509 crore was spent for recent acquisitions.

With this, I would like to open the floor for Q&A .

Moderator: We take the first question from the line of Tushar Manudhane from Motilal Oswal Financial Services .

Tushar Manudhane: Sir, firstly on Lucknow, where we already have acquired Sahara, where you also plan to add 140 beds and then another tower of ~350 beds. So , if you could just explain conceptually the need for the buying land at Shaheed Path, Lucknow, that is my first question.

Abhay Soi: Firstly, the size of UP itself is the size of Europe, and it is an extremely underserved market. It has a major supply of clinical talent because traditionally they've had many government hospitals and medical colleges, etc . It drains a lot of patients from various places, right up from Bihar to Nepal and so on. We believe that there is an immense need for quality infrastructure over there. And furthering our presence in the Lucknow market, in the UP market, will add to further volumes.

I mean, we've barely been there since the 7th of March and in the last couple of months itself, we have moved up the occupancy by 8%. Just our presence right now has taken it up from 54% to 62%. That's the extent of ramp -up that we expect . Having said that, I would also like to tell you that our infrastructure is AA A+ over there. It is right in the heart of Lucknow , if you're familiar with Lucknow.

This is in the heart of Gomti Nagar, 27 acres of land and with access like no other. In terms of infrastructure , it is superior. In terms of location, it is superior to anything else over there. And we believe that there will be a lot of traction. In the state of UP itself , the kind of development, which is happening that you see -- 5 airports have been launched, another 4 are

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being added. The development which is happening is huge , the state is aiming to be a \$1 trillion economy by 2027.

It's not a matter whether they do it by 2027 or 2028 or 2029, but the rate of development over there is second to none -- I mean not even second to none, but it's right up there. And for development of any state, any economy, quality healthcare is a prerequisite. There is a lot more demand over there.

Tushar Manudhane: Understood, sir. Sir, but just that when we could have spread across maybe other cities of UP because peers are also trying to at least build -up 500 bedded hospital if not in Lucknow but in other smaller cities so that can sort of reduce the pace of patient flow to Lucknow, and they are also coming up with super specialty hospital like that.

Abhay Soi: The demand is there across the country, right? The supply of clinicians is where the problem is. Lucknow is where the clinicians reside. That is where the government hospitals, the medical colleges have traditionally been. Now our belief is that as far as tertiary care is concerned, the entire tertiary care of the region will be serviced through Lucknow. You cannot set up tertiary care hospitals in tier -3 cities, in smaller cities , because doctors aren't willing to go there and stay there because the social infrastructure isn't there, the school isn't there and so on.

And given the kind of roads which are being made, the connectivity which is there, it will act like what Delhi is in NCR. You get people from various places in Delhi . Similarly, in Lucknow , you'll have many more hospitals. I mean for the entire state of ~240 million people, there are 2 corporate hospitals and both of them are in Lucknow. We haven't heard of a third corporate hospital in all of UP, right?

I believe that you can have 5 more super specialty hospitals, why 1 or 2 more. The only question that I needed to ask at that point in time when

we were even bidding for this piece

of land is that does it deserve another super specialty hospital , what if somebody else gets it , etc.? Similar to in Delhi, our approach is a cluster approach. We believe that Lucknow could be the next Delhi NCR. And frankly, it can do with many more hospitals.

Tushar Manudhane: Got it, sir. That's helpful. And secondly, with these beds sort of coming up and just to refresh per se in terms of the EBITDA growth for, say, '25, '26, the kind of expenses that will be coming on board, particularly for FY '26. So , if you could share in terms of whether we'll be

able to grow on the EBITDA front even in '26?

Abhay Soi: You're talking about FY26 or FY25 now?

Tushar Manudhane: FY26 , particularly.

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Abhay Soi: Let's imagine what's happening in FY26 . You would have had Dwarka, which starts now in the month of June 2024 and we breakeven by the end of the fiscal year. And next year, we're definitely in the positive and making money. We put all the building blocks of Lucknow and Nagpur in place . Both of them are profitable in any case , and they become more profitable in FY25.

But look, it will have a tremendous growth in FY26 simply because in this year, we'll be fixing everything . Then we have three capacities coming in, in beginning of FY26 effectively, for which we will get the benefit of the whole year – 155 beds in Mohali, 241 beds in Nanavati and 375 beds in Max Saket.

All three additions are brownfields , adjacent to properties, which are already fully occupied.

We should see the same sort of results that we've seen in any of our brownfields . In fact, FY26 will be a very exciting year for us . This year, if you look at FY24, was always meant to be a year of incremental growth. We're very fortunate we were able to add new hospitals . In fact, FY25 would have also been incremental growth if we hadn't added Lucknow and Nagpur. Now, we'll have the EBITDA and revenue kicking in from both of these. Then we add Dwarka, where we will have some losses up to breakeven in the first year, which will get more than absorbed by Lucknow and Nagpur, but next year all three will be exploding.

And in relation to that, we have three brownfields, which don't take time to breakeven and you have seen that in the past.

Tushar Manudhane: Understood, sir. That's helpful. And just lastly, what may push or in fact, prepone the Max Smart by almost nine months. If you could just elaborate on that.

Abhay Soi: As you are aware, we were coming up with a much larger capacity in two phases in Max Smart, and we were also coming up with the first phase of Max Vikrant capacity. Now , the fact is that we got delayed on Max Smart. So , what we've done is instead of doing one phase of Max Vikrant, which is contiguous, on the adjoining land, and doing two phases of Max Smart, we are doing one phase of Max Smart and two phases of Vikrant.

So, what we've done is essentially we have preponed. We are not going to be making as many basements and so it will take less time and we have managed to prepone the entire project. I mean, we'd rather have 375 beds sooner than have 600 beds later and then another 300 beds coming through Vikrant at an even later stage. Right now, we have sort of caught up with Vikrant timelines , which was supposed to be later. And that's how we've done this.

I'd like to add that it's a pleasant surprise that we've been able to prepone Max Smart. A lot of people were expecting delays, but let alone delays, we have preponed this entire project.

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Moderator: We'll take our next question from the line of Damayanti Kerai from HSBC Securities and Capital Markets India Private Limited.

Damayanti Kerai: Abhay, you mentioned in your opening comment, there were some visa issues in two markets, which slightly impacted the international segment -- international patient segment. Can you please

elaborate on it?

Yogesh Sareen: Damayanti, basically there are markets of Iraq, etc., where we have accounts receivable build - up. We took some calls on credit control and we decided that we will not take more patients till such time we recover our money . And that's what Abhay was alluding to. There are also some markets , where there were some disturbances on the visa side . Because of the elections in these markets , visas were not being issued.

These are the 2-3 markets that got impacted . You will notice that overall, we've been growing the international market revenue by around 23%. This quarter, it is 14% growth , which is a bit subdued because of these two reasons.

Damayanti Kerai: Okay. And these are more temporary in nature, right?

Yogesh Sareen: Yes, very much.

Abhay Soi: We continue to see great traction with new offices that we are opening overseas . It is very positive and we intend to sort of double down on that further.

Damayanti Kerai: Okay. In all focus markets, you will be doubling down your effort to get more footfall.

Abhay Soi: We started these direct -to-fly offices , as you recall last year, and we've seen great traction and great throughput through those. We're seeing a lot of success through the new markets where we're setting up our own offices. We will continue with this strategy.

Damayanti Kerai: Okay. Abhay, is the way you mentioned attractiveness of Lucknow market in terms of like demand and then availability of clinical talent, etc. Can you talk a bit about how do you see the Nagpur market? And do we have similar supply of clinical talent in that market, too?

Abhay Soi: Yes, you do have supply of clinical talent. But if I look at India, Lucknow that way is unique,

as in perhaps it has the most amount of supply -- just to give you an example, at least 20% or so of our own doctors in Delhi, which work for Max, have come from Lucknow. So, it has traditionally been the repository for talent. Of course, Nagpur is also a repository for clinical talent, but not to the same extent as Lucknow. You don't have other places with the same sort of supply. I would compare Nagpur more to Dehradun, where we have a hospital which is doing quite well. So, I would compare it to that market rather than Lucknow.

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Having said that, it is emerging as the transplant capital of the country. Because there is a huge amount of awareness of organ donation and it seems to be a cultural thing there, with a lot of people donating organs. Also, it has been a big centre for mouth cancer because of much higher consumption of gutka, etc. We're seeing a lot of oncology patients in Nagpur. And with oncology being our mainstay, I have no doubt that we will do well there. Let me also clarify that Nagpur also has much fewer quality health care assets.

I don't think there is anybody even compared to Alexis Hospital over there. Competition is also significantly lesser. In a way, we will have first dibs on most of the clinical talent over there. We are quite comfortable that even though the supply may be less than Lucknow, but nevertheless, we'll get the best. We don't have a challenge in getting the clinical talent. And the market itself is quite big. It also draws a lot from Madhya Pradesh – Chhindwara, etc. And of course, it also has great connectivity and infrastructure.

Damayanti Kerai: Okay. And my next question is a quick clarification on occupancy. So now you're giving it on like-for-like basis, 75% for fourth quarter and then you have given separate numbers for the new units. Can you just for understanding purpose, on a blended basis, what is the number occupancy number for fourth quarter?

Yogesh Sareen: Around 74%.

Abhay Soi: Please do keep in mind that Nagpur has only been there since 9th of February and Lucknow since 7th of March. So, you don't get the full impact of the fourth quarter,

right?

Damayanti Kerai: Got it. And my last question is like your upcoming hospitals, most of which are located in very attractive locations, brownfield location, etc. For both, we are assuming breakeven within a year of start of the operation. So very broadly, like what kind of loss you generally incur when hospitals are ramping up maybe like loss per month or any indication. For your upcoming hospitals, most of which are brownfield. So, I just want to understand while hospitals are ramping up, what kind of loss you generally incur.

Abhay Soi: I'll give you the example of the last brownfield we did. We added about 122 beds on a base of 280 beds. That would have broken even in 10 or 15 days. And by the 40th day, the additional beds were generating 40% EBITDA margin. And in the whole year, we have 78% average occupancy on the new and old beds. So, brownfields don't take time to breakeven. They don't take years; they take a month. We normally say a quarter at best, but we have kind of beaten that every time.

Damayanti Kerai: Yes. Just like in NCR, I can understand this kind of performance we may expect. But is it true for, say, Nanavati or Mohali, etc, also, we should see similar ramp-up?

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Abhay Soi: No doubt. I mean Nanavati is in Mumbai, right? In the heart of Mumbai, we have another 375 quality beds coming in. And in a place where no new hospital has come up in 20 years, I don't see a challenge in breaking even. Mohali again -- it's the highest ROCE, the highest occupancy that we have in the Network right now. It's not Delhi or Mumbai.

Moderator: We'll take the next question from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Given the background you gave on Lucknow is it fair to assume that Lucknow can get to what we have margins for our corporate average much faster probably, let's say, in FY26 itself. Would that be a fair assumption? Or do you think because of the addition, etc, that could take a little bit longer?

Abhay Soi: See, addition is separate altogether. Addition is a capital expenditure, right?

Neha Manpuria: Sir I mean -- associated with the addition, etc?

Abhay Soi: I didn't get the question. Can you repeat the question?

Yogesh Sareen: Neha, basically, in terms of margin, we can probably get to the overall similar level of margin very soon; not probably in the initial part of the year, but maybe by the end of the year. But the EBITDA per bed will have to wait because the ARPOB there is lower. ARPOB of Sahara Hospital when we acquired it was Rs. 47K.

First, the ARPOB needs to go up. Higher margin on lower ARPOB doesn't mean much because we generally look at EBITDA per bed metric. So, EBITDA per bed will have to wait. I would say it will take some time to get to that level. But in terms of margin, probably by the end of the year, we should be closer to what the margins are on overall basis.

Abhay Soi: On pretty much all, including ARPOB and other metrics, we will be able to catch up in the next two years. And when I say catch up, I mean we know the numbers of some of our peers

over there. So, we should be able to get there, if not further.

Neha Manpuria: Okay. Understood. And what about Nagpur, given the margins there at least the numbers that you gave seem close to 14%, 15%. And it's probably not as flourishing as UP, would that take a little bit longer, a much slower pace to Lucknow.

Abhay Soi: Nagpur or Lucknow?

Neha Manpuria: Lucknow.

Abhay Soi: So, Nagpur is a different ARPOB, and different ARPOB means different ROC E as well. You can compare it similar to a Mohali or Dehradun rather than any other location. You can't

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compare ARPOB of Nagpur to Delhi or Mumbai for that matter. You'll be looking at obviously more subdued ARPOBs, but given the price you pay, you'd look at ROC E.

Neha Manpuria: So, if I were to look at the

Nagpur ARPOB, it is very similar to where Lucknow is, right? So

that's why I was asking. Will it take the same trajectory as Lucknow?

Abhay Soi: Today it is, but Lucknow has potential for a lot more, right?

Like I said, for Lucknow – even if we look at some of our peers who are operating there, they operate at a significantly higher ARPOB.

Neha Manpuria: Yes.

Abhay Soi: Nagpur as a market, well -- you can use maybe Dehradun as a surrogate.

Neha Manpuria: Okay. Understood. Got it. And in terms of incremental M&A, now that you've announced this in land in Lucknow, you've always said we'd rather be the second or third player in the market. Any markets that you think are interesting, particularly given you've done two of these Nagpur and Lucknow now, which could interest you?

Abhay Soi: In the 19-20 cities, where at least two or three of our peers have proven viability and are doing fairly well. We intend to go there if we find the right targets. And we are quite confident we'll be able to perform better than them.

Neha Manpuria: And Abhay, what would be your target leverage? I mean, let's say, if you were to get multiple assets, what leverage are you comfortable with in terms of when you're looking at M&A, particularly given the strong cash flow generation?

Abhay Soi: Yes. So, we are quite comfortable going up to 2.5x debt-to-EBITDA. If you see our EBITDA at present, we will be circa Rs. 2,000 crore. We are at Rs. 1,907 crore presently, and if we add up 11 months of EBITDA for both Nagpur and Lucknow, we'll be closer to Rs. 2,000 crore plus. And then we have growth in the current year.

But even if we take 2.5x that, we get to around Rs. 5,000 - 6,000 crore of debt that we can use. Please do keep in mind, anything bigger than that would mean that we're acquiring something that is listed. And if it's a listed entity in any case, you'll have to look at a merger because you can't have a step-down subsidiary of a listed company, being a listed company.

Moderator: We have a next question from the line of Andrey Purushottam from Cogito Advisors LLP.

Andrey Purushottam: Okay. I have two questions. One is, could you comment on the valuation parameters in acquiring these two hospitals, they are at two different EBITDA multiples. Are they meant

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to be ROC E accretive? Or what is the philosophy in -- financial philosophy apart from growth, obviously? That is my first question.

And my second question is going forward in the next 12 months, how do you see the lever s for profit working in your favour, whether it is the increase in mix of international patients, whether it's a better mix, whether it is a lower proportion of government cases, etc. I just wanted to gain your general outlook on the trend that you see in the next 12 months that will lead to profitability growth.

Abhay Soi: Two aspects here. One is on the financial aspect; we seek 20% to 25% pre-tax ROCE within 4-5 years from acquisition. In both these cases, our belief is that we will be able to hit that number far sooner than that. And we do these transactions by seeing what is the value we are paying versus what we believe with adequate amount of intellectual integrity, what is the business plan that we are able to or capable of underwriting. And then we do a goal seek for that number at 20 %-25% pre-tax ROCE, and that's how we come up with the maximum that we are willing to pay for an asset. Of course, we wouldn't pay that maximum. We try to negotiate, but that's where we are on the financial side. I hope that answers your first question. Now as far as drivers for the year is concerned, it is all of the things that you -- sorry?

Andrey Purushottam: For the next 12 months.

Abhay Soi: Next 12 months is this financial year, effectively. Well, instead of 12 months I will draw a line at 10 months because that's the number of months left for the

current financial year. All

of these factors that you mentioned – higher international patients, which means better payor mix, better clinical mix as we are seeing more robotics happening, we are seeing more growth

in oncology , higher -end surgeries , etc. So, all of these things should be in play. We're also hoping and believe that CGHS rates will get reworked. So even on the institutional side, we're expecting better rates. So , all of these are levers for us in the current year.

Other than that, of course, compared to last year, we would be adding incremental EBITDA emanating from both Lucknow and Nagpur, whilst Dwarka will have some level of initial losses at start-up. But those initial losses at start-up, we will be able to more than absorb it with our profits that we're generating through Lucknow and Nagpur.

Andrey Purushottam: Right. And do you see a drag on profitability?

Abhay Soi: No. Like I said, of course, Dwarka will take time to breakeven, but we've added two new capacities, with ~500 beds in total , which are already generating profits and free cash flows. We will be adding to that. That number is only likely to go up under our watch.

Moderator: We'll take a next question from the line of Bino Pathiparampil from Elara Securities (India) Pvt Ltd .

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Bino Pathiparampil: Just two clarifications. One, did you say you're adding -- planning to add more beds in Nagpur over there over and above between 200 beds there?

Abhay Soi: That's right. We're going to add another 140 beds on top of the 200 beds that we have.

Bino Pathiparampil: By when would that be?

Abhay Soi: We are working that out at present, but definitely within the next 24 -30 months .

Bino Pathiparampil: Okay. And I didn't quite understand the Saket Smart and Saket Vikrant. So , both put together, how many beds are now getting added in FY26 ?

Abhay Soi: No, Vikrant is not getting added by FY26. Smart is getting added in FY26 now. Vikrant wasn't meant to be commercialized in FY26 . Whereas Smart , which was going to be added in FY27 (after initial delay ), has now been preponed by nine months and is going to come through in Q1 FY26 .

Bino Pathiparampil: That is 350 beds?

Yogesh Sareen: 375 beds.

Bino Pathiparampil: Sorry, 375, okay. I was looking at your Feb investor presentation, that shows Vikrant 300 beds in FY '26. Is that old one?

Abhay Soi: Maybe you're looking at the older presentation.

Yogesh Sareen: No. I think in the investor presentation , we are yet to change to 375 beds as well as other changes . That will be updated now with whatever we have done in terms of preponing the beds. There are some beds being preponed and some being postponed. I would say the new presentation will get updated in another week's time . And that is the one that should be referred to. As Abhay mentioned, we've done some re-setting in terms of some beds coming in advance and some beds getting delayed for that reason.

Bino Pathiparampil: Okay. Understood. I will refer to it.

Abhay Soi: Your total number of beds remain the same overall . But like I said, this gets preponed. The total number of beds don't change.

Moderator: We have the next question from the line of Rishabh Tiwari from Allegro Capital Advisors.

Rishabh Tiwari: So, I have two questions. Firstly, what would your EBITDA look like now post Ind-AS for this quarter and for the year?

Abhay Soi: We don't provide forward -looking projections.

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Rishabh Tiwari: No, I'm asking for Q4 FY24 and FY24 .

Yogesh Sareen: For Q4 , EBITDA was Rs. 500 crore from the existing hospitals and another Rs. 3 crore of EBITDA came from these two hospitals that we added in Q4 – one in February and one in March . And the Rs. 3 crore is actually net of Rs. 5 crore of one-time transaction expenses . The overall EBITDA for the year is Rs. 1,907 crore .

Rishabh Tiwari: T

his would be post-Ind AS right?

Abhay Soi: Yes.

Rishabh Tiwari: Yes, I'm asking post Ind-AS and pre Ind-AS.

Yogesh Sareen: For pre-Ind AS, you will have to reduce the number by around Rs. 70 crore . So, if you want to take the leases up, that's the only difference in our case. The lease cost would be around Rs. 70 crore .

Rishabh Tiwari: For the year or for the quarter?

Yogesh Sareen: For the year.

Rishabh Tiwari: And next question is Abhay you mentioned that we took the occupancy up from 54 % to 62 % in Sahara Hospital. I just wanted to know more about it – was this through operational levers?

Or did we see some improvement in seasonal mix ? How do we take this up within Q1?

Abhay Soi: No, it is the result of changing some basic processes. You always have some very early and easy wins when you walk in, because you would have seen those things earlier in your existing hospitals . So those changes kick in. These are very cheap , early wins.

Rishabh Tiwari: Okay. And are we looking to add any beds here as well in Sahara?

Abhay Soi: We will be adding beds. Like I said, we will be adding 140 and then another 450.

Moderator: We move on to our next question from the line of Amey Chalke from JM Financial Institutional Securities Ltd .

Amey Chalke: Congratulations on a good set of numbers. The first question I have is on the volume growth.

So IPD volume seems to be around low to mid-single digits. Anything to read there? Or do you expect the volume growth to remain at this kind of level? And second question I have is on the safety selection. I understand we generally map 20-odd cities across country. What would be the cities you would not like to enter into? And what are the reasons for the same?

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Abhay Soi: First and foremost, you have to keep in mind that our occupancy growth till we come up with new capacity, brownfield or otherwise, will be fairly muted because we have capacity constraints. We don't have beds available to put more patients. That's why we are doing the brownfield.

Having said that, because of constraints on capacity, our occupancy ramp-up will be limited, whilst it will have a play, a positive impact on ARP OBs. What tends to happen is that the lower-end surgeries get posted at a later date, whilst the major ones get priority, as they should. Therefore, the lower-end ones sometimes have a tendency of evaporating, because the patients then decide to go to some other facility or some other brand, because they may not be very important sort of surgeries as well.

What happens is that -- while the occupancy does not ramp up, you see a significant growth in ARPOB. But when you come up with brownfield capacity, the interplay which happens is that the occupancy ramps up immediately, but your ARPOB growth becomes more muted. Having said that, the impact on overall EBITDA per bed is positive simply because you have huge operating leverage emanating in the new capacities that you add, since the management cost, clinical costs, etc., are already being incurred by the existing capacity.

I just wanted to answer that question as far as occupancy is concerned and assure you that there's nothing to read into it. It is simply that we don't have capacity. So, our occupancy will go up at a slower pace.

Your second question was with respect to which cities we would not go to -- so conversely, we would not go to cities where our peers have not proven viability or to uncharted territories because we don't see ourselves as pioneers going into new places and taking chances. We would rather have somebody else define the market, demonstrate success and then we would go there and try to do it better. We've been very comfortable in doing that. And we've been successful in doing that.

Amey Chalke: So, is there any city in which you think is oversupplied with the quality of the beds, etc.? Or is just

that the -- because it's uncharted territory, that's why you don't want it.

Abhay Soi: Let me give you an example. The maximum number of beds, in any geography, is in Delhi NCR. We have 11 hospitals over there with ~2,500 beds. We are the largest player by far. We are equal to next three of our peers put together and multiplied by two in terms of number of locations, and we are equal to all three of them put together, as far as number of beds are concerned.

And the next 15 players have similar number of locations and a similar number of bed capacity. Yet we only have ~2,500 beds -- 10% of which are free, 10% of which are catering

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to international business, 40% of which are catering to upcountry. Essentially, if the largest player has ~800 beds catering to the NCR, and the next three peers of ours also have 800 beds and then the next 15 people have 800 beds, that is like 2,400-2,500 beds for a population of ~48 million people, which is ~85% of the U.K.'s population.

And this is a place where you have the maximum amount of supply. Look at the places we're going to -- if we are going to Lucknow, literally there are 2 corporate players, then we've come up with a hospital and in a state with a population of 24 crore people. In Dehradun, we are pretty much the only players over there. In Mohali, it is us and one more player. If some of our peers in Patna, then they are the only players over there.

If you look at places like Kanpur or Pune, I mean, every place is underserved, right? Even in a place like Mumbai, no new hospital has come up in 20 years. I don't think in India, we have a problem of oversupply anywhere.

Amey Chalke: Right. But in generally, speaking about the Mumbai, like we generally don't see except -- I'm talking about the private hospital, we don't see hospitals being overrun or something like that or there is a like we have not seen many hospitals getting added, but we have not seen capacity getting over --

Abhay Soi: No, no. You haven't seen hospitals being added because to make a 400-bed hospital, you need four acres of contiguous land. Firstly, you can't find four acres of contiguous land in Mumbai.

And even if you find it, you'd rather make a residential complex or a commercial building there .

I mean no new hospital has been made for 20 years . It's the same hospital in Hinduja, the same in Breach Candy . They haven't added one square inch because where is the space. Even H.N., they brought down the earlier hospital and made a new hospital . Bombay Hospital is the same one, Jaslok is the same one. Nanavati is the only one which has the land , so we're building on it.

Moderator: We have a next question from the line of Kunal Dhamesha from Macquarie Capital.

Kunal Dhamesha : So just kind of continuing on the previous question. So mature hospitals, we are at 75%. And we are seeing that there is not enough capacity for us. But we have done quarters with 77%, 78% occupancy, right? So , what is it that -- is it a mix of hospitals where our good hospitals are already full and some of the hospitals are below that 75 % run rate ? And secondly, if it's kind of across the hospitals, we are facing difficulty in terms of capacity, then why our peer mix improvement is not accelerating in terms of reducing the institutional patients?

Abhay Soi: Because it is a seasonal business . There are some quarters which are surgical quarters and some quarters which are medical quarters . In a medical quarter, you require rooms while in

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a surgical quarter, you require more ICU beds. And every kind of capacity is not fungible across every hospital.

In fact, some hospitals may have certain constraints that they're operating at 75% because maybe they have lesser single rooms where there's demand for it and more ward beds where you don't have demand for , etc. But whereas in a dengue season, even t

he lower category or

multi-share rooms get absorbed. But that doesn't happen in the surgical season. That's the reason that we are constrained by seasonality as well as the kind of infrastructure that we have.

Now as far as the pay or mix is concerned, the pay or mix is a slow churn now. If I look at ex-Shalimar Bagh, ex-new capacity that we've come up, it's come down from 29% to 27%, but there's been a 6% reduction as far as the OBDs are concerned. What we are seeing is – a turn perhaps more so on the OBDs than anything else.

Kunal Dhamesha : Sure. But let's say, when you say the 122 beds added in the Shalimar Bagh capacity or hospital. And we say that it's been EBITDA positive in 10 days. Is it more on a contribution margin? Because if I back calculate our pay or mix for that 122 beds, there'll be roughly 46%, 47% of those beds have been occupied by institutional patients, right? And we all know that the ARPOB roughly 50% of what you get in other channels.

So, is it on a contribution margin we are seeing that the EBITDA turned positive in 10 days ? Was it loaded with the management cost? Or how should we think about that?

Yogesh Sareen: Kunal, it is on EBITDA. It's after the incremental cost that you have to incur for those beds.

So, it's not contribution , it's EBITDA. We had two options. We had an option to open only 50 beds to start with, and not all of the 122 beds . But what we did is that we opened all the beds and filled up the balance also through the institutional patients.

There were two options and we chose to open all 122 beds and then try and fill up and then filter , then distil -- that's where we are today. But when we say the number, 40% is EBITDA margin, after incremental cost. If there are 122 beds being opened, there is more manpower being recruited, right? There are nurses required. There will be GDAs required, maybe less doctors, but obviously there will be extra costs, etc . So, the number reported is after all those costs.

Kunal Dhamesha : So even with 46% kind of institutional payor mix, we are making 40% EBITDA there. Is that the correct...

Yogesh Sareen: Because the incremental cost of running those beds, which we opened, is very less .

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Abhay Soi: Let me explain this separately. Let's say, if you look at bed-by-bed, unit wise, right? Or let's say, all 100% of the beds over there, you were operating only for institutional business. It will still be EBITDA positive.

It will be EBITDA positive simply because you have operating leverage. The only thing you're getting over there necessarily is the nurses, which is not very expensive, is the resident doctors, again, not very expensive, and allocated to those beds alone, which are operational. What you're not doing is you're not fitting out beds and you're not sort of staffing beds where you don't have capacity.

You basically open floor by floor, whatever you have demand for. There's no fixed cost associated or fixed variable costs associated with beds which are not operational. And the ones which are operational, we have only the low-end cost. It does not have high-end sort of clinicians, etc., or their minimum guarantees, which are being taken on board . Because those clinicians already exist, their minimum guarantee is already being paid. Utilities are common.

The general set of management costs has already been incurred. You don't operate with a separate CEO, separate Medical Director, separate HR, separate Marketing, nothing for that. Kunal Dhamesha : And if I may, just one more on the Supreme Court matter. Now it seems that the next date for that hearing of PIL seems to be somewhere around September. So, what's your internal assessment? Is it kicking the can down or would you say that the risk of adverse outcome has reduced meaningfully in your view? How do you view that? Abhay Soi: We would not like to comment on

it. It's a Supreme Court matter and currently sub-judice. We've seen the rap IMA got, so we definitely don't want to be speaking about it. But in the last hearing, the comments, the observations of the judge, etc. were for everybody to hear and make their own inferences.

Kunal Dhamesha : Sure. Thank you, all the best.

Moderator: We'll take our next question from the line of Shubham Harne from Purnartha Investment Advisors.

Shubham Harne: Sir, on Dwarka, I want to ask questions. So that is getting added in tranches or it will be on one go?

Abhay Soi: In tranches. Because our occupancy will also have been in tranches only.

Shubham Harne: Okay. So initially, earlier, you have said 160 beds will be added and then 140 like -- something like that only?

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Abhay Soi: Yes, since occupancy also ramp -ups in that manner. No point staffing all the 300 beds when we don't have occupancy on day one for all 300 beds.

Shubham Harne: Got it. And on Gurugram sector 56 hospital, by when it will get commissioned or something from -- around that?

Abhay Soi: Gurugram Sector 56, we are delayed by 6 months. So, it should be second quarter of FY26 now.

Keshav Gupta : Yes, Q2 FY26.

Shubham Harne: Okay, thank you sir.

Moderator: We have our next question from the line of Rishabh Tiwari from Allegro Capital Advisors.

Rishabh Tiwari: Sir, just one follow-up on the lease impact. So Rs. 70 crore is what you mentioned for the year, is this taking into account the recent acquisition? Could you please help us with the like-to-like number?

Yogesh Sareen: The recent acquisition does not have any lease rentals. The land is getting converted into free hold. We have already put in our application for same. Rs. 70 crore is on the existing hospitals. Rs. 71 crore is the total impact since there are one or two more entries which come below the line. So, I'm taking all those -- there's also the liability for donations that we have. So, Rs. 71 crore is total impact of the Ind-AS movement. If I was to do IGAAP accounting, then Rs. 71 crore will come before the EBITDA.

Abhay Soi: Just an important point to note. I don't know if you picked up my statement earlier or not that during the fiscal year, we generated Rs. 1,336 crore of free cash flows from operations. This is after interest, tax, working capital changes and routine capex also, let alone leases. Our translation of post Ind AS EBITDA to free cash flows, after the pre Ind AS impact, post interest tax, working capital changes and routine capex, is 70%. This is a significantly high number.

Moderator: Thank you. We'll take our next question from the line of Senthilkumar Natarajan from Joindre Capital Services.

Senthilkumar Natarajan : My question is what is the management policy on writing off goodwill and other intangible assets over a certain period of time as a matter of prudent accounting policy?

Yogesh Sareen: Under the Ind AS method, Goodwill is only tested for impairment. Intangible assets represent some of these contracts that we are writing off over the period of the respective contracts.

But there is an element of Brand in the intangibles, which is again tested for impairment. Ind AS accounting standards required to only test for impairment for Goodwill and Brand. And

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for the other intangibles, you will find that they are coming down progressively year after year because we're putting them in the amortization charge.

Senthilkumar Natarajan : Yes. OK, understood. Thank you. That's it from my side.

Moderator: Thank you. We'll take our next question from the line of CA Vipul Makwana from Makwana.

CA Vipul Makwana: Apart from all the financial queries which my peers have asked, I just want to ask on the qualitative aspects like what is Max doing separately or concisely different than the others were seeing a good number set, like on

ARPOB and everything. So, Abhay if you could just throw some light on it.

Abhay Soi: First and foremost, the fact that we have significantly higher occupancy levels than any of



our peers means that we are doing well on qualitative aspects as well as the value proposition that we are able to provide to our patients .

And it's not only in Delhi or Mumbai . Our highest occupancies also continue to be in Tier 2, Tier 3 cities. Yes, it's for the patients to perceive more than what we can say in terms of what are the qualities that they appreciate . And we continue to cater to a lot of free patients – we treated ~35,000 patients in OPD and ~1,200 patients in IPD in the last quarter itself.

All our hospitals are teaching hospitals. We have 500+ MBBS doctors as part of our DNB program – these are post-graduate students. We write a lot of -- you've seen the number of publications that we're doing, which is the highest ever that we did last year, both for international and domestic journals. We continue to do academics and research as well besides, of course, the financial parameters that we offer.

CA Vipul Makwana: That helps. Thank you so much.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to management for closing comments. Over to you.

Abhay Soi: I would like to thank each one of you for either covering the company or having invested in the company. We appreciate that. And we believe that the next two years will be years of exponential growth for us, particularly FY26 , where a lot of things are going to be coming on stream. So , we look forward to an extremely lucrative journey going forward. Thank you.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

## Call: Aug 2024

Max Healthcare Institute Limited  
Corp Office: 2nd Floor, Capital Cyberscape,  
Sector - 59, Golf Course Extension Road,  
Gurugram - 122102, Haryana  
T: +91 -124-620 7777

www.maxhealthcare.in Max Healthcare Institute Limited  
Regd. Office: 401, 4th Floor, Man Excellenza, S. V. Road,  
Vile Parle (West), Mumbai, Maharashtra - 400 056  
T: +91 -22 2610 0461/62  
E: secretarial@maxhealthcare.com, investors@maxhealthcare.com

(CIN: L72200MH2001PLC322854)

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Listing Department ,  
National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,  
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Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on August 2 , 202 4  
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on August 2 , 202 4, on financial results of the Company for the quarter ended June 30 , 202 4.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/investors/investor -resources](http://www.maxhealthcare.in/investors/investor-resources) .

Kindly take the same on recor d.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Aroraa  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited  
Q1 FY25 Earnings Conference Call  
August 02 , 2024

Moderator : Ladies and gentlemen, good day and welcome to Max Healthcare Institute Limited Earnings Conference Call.

Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you and over to you, sir.

Suraj Digawalekar: Thank you, Neerav. Good morning, everyone and thank you for joining us on Max Healthcare's Q1 FY'25 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director, Mr. Yogesh Sareen – Senior Director and Chief Financial Officer and Mr. Keshav Gupta – Senior Director, Growth, M&A and Business Planning.

We will begin the call with opening remarks from the management following which we will have the forum open for an interactive Q&A Session.

Before we start, I would like to point out that some statements made in today's call may be forward-looking in nature and a disclaimer to this effect has been included in the Earnings Presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi: A very good morning to all, and a warm welcome to Max Healthcare's Earnings Call for the Q1 FY 25.

It's been a very promising start to this financial year. I am pleased to share that we have operationalized Max Super Speciality Hospital, Dwarka on July 2nd. Located in the heart of South Delhi, it is a 303-bed hospital equipped with the latest high-end technology and currently has 125 doctors and ~480 nurses, paramedics and support staff.

The hospital is seeing good traction since its launch. Presently, the hospital teams are engaged on various fronts including patient outreach, empanelment with insurance companies, clinical accreditations, etc.

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Yesterday, we announced a built-to-suit transaction for setting up a 250-bed hospital in Mohali. This 2.75 lakh sq. ft. building with attendant parking will be constructed by the developer as per our requirement and specifications. This is a long-term lease, extendable up to 50 years at our option, and we expect to commission the facility by FY28. The hospital will cater to surrounding areas of Himachal Pradesh, Punjab and Haryana. This is in line with our asset-light strategy, which allows us to expand while insulating against development risks.

Further, in a strong demonstration of our turnaround capabilities, both Lucknow and Nagpur hospitals that we acquired at end of Q4 last year are ramping up well, with an average occupancy hovering around 60%. For Q1, these hospitals contributed Rs. 99 crore to the revenue and Rs. 18 crore to the operating EBITDA, reflecting a year-on-year growth of 21% and 64% respectively. And more importantly, this was done within the first three months of us acquiring these facilities. Collectively, their ARPOB stood at Rs. 45,300 and annualized EBITDA per bed at approximately Rs. 30 lakhs. A large part of the post-merger integration activities has been completed now, the results of which will be more visible in the quarters to come and further augmented as new clinical teams join and infrastructure is upgraded there. Now, coming to the Q1 performance. This is our 15th consecutive quarter of year-on-year growth.

Overall network gross revenue stood at Rs. 2,028 crore, registering a growth of 18% year-on-year and 7% quarter-on-quarter.

Network operating EBITDA was Rs. 499 crore, net of one-time charge of Rs. 6 crore towards pre-launch costs at Max Dwarka. This reflects a growth of 14% year-on-year and a marginal decline of 1% quarter-on-quarter.

Profit after tax (PAT) stood at Rs. 295 crore compared to Rs. 291 crore in Q1 last year and Rs. 311 crore in the previous quarter. While we expect substantial improvements during the course of this year, we are also infusing equity in the Lucknow subsidiary to bring down the finance costs.

All numbers from here on are going to be now shared for a like-to-like basis, excluding new hospitals.

1. Our average occupancy for the network increased to 77% from 74% in Q1 last year and 75% in the trailing quarter. While occupied bed days grew by around 5% year-on-year and 2% quarter-on-quarter.

2. Average Revenue Per Occupied Bed (ARPOB) for the quarter improved to Rs. 80,100, growing by 7% year-on-year and 3% quarter-on-quarter. Year-on-year improvement in ARPOB was largely due to growth in oncology, orthopedics and

renal sciences as well as increased number of robotic procedures. This was coupled with tariff revisions for self-pay, insurance and institutional segments.

3. Network gross revenue was Rs. 1,929 crore compared to 1,719 crore in Q1 last year and Rs. 1,847 crore in the previous quarter. This reflects an increase of 12% year-on-year and 4% versus the trailing quarter.

4. The international patient revenue stood at Rs. 158 crore. There was a temporary impact on patient flows from some countries due to political situations and credit-risk management related actions taken by us, which accounted for the muted growth in this segment.

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5. Network operating EBITDA stood at Rs. 487 crore, reflecting a growth of 12% year-on-year while declining by 2% quarter-on-quarter. Operating EBITDA margin stood at 26.5% for the quarter. Growth in EBITDA was impacted due to annual merit increase, GST on variable management fee and drop in footfalls for immigration health checks owing to change in visa regulations by some countries, particularly Canada and UK.

6. Annualized EBITDA per bed stood at Rs. 74.7 lakhs, clocking a growth of 6% year-on-year.

7. PAT was Rs. 316 crore versus Rs. 291 crore in Q1 last year and Rs. 322 crore in the

previous quarter.

8. Free cash flow from operations was Rs. 258 crore. Of this, Rs. 213 crore was deployed towards the ongoing capacity expansion projects and upgradation of facilities at new hospitals. Consequently, net cash position stood at Rs. 66 crore at the end of June 2024.

9. Continuing our efforts to support the local communities, we treated approximately 36,800 out-patients and 1,150 in-patients from economically weaker sections of society entirely free of charge.

10. Both our strategic business units (SBUs) continued to report significant growth in the revenue and profitability:

- Max@Home reported a top line of Rs. 49 crore, reflecting a strong growth of 23% year-on-year and 6% quarter-on-quarter. This SBU offers 14 service lines over 10 cities and continues to experience a very high rate of repeat transactions.

- Max Lab, the non-captive pathology vertical, now offers its services in nearly 50 cities and has a network of over 1,100 collection centers & active partners. This SBU reported a gross revenue of Rs. 41 crore, reflecting a strong growth of 21% year-on-year and 6% quarter-on-quarter.

11. Now, coming to the status of expansion projects:

- For 590 beds at Lucknow hospital, the work on refurbishing the existing facility is underway. Finishing work for operationalizing additional 140 beds has started. We have also received the environmental clearance (EC) approval for setting up a new 450-bed tower on this site and we expect to complete the development of the new tower within 24 months.

- For 140 beds at Nagpur hospital, work has been initiated to add around 25 beds through internal reconfiguration by Q3 FY25 and environmental clearance (EC) application for adding 115 beds on two additional floors has been filed. We expect approval over the next 2-3 months and the project completion within 24 months.

- For 241 beds in Nanavati (Phase I), the hospital structure will be up in this month and the project completion is expected by end of this fiscal year, as

communicated earlier . The project continues to be on schedule.

- For 375 beds at Max Smart (Saket Complex ), the project was fast-tracked in Q4 and we expect its completion by Q1 FY26. This is a steel structure building and the installation of columns has already started. The site is fully mobilized and we don't expect any further delays.

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- For 155 beds at Mohali , work on the 2nd floor slab is underway , while the work on ramp area side is reaching ground level. All high-side equipment has been ordered and the project is on schedule. We expect the completion by Q1 FY26.

- For 300 beds at Sector 56, Gurgaon (Phase I) , basement slabs are nearing completion and the project is progressing as per plan. Orders for all high lead-time items have been placed. We expect completion of the first phase by Q2 FY26.

- For 250 beds at Patparganj (Phase I), post issuance of the no objection certificate (NOC) by the fire and water departments , we have submitted the drawings for approval and the tendering process for this project is on.

- For 415 beds at Max Vikrant ( Saket Complex) (Phase I) , the environmental clearance (EC) and consent to establish (CTE) have been received. The barricading and D-walls , etc. has started. We have applied for forest approval and are still awaiting the same. We expect some delay due to ongoing litigation involving DDA and Delhi Government regarding cutting out trees in the eco -sensitive zone near Asola Bhati Wildlife Sanctuary and the Ridge areas without approval.

With this, we open the floor for Q&A.

Moderator: We will now begin the question -and-answer session. The first question is from the line of Amey Chalke from JM Financial.

Amey Chalke: The first question I have is in Lucknow Sahara unit. I understand that we have a plan to expand the unit to 550 beds and then there i

s another tower which could come up.

But what is the near -term action plan for the existing units of the 250 bedded unit in terms of the speciality addition , etc. ARPOB , I believe , is substantially low, so what would be the driver s going ahead for this unit to improve the ARPOB?

Abhay Soi: Clinical programs is clearly one, adding doctors is the other and that's how you achieve growth in clinical programs , and improvement in infrastructure. We expect to refurbish the present infrastructure as well as add 140 beds by end of this calendar year. As far as clinical programs are concerned – oncology represents less than 2% of the total revenue in Max Lucknow. So, that's a major driver for us. As you know, it is a high ARPOB business and contributes more than 24 -25% to our overall revenue. So, if you look at implying that to Max Lucknow , you will see a major fillip in ARPOB. Like I mentioned, we've seen that in the first three months literally . We acquired it in end of March , and in April, May, June compared to April, May, June last year, there's already been a ~58% increase in EBITDA, and this has largely come through all the low hanging fruits, etc. , which were there.

The real changes that we may have made, you're going to have all of those pay dividends in the quarters to come . Also, typically we have a higher proportion of surgical business in our hospitals and a lower proportion of medical business, whereas in both Nagpur and Lucknow, the situation is the other way round. So, when your surgical mix moves up with respect to the medical mix , you see higher ARPOBs in any case. So, there are plenty of levers. We are not sort of concerned about that. And frankly, the kind of traction that we are getting and the early wins that we have been able to demonstrate over there, gives us a lot of confidence that we should be able to ramp up both ARPOB as well as profitability there.

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Amey Chalke: So, this oncology addition, etc. should we expect it in the existing unit or it will happen once the expansion for the bed addition happens?

Abhay Soi: No, no. In the existing unit. We already have bunkers for radiation, etc., which are non-operational. We will just operationalize this. The 140 beds that we are adding as well as refurbishment that we are doing is going to happen by the end of the year.

Actually, even the new tower that we are building is going to be done over the next 24 months because it is 27 acres of land and we don't have to build so many basements, similar to what we are doing at Max Smart. But yes, the oncology business should ramp up in the current year itself. It may not ramp up to 25%, but you will see ramp ups in the quarters to come immediately, as well as in the surgical business. Even after we take over these, when we start adding doctors, when we issue letter of intents and they sign up to join us, they still have 2-3 months' notice period. So, it takes them time to join us and as the infrastructure, the new equipment that has been ordered, etc. comes through, you will see more and more teams coming up. Right now, all the technology, all the equipment, etc. is being replaced, right?

Amey Chalke: The second question I have is on the Dwarka unit, which has been recently commissioned. If you can provide some timeline over there, how the ramp up will happen? And in terms of the Speciality, is it at par at present with the other units or do you think the improvement will happen over a period of time?

Abhay Soi: So, in terms of the infrastructure, it is at par. I think the bunker is not ready as yet. That is going to be ready in due course. But other than that, I think as far as the infrastructure and technology is concerned, not only it is at par, but it's also the new generation. I would say it's perhaps one or two steps ahead of the existing infrastructure. We have again seen very good traction

over there. It is a massive facility. It's the best infrastructure of its kind in the region of Dwarka. There's a huge amount of population. Again, fantastic traction is what we have seen in the first month itself. And I think that should continue. We should have a break even within 6 to 9 months.

Moderator: Next question is from the line of Rishi Mody from Marcellus Investment Managers.

Rishi Mody: Abhay, just on the Mohali hospital that you are planning to add four years out, right, is this going to be firstly an O&M agreement the way we have with Devki Devi and those kinds of hospitals or is it an outright rental sort of a model that we see in retail where we don't incur any CAPEX, we have to just put in a security deposit, and we start operating on it?

Abhay Soi: It is the latter, we're leasing it. It's an outright rental / lease model where somebody else is putting the infrastructure and has invested in the land making a built-to-suit. It's a 50-year lease agreement. The only difference is normally you have commercial leases for 9 or 15 years, but here there's a 50-year lock in.

Rishi Mody: So, just from a broader conceptual point of view, right, are we seeing incremental interest from, say, the builder or real estate community towards having this sort of models or like if you could just share your observations from across north India or rest of the country?

Abhay Soi: It depends on the tenant. Today, say you tell a builder to make a built-to-suit and we will give you 8-8.5% yield, with 3-4% increment every year, which is what the market is, as far as commercial real estate is concerned. As far as the builders or developers are concerned, they are happy to do it for commercial real estate, for retail real estate or for hospitals. The question is how many hospital chains can offer this. Your own

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ROCEs have to be significantly higher than what you are paying out over there. In our situation, we are able to take up these hospitals because our ROCEs are significantly higher. And so, this built-to-suit model works for us. And it certainly works for the developers. They are taking a punt on our balance sheet effectively and that is where they're getting the confidence that we will be around for 50 years, and they'll be able to, even after making that particular hospital, dedicate it to us for 50 years. Because do remember, this is a built-to-suit, so it's as per our design. If somebody is building something under certain specs and leasing it out to you for 50 years, he has to be very confident that you will be around for 50 years and your balance sheet will be able to hold it. I think that's the punt a developer needs to take because eventually you're a tenant, but here you're a tenant for 50 years. It's not as if you can move this hospital and you will start using it for a mall or give it for some other purpose or hotel or whatever else it is.

Rishi Mody: I think you went through the capacity addition plans and updates pretty quickly, but just on an overall basis versus Q4, any delays or any preponement, if you could highlight that piece?

Abhay Soi: Not really. So, we're not expecting any delays or preponements. I think, except for Vikrant, which is the last one in any case and is a 2027-28 generation hospital where we're expecting maybe a couple of quarters of delay. Other than that, we are not expecting any delays. So, you should have Mumbai, Mohali, as well as Smart up by

Q4 FY25 , Q1 FY26 .

Rishi Mody: Yogesh, I had a question for you on the tax rate piece. So, I am seeing a tax rate for this quarter has come up to 26%. Just wanted to understand because I am under the impression that we have a tax benefit in the BLK and the Nanavati and now with both, the Dwarka unit. So, is my understanding correct and this is just a factor of the profitability for these two charitable hospitals, if you could

just help me understand the tax piece for you all?

Yogesh Sareen: That's right. So, overall rate is 22.8% . The rate has gone up this quarter because we have losses in the new entities, mainly the Sahara hospital entity. Sahara hospital transaction was Rs. 993 crore. We funded this by Rs. 840 crore of loan into the entity. And as Abhay alluded to in his speech, we are changing the cost and capital structure in the company. So, since we have losses in the company, we don't do the DTA as per our policy till such time the company is profitable. That means that you have these losses, but you have tax on the other entities. That is the reason why the rate has gone up, but otherwise the rate was same as in Q4 . Q4 was 21.8% and we do think that going forward it will be around 22%. Also, for BLK, the agreement is that whatever is the net profit , 98% of the profit comes to us. So, the moment we withdraw that money from there, there are two taxes on that : one is that there is GST and also other is that when the money comes on this side, then we have to pay tax on it , right? So, to that extent, BLK doesn't allow me any tax shields.

Yogesh Sareen: We don't trap money in trust. We take it and we pay full tax on it.

Rishi Mody: For example, now in BLK, there's no capacity expansion plan, so you are taking the money, repatriating it to the holding company, but Nanavati, we have a plan of expansion, hence we are not repatriating, but once that expansion is done, it will also become like BLK, is my understanding correct?

Yogesh Sareen : That's right, but also this formula is on cumulative basis . So, what happened is that BLK wiped out their losses on a cumulative basis in Q2 last year. So, the withdrawal has happened from Q3 onwards . Similar thing will happen for Nanavati, the

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withdrawal will happen but that will happen once the cumulative profits are made in that trust.

Rishi Mody: So, maybe 1 -2 years down the line, it will start paying off?

Yogesh Sareen : Yes.

Moderator: Next question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets (India) Private Limited .

Damayanti Kerai: My first question is on the margin contraction which we have seen in 1st Quarter . Can we attribute most of that for the new facility cost or between 4Q and June quarter, have you seen any major change in the business mix say in terms of medical and surgical volumes which are coming to your facility?

Abhay Soi : We have clearly seen a change in the clinical mix. It's moved to a higher -end clinical mix. And like I've mentioned in the past, when you do higher -end surgeries, you do more robotics and more oncology, etc., they typically have lower margins in percentage terms, but higher margins in value terms. Whilst you see a flattening of EBITDA margins, you see an increase in EBITDA per bed on the existing hospitals as well. The takeaway should be that what is the increase in EBITDA per bed because eventually we have got two things - we have got inventory and we have got number of days in a year. You would rather do a 20% margin on a Rs. 10 lakhs surgery than do a 50% margin on a Rs. 2 lakhs surgery, right? So, the percentage is irrelevant. Other thing is that we had a reduction in OP footfalls, like I mentioned , in the current year and these OP footfalls were largely from immigration because of disruption in visa from Canada and from UK and a little bit in Australia. That has also had a couple of percentage points impact.

Damayanti Kerai: My second question is a couple of your hospitals are scheduled to start in another 6 to 12 months if I may say, Nanavati, Smart . So, just want to understand when you are about to start a new facility, when do you start your pre -launch activities and what kind of cost you are anticipating for these upcoming units?

Abhay Soi : Unlike Dwarka, which is a new facility and you need to actually

have a minimum

number of doctors and staff, i.e., you pretty much need to staff the entire hospital. In that case, before you apply for even a nursing home license, you are sitting fully staffed, then you make the application for the nursing home license and then you need to promote the facility by advertising it, etc. by onboarding doctors and getting all the insurance companies and TPAs onboarded , by getting institutional

empanelment , by getting corporate tie -ups, etc. You are really starting from ground zero. All of the facilities that you have alluded to right now, be it Nanavati, Mohali as well as the Max Smart , they're all existing facilities. So, we don't need to promote the existing facilities. There are enough people waiting in the ER over there, waiting for beds . In fact, it's going to be very similar to what happened in Shalimar Bagh. In Shalimar Bagh , we didn't have to incur any pre -op expenses or promote the facility or even wait to get the contracts, etc . All of the contracts were already signed up at the existing facility. So, you won't see any pressure on margins over there. I think almost from day one you are good to go.

Damayanti Kerai: My last question is a clarity on your Mohali planned hospital. So, you said it's a 50 - year long lease and it's a built to suit premise which you will be getting. So, there will be no CAPEX once you get the facility lead, just say, equipment and then people cost which will be going towards this?

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Abhay Soi : That's right. We will be providing the medical equipment , the security deposits, which is interest bearing , and loose furniture . The entire project cost will be incurred by the developer, including the land and building, etc. More importantly, any delays, any cost and time overruns, everything will be on to their account. This is very similar to what we have done in Dwarka .

Damayanti Kerai: The way like Dwarka, it's O&M facility which you got into your network, but -

Abhay Soi : This is not an O&M; it is a lease . But the same arrangement like Dwarka, in the sense that developer will develop it and incur the project cost . We only bring in the equipment and fit-outs. Whether you look at O&M or a lease, the commercials are pretty much the same and the arrangement is asset -light.

Moderator: Next question is from the line of Kunal Randeria from Axis Capital.

Kunal Randeria: Abhay, in the last few quarters, the number of your institutional bed is stagnant around 29%. Do you see headroom for some improvement over here?

Abhay Soi: We do see headroom for improvement here, but usually Q1 and Q3 are weaker quarters. So, compared to a Q4, you take your foot off the accelerator since that's not the time because your occupancy levels subside at that point in time and you are not going to churn your patients. Typically, you will see churn happening in the Q2 and Q4 .

Kunal Randeria: Because even I think in last quarter it was somewhere around 29%, right in the last five or six quarters somewhere in this range.

Abhay Soi: But that's a Q4, right, the fourth quarter is a strong quarter. So, if you see Q3 versus Q4, there may have been some change. But if you look at Q4 versus Q1, you won't have a change. So, typically now again from Q1 to Q2, you will see a change, but Q2 to Q3 you won't see a change.

Yogesh Sareen: I would also say there's a change that you don't see. That change is in the ARPOB of the business, right? We do the PSU business, but while you may see the same beds, but the throughput of those beds has gone up. If I see the improvement in the ARPOB year-on-year, it's a 10% improvement in the ARPOB of the PSU. This means that within those beds we are trying to do higher-end work.

Kunal Randeria: Second question is on ARPOB. This quarter saw around 7% growth on your existing units. So, is this the new normal that we should expect going forward?

Abhay Soi: No, this is because of immigration and visas, that's had maybe a 2-3% impact on ARPOB. That is a business we had last year. But this year there's been some disruption because of Canada and UK not issuing immigration visas, and we have seen the same with Australia as well. We believe it's a temporary phenomenon and that business is a little subdued . But if it doesn't, then we kind of repurpose that space to put more dialysis beds , etc. We will catch that up. I think 7% would be a little muted compared to perhaps what we have had in the past.

Kunal Randeria: When you mention that your growth should be higher than 7%, does it include all the beds or just the existing units?

Abhay Soi: New units will be a significantly higher ARPOB growth . New units are starting at a low base. So, if you combine the two, then your growth should be more. You had a revenue growth of 21% in the new units .

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Kunal Randeria: So, you are saying that maybe Lucknow and Nagpur whose ARPOBs are lower should grow at a much faster pace than your existing hospitals?

Abhay Soi: And Dwarka . It's the new hospital, right. Since your percentage growth, the absolute base is lower, they're starting on a lower ARPOB. If you add a 21% and 64% growth in Lucknow and Nagpur on revenue and EBITDA alone so you can imagine huge



amount of growth coming from that.

Kunal Randeria: I think Lucknow seems to be like a go to spot for a lot of your peers also. Do you think that so many beds coming up in that area, there is enough medical talent available both from the doctors as well as from the nursing perspective?

Abhay Soi: Actually, I don't know of any other hospital coming up there. And besides Medanta, which has been there for a while and a smaller Apollo facility which has again been there for 5-6 years, and Max Lucknow (erstwhile Sahara hospital) was also an existing facility and other than that we have only bought the land in Shaheed Path. But I haven't heard of any other hospitals really coming up there.

Yogesh Sareen: But nevertheless, we see an abundant supply of talent there. In fact, the doctors that we employ in Delhi, a lot of them are from SGPGIMS, Lucknow. So, to that extent, we haven't really –

Abhay Soi: Historically, Lucknow has been the place where we have got most doctors. Around 25-30% of our Delhi doctors are from Lucknow. Historically, Lucknow has had medical colleges and government medical hospitals, etc. and therefore a big base of doctors there.

Moderator: Next question is from Dheeresh Pathak from Whiteoak Capital Management.

Dheeresh Pathak: Sir, which quarter do we take the annual increments -- is it this quarter or -?

Abhay Soi: 1st of April.

Dheeresh Pathak: For the Mohali asset that we mentioned, would it be possible for you to share how much the real estate partner is investing in the land and building for the 250 beds?

Abhay Soi: I can't share that. He owns the land. We have given our specifications and our BOQs, but they don't share what they're spending. But the building alone will cost about 80% of the total project cost if we were to do it.

Moderator: Next question is from the line of Adrit Chaturvedi from Nomura.

Adrit Chaturvedi: So, my question is specifically regarding the inpatient business. So, you have reported this quarter around Rs. 1,600 crore of IP revenue. So, that's around 21% year-on-year growth. And also, the volumes have grown by roughly 17%. So, that translates to about like 3.5% growth in realization. Now, this 3.5% is sort of one of the lowest in the preceding quarters, but at the same time, the volumes are like highest in the preceding quarters. So, is this the sort of business profiling that we should come to expect as you grow and you expand that your realization per patient on the IP business is going to be around 3-4% but you are going to focus more on volumes?

Yogesh

Sareen: I think you are looking at the overall number, which includes the new facilities. You should compare the number on year-on-year basis for the existing hospitals separately because the new hospitals' ARPOB is obviously lower. To that extent,

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you will find a volume increase but not an increase in the revenues because of the ARPOB issue. As Abhay mentioned that as we change the mix of the business in those new hospitals, we also put in oncology there and have more talent there, the ARPOBs will start inching up slowly. So, there's a difference of Rs. 80,000 to Rs. 45,000. The new hospitals have ARPOB of Rs. 45,000 and our existing hospitals have an ARPOB of Rs. 80,000. We think that Rs. 45,000 will surely come up fast. So, it should be anything between Rs. 60,000-65,000 let's say in a year's time and that's how this will catch up. Then you will find that the volume increase and the overall revenue increase will start to have lesser differences.

Adrit Chaturvedi: So, the backdrop also on the levers for these realizations, right? So, like if we look at the sum of shares of oncology, cardiac sciences, orthopedics, neuroscience and renal, the share has come down this quarter on year-on-year basis and that's the first in a while and -?

Abhay Soi: Because you included the new hospitals, which has less than 2% oncology share.

Yogesh Sareen: Because the new hospitals share is very different from what we have in the existing hospitals.

Adrit Chaturvedi: When do you suspect like, as the expansion ramps up like we would have a sort of a resting rate where as you said ARPOB grows at roughly 9-10%. So, I guess the realization grows at 8%.

Abhay Soi: No, it doesn't work like that. Please understand what it is that we focus on? We focus on acquiring facilities, turning them around, and getting higher ROCEs over a period of time, but we look at it on an incremental basis, right? If we bought something with a lower ARPOB and we are demonstrating an increase in that ARPOB, it may or may not get to the same ARPOB levels as the Network average. For instance, Nagpur hospital will not have the same ARPOB, eventually, that the Delhi hospital does nor will it have the same EBITDA per bed. What you have to see is vis-à-vis what you

paid for it, what is the sort of return you are going to get. What we are seeing is that levers for doing all of that is going to be better clinical mix, doing more surgical work, higher oncology business, etc. Now, you may get to that mix of 25% or 24% oncology, okay, but you may not get to the same ARPOB as Delhi because the rates in Nagpur are cheaper. I am not even chasing the Delhi ARPOB for it, right?

Adrit Chaturvedi: Lastly, so a lot of your peers have mentioned that they're sort of translating a lot of the lower ALOS businesses to like day care. And right now, your IP business is roughly 80% of overall. So, you don't give any separate numbers there. So, do you have like any kind of qualitative color regarding what's the OPD margin profiling versus daycare and how it helps your margins, if you sort of translate a lot of those lower ALOS to daycare and non-census beds?

Abhay Soi: Look, the lower ALOS business has its own sort of tenets with it, because currently insurance companies don't cover procedures where you don't spend the night in the hospital. So, you need to spend the night for insurance companies to settle the claim.

Daycare may not be covered by the insurance companies. That itself is kind of a hindrance. The longer you stay in hospital, the hospital loses money since the surgical part is what makes money. So, if you can translate anything into a daycare, get a person in the morning, do the procedure and have him out by the evening, your yield is significantly higher. The question is, why is it that you can't do it, because for most of the procedures, you need to stay in the

evening night in the hospital to get to claim insurance.

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Adrit Chaturvedi: So, on the cash pay patients is there a targeted strategy there to like to move a lot of these lower ALOS to daycare or is it something that's not the focus right now in terms of your non-IP business?

Abhay Soi: So, we do that as hygiene, right? I'm not sure exactly what you are seeking. If you are able to articulate a little better, then perhaps I can answer your question.

Yogesh Sareen: For us, ARPOB is the one that we pursue. The daycare procedures will be all run-of-the-mill procedures. Our focus is to go up the value chain. So, we will be pursuing liver transplants, oncology, neuro, cardiology, cardiac surgeries, etc. I think those are the sort of procedures that we pursue. So, while obviously there'll be shift of some procedures that you do in one day stay to daycare. Yes, we can and we do, but that's not the focus of the hospital administration to really pursue.

Abhay Soi: It really depends on what your proportion is. For everybody, the ALOS is different because the clinical programs are different. Now, if you do a liver transplant, your ARPOB is higher, your EBITDA per bed is higher. But the fact of the matter is that the patient stays much longer. Now, would I replace that with a dengue patient, which has maybe a lower ALOS and higher percentage margin but lower sort of EBITDA per bed and ARPOB? No.

Moderator: Next question is from the line of Prashant Nair from Ambit Capital.

Prashant Nair: As your new projects now start coming onstream over the next few years, if you take some of your larger brownfield projects like say in Saket or Nanavati, when the new beds come online, do they have similar case mix, payor mix, ARPOB, etc., as the older base, or would we start at slightly different levels and then match up to how the existing hospital beds does?

Abhay Soi: So, you are going to have a different mix to start with, because first and foremost you try to ramp up occupancy over there in the new beds. And when you do that, then you are not kind of very strict on the payor mix as well as the clinical mix. People who may not be getting priority today because that's a lower end payor mix or a lower end clinical mix, you open your doors to it. So, your ARPOB from the incremental beds is lower. Having said that, the EBITDA per bed is higher. So, your profitability is higher because you get huge amount of operating leverage over there. Your main costs, associated costs, etc. are already being incurred by the existing hospital.

Prashant Nair: Second question is in these projects, would the investment in the beds follow a similar pattern for say a new Greenfield project where you front load everything, right? So, in brownfield beds, is there a period maybe after the first year, second year when you have to step up either investment in people or equipment for a ramp up, or are they all done at the beginning?

Abhay Soi: This is all done in the beginning. So, you don't have to step up anything. For brownfield, you are operating with the same set of people, the same majority of the equipment is already there.

Moderator: Next question is from the line of Tushar Manudhane from Motilal Oswal.

Tushar Manudhane: So, on international patient ARPOB if you could just help us understand like how much decline in the ARPOB that has happened because of these issues related to

visa's over there year-over-year or quarter-on-quarter?

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Abhay Soi : International business has got nothing to do with immigration business. The se are two separate streams altogether. The immigration business is where people come for visas, they have to get thems elves tested , etc. in our immigration departments. International business is the incoming business that you have. I mean, immigration business is if you have to relocate to Canada, then you h

ave to get your medical

tests and all done. It's outpatient business. The international medical tourism is people who are coming to India. So, these are two separate business streams.

Tushar Manudhane: So, as far as international business, just to understand what kind of ARPOB growth we have seen in the last two or three quarters?

Yogesh Sareen: Typically, the ARPOB in the international business is one and half times of the overall ARPOB. International business growth on ARPOB would be 7% to 10% if you see last year. But it also changes from quarter to quarter , depe nding on what is the procedure configuration that you are getting . So, it will be overall growth. These tariffs are linked to the self-pay tariff.

Tushar Manudhane: So, largely similar to that of the domestic patient in terms of growth, not in terms of value, but in terms of growth, we are largely similar in both international as well as domestic patient, is that the right way to look at?

Yogesh Sareen: Pricing is linked to the self-pay tariff, right? We charge a premium on that, but it's linked to self-pay.

Tushar Manudhane: How much is the OPEX at Dwarka like operational costs when you say break even within six to eight months, so if you could share how much OPEX and how much of that is already into the P&L? You said that we need to get the licenses and also you need to have doctors and nurses already onboard. So, does it mean that it is already in 1Q?

Yogesh Sareen: In this quarter , we have Rs. 6 crore of pre -launch costs. These are the cost s of the doctors and the staff that we had hired. Before you get the nursing home license, you are supposed to have people on your payroll . You need to demonstrate to the Delhi Health Department that we have so many people available for treating patients. So, this is that cost. We haven't had any revenue from the hospital in Q1. This cost will obviously build up . This is the cost which is for the quarter. But obviously if you take June month, the cost will be higher than the average . Rs. 6 crore is not for the full quarter. If I take July, you will have further increase in the cost because we are adding more people. So, we opened 94 beds at this point of time and I can't give you a P&L statement in ter ms of what the cost would be, but we do think that by the 8th to 9th month we should be in a breakeven state, which means that in Q4 , we should start to see breakeven numbers at EBITDA level for Dwarka hospital.

Tushar Manudhane: Trying to understand till that time how much drag will be there on the existing EBITDA of say 500 crore, so from that perspective just wanted to understand?

Yogesh Sareen: We said Q4 will breakeven, right? So, that means the drag will happen till Q4. So, in Q4 you will see that loss becoming kind of breakeven for Dwarka.

Tushar Manudhane: And how much further investment would be required in Lucknow for these 450 beds once the EC approval is there?

Yogesh Sareen : So, I think this will be around Rs. 700 crore plus for the beds.

Moderator: Next question is from the line of Amit Kadam from Canara Robeco Mutual Fund.

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Amit Kadam: Sir, my question is like I am just reading the Slide #10 and I am trying to refer the outpatient growth what we have seen, the fourth chart, where it shows that 4.5% has been the growth for the like -to-like the existing hospital. Whereas when I compare it for the Q1 FY'24 a year back, this run rate was like almost 13%, 14% growth rate. How do I read this particular thing 14% versus 4.5% right now?

Yogesh Sareen : OP consults have come down and Abhay alluded to it in his speech as well. We have immigration business, which is down by around 48%. We have three centers for immigration: two in Punjab and one in Delhi. Now, the footfalls have come down by 48% there. So, obviously that has an impact on these numbers because that's OP business. So, we do think that

this imp act will go on till Q3 . Q4 was the normal quarter

because we have this change in the visa regulations from 1st of January this year.

So, there will be some impact that you will have.

Abhay Soi: But the good part is this is not the OP business which leads to in-patient admissions, right? I mean, for immigration business, OP patients come do the consult and go out

the same day . So, they don't get translated. Normally reduction in OP business or in outpatient footfalls would be concerning because what that means is you are going to have lower inpatients going forward, but this is not the category of people which convert to inpatients. Also , last year , I think we opened Shalimar Bagh and so 3% of that was because of the capacity expansion , as Keshav pointed out . It was new capacity , which had opened in this quarter last year.

Amit Kadam: So, like the kind of capacities what we are going to see in next one odd year, what is the metrics or what number you would like to see because for a kind of a run rate in terms of volume what we want to see and partly driven by ARPOB, but partly driven by volume, this number should be high single digit, that's what could be a number for maybe a steady state growth?

Abhay Soi: Are you talking about the existing hospitals or the new hospitals or combined?

Amit Kadam: Existing, sir, because combined may not be correct reference but like when I just see that inpatient and outpatient both and you say that partly would be the international but then I am just like little confused that international patient coming off may not have a so much -

Abhay Soi: So, the material part is the inpatient.

Amit Kadam: Yes, so inpatient is 5.5 %. That's what is the second part of the question is then what is the run rate I need to see in this particular number to maintain our steady state growth rate as a tracking point?

Yogesh Sareen : So, I will just give you an example of what happened in Shalimar Bagh. This is the hospital where we added 43% bed capacity, and the moment you had the capacity available, the capacity got filled up, the inpatient volumes and the occupancies went up by 34 -35%. That means the outpatient and inpatient footfalls went up by 34 -35%. That's the same thing we expect here to the extent that if there is a brownfield expansion and let's say in a 300-bed hospital, we are adding 150 beds, and we plan to open all those 150 beds in one go , so to that extent you will find that the volume will expand and within 2 -3 quarters we should be reach normal occupancy levels as well –

Abhay Soi : If you want to do apples -to-apples , like you added Shalimar Bagh capacity last year same time, this year you will add Lucknow and Nagpur and then see what the overall growth is. That's why I asked you the specific question, because you are not doing

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apples -to-apples, right. If you are looking in existing hospitals last year, then you have to look at existing hospitals without the Shalimar Bagh expansion.

Amit Kadam: But sir, this is a volume count, right? So, it is irrespective of what hospitals I am trying to count because even I give that benefit of Shalimar Bagh also now part where in the existing part, and if it is 5.5.

Abhay Soi : I just look at it differently. So, let's say last year, you had 3,400 beds and then you add let's say 200 beds to it because you have done a brownfield . Now, you have moved to 3,600 beds. You are not comparing the 3,400 to the existing beds today, you are comparing the 3,600 beds to the existing today, but you had that jump of 200 beds last year. Likewise, if you take a jump of 900 beds this year, then you need to look at overall footfalls.

Yogesh Sareen : Let me convey it the other way around and explain this. So, basically, we have the highest occupancy among our peer group , which means that we cannot have more in-patients. So, what needs to co

me first? You need to have capacity to have more

in-patients. So, to that extent we are limited by the number of beds that we have.

And the example that I gave to you was Shalimar Bagh. The moment we added beds there , the volumes went up. So, as the capacities will get added, we do think the growth in the volume will materially change because we don't have beds right now.

Abhay Soi : If you don't come up with brownfield , if you don't come up with capacity addition, we're already operating at 77% occupancy. I don't have room for my beds ' occupancy to go up. It can only be marginal .

Amit Kadam: This explains that particular thing because maybe on the flip side, I was maybe trying to see is there a shortfall in terms of whatever the demand expectation is.

Abhay Soi : No. I mean, there's a shortfall of supply and that's why we are doing all these brownfields.

Yogesh Sareen : And that's where the Shalimar Bagh fitted .

Amit Kadam: And you mentioned that inpatient and outpatient doesn't have a high correlation with that particular thing?

Abhay Soi : No, it has a very high correlation, but the outpatient business that has reduced is the visa immigration business. So, let's say if you immigrate to Canada, you will come to our hospital and get the medical certificate from us because we were accredited with

the Canadian Embassy. Now that person was not going into inpatient, he was not getting admitted for a procedure. Since Canadian government is not issuing visas right now, so people are not coming for that medical certificate.

Amit Kadam: If you would have added that missing part, what run rate this number 4.5 would have been ?

Abhay Soi : My inpatient would be the same.

Amit Kadam: Not inpatient. Outpatient, then, sir. That 4.5 would have jumped up?

Abhay Soi : Yes. Then that would have gone up.

Moderator: Next question is from the line of Sumit Gupta from Centrum Broking.

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Sumit Gupta: So, just want to understand on the bed share mix. So, basically from the existing universe bed share has remained majorly between 29% to 30% for the institutional segment, however overall institutional bed share mix has come down to 27%. So, how do you see it going forward over the next two to three years, once all the facilities get integrated and they are in full stream?

Abhay Soi: There are two aspects. Like we said that Q1 is a weaker quarter, so you have less demand. Therefore, you don't put your foot on the accelerator and start driving out institutional business, you accept and accommodate because you want to fill your beds. Normally, you see this happening in Q2 and Q4 where you have reduction in institutional business. Having said that, like I mentioned, when you come up with new brownfield capacity, then occupancy is the criteria, not the caliber of clinical mix as well as the payor mix. So, at that stage, we will be taking more institutional. But having said that, even for institutional business, the EBITDA per bed which we're able to eke out is significantly higher than what it is for existing beds, and that has been our experience in Shalimar Bagh also. So, when we opened the new beds there, we saw the incremental ARPOB of the new beds come down because you were taking institutional as well as lower payor mix, etc. but the EBITDA per bed pertaining to those incremental beds was higher than the EBITDA per bed of the existing hospital.

Sumit Gupta: And for the international patients also, you see bed share mix going upwards or remaining at the same level and like how do you see this international business going forward?

Abhay Soi: The growth in that business has been outstripping growth in other segments, so percentage share is incrementally going up.

Yogesh Sareen : So, I think Q1 is an aberration there, but growth has come back as we speak and there was some temporary blip because of situation in Bangladesh, Yemen, Somalia,

India,  
etc. –

Abhay Soi: Nevertheless, as long as your growth in international business is more than growth in your other business, you will see the bed share only go up.

Moderator: Next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: We have added two new cities to our network in the past six months. Other than these two, our focus seems to be on growing in the existing market at least over the next four to five years. The question is beyond what we have already announced so far on the expansion front, is it fair to expect a relatively higher expansion announcement coming through towards the newer markets?

Abhay Soi: I think growth in our existing hospitals is not a focus, growth in our existing hospitals is a necessity, because we have run out of capacity. We are already operating at 77% and we need to increase bed strength in our existing hospitals. That's why you are seeing this incremental expansion happening and this is something we embarked on 2-3 years ago. So, they've been under construction and maybe between the next 8 to 15, 20 months you are going to see a lot of that capacity come through. Now, having said that, we still have our balance sheet. You know all the expansion is being incurred through internal accruals. So, we are intending to go to other markets. They are high ROCE markets, high opportunity markets, and frankly, you can imagine whatever multiple we have paid for these two hospitals, has already come down by 60-odd% simply because we have moved the EBITDA up in the first three months. So, we see opportunities in other markets as well and we are focusing on them. That's where the focus is going to be. This doesn't mean that we don't look

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at metros. The point is that in metros, it's going to be that much more difficult to be able to acquire hospitals or to acquire land because it's not available.

Alankar Garude: So, maybe a link to that one, how are you looking at this expansion from an operational standpoint, I mean any need to beef up the senior management team,

can senior management bandwidth be increasingly an issue going forward?

Abhay Soi: On the contrary, I think if you actually see the number of locations that we have added in the last 5-6 years, is essentially three; one is Dwarka, which is opened, one is Lucknow and one is Nagpur. So, from maybe 17 hospitals, we have gone to 20. Everybody wants career progression, right? We have got close to 30,000 people working for us, and if it's one unit head, he also wants, over 5-6 years, his area of influence to increase. He's handling one hospital, now he wants to handle two, the person who is handling two, wants to handle three or four. So, I think providing that growth, providing that segue for people's career path, is a necessity today. And essentially, a lot of our expansion is brownfield. Now if I increase 800 beds by another 400 beds in Saket, it's the same location, you are not really operating another facility. So, from 200 beds in Mohali, you go to 355 beds, you don't require management bandwidth for it. I mean, you are just doing more of the same effectively. And 85% of our growth coming through going forward is coming through brownfield. So, it's your existing capacity that you are increasing. From a leadership standpoint or down the line, with three locations getting added on top of 17, there is not much of a bandwidth issue. On the project side, of course we have augmented the teams, because when we embarked upon all of these expansions, we needed to fortify those teams. So, that we focused on. Other than that, not really. And of course, we have strengthened the teams, etc. but that happens through due course. I don't see bandwidth issues because frankly the expansion hasn't been that much in terms of locations.

Alankar Garude: So, maybe that question becomes more relevant

as we go beyond these 20

locations. So, the second one then, Abhay is we have been increasingly hearing about resistance from insurance companies on rising healthcare tariffs. Over a period of time, do you expect any change in the balance of power when it comes to negotiations between corporate hospital chains like us and the insurance companies?

Abhay Soi: Insurance companies by the end of the day piggyback on what your schedule of charges is, what your rate list is for cash paying patients. And frankly, I think you have the most democratic system over here because no hospital has a monopoly. Patients are choosing to go where they want basis the value proposition that they are able to get. It's not as if people are not kind of reducing their prices to get more patients. Everybody does that. None of the big chains are price makers, we are all price takers. Eventually, 90% of all healthcare is provided by smaller nursing homes, etc. Every year they know what the inflation is, they come up with a particular price, and when we seek price data, that is the basis on which we increase our rates. From our standpoint, do keep in mind that our EBITDA per bed is still 55% better than the second-best player in the industry. Our ARPOBs are maybe 20% better than the second-best player in the industry. So even if there is price negotiation, that sort of comes down for us. I frankly think that the insurance companies are a lot more fragmented.

Moderator: Next question is from the line of Shubham Dugad from Purnartha Investments.

Shubham Dugad: I want to know how many nursing seats you have across hospitals?

Abhay Soi: You mean nursing colleges?

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Shubham Dugad: You have some nursing facility in Lucknow, I think.

Abhay Soi: We have one nursing college in Lucknow and that's around 100 seats.

Shubham Dugad: And do you want to increase that or something like planning there?

Abhay Soi: No, we have tie-ups with many nursing colleges.

Shubham Dugad: How many beds being operationalized in Dwarka?

Yogesh Sareen: So, it's a 303-bedded hospital, and so far, 94 beds are operational.

Moderator: Ladies and gentlemen, I now hand the conference over to the management for closing comments.

Abhay Soi: So, thank you everyone for joining us for the Q1 FY25 Earnings Call. We look forward to catching up again next quarter. Thank you.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

## Call: Nov 2024

Max Healthcare Institute Limited  
Corp Office: 2nd Floor, Capital Cyberscape,  
Sector - 59, Golf Course Extension Road,  
Gurugram - 122102, Haryana  
T: +91 -124-620 7777

www.maxhealthcare.in Max Healthcare Institute Limited  
Regd. Office: 401, 4th Floor, Man Excellenza, S. V. Road,  
Vile Parle (West), Mumbai, Maharashtra - 400 056  
T: +91 -22 2610 0461/62  
E: secretarial@maxhealthcare.com, investors@maxhealthcare.com

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Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on November 7 , 2024, on financial results of the Company for the quarter and half year ended September 30, 2024.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/financials#earnings-call](http://www.maxhealthcare.in/financials#earnings-call).

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Arora  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited  
Q2 & H1 FY25 Earnings Conference Call  
November 07 , 2024

Moderator: Ladies and gentlemen , good day and welcome to the Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded.

I now hand the conference over to Mr. Anoop Pujari from CDR India. Thank you, and over to you, sir.

Anoop Pujari: Thank you. Good afternoon, everyone and thank you for joining us on Max Healthcare's Q2 and H1 FY25 Earnings Conference Call .

We have with us Mr. Abhay Soi – Chairman and Managing Director , Mr. Yogesh Sareen – Senior Director and Chief Financial Officer and Mr. Keshav Gupta – Senior Director , Growth , M&A and Business Planning of the Company.

We will begin the call with opening remarks from the management , following which we will have the forum open for an interactive question and answer session. Before we start , I would like to point out that some statements made in today's call may be forward looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks.

Abhay Soi : A very good afternoon to everyone and a warm welcome to Max Healthcare 's Q2 & H1FY25 earnings call.

This financial year was projected to be one of limited growth. Our brownfield capacity expansions were scheduled for completion at year -end and our greenfield hospital in Dwarka would still be in its early stages of operation. However , the first half of the year has seen stellar growth fuelled by recent acquisitions and their successful integration into our Network . By expanding into new , but proven geographies , we have supplemented our growth momentum.

Max Lucknow and Max Nagpur have been fully integrated into the Network and have significantly improved their performance. Combined revenue reached Rs.130 crore for these two hospitals in Q2 , representing a growth of 40% year -on-year and 32% quarter -on-quarter. Compared to the previous year , the combined EBITDA nearly doubled to Rs. 33 crore in Q2 with a margin of 26%.

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The newly operational Max Dwarka hospital reported a revenue of Rs. 33 crore and EBITDA loss of Rs. 18 crore , with occupancy of 41% and Average Revenue Per Occupied Bed ( ARPOB) of Rs. 80,000. This hospital has been performing very well and we believe that it will break even much before the end of the year and perhaps will be the fastest breakeven in our history.

We are excited about the recent acquisition of Jaypee Hospital , Noida. It is a marquee asset and will strengthen our presence in the National Capital Region. The enterprise value for this acquisition was Rs . 1,660 crore and we expect to integrate the hospital into the Network quicker than others . In the near term , our plan is to increase the operational bed capacity from 376 beds currently to 430 beds by March 2025 and upwards of 480 beds by December , 2025. We are simultaneously strengthening the medical programs in urology , medical oncology , gastroenterology and neurosciences through a combination of external hires , infrastructure enhancement and technology upgrades.

Now coming to Q2 performance highlights ; this is our 16th consecutive quarter of year-on-year growth.

Overall network revenue stood at Rs. 2,228 crore , registering a growth of 22% year -on-year and 10% quarter -on-quarter .

Network operating EBITDA was Rs. 566 crore , but more importantly , excluding the extraordinary items comprising of Rs. 18 crore of start-up losses at Max Dwarka and Rs. 7 crore of transaction costs for the Jaypee deal, the operating EBITDA stood at Rs. 591 crore , reflecting a growth of 19% year -on-year and 17% quarter -on-quarter. Profit after tax (PAT) stood at Rs. 349 crore compared to Rs. 338 crore in Q2 last year and Rs. 295 crore in the previous quarter. Yet again , excluding the extraordinary items as mentioned above , PAT stood at Rs. 383 crore , reflecting a growth of 13% year-on-year.

From here on , I will mention all numbers and percentages excluding the extraordinary items.

1) Our average occupancy for the Network increased to 81% from 77% in Q2 last year and 75% in the trailing quarter , while the occupied bed days (OBDs) grew by 18% year -on-year and 8% quarter -on-quarter .

2) ARPOB for the quarter stood at Rs. 76,100, growing by 2% year -on-year and remaining flat quarter -on-quarter . Like -for-like ARPOB grew by 7% for the



existing hospitals .

3) Network gross revenue was Rs. 2,194 crore compared to Rs. 1,827 crore in Q2 last year and Rs. 2,028 crore in the previous quarter. This reflects an increase of 20% year -on-year and 8% versus the trailing quarter.

4) The international patient revenue stood at Rs. 178 crore . This reflected a growth of 12% both year -on-year and quarter -on-quarter , despite contraction in patient footfalls from Bangladesh and Yemen due to political unrest .

5) Network operating EBITDA stood at Rs. 591 crore , reflecting a growth of 19% year-on-year and 17% quarter -on-quarter . Operating EBITDA margin stood at 28.2% for the quarter.

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6) Annualized EBITDA per bed stood at Rs. 75.5 lakhs , remaining flat year -on-year and increasing by 6% versus the trailing quarter. Like-for-like EBITDA per bed grew by 4% for the existing hospitals.

7) Profit after tax was Rs. 383 crore versus Rs. 338 crore in Q2 last year and Rs. 295 crore in the previous quarter.

8) Overall free cash flow from operations was Rs. 464 crore , of which Rs. 217 crore was deployed towards ongoing capacity expansion projects and upgradation of facilities at acquired hospitals. Consequently , net cash position for the Network stood at Rs. 313 crore at the end of September 2024 compared to Rs. 66 crore at the end of March , 2024.

9) Continuing our effort to support the local communities we treated approximately 39,600 outpatients and 1,300 inpatients from economically weaker sections of society entirely free of charge.

10) Both our strategic business units continued to report significant growth in the revenue and profitability.

- Max@Home reported a top line of Rs. 53 crore , reflecting a robust growth of 24% year -on-year. It offers 14 specialized service lines across 12 cities , with over 50% repeat transactions.

- Max Lab reported a gross revenue of Rs. 47 crore , reflecting a strong growth of 21% year -on-year. It provides services in 50 cities through its network of over 1,100 collection centres and active partners.

11) Now coming to the status of our expansion projects :

- For capacity augmentation at Max Lucknow , finishing work for additional 140 beds is in progress and will be completed by December 2024 , as communicated earlier . Further , environmental clearance (EC), consent to establish (CTE) and fire no-objection certificate ( NOC ), etc. have already been received for the additional 450 -bed tower at Max Lucknow . We are completing the drawings for the same and will be appointing the contractors shortly .

- For 140 beds at Nagpur , 12 beds have been added in October 2024 . For the balance beds on additional floors , applications for EC, etc. are in progress .

- For 268 beds at Nanavati in Phase 1, we have completed most of the structural work. The project continues to be on schedule and we expect completion by end of the financial year , as communicated previously .

- For 400 beds at Max Smart , this project is on track. We expect its completion within Q1 FY26 .

- For 155 beds at Mohali , most of the structural work is complete and we

expect its completion again by Q1 FY26 .

- For 501 beds at Sector 56 Gurgaon , the structural work is complete up to the plinth and above. We expect completion of the first phase of 300 beds by end of Q3 FY26.
- All of these are on schedule and we will see, over the next 12 to 15 months , significant ramp up in our capacity , most of which via Nanavati , Mohali , as well

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as Max Smart (Saket) should be coming through in the months of April-June 2025 .

- For 367 beds at Patparganj , post issuance of NOCs by fire departments and water departments etc. , we have already submitted our plans and we have received the environmental clearance . This project is largely on schedule as well.

- For 550 additional beds at Max Vikrant (Saket) , EC, CTE, etc. have already been received and barricading work is on. The forest approval over here is delayed due to Supreme Court proceedings in relation to tree felling involving DDA and the Lieutenant Governor of Delhi. For the past 6 months , they have not permitted anybody to remove any trees. But we think that this should get resolved fairly soon.

And finally moving on to the overview of the company performance for the first half of financial year 2025:

12) Network gross revenue stood at Rs. 4,222 crore , reflecting a growth of 19% year -on-year.

13) Overall Network operating EBITDA grew by 17% year -on-year to Rs. 1,089 crore , reflecting a margin of 27.2%. While EBITDA per bed stood at Rs. 72.8 lakhs per bed .

14) In the first half , we generated Rs. 722 crore of free cash flow from operations , after interest tax , working capital changes and routine CAPEX , of which Rs. 430 crore was deployed towards ongoing expansion projects.

With this , we open the floor for any Q &A.

Moderator: Thank you. The first question is from the line of Amey Chalke from JM Financial Institutional Securities Limited .

Amey Chalke : The first question I have on Nagpur and Lucknow units , both have done well during the quarter. I understand that Nagpur might be going through the seasonal peak , but both the units are now delivering 20% + margins. Is it possible for us to give some colour – what would be the potential margins for optimally utilized unit both at the Nagpur and the Lucknow unit? Specifically , Lucknow , can it

match the profitability of

our existing hospitals in Saket , etc.?

Abhay Soi: Firstly, we do not give forward looking statements . But intrinsically , margins and percentage margins are only one part . They are still operating at lower ARPO Bs compared to the rest of the Network and compared to the potential in the market over there as well . In the coming quarters , we will see improvement in that. There will be improvement towards better quality programs , that means higher -end surgical mix, more robotics , more complicated surgeries , as the new teams that we have hired come in and kick in, some of them being very recent hires . We will see all the benefits coming through. For Lucknow so far , we have a bunker over there but oncology has not been a big play . It accounts for only 7-8% of revenue as against 26-27% across the Network. As we get the bunker ready and the new LINAC machine comes in place , you will see this number move up . And so, you will automatically see an increase in ARPOB . Our preoccupation , as you are aware , is never with margins in percentage terms , it's with EBITDA per bed . And we think there is a significant amount of juice over there with respect to EBITDA per bed.

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Also, I just want to add that as we are going to get 140 more beds over there , which is almost 50% more capacity , our operating leverage on the incremental beds is much more , so the EBITDA per bed for incremental beds will be significantly more.

You will see the margins expand because of that as well.

Amey Chalke : So, do you see the Rs. 70 lakhs per bed kind of EBITDA to be achieved in Lucknow ?

Abhay Soi: Well, we will not give you a forward -looking statements on that.

Amey Chalke : The second question I have is on the Sake t Complex . So, here we are adding almost 1000 beds or maybe 800 to 900 beds o ver the next 2 to 3 years. So , should we consider this as a brownfield expansion? Because considering this much number of beds are getting added , I believe that everything like utility , doctors , everything should be added as well. So , the delta in the margins will not be significant. Is my understanding correct or you think that this is like a plain brownfield expansion which is happening in this locality?

Abhay Soi: No, it works like a plain brownfield expansion. When you look at u tilities , you are looking at capital cost. As far as the operating cost is concerned , there will not be a significant increase in operating cost. If you look at the P &L, about 15% of our costs are related to doctors. And out of the 33% of indirect cost s, 23% is non-medical personnel – most of it is being management cost. About 25 -30% of the operating cost may increase on incremental basis , but we will still have signifi ca nt amount of room . In each one of the brownfield s that we do , the utilities are the same. We have to increase our utilities , whether it is chillers for air conditioning or sewage treatment plants (STPs), etc . but it is all in the same complex. Yet you have these benefits of a brownfield . Also, please note that it is not 800 -900 beds over the next 3 to 4 years. You will have to look at it slightly differently. We will have 375 beds coming up in the month of April -May 2025 . We expect a very quick ramp up of th ese 375 beds , as we have witnessed in our other brownfield expansion s. The next set of 550 beds will come up after 3.5 years. Basically , we will have a run on these 375 beds for the next 3.5 years. Our belief is that these beds are going to get occupied in very quick time and we a re going to run out of capacity way before our new beds , which is another 550 beds in Vikrant , are going to come out.

Amey Chalke : And the third question if I can squeeze in , our profitability for the core business is still healthy at 20% + margin but it has dropped slightly year -on-year. If you

have to call out anything there.

Abhay Soi: Again , the point I would like to highlight is that you need to change your view o n the margins. When one starts doing high -end surgeries , or high-end payor mix, margin in percentage term come s down but it goes up in absolute terms. We would rather do a Rs. 10 lakh -surgery with a 20% margin than a Rs. 2 lakh -surgery with a 50% margin. If you look at medical patients , medical patients give you more margin in percentage terms but less in value terms. Whereas a surgical patient , say a liver transplant or a heart transpl ant patient , gives you less in percentage terms but more in value terms. But that's the patient you would want . You will need to see it in context of three aspects . First, you need to look at EBITDA per bed . Secondly , you have to compare it to the same time last year rather than on a quarter -on-quarter basis. Q2 is always a medical quarter and ARPOB s are lower in Q2 , but it is always also characterized by higher occupancy. And the third aspect is that if you are going to look at EBITDA per bed , you have to take out at least the newer acquisitions that we have done because be it Dwarka, which has started its operations only right now , or Lucknow and Nagpur , which have been acquired and are still in the building stage. So, they enjoy lesser EBITDA per bed. Other than that, if you look at the EBITDA per bed , it's fairly healthy.

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Yogesh Sareen: EBIT DA per bed has g rown by 7% YoY and this is despite the fact that our immigration business , which we mentioned last quarter also , has dropped by 44%. And it was very high EBITDA margin business for us . So, this has had some impact but we think that it will get normalize d from Q4 onwa rd because we had the drop in busines s from Q4 last year. So, we think that once that gets normalize d, you will see that even EBITDA per bed growth would be much better than what you see today , which is 6% YoY.

Ameya Chalke : So, going ahead for the existing business , the EBITDA per bed improvement would be largely inflationary or you think there is there would be improvement in terms of the productivity as well.

Yogesh Sareen: ARPOB is growing by 7% and if we really adjusted for the drop in immigration business, it would be 8-9% growth in APROB , of which only 2% is because of price . Rest all is efficiency in terms of process es, as well as payor mix and case mix. If this is what you are referring to as efficiency, then this will continue. And we do not think that we will see any big change in these numbers in terms of growth in ARPOB going forward.

Moderator: The next question is from the line of Neha Manpuria from Bank of America Securities .

Neha Manpuria: On the J aypee asset acquisition , I just wanted to get a sense on do you think if I look at the numbers E BITDA per b ed obviously can be better but in terms of occupancy ,

ARPOB, it seems pretty decent. Obviously, it can be higher. But what do you think is required here from either do we think we need more medical equipment, more doctor teams, does it need upgradation? How are you thinking about ramping up the Noida asset?

Abhay Soi: I think all of the above. It has been a company, which has been in liquidation for a significant period of time. So, most of the equipment is the end of life. As far as the clinical programs are concerned, again these have been operating with just (bare minimum) gravity. So, there is a huge amount of upside over there. Separately, it has been operating as a 376-bed hospital and we believe that we can move it to a 500-bed hospital very quickly, within the current structure, without making any significant changes. And this can be done in matter of a few quarters itself. So, if we ramp up the capacity, we move up the clinical programs, we move up ARPOB and payor mix, etc., we think we will be able

to get there soon. To us, given the location, the access and the quality of infrastructure, it is a marquee asset. So when you have this kind of advantage, it becomes easy. We have no doubt that this will play out the same way as Lucknow, Nagpur or even better.

Neha Manpuria: And are there any litigation etc. that we need to be worried about which needs resolving or which could be a liability in the future that we need to be mindful of?

Abhay Soi: None whatsoever. I mean there are no pending litigations. That's the benefit of buying through an NCLT process that if there are any creditors, if there are any claimants, etc., there is a time-bound manner in which they have to raise their hand and put in their claims, be it any department, even a government regulated department. So, the NCLAT process allows claimants to step up and state what their claim is. And after the expiry of time mentioned by NCLAT, those claims are not entertained. In this case, all the claims that were entertained at the NCLAT proceedings, all of them have been satisfied and more importantly, even post the proceedings there are no further claims that have been raised. When you get something through a High Court order, i.e., NCLAT is a High Court, it cannot be questioned.

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Neha Manpuria: So, it's a fairly clean, from a litigation perspective, there's no big overhang that we need to be worried about then?

Abhay Soi: Since we are getting it through the High Court, the title is clear. And there were no creditors which were dissatisfied creditors either. All the claims have been fulfilled.

Neha Manpuria: What about the two, the Bulandshahr and the Anoopshahr and Bulandshahr is commercial. So, would that be meaningful enough? Should we be considering that when we are looking at the math?

Abhay Soi: No, they're not meaningful enough. And we did not even attribute any value to it. We are trying to determine whether we should focus and spend CSR time and money on this or just wrap it up.

Neha Manpuria: And last on Jaypee, we have acquired part stake. The rest of it what is the process for that? By when do we see the full acquisition?

Abhay Soi: I think in the next few days we should have that wrapped up.

Yogesh Sareen: There is a call and put option, and they have exercised the put. We think that hopefully by next week we should be wrapping it up.

Moderator: The next question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets (India) Private Limited.

Damayanti Kerai: My question is on Dwarka, so Abhay in your opening remark you mentioned Dwarka will still be in ramp up phase for most of this FY25. So, just want to understand like on TPA and insurance empanelment I believe discussions are underway. So, when do you expect this 18% revenue share will likely move up to the Network level 30% when you empanel the required number of channels?

Yogesh Sareen: So, presently the TPA empanelment is underway. We are also waiting for the NABH certification as some of the TPAs have a requirement of NABH certification. We do think that by end of December 2024, we should have all of them in place. And January 2025 onward we think we should see more than 30% revenue coming from TPAs.

Damayanti Kerai: So, by December most of the contracts will be signed and then you will build up from them?

Abhay Soi: Yes.

Damayanti Kerai: And the ARPOB which you mentioned in your presentation Rs.80,000. That is because we understand most of the patients so far are with the cash channel patients. And maybe ARPOB will settle down more at your Network level for this hospital also?

Yogesh Sareen: Yes, there are two aspects . One is that we have very high outpatient consults and since we do not have TP A empanelment , the outpatient consults were happening in Max Dwarka but in-patient

t admission was happening in some other Max hospital.

These doctors did have some attachment in other Max hospitals and were admitting the patient s in the other hospital . For example , the neurosurgeon was attached with Max Gurgaon , so we admitted the patient there . Hopefully when we have everything in place , then the ARP OB will moderate a bit. But it will not be a very big change because it will be closer to the national average .

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Abhay Soi: But it's also a play between occupancy and ARPOB . Our belief is , like I said earlier, that this will probably be the fastest greenfield breakeven that we have witnessed .

We have seen very good traction and it is only a matter of time when all the TPAs signed up . We started end of July , so basically beginning of August. It does take 3 to 6 months to sign all the contracts . I mean a lot of it is in play already , and like Yogesh mentioned , by January 2025 we expect all of the empanelment to be done.

Dam ayanti Kerai: My next question is on your Network level revenue contribution from TPA and corporate channel. So , it's around I guess 38% in the first half. So , looking ahead I guess when we are adding a lot of capacities into the Network where I guess this payor mix etc . will take some time to build up. So , say in next 2 to 3 years , what kind of contribution we can assume from this particular channel , from 38% can we assume it can comfortably go up to 45% or so or how do you see this pie moving up?

Abhay Soi: So, we cannot give you any forward -looking guidance but we can tell you what our experience has been . When you open a new brownfield capacity , you essentially start taking in a larger amount of lower -end payor mix as well as lower -end clinical mix. One of the characteristics of a brownfield is that you are sort of finished with distilling your payor mix and you are doing very little or none of institutional business in the existing hospital . Like in Saket , in the main hospital , we have stopped doing it , in Nanavati , we have stopped doing it. The minute you put up additional brownfield , one of the things you are going to do is that you are going to switch on the tab for institutional over there , which is a lower -end payor mix. But because of operating leverage , even with the lower -end payor mix, which means with lower ARPOB, the EBITDA per bed is high. That's what we witnessed in Shalimar Bagh when we opened the brownfield . If you actually see what has worked for us in Nagpur – it was operating at maybe 55 % or 60% occupancy , and the first order of business after taking over operations was to sign up the institutional business , which is again lower ARPOB but provides higher occupancy. Now , the result is that ARPOB got somewhat muted , occupancy went up to 90 %+, but the EBITDA moved up by more than double . So, I think that is what we have seen is when you occupy incremental beds , the cost of which is only incremental , that even with a lower -end payor mix, it comes down to the bottom line and your margin improves. To what extent that I will not guide you.

Dam ayanti Kerai: And my last question is in some of your near-term upcoming capacities. So , Lucknow and then I guess Nanavati, those are coming in near term. So, for these facilities when do you start putting up a new set of doctors , medical team etc. or it happens very near to the commencement of these units.

Abhay Soi: We have new capacities coming up , firstly in Nanavati , in Max Smart ( Saket ) and Mohali as well as Lucknow is coming by December. But the first three are coming up at the same time in the month of March -May 2025 . These are brownfields. Unlike Dwarka , which is a greenfield , where you are going to get doctors , nurses , other staff, and they will be on your books for about two months even before you commence operations , and even once you commence

operations , it takes you time

to sign up TPAs and there's a ramp up required . So, there are some limitations in ramping up , which are not limitations of the operational level , but there are limitations of enabling that occupancy . However, in a brownfield you do not have any of these limitations . It is the same existing contracts , the same licenses , etc. And most of your doctors are the same. The more expensive doctors are the chair persons or heads of the clinical programs . It is not the junior consultants or resident doctors . In the first month itself , you get these junior doctors and absorb them. So, we do not see a major impact because of it, unlike Dwarka .

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Dam ayanti Kerai: So, any hiring in these Brownfield s as you said maybe the department heads or the really senior position but most of the team is already in place ? That's why like there won't be any meaningful delta in terms of cost?

Abhay Soi: That's right. Even on the management side , it will be t he same heads for facility , F&B, nursing , engineering , medical superintendent , etc. It's not a significant amount and particularly in Smart, where we are just expanding beds. The team there is already in place .

Moderator: Next question is from the line of Sumit Gupta from Centrum Broking Limited .

Sumit Gupta: Couple of questions , Firstly, can you tell what is the international patient's bed share and what kind of trend we can expect?

Yogesh Sareen: International bed share is 5.5% a nd it constitutes around 9% on the revenue side . But the bed share is 5.5% as the ARPO B is higher than the national average when it comes to international patients.

Sumit Gupta: Second is on the J aypee hospital. So , can you explain the ca se and the payor mix of this hospital in Noida?

Keshav Gupta : For Jaypee hospital patient case m ix, about 25% of revenue comes from institutional business and about 10 -14% of business month -on-month comes from international patients .

Sumit Gupta: And just want to understand on the broader aspect like how the competitive scenario in Noida just like so there are hospitals which are also going to expand in this geography , so how do you see that panning out? Will there be any impact that you see on the volume side?

Abhay Soi: I think there is still some time before the other hospitals come up . But Jaypee is a AAA+ location and you cannot get a location better than this in Noida or Greater Noida. It is an 18-acre complex , with 9.5 lakh square feet of built -up area. H onestly, we think none of us would ever build a hospital like that. We would not want to. The size and scale of it is something which is first class . So, from that standpoint the benefit is that you get something with that size , that scale , that location , that infrastructure , which nobody is going to operate. It's like you can make a new hotel in Jaipur , but there is only one Rambag h Palace . So Jaypee is that Ramba gh Palace in Noida . So, it's an easy sell , easy access , easy everything from that standpoint . It already has a start , even with a lot of limitations that it operated in, under liquidation for over 10 years , very little medical equipment , etc. yet you have seen halfway decent results coming out of this hospital. The reason for that is just fundamen tally structural. I mean it's like getting 7-star's treatment at maybe 4-star, 5-star prices or quality of service s.

Sumit Gupta: Lastly on the Dwarka hospital , just like in the presentation you have mentioned that the hospital is expected to open nearly 200 beds by end of FY25. We just want to understand on this , was like you're going to add like 200 b eds or what 1 41 beds are going to be operational capacity to get to 200 b eds?

Yogesh Sareen: No, it's a 303 -bedded hospital. So , that in a greenfield situation we don't open all the beds , because when you open

more beds that means there's a cost attached to it.

So, we open beds as are we able to fill it up. So, I think what we s aid is we have 141 beds open as of now and we think by March 2025 we will open 200 beds in total .

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Abhay Soi: What happens is that you have to get a license . If we want to get a license for 300 beds, then we need to staff all 300 beds. Then you have to prove it to the licensing authority that you have staffed all the 300 beds. But if you have occupancy over 150 beds , then you'll just say , give me a license for 150 beds because you will staff accordingly then. There's no point carrying staff of nurses and resident doctors for 300 beds , when your occupancy only 120 -150 beds because you are waiting for all the TPA empanelment, etc. to happen.

Moderator: The next question is from the line of Prashant Nair from Ambit Capital Pvt Ltd .

Prashant Nair: So just a follow up on the J aypee asset . When you acquired it , you had mentioned that there is scope to raise bed capacity meaningfully here. I think you also alluded to that now. Have you worked out over what period of time you will be looking to expand the bed capacity here? Also, second part , for the current hospital how much do you think you would need to invest in order to upgrade medical equipment , clinical capabilities etc. over the next couple of years?

Abhay Soi: On the first part of the question , we will look at expanding the capacity at the earliest. We will be limited by the actual time it takes to construct. So, right now we are looking at moving up to 480 odd beds from the current capacity and thereafter it should take us 2-2.5 years to build another 450 -500 beds , which we will embark on fairly soon because we see the current capacity being filled out in a matter of months and way

before the new capacity that we are looking at because it takes time to construct .

Yogesh Sareen: As far as the BME medical equipment upgrade is concerned , we already have some plans for that. There will be Rs.150- Rs.200 crore spend that we are planning to expand first to 480 beds and as Abhay mentioned , after that we will start the work to build additional beds.

Abhay Soi: But this includes the medical equipment over Rs.150 crore.

Yogesh Sareen: Yes.

Abhay Soi: Because you have to appreciate that almost all of the medical equipment is end of life. It requires replacement.

Moderator: Next question is from the line of Nitin Agarwal from D AM Capital.

Nitin Agarwal: We've had a pretty busy last few quarters in terms of these acquisitions that we've done. And with a bunch of new assets coming on stream next year as you mentioned , are we looking to take a pause on the inorganic growth part of plans or we still continue with inorganic growth activity for the next few quarters ?

Abhay Soi: We have a pipeline . If we keep getting opportunities to add marquee assets , we will continue to do that. We certainly have the balance sheet and we certainly have the bandwidth.

Nitin Agarwal: Secondly on Jaypee , so we mentioned a Rs.1,660 crore if I remember as the EV for the business acquisition. This includes the entire pay-out for the entire equity and whatever debt we've taken on for the asset.

Abhay Soi: That's right. That's the enterprise value.

Nitin Agarwal: And beyond this we mentioned there is going to be some registration and transfer charges. So, how much of that will really account to?

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Abhay Soi: It has already been paid.

Yogesh Sareen: Rs. 62 crore , which is over and above. Yamuna Expressway Industrial Development Authority (YEIDA) has a rule where even if there's a change in shareholding , there's an amount we have to pay for the transfer , which is Rs. 62 crore that we paid.

Abhay Soi: That's already paid out , no further transfer charges.

Nitin Agarwal: So, I mean how should we think about the assets? So , w

e paid about Rs1,700+

including the upgrades and all about Rs.1,800 odd crore for buying out an asset for about 500 bed asset. So, is that the way we should think about it? Apart from obviously it's a running asset that gives us immediate EBITDA versus a greenfield investment that we would have done. But in your mind how should you evaluate this versus assuming you were to make a same greenfield investment assuming there was an opportunity like this in Noida to versus acquiring this asset for the price if you paid?

Abhay Soi: If you look at it from a return on capital employed ( ROCE ) standpoint . Essentially what you are doing is you are looking at 500 -bed asset , which is renovated with new equipment etc. and capacity to add another 1 ,500 to 1 ,700 beds on the same land . So, very clearly if you want to look at it over a 4 to 5-year plan, you look at 500 beds plus another 450 beds that you would be able to add since you have already got the land for it. And then you look at what is the EBITDA and what sort of ROCE you are able to get. If you're able to generate more than 20-25% ROCE over that period of time, then it is a good acquisition. That's how we look at it. It so happens that this one is the easiest yet perhaps because it is a marquee asset with that location. But no matter how you cut it , no matter how you look at it , even if you look at it sans any expansion , a 500-bed hospital in this location should give you about Rs. 300 odd crore of EBITDA.

Yogesh Sareen: So, let me just tell you how we look at it . Basically , we have a hospital in Vaishali , which is a 387-bed hospital and Jaypee is going to be 480 beds . Now, 480 beds with all the money that we are going to spend is going to be roughly around Rs. 1,900 crore . And the hospital in Vaishali does roughly around Rs.380 crore of EBITDA every year and it is a 387-bed hospital.

Abhay Soi: So, this is actually a better location , better access , bigger , better infrastructure than Vaishali.

Yogesh Sareen: More beds and on 18 acres , while Vaishali hospital is on a 2.5-acre piece of land. So, you can obviously can see the kind of numbers that we are expecting from Jaypee, given that we have a benchmark in the same market already giving that level of EBITDA and margins .

Abhay Soi: Our normal bed size average is about 1 ,100 square feet per bed. This is close to 2,000 square feet per bed , already constructed.

Nitin Agarwal: Abhay when you are looking at , obviously you are looking at consistently evaluating a pipeline of various sort of assets. So , the kind of transaction that you have gotten

in the Jaypee Hospital, is more of a one-off transaction or there are similar such transactions which are there for taking in the market?

Abhay Soi: Well, if Jaypee is a one-off then I guess Sahara was a two-off and then Nagpur is a three-off. So, I think we keep doing these 'one-offs', which we like to do every once in a while. I think Sahara Lucknow was very similar, and so was Nagpur.

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Nitin Agarwal: No, I got to appreciate that. And lastly on Dwarka you talked, it's going to be our fastest probably breakeven for a greenfield. So, what is the time horizon looking at for the breakeven here?

Abhay Soi: We are looking at it before the end of this financial year.

Moderator: Next question is from the line of Andrey Purushottam from Cogito Advisors.

Andrey Purushottam: My question is really a follow up from one of the earlier questions, in terms of the levers that you have to increase EBITDA per bed. If you could just expand a little bit on that and also answer two specific questions. One is, has your proportion of your low margin CGHS kind of business actually diminished over the last year or two?

And how do you intend scaling up your international business per se and also perhaps to compensate for the possible reduction in volumes from

places like

Bangladesh and how important is Bangladesh to your total share of patients of the international business per se?

Abhay Soi: I'll take the last one first. Bangladesh used to be less than 1% of our total business.

So, it really has not impacted us that much. And we have been able to compensate that through other markets and all our initiatives everywhere else. So, Bangladesh is not affecting us. What did affect us in the past was Afghanistan which was 12% of our international business. But we have not had that business for many quarters now. So, you don't see the impact of that. Hopefully as and when it comes through, there will be a positive impact because of it. Having said that, levers for EBITDA per bed, let's start from the top with revenue lever. And perhaps most of it are revenue levers, which include adding clinical teams, adding occupancy, increasing occupancy, increasing more beds. I mean every revenue lever that you can think of will play into the bottom line and increase your EBITDA per bed. Because I think as far as costs are concerned, we are pretty much in line with where we want to be, any benefits over there will be only incremental.

Institutional business has diminished marginally over the last few quarters. But the ARPOB of the institutional business has almost doubled. The rates are becoming closer to our cash rates. More importantly, in a lot of our metro locations where we already have very high capacity, we have already distilled away from that. So, if you look at Mumbai, we do not do any institutional business. But once we start the new capacity, we will start the institutional business, and we just occupy the beds with it till such time that we have idle capacity. Once that kind of runs out, we start distilling it again, and go back to cash business. At Saket, in the main facility, we do not do any institutional business. At some stage, once we come up with new capacity at a Max Smart, we will start taking on more institutional patients. But again, over a period of time, we will distill it. But then, in the third phase 3-4 years later, when Vikrant comes again, we will start doing taking institutional business. So, you keep playing that game. But more importantly, as you start running out of capacity, you start distilling payors and within those payor groups also you start distilling the clinical mix.

Andrey Purushottam: And going back to the international business, you said that you have a 5.5% bed share and a 9% revenue share. Now if you're looking to increase this share, what was the approach that you're following? Is it largely walk-in business or are you making a fair amount of proactive efforts and other geographies to attract patients there?

Abhay Soi: We have set up offices over last 2-3 years in about 16 countries. We have our own offices as well as partner offices. And that is the reason that we have seen an outsized sort of growth in our international business. It may be 9-10% of our revenue, but our revenue has also been growing significantly over the last 4-5 years. This

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business has been growing a 20% CAGR for us. It sort of outstrips are overall growth and the reason is the investments initiatives that we have taken in terms of setting up these direct-to-fly offices overseas.

Andrey Purushottam: One hears about lots of problems that people face with the NHS in England and similarly in places like Canada etc. So, given the fact that we can provide outstanding medical care at a fraction of the cost, do you see more traction coming from developed countries like UK and Canada etc. where it has a lot of bureaucratic



problems about getting service in their own countries?

Abhay Soi: It's a question of whether you want bad healthcare for free or you want to pay for good and upfront healthcare and you want to come to India for it . We are

setting up

some beachhead operations in the UK as we speak , because of the same reasons that you mentioned. But we are not expecting some floodgates to open . It is more a long-term thing that we need to have some sort of presence over there to at least get people of Indian diaspora to India to be able to treat them . But we do not see people coming in drones because there is waiting time in NHS . There is waiting time but ultimately it is free.

Moderator: Next question is from the line of Madhav Marda from Fidelity International .

Madhav Marda: I just had one question , on the Dwarka unit it's quite encouraging that a greenfield asset can breakeven in a very short period of time. Could you just give some colour in terms of what guides us because I guess a general understanding for everyone has been that greenfield assets take let's say 2-2.5 years to reach EBITDA breakeven ? But since we can do it at a very good pace , just what are things that are working in our favour , just for my understanding ?

Abhay Soi: I do not think it is 2-2.5 years. Typically , it should take about 18 months. That's par for the course. But having said that , Dwarka is again an excellent location. The first factor is always a location. Second is the infrastructure. The third is how much underserved that market is. And fourth , I tend to believe that our brands are NCR centric . We already have around 14 facilities over there , which makes a big difference . There is a very strong brand recall . And our belief is that we would have already broken even if we had all the institutional tie -ups, NABH certification, TPA empanelment, etc . Typically , you need 6 months of data to get NABH. Once you get NABH, you get higher rates from institutions. So , a lot of it is also process related.

But otherwise, for us again , once the location is good , once the infrastructure is good , access is good , then it should not take much time to ramp up. Also, the team of doctors we have been able to access and get very good teams of doctors . But that's the Max advantage , particularly in NCR, that we are able to attract quality talent.

Moderator: Next question is from the line of Amit Thawani , an individual investor .

Amit Thawani : I particularly enjoyed the response to the question whether Jaypee was a one off. But jokes apart , I also enjoyed how we've broken up the business into existing and new units. So, at 81% kind of capacity utilization, we are little bit maybe constrained for volume growth in the existing units. A little bit of a forward -looking statement , what would the growth that you estimate in the existing units in volume and ARPOB going forward? I mean just a ballpark even a range is fine and even I'm looking for more of a longer -term number rather than a short -term number.

Abhay Soi: Firstly , 81% is a seasonal number , because of seasonal flu , etc. Normally , Q2 has higher occupancy. You will see in Q3 perhaps the occupancies will get a little more muted than that and that's traditionally been the case. Whatever is the occupancy in Q2, Q3 is lower , Q1 is lower and then Q4 again you ramp up. So, yes, the other

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characteristic of having a capacity constraint is that you have a higher move up in your ARPOB simply because when you have the capacity constraint , the lower -end clinical mix and payor mix are perhaps not given priority. And when you are unable to give the mix priority , you post those surgeries and procedures at a later date , they are likely to evaporate. When you have capacity , all of that lower -end payor and clinical mix comes through. But like I said , it's also characterized by higher EBITDA per bed. So , it works out better to have higher occupancy than a ramp up in ARPOB . The fact of the matter is that we have been operating at these kind of capacity levels for the last couple of years with the 1 -2 percenta

ge points here or there as far as

occupancy is concerned , and that is always because at the margin there will be some elasticity but it is diminishing returns. And we need to be cognizant of the fact that we have capacity constraints. We are cognizant of the fact that we have capacity constraints and therefore, we embarked on this massive journey of brownfield expansion . Some people will ask me the question like you are asking me that look you're at full capacity , where do you go from here? So , the answer to that is that's what the brownfields are for and some other people tend to ask that look there's so much capacity coming in the country , why are you putting up more capacity? Well , this is the reason we are putting it up. Because we have run out of capacity.

Amit Thawani : But any quantitative guidance. Would you want to venture in that?

Abhay Soi: No, we stay away from providing any forward -looking guidance.

Moderator: Next question is from the line of Alankar Garude from Kotak Institutional Equities.

Alankar Garude: Just one question for Yogesh. With Dwarka coming online , what will be the annual lease payment at the Network level?

Yogesh Sareen: I think the overall number for the year would be around Rs. 95 crore . So, specifically for Dwarka, this number is around Rs. 28-29 crore .

Alankar Garude: And the number for Dwarka when you say Rs. 95 crore , would be for nine months , right?

Yogesh Sareen: No, I'm saying Rs. 28 crore is Dwarka, total is Rs. 95 crore for the overall Network.

Abhay Soi: And Rs. 28 crore is for the year.

Yogesh Sarin: I'm talking all annual numbers.

Alankar Garude: So, basically for FY25 the number would be Rs. 7-8 crore lower. Got your point.

Moderator: Thank you. As there are no further questions , I would now like to hand the conference over to the management for the closing comments.

Abhay Soi: Thank you everyone for joining us on the Q2 results. We look forward to interacting with you again next quarter. Well appreciated , thank you.

## Call: Feb 2025

Max Healthcare Institute Limited  
Corp Office: 2nd Floor, Capital Cyberscape,  
Sector - 59, Golf Course Extension Road,  
Gurugram - 122102, Haryana  
T: +91 -124-620 7777

[www.maxhealthcare.in](http://www.maxhealthcare.in) Max Healthcare Institute Limited  
Regd. Office: 401, 4th Floor, Man Excellenza, S. V. Road,  
Vile Parle (West), Mumbai, Maharashtra - 400 056  
T: +91 -22 2610 0461/62  
E: [secretarial@maxhealthcare.com](mailto:secretarial@maxhealthcare.com), [investors@maxhealthcare.com](mailto:investors@maxhealthcare.com)

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Listing Department ,  
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Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
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Sub.: Transcript of Earnings Call held on January 31 , 202 5  
Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on January 31 , 2025, on financial results of the Company for the quarter and nine months ended December 31 , 202 4.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/financials#earnings-call](http://www.maxhealthcare.in/financials#earnings-call).

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Arora  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited  
Q3 FY25 Earnings Conference Call Transcript  
January 31 , 2025

Moderator : Ladies and gentlemen , good morning, and welcome to the Max Healthcare Institute Limited 's earnings conference call.

Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you, and over to you , sir.

Suraj Digawalekar : Thank you, Ryan . Good morning , everyone, and thank you for joining us on Max Healthcare Q3 and 9 M FY '2 5 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director, Mr. Yogesh Sareen – Senior Director and Chief Financial Officer and Mr. Keshav Gupta – Senior Director, Growth, M&A and Business Planning.

We will begin the call with opening remarks from the management , following which we will have the forum open for an interactive Q&A session.

Before we begin , I would like to point out that some statements made in today's call may be looking forward in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks . Thank you, and over to you Abhay .

Abhay Soi: A very good morning to everyone and warm welcome to Max Healthcare's Q3 FY25 Earnings Call .

We are considerably pleased by our performance this quarter with over 30% year -on-year growth across parameters such as revenue, EBITDA and occupied bed days , notably supplemented by the growth momentum of our recent acquisitions. We are happy to share th at we achieved EBITDA breakeven in December 2024 , within a record period of 6 months from the launch of our greenfield hospital in Dwarka . This hospital reported a revenue of Rs. 59 crore s and an EBITDA loss of Rs. 5 crore s in Q3.

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Max Lucknow demonstrated year -on-year growth of 58% in revenue and 94% in EBITDA, while Max Na gpur reported year -on-year growth of 22% in revenue and 50% in EBITDA in the third quarter.

Additionally, Jaypee Healthcare Limited became a wholly -owned subsidiary of the Company during the quarter. Jaypee Noida is presently being integrated into our Network and reported a gross revenue of Rs . 112 crore s with an operating EBITDA of Rs. 23 crore s at a margin of 21% in the third quarter.

We have now been able to demonstrate remarkable operating efficiencies across all formats of inorganic growth, namely greenfields, acquisitions and brownfields. This fortifies our confidence for the upcoming phase of accelerated growth driven by significant brownfield additions wi thin the next six months.

Consequently, we continue to strategically pursue inorganic opportunities. We are expanding our footprint in the Mumbai Metropolitan Region through a foray into the attractive Thane micro-market , given its rapid urban growth and proximity to Mumbai. Our Board has accorded its approval to enter into an asset -light 'built-to-suit' agreement for a 500 -bed hospital at a prime location in Thane , to be set up by the partner as per our specifications on a built -up area of appr oximately 6 lakh square feet. The hospital is expected to be commissioned in 2028. This marks our third asset -light transaction , designed to drive future growth and maximize potential return on capital employed (ROCE) with minimal investment.

The Board has also provided i ts approval for enhancing the capacity of our upcoming asset -light 'built-to-suit' hospital in Mohali (Zirakpur ) to 400 beds from 250 beds planned previously.

Now, coming to the Q3 performance highlights, which is our 17th consecutive quarter of year -on-year growth.

1) Our average occupancy for the Network stood at 75% versus 73% in Q3 last year and 79% in the trailing quarter, while the occupied bed days (OBDs) grew by 36% year -on-year and 8% quarter -on-quarter.

2) Average revenue per occupied bed (ARPOB) for the quarter stood at Rs. 75,900, remaining relatively flat both year -on-year and quarter -on-quarter. Like-for-like ARPO B for Existing Units, however , grew by 7% year-on-year and 3% quarter -on-quarter .

3) Network gross revenue was Rs. 2,381 crore s compared to Rs. 1,779 crore s in Q3 last year and Rs. 2,228 crore s in the previous quarter. This reflects an increase of 34% year-on-year and 7% versus the trailing quarter.

New Units reported a gross revenue of Rs. 323 crore s. While Existing Units registered a year -on-year growth of 16% in revenue, driven by 8% growth in OBD s and 7% growth in ARPO B.

4) The international patient revenue stood at Rs. 201 crore s, registering a growth of 28% year -on-year and 8% quarter -on-quarter , despite contraction in patient footfalls from Bangladesh and Yemen due to political unrest.

5) Network operating EBITDA stood at Rs. 622 crore s, reflecting a growth of 32% year-on-year and 10% quarter -on-quarter. This includes Rs . 60 crore EBITDA contribution from New Units.

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6) Network operating EBITDA margins stood at 27.3% for the quarter. Existing Units improved their EBITDA margin by 70 basis points to 28.6%.

7) Annualized EBITDA per bed for the network stood at Rs. 73 lakhs . Like -for-like EBITDA per bed for Existing Units stood at Rs . 82.6 lakhs, reflecting a growth of 9% year-on-year.

8) Profit after tax before exceptional item was Rs . 390 crore s versus Rs.338 crore s in Q3 last year and Rs.349 crore in the previous quar ter, reflecting a growth of 15% year -on-year. The exceptional item of Rs . 74 crore was towards charges paid to Yamuna Expressway Industrial Development Authority (YEIDA) for securing permission for a change in shareholding of Jaypee Healthcare Limited prior to acquisition.

9) Overall

free cash flow from operations was Rs. 303 crore s. During the quarter, Rs. 362 crore was deployed towards ongoing capacity expansion projects and upgradation of facil ities at acquired hospitals , while Rs. 146 crore was distributed as dividend and Rs. 1 ,716 crore (net of cash at Jaypee Healthcare Limited ) was used for Jaypee acquisition. Consequently, ne t debt for the Network stood at Rs . 1,608 crore s at the end of December 2024.

10) Continuing our effort to support the local communities, we treated approximately 37,500 outpatients and 1 ,300 inpatients from economically weaker sections of society entirely free of charge (worth Rs . 52 crore s at hospital tariff ).

11) Both our strategic business units continued to report significant growth in revenue and profitability.

- Max@Home reported a top line of Rs . 55 crore s, reflecting a robust growth of 24% year-on-year. It now offers 15 specialized service lines across 14 cities , with over 50% repeat transactions.

- Max Lab reported a gross revenue of Rs. 41 crore s, reflecting a strong growth of 22% year -on-year. It provides services in 48 cities through its network of more than 1,200 col lection centres and active partners.

12) Now, coming to the status of our expansion projects :

- 128 beds at Max Lucknow – 64 beds have been commissioned in January 2025 , and the balance 64 beds will be added in February 2025. Further, we are awaiting in -principal approval for the existing 13th to 17th floors for hospital use, which will add another 140 beds almost immediately.

- 127 beds at Max Na gpur – 12 beds have been added in October 2024. For the balance beds on additional floors, we are expecting the environmental clearance (EC) to come by March 2025. Project completion should take another 24 months thereafter.

- 268 beds at Nanavati in Phase 1 – Interior fit -out work s are in progress

currently . The project continues to be on schedule and we expect completion within the next 3 to 4 months.

- 400 beds at Max Smart (Saket Complex ) – Majority of the structural work is complete. The project is on track and we expect its completion within Q1 FY '26.

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- 155 beds at Mohali – Interior work is in progress and we expect its completion again by Q1 FY '26.

- 500 beds at Sector -56 Gurgaon – Structural work is in progress. We expect completion of the first phase of 300 beds by the end of Q3 FY '26. All of these are on schedule, and we will see a significant ramp up in our capacity over the next 12 months , a large part of which is coming through Nanavati , Mohali and Max Smart within 6 months.
- Thereafter, 367 beds at Patparganj – Post receipt of environmental clearance (EC), tendering work is in progress currently . This project is largely on schedule.

- 550 beds at Max Vikrant (Saket Complex) – Forest approval is delayed due to Supreme Court proceedings in relation to tree felling involving DD A and the Lieutenant Governor of Delhi for the past 6 months. They have not permitted anybody to remove any trees in Delhi , but we think that this should get fairly resolved soon. All other statutory approvals are in place.

- 400 beds at Zirakpur, Mohali – No objection certificate (NOC) from fire department has been received. Project is expected to be completed within 30 months.

And finally, moving on to the overview of company 's performance for nine months ending December 2024 :

13) Network gross revenues stood at Rs . 6,636 crore s, reflecting a growth of 25% year-on-year. New Units contributed Rs . 585 crore s to the gross revenue.

14) Overall network operating EBITDA grew by 20% year-on-year to Rs. 1,687 crore s, reflecting a margin of 26.6% , while EBITDA per bed stood at Rs. 71.5 lakhs. Existing Units reported an EBITDA margin of 27.7% and EBITDA per bed of Rs . 78.5 lakh s.

15) Max Lucknow demonstrated year -on-year

revenue growth of 41% in revenue and 67% in EBITDA , while Max Nagpur reported a year -on-year growth of 26% in revenue and 118% in EBITDA within 9 months of acquisition.

16) Since becoming operational in July, Max Dwarka clocked a revenue of Rs . 92 crore s and EBITDA loss of Rs . 29 crore s. This greenfield hospital achieved EBITDA breakeven in December 2024, a record 6 months from its launch as highlighted previously.

17) During the 9 months, we generated Rs. 1,025 crore s of free cash flow from operations after interest, tax, working capital changes and routine CAPEX . Rs. 793 crore was deployed towards ongoing expansion projects and upgradation of facilities at acquired hospitals , Rs. 146 crore was distributed as dividend and Rs . 1,716 crore was used for Jaypee acquisition.

With this, we open the floor for any Q&A.

Moderator: Thank you. Ladies and gentlemen, we will now begin the Q&A session. The first question comes from the line of Amey Chalke from JM Financial Institutional Securities Limited . Please go ahead.

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Amey Chalke: Hello, thank you so much and congrats to the management on good set of numbers. So, the first question I have is on the revenue from the existing units. It seems that quarter -on-quarter from Q2 to Q3 the revenue has remained largely flat despite being the weak quarter of Quarter 3 . Is it possible to give an explanation on the front where we have seen the improvement or performance improvement during this quarter for

the existing units?

Yogesh Sareen : Amey , as you would know , Q3 is typically a weak quarter because you have festivals in this quarter , and if you see the history , you will find that typically the revenue s come down by 2 -3% and EBITDA also drops by 3-4% in this quarter. Despite the history, this time it is flat and in fact the overall EBITDA has improved over Q2.

That way , the performance has been much better and it is mainly because of the fact that in the Diwali month, typically you will see the occupancy drop to around 65 -70% range , but this time we had very healthy occupancy even during Diwali , and that is what has made all the difference in this quarter.

Abhay Soi : You must look at it on a year -on-year basis because of seasonality. Every quarter must be seen on a year -on-year basis rather than sequentially quarter -on-quarter .

Amey Chalke: So, no, because I was expecting a drop this quarter considering it is a seasonally weak quarter. That's why the question was.

Yogesh Sareen : Occupancy levels in the Diwali month made a lot of difference. Typically , the occupancies do drop, but this time it did not happen. It is also because of the fact that Diwali was at the very end of the month, which also helped in a way. If it is in the middle of the month, then you will have more impact.

Amey Chalke: Sure, and second question I have, we were expecting a price increase for some of the insurance schemes. So , have that been taken place or do you expect in the next one year any price increase to happen on the insurance side?

Abhay Soi: We are not expecting a price increase on the insurance side , we were looking at a price increase on the institutional side. We are still expecting that as it is long overdue. Our belief is that it should come within a month or two. But let's see what happens on that.

On the insurance side, it happens on a rolling basis. That means whichever insurance contracts come offline every two years, you get the new rates over there.

So, that's happening as course of hygiene.

Amey Chalke: Sure. And if you see our therapy mix, it is continuing to improve. The oncology mix has also improved from Quarter 3 of last year to this year. Considering the new bed additions which have happened would have a little bit lower oncology proportion , I believe for existing hospital, the mix would have moved up sharply. So , where should

we see the optimized mix for the oncology revenues going ahead?

Abhay Soi: In the new hospitals , it should be increasing considerably in fact. In Lucknow , for example , the new bunker is yet to get ready and is expected to be operational by July. Thereafter, oncology revenue should increase . What happens is that without radiation oncology , even the other programs suffer in as far as oncology is concerned. So , you will see a major uptick over there.

We are looking at a new bunker coming on stream even in Dwarka. The facility is without a bunker right now, so there is no radiation oncology there. It needs to kick

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in over there as well. Besides that , even for Jaypee and others, we are going to see further increase in oncology.

You are absolutely right. The current pie although takes into account improvement or increase in oncology business , but that number is kind of subdued or pulled down by the new hospitals, new acquisitions where we are looking at perhaps significant increase in oncology business.

Amey Chalke: Right. Just to add more , like, which would be the hospital which would have highest oncology mix and what would be that number so that we would know the upper limit for the oncology ?

Yogesh Sareen : We can't give you hospital -wise numbers, but the very fact that we have 25% + as percentage share of oncology in the overall setup, obviously there will be hospitals that will be in the range of 29 -30% as well . The new hospitals will be lower . For example, Dwarka would be around 12% . These are the kind of ranges that we have.

Abhay Soi : Even Lucknow will be in single digits again ?

Yogesh Sareen : Yes, Lucknow will also be around 10 -11%, so it is half the network average.

Amey Chalke: Just last question . The PHF profitability has pulled down a bit this quarter. Any reason for the same?

Yogesh Sareen : No, you will have to see it in the overall aspect. If you read the notes given , some of them have donated money to the other trusts . Also, we have revised the fee structure for two of the PHFs, which means that there is more upstreaming happening in the first column , that is we are getting more fees into MH IL, especially from Balaji and DDF.

Amey Chalke: Thank you so much. I will join back the queue .

Moderator: Thank you. The next question comes from the line of Sumit Gupta from Centrum

Broking Limited . Please go ahead.

Sumit Gupta: Sir, on the Lucknow performance specifically, I just want to understand how the market is panning out and what kind of trend s to expect in the overall profitability going forward?

Abhay Soi: We are not going to give any forward -looking guidance, but the market is panning out very well. Like we said, we are launching another 140 beds. The reason we are doing that is because the occupancy requires that. 64 beds have already been commissioned in the month of January. These are new beds and another 64 beds will be commissioned in February, that is in the current month. Thereafter , we are looking forward to another 140 beds , which can immediately come online post approval.

We have the requirement for these beds and, that is what we are anticipating.

Therefore , we are getting these beds online.

We are also looking at the new bunker to come on stream over there. So , with the new bunker, the radiation oncology business, the day care business, etc. increases.

And we are seeing very good traction with clinicians, bringing in new clinicians and so on. So , I think , Lucknow has a significant amount of meat over there.

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Sumit Gupta: Okay . So, what are the competitive scenarios there on the addition al intensity being panning out like it is going up. What is the trend of the int ensity?

Keshav Gupta : It's the same as earlier, so there is an Apollo and there is a M edanta. The same hospitals are still there.

Abhay Soi: And there

are some other smaller nursing homes and hospitals.

Sumit Gupta: Okay. And s ir, of the Nagpur facility , the Q-o-Q, there is a decline in the overall profitability . What has led to that decline in the growth ?

Abhay Soi: Quarter -on-quarter , like we said, is a seasonal business . You must look at it on year - on-year basis .

Sumit Gupta: Okay , understood. Thank you , sir.

Moderator: Thank you. The next question comes from the line of Damayanti Kerai from HSBC Securities and Capital Markets (India) Private Limited . Please go ahead.

Damayanti Kerai: Hi, thank you for the opportunity. My first question is on your debt side, so Rs.1,600 crore net d ebt after payment to the Jaypee etc. So , now in view of multiple projects coming in like coming quarters or years, how should we look at the funding side ? And then maybe you can just like give your upper limit for net debt to EBITDA , like what will be your upper tolerance level there?

Abhay Soi: So, our upper limit is 2.5 x net de bt-to-EBITDA , but we are far from that. The new ones that we have announced such as Mohali (Zirakpur ) as well as Thane, they are both asset -light models. The developer incurs the cost and we are essentially leasing these spaces from them thereafter. Dwarka expansion again is on a similar line, which is an asset -light model. As you are aware that Dwarka itself is an asset -light model.

Now other than that , we are looking at Rs. 500-600 crore over the next 3 -4 months of CAPEX towards the brownfields and then thereafter. But yes , our overall cap is 2.5x net debt-to-EBITDA. This would include not only current CAPEX but also any further inorganic growth or whatever else we may do , including on and off-balance sheet de bt.

Yogesh Sareen : We are at 0.65 x at this point in time after the Jaypee acquisition.

Damayanti Kerai: Okay, so comfortable headroom .

Abhay Soi: Yes, and we are very conservative on the de bt side.

Damayanti Kerai: Okay , and just if you can remind us like what kind of cash is currently generated from the existing business.

Abhay Soi: This quarter we generated Rs. 303 crore s of free cash flows. Thi s is after tax, working capital increase, maintenance CAPEX and any interest.

Damayanti Kerai: So, on an average, we can assume like Rs.1,200 crore of cash per year is getting generated against your all growth needs.

Abhay Soi: Hopefully more , given the year-on-year growth that we have.

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Damayanti Kerai: Okay . My second question is on price increase which you mentioned on the institutional channels. Can you bit elaborate, are you expecting something to come up on the CGHS rate or what is it regarding ?

Abhay Soi: That's right . We are expecting some revisions to come up in CGHS rates that will also impact the other PSU business. The gap earlier used to be 44% betw een our cash rates and our institutional , and now it has come down to 36%. The delta has



reduced. On top of that, we are expecting better rates coming through on CGHS now. But we have been expecting this for some time. Our hope is that this comes in this quarter.

Damayanti Kerai: Have you heard any announcement from the Government? You mentioned next two to three months, right? You are hoping to hear something?

Abhay Soi: We will just have to wait until it comes through.

Damayanti Kerai: Okay, great. And then my last question will be on institutional bed share, which is around 30% for the quarter. So, how should we look at, because earlier you mentioned your endeavour is to bring it down, right? But eventually, I understand you will take up scheme patients even when you have beds to ramp up. But any like guidance or any target in your mind for this part of the business?

Abhay Soi: See, what happens is that as we add new hospitals or new capacities, your institutional business is going to go up with that. Our endeavour is not

to reduce

institutional business. Our endeavour is to accommodate growth in our preferred channels of business. But if we can do both, we do not have a problem doing it. You distill it when you have a capacity constraint. If you are able to add more and more capacity to it, then it is not a problem because even the institutional business is contributing towards your fixed costs. The idea is not to, unless it was a loss-making business, then you wouldn't be doing it in the first place. The first choice is to create more capacity to accommodate that business as well as any growth in your preferred channel business. And wherever you cannot do that, you start distilling that business.

Damayanti Kerai: Okay. So, you have the flexibility to play around with this mix, right, to optimize the asset utilization?

Abhay Soi: We have done that previously. You see plenty of facilities where we have brought it down to zero, but you will see that some facilities that we open up now, let's say Mumbai, we will start institutional business because we are coming up with new capacity. The first idea is to fill the beds, because this business contributes towards fixed costs.

You have seen it in the Shalimar Bagh brown field as well. But having said that, even with the lower rates, your EBITDA per bed is higher simply because you have got operating leverage when you are adding brown field capacity on existing hospitals. So, if you take an example of Nagpur, it used to operate at 55-60% occupancy. But we have been able to ramp up the occupancy by taking in institutional business. The hospital never used to do so before our acquisition. And the immediate fallout of that is that it all percolates down to your EBITDA. So, you are better off taking where you have idle capacity. So, if we can create idle capacity, then it is great. You take institutional business, it percolates down to EBITDA and you have higher EBITDA per bed from the incremental institutional business.

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Damayanti Kerai: Okay, thank you. That's very clear.

Moderator: Thank you. The next question comes from the line of Prashant Nair from Ambit Capital. Please go ahead.

Prashant Nair: So, my first question was on the Mumbai project and the additional beds that you intend to add in Mohali. As both are built-to-suit projects, what would your investment outlay be for these two assets?

Abhay Soi: Essentially, it's going to be medical equipment, which will be let's say about Rs. 150-200 crore. But that only happens at the end when it's constructed – the last 3-6 months or whatever.

Abhay Soi: Yes, it's about Rs. 30 lakh per bed, but it's all back-ended. Medical equipment comes after the entire project is almost complete.

Prashant Nair: And Mumbai you intend to operationalize in fiscal '28, is that right?

Abhay Soi: End of '28, that is right. So, it takes about 3-3.5 years to build, including permissions.

Prashant Nair: And Yogesh, for your business, would say 65% cash conversion be kind of a reasonable number to work with, assumption to work with, or can this change, say, at any point over the next few years?

Yogesh Sareen: Yes, I think 65% is the right number. I mean, we only hope that the tax numbers will come down going forward in terms of cash outflow. So, this should be okay.

Prashant Nair: All right, great. That's it from me. Thank you.

Moderator: Thank you. The next question comes from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane: Thanks for the opportunity. Sir, first one on the hospitals which are coming up over the, say, next six months. So, with respect to those, what kind of operational cost addition can be factored for FY '26?

Abhay Soi: Is this for the brown fields ?

Tushar Manudhane : Yes sir, the ones which are coming up in say Q1 FY '26, Mohali , Max

Smart , Saket ?

Abhay Soi: Marginal operational cost increase because these are all brownfields. We already are incurring all the major costs in the existing hospitals.

Tushar Manudhane : So, secondly, with respect to this build-to-suit where the investment is lower, but accordingly, I mean, subsequently as and when let us say the steady -state occupancy of say 55 -60% as and when that happens, what kind of margins, you know, we are sort of assuming this build -to-suit because there would be rent share or there could be some revenue share with the partner unlike, you know, own green field, brownfield expansion ? So, you know, basically trying to understand what kind of margin dilution happens?

Abhay Soi: You have to look at what we get vis-à-vis what we invest. So , you have to look at it on a ROCE basis . Now, if 80% of the capital cost , which is land and building in this case , is incurred by the partner and you are locking it at 8 -9% yield , and we enjoy a

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35% ROCE in a stable state on the whole hospital , so you can imagine what it does to our 20% contribution. Our ROCE goes beyond 100% effectively . And most importantly, you are insulated against any cost and time overruns because that is to the partner's account and not to your account.

Tushar Manudhane : Understood. And what kind of ARPOBs , you know, do you think this location can drive it compared to say metros or tier 1s where currently MHIL is at Rs.75,000-76,000? What sort of assumption for this location ?

Abhay Soi: I don't think there is any difference between Thane and what any of the Mumbai hospitals will be operating. And you have some listed players like Jupiter , which are operating in Thane.

Rs. 75-80,000 is the ARPOB at present. Three years later, hopefully, it will be more. In Mohali, it should be no different from our existing hospital in Mohali, which is at an ARPOB of about Rs. 55,000 at this point of time. Three years later, it should be more, as you are aware that ARPOB is growing by 7 -8% every year.

Tushar Manudhane : Got it , sir. And just one more on Lucknow . Given the current occupancy and current profitability at least at the existing centres , so the growth will be more driven only by beds now, given that the operational efficiency is largely in place or you think there is still some more efficiency which can drive the profitability while beds will drive the volume growth ?

Abhay Soi: So, when you have higher occupancy, the cost also defrays over a larger number of beds. So, automatically, you have operational efficiency coming through. Other than that, like we mentioned, that there is no bunker over there right now, which should come on stream shortly and improve day care, radiation, etc.

It is coming on stream in July. And thereafter, you will have a higher amount of ARPOB emanating from that particular facility. There are other clinical programs' equipment which are online and coming onstream. We have ordered the equipment , but it takes time for it to come. All of this will contribute to higher ARPOB, better operating efficiencies , etc.

Yogesh Sareen: Also, some of the doctors have joined during Q3. So, there will be a full quarter impact going forward.

Tushar Manudhane : Understood. And just one more, if I may, on Nagpur, given the kind of size of revenue, in fact, it's relatively smaller in the overall scheme of things, but just to understand here, in terms of, you know, the current, the occupancy, which is sort of dragging down and subsequently having an impact on profitability, even if I leave aside its seasonal impact, still, just to understand, you know, the potential to add another 115 beds here.

Abhay Soi: The occupancy on a year -on-year basis has increased.

Yogesh Sareen: Q2 was 91% , and this quarter is 79%. They had higher incidence of vector borne diseases last quarter. This does happen

seasonally.

Abhay Soi: 79% is by no means a low occupancy in a slow season. 79% is midnight occupancy and like we said, it's going to take us 24 months to build the additional beds. We have no doubt that even if you take a 5% increase in occupancy over the next two years, we are going to be fully occupied out. And do keep in mind that you create

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infrastructure for your peaks, not for your troughs. So, our peak occupancy was 91% last quarter.

Tushar Manudhane : No, I meant to say that at 79% occupancy, we are at say Rs. 54 crore revenue, Rs.11 crore EBITDA, which is like roughly 20%. So, from a profitability point of view, we are more or less there and then given the kind of profitability here, we intend to still add 150 beds is what I am trying to ask. Or is the EBITDA margin still possible to get better at this site?

Abhay Soi: No, of course, it is possible to get higher EBITDA margins. Also, if you say occupancy is not an issue, then as far as the higher occupancy is concerned, you also get huge amount of operating leverage, because of the brownfield. EBITDA per bed is significantly higher then. You also have to visualize this basis the entire ROCE that we are shooting for, which is 20 -25% within 4 years.

Yogesh Sareen: The other important aspect is that we still have to get to the respectable ROCE there. Our target is 20 -25% range, and we are at 10 -11% range as of now. So, we have to add those additional beds to get to that ROCE level. And that's how it was all planned when we acquired the hospital.

Abhay Soi: Keep in mind that this is only the 9th month, right?

Tushar Manudhane : Got it. Thanks. That's it from my side.

Moderator: Thank you. The next question comes from the line of Rishi Mody from Marcellus Investment Managers. Please go ahead.

Rishi Mody: Yes, so Abhay, just wanted to understand this partner healthcare facility (PHF). So, we have used the profitability there to fund the new Vikrant and the other which is again a PHF facility. So, wanted to understand like do we just control the cash flow, or do we have ownership over that free cash flow that these Balaji Society and all of these guys create? Like can you give out that money as dividends to the shareholders or you are not allowed to do that?

Yogesh Sareen : No, there is no dividend that can be declared for any society. But one society can contribute to the other society if their objectives are same. For example, Balaji Society and Modi Society objectives are the same. So, if they have ample cash, they can donate to other societies to use it for their construction and other purposes and that's what they did. But what can be given as dividend is basically you have to upstream to the main company. But the moment you mainstream it to the main company, then you get only 75% of it because you have to pay tax on it.

So, our endeavour is always to, if you need cash in a society, you move it from one society to the other society. But you can upstream it, you have the ability to upstream.

Abhay Soi : We have the ability. Your question is can it be dividend? Yes, it can be upstreamed and then given as dividend, but of course we will be paying a 25% tax.

Rishi Mody: Okay, got it. And second, you mentioned on the PHF the fee has been revised.

What's the change like today, I am guessing we are getting around 25% to 28% of that revenue out as a fee which gets recognized in our Max Healthcare revenue books. So, just what's the change?

Yogesh Sareen : Every two years the fees get revised for each of these PHFs. There is a method to the madness. We review the cost and their cash flow, and based on that the fees are

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revised. For Balaji, annual impact will be around Rs. 25 crores for the fee revision.

Now I don't know the percentage, but that's the absolute amount of PBT, which

will

be upstreamed into MHIL through the MSAs that we have.

Rishi Mody: Okay, so we will get an extra Rs. 25 crores going forward from Balaji.

Yogesh Sareen : Yes.

Rishi Mody: Okay. Second on the Thane Hospital. So, I read that the lease is only for five years, the initial lease, and then you have two renewals versus when I look at the Zirakpur one, we have around a 20-year lease with some 20-year renewal. So, just wanted to understand like is there a risk of that property post, you know, we getting it being taken away by the developer and being given to someone else for a higher rental? Or like why did we get into only a five-year lease? And secondly, the two renewals post those five years, how many years are they for and who has the right for that renewal?

Abhay Soi: There are two things. It's not five years or three years. It's actually 15 years.

Keshav Gupta : 5 + 5 + 5, so 15 years lease.

Abhay Soi: Second is that after 12 months of operations, we have a call option on it. We can acquire it at any point of time. We can acquire it and sell it down to a REIT. We can acquire it and nominate somebody else or whatever else it is.

Yogesh Sareen: Otherwise, you have to understand the commercial logic of it, because if we do a 15-year lease in that state, then we have to pay higher stamp duty. Since we have the option to buy, we said we will take a call later and then renew it. It's at our option.

Rishi Mody: Okay, so like five plus five plus five. Okay. All right, just fine. Second, the last

question from my end is the IRDAI regulations for the guidelines that have come through.

Abhay Soi: Sorry, I just want to go back to that point. It's 15 years with a call option at our choice.

Rishi Mody: Yes, to buy the property within 12 months. I heard that. Yes.

Abhay Soi: Yes. So, don't look at it as only 15 years. We can always, if the 15 years is not extended, buy it or we can actually nominate somebody else to buy it on the same lease.

Rishi Mody: And what would be that amount if you exercise the call option, like how much do you have to end up paying to the developer?

Abhay Soi: It's at cost.

Keshav Gupta : It's at cost and a very nominal yield on the real money spent on the project.

Rishi Mody: Okay, all right . So, 1 crore per bed would, effectively Rs.1, Rs.1.5 crore per bed would become the cost if you end up acquiring it.

Keshav Gupta : Yes, that's the broad range. The real cost for putting a project up is about Rs.1.9-2 crore per bed for a greenfield.

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Rishi Mody: Okay . Last on the IRDAI guidance that's come out today or yesterday where they decided to cap the increment on senior citizen 's insurance policy price hikes and they have asked these insurance companies to get back together and negotiate with hospitals and bring it closer to the PM-JAY rates. So , just wanted your view on , firstly , how much of our revenue within the insurance pool comes from senior citizen . And secondly, do you see any rate negotiation impact from this instruction?

Abhay Soi: No, there is no rate negotiation impact. In fact, we see a benefit from it because the number of people who may be going out of the insurance net because their premium goes up, there is a step jump in that premium , now won't go out of the insurance net because of this . It's more palatable . As far as premium is concerned, there is no rate negotiation for different age groups. It's not cut that way. I mean, the cost is cost right, whether it's for a senior citizen or a junior citizen from our standpoint .

Keshav Gupta : And they are giving a price hike of 10% per year. What we negotiate with insurance companies is around 10 -12% every two years. If the underlying logic was to continue through and through , the reflective increase that the hospital seeks and most of the hospitals seek is lesser than what they

are allowing anyway.

Abhay Soi: The price change on insurance every two years is about 12 -13% at best. So, that comes to a medical inflation of about 6% per year effectively, at best, 5.5 -6% a year.

Rishi Mody: Right, yes, I got that. So, that 's the secure part just like these guys collectively, the insurance companies collectively come in and bargaining, does that impact?

Abhay Soi: That would be a Competition Commission issue, right? I don't think you can cartelise . But no, if that was to happen, then they should be kind of getting together and negotiating overall current rates? Why would they take only one segment? Like Keshav said, if our increase is 6% per year and they are being permitted 10% per year, so where is the problem?

Rishi Mody: Okay. All right. Understood. Yes, that's it from my end. Thank you for taking my questions.

Moderator: Thank you. The next question comes from the line of Andrey Purushottam from Cogito Advisors. Please go ahead.

Andrey Purushottam : Hello , this is Andrey. I had one question regarding the payor mix. How much of the payor mix is in your control? And to the extent that is in your control, what are the steps that you are taking next year or so to hopefully in favour of higher profitability?

Abhay Soi: No, it is not . It's the burden of disease, right? Effectively, we start adopting and align ourselves with what the market demand is. You move towards higher technology and perhaps more robotics and higher end programs. That is what drives the clinical mix.

Andrey Purushottam : No, I am talking about the payor mix.

Abhay Soi: Sorry, as far as the payor mix is concerned, we have seen there is an increase of 28% in international business. So, this is because we have engaged deeper with various geographies that we get patients from as far as the domestic patients are concerned. Upcountry is 40% of our NCR business . All of that is growing at a faster pace and so we want to have deeper engagements with upcountry. And other activities that we do to engage with the communities that should enhance the preferred channels for us.

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Andrey Purushottam : Okay, thank you.

Moderator: Thank you. The next question comes from the line of Vaibhav Saboo from Nippon AIF. Please go ahead.

Vaibhav Saboo: Yes, thanks for giving me the opportunity and congrats for a good set of numbers. It was asked previously also, but just want to understand, you know, that on a consolidated basis, for example, our EBITDA margin is somewhere around, like, 20-25 percent, but you told to assume for mature hospitals, it would be around 29-30%. I just wanted to understand for the asset-light model, while I understand completely that ROCE would be very much attractive, I just wanted to understand what would be a similar, what would be the EBITDA number for the mature business for asset-light models. It could be around 20-25%, just wanted to understand that range.

Abhay Soi: This is post Ind AS, so the rental line comes below EBITDA.

Vaibhav Saboo: But for pre Ind AS basis, what would be the EBITDA margin?

Yogesh Sareen: It should depend on what the maturity of the hospital is, but I think it should be lesser by 5-6%.

Vaibhav Saboo: That's it from my end. Thanks, and all the best.

Moderator: Thank you. The next question comes from the line of Neha Manpuria from Bank of America. Please go ahead.

Neha Manpuria: Yes, thanks for taking my question. Abhay, just one clarification on the Mumbai expansion. I think once the new tower is commissioned, we are planning to phase out some of the older ones and rebuild that. So, would the net addition still be the 268 beds that we have talked about or should I also adjust the beds that will be going off for some time till we get it back? How should that phasing happen?

Abhay Soi: No, so it is going to be a net reduction there. When

we start Phase 2, which should

be almost immediately, you will have a reduction of 160 beds.

Keshav Gupta: Yes, we are currently adding 268 beds. After that is commissioned, we have to tear down 160 beds. The next we will be adding 271 beds. That's a phase of about 1.5-2 years period. So, net reduction eventually will be 111 beds in Phase 2.

Yogesh Sareen: So, for the time being, you should take 268 beds. There is no reduction there. We have to take a call on reduction. Once we do the Phase 2, then the reduction will happen.

Neha Manpuria: So, the Phase 2 is not happening immediately?

Abhay Soi: Yes, at that stage you will bring down about 160 beds.

Neha Manpuria: Okay, and this is over the next two years. This entire process will get completed in the next two years.

Abhay Soi: That's right. But the beds which are being pulled down are more ward style. One of the things we have been able to do through this expansion is right-size the type of beds. Because the type of beds that we have a demand for and occupancy for are single beds, deluxe beds and so on. Whilst the present facility, one of the reasons that the occupancy levels typically have been lower at Nanavati compared to the rest

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of the network, is because a lot of the beds were old style, ward style, nightingale ward beds. Those are the beds that are getting pulled down.

So, we are right sizing the beds. So, the net impact shouldn't be that much. You know, you are getting what is required where we have almost a 90% occupancy on single beds, ICUs, etc. Those will continue. While the ward style, nightingale wards, etc., come down, which in any case weren't in demand.

Neha Manpuria: Okay. So, there won't necessarily be a financial impact from it because of the right sizing beds, but then how should I think about the improvement because I think one of the areas that we wanted to improve was also the margin of the Nanavati Hospital. So, does that start happening once we have commissioned all of the when you completed Phase 2 fully?

Abhay Soi: Absolutely, and it happens on two counts, because one is your number of beds increases. Let's say if your operational beds are 280, and even if you take an increase of about 140-150 beds on top of that, net increase, you are looking at 50-60% more capacity addition. So, you will have the same cost structure defrayed over more number of beds and even the higher cost structure defrayed over the more beds and the right size beds. So, you will get the operating leverage, and your margins will certainly go up.

Neha Manpuria: Understood. Okay, that's helpful. My second question is just an extension of the previous question. I think there has been some chatter among insurance companies about getting together and trying to negotiate pricing, even using IRDAI as one of the agencies that does that. I know you mentioned Competition Commission, but do you see that as a risk when we are thinking about pricing with insurance? Could that 10% over two years be much lower as we think about the next, let's say, three to four years depending on how this process progresses?

Abhay Soi: Not at all. We haven't seen any approaches, any discussions, anything other than

that. IRDAI is actually the regulator for insurance companies. So, I don't think insurance companies can get into the umbrella and have IRDAI negotiate that or get into the discussion. Because I think it is far beyond their mandate, right? Other than a conversation like this, where you are mentioning a rumour, and that also far and few that have come our way, there has never been any approach on that. Secondly, and most importantly, the moot point is that medical inflation is 6-7%. That's the increase we get every year. You just get it every two years, so it shows up at a 12% increase every two years. But the fact is, it's for two years. O

n an annual basis, it's

still 5-6%. Any increment that you see is on real growth, which is new technology, robotics coming in. Of course, in terms of percentage margins, you get less margins. But in terms of value, you get more.

Because do understand, eventually all the innovation that happens in the healthcare sector eventually has to pass through the doors of hospitals. The same hospitals 30 years back who used to do conventional surgeries are now doing robotics. The same hospitals which used to do CABGs at best are doing transplants now. We used to have general surgery surgeons working on oncology. Now you have organ specific oncology. You have got radiation, chemo, etc. You have got other robotics doing those surgeries. When you do all of that you go up the value chain as well. So, there is real growth.

Yogesh Sareen: Neha, Also, if the insurance companies get organized, on the other side, the hospitals can also get organized, right? There can also be a collective bargaining on the hospital side. That's easier to do in fact. Hospitals are far lesser in number if you take organized chains.

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Neha Manpuria: Yes, the demand supply equation, I guess, is in your favour.

Abhay Soi: Not only that, also the density of beds, right? We are dominant players in Delhi NCR.

And similarly, some of our peers are in their own markets. If you are kind of dissipated across the country, then it's a different matter. Today, we have 15 facilities in Delhi NCR, which are twice the number of facilities of the next three listed players put together. So, literally three players put together and the next 20 players are not even the same size. Your bargaining power is slightly different in that case. But it's not even a question of bargaining power. I think it's a question of what is right and what isn't. 5-6% growth in pricing every two years is not something that you can dispute, right? How much lower do you want to be is the question.

Neha Manpuria: Yes, fair enough. Thanks so much, Abhay.

Moderator: Thank you. We take the next question from the line of Kunal Dhamesha from Macquarie. Please go ahead.

Kunal Dhamesha: Hi, thank you for the opportunity. The first one on the D warka. While we have done a very good job of breaking even, based on our reported number, if I look at the indirect cost in the D warka Hospital for the 140 beds, it seems to be roughly around Rs.150-Rs.160 crore a year, assuming that our direct costs are similar in line with the network average. So, my question is given that we are going to add almost around 1,400-1,500 beds next year, how should we think about the indirect cost related to that? I know there are some brownfields etc as well, but again these brownfields are separate towers versus the current towers and all that, so there might still be higher kind of indirect cost there. So, let's say, all in all these 1,500 beds, what is the indirect cost that we should assume whether it's Rs.1,000 crore or Rs.800 crore or how should we think about that?

Abhay Soi: No, I think the way you need to think about is that indirect cost is not linear. Right?

Kunal Dhamesha: It would come primarily in the first year?

Abhay Soi: It's already there. Whatever indirect cost you have been incurring up till breakeven, we have incurred the indirect cost. It is that indirect cost on 140 beds that we have broken even on. Thereafter, every bed that you add, your indirect cost is not going to be linear. It's only the direct cost which is going to be linear. Now indirect cost will be just some nurses and some resident doctors etc., which is a marginal cost.

What is going to happen is that every bed that you add, the revenue from that bed will give you a lot more leverage onto your EBITDA. A lot more which means almost your entire contribution margin. Let's say if you are operating at a

60% contribution

margin, 50% of your top line, or that means almost your entire contribution margin will flow to your EBITDA, because your indirect cost is not linear. And that is the basic principle of any brownfield or opening any hospital. And frankly, over the next few quarters, you are going to see that being demonstrated.

Keshav Gupta : And those beds are not coming in new tower. The existing structure already has 300 beds .

Abhay Soi: It already has the beds. The beds need to be fitted out.

Keshav Gupta : We only opened 140 beds.

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Kunal Dhamesha: Correct. So , once you operationalize another 160 beds, there would still be some incremental cost related to it, right?

Abhay Soi: If you have a hospital which has 8 floors, and you have operationalized 4 floors. Now you need to operationalize another 4 floors. All your costs of your clinicians, management, utilities or common areas, all the support functions, kitchens, everything is already incurred.

When you open up another floor, what do you do? You get nurses and resident doctors , which don't cost us much . Effectively, you are looking at maybe 8 -9% of your revenues attributed to this cost. If your contribution is 60%, you take out 8% from that. So, 52% of your top line is going straight to your bottom line.

Kunal Dhamesha: When you ramp up.

Abhay Soi: Obviously when you ramp up, right? So, every incremental bed will give you more and more.

Yogesh Sareen: Post the breakeven stage.

Abhay Soi: Post the breakeven stage. And that's not only a hospital , any other venture works like that . That's basic economics.

Kunal Dhamesha: Sure. Another one on the institutional mix and the payor mix or bed share and the payor mix, if I look at the nine-month s payor revenue from the institutional patients have grown at roughly around 34% year-on-year, and the bed share has only grown at around 20 %. So, my view is that there is a decent amount of kind of rebasing of the pricing of some sort was happened , and I also see there was some order passed in February '24 from the Delhi Government advising the CGHS rate. So , what are we looking forward in the next one or two months incrementally?

Abhay Soi: Delhi government has nothing to do with CGHS rates. CGHS rates are under Central Government .

Yogesh Sareen: Yes, and Kunal, Abhay already mentioned that the gap between the cash, the self - pay and the institutional segment has come down in terms of ARPO B, right? Earlier, it used to be 44%, now it's only 36%. That means there is an 8% improvement in terms of the ARPO B. Basically, we are doing more oncology . So, as a result, you find that the bed growth is not there, but there is a revenue growth. That obviously means that there is an ARPOB growth which is happening, and it's not because of rates. There is nothing happening on the rates. It's only because of the change in the mix of the patients.

Kunal Dhamesha: Sure, I have some document . Probably I will share it in more detail on that. I have just one more question , if I can.

Yogesh Sareen : Yes.

Kunal Dhamesha: Sure . So, sir, this Rs.40 crore donation that we have done from Balaji and Devki Devi Society , and that kind of , you know we have taken it pre -EBITDA , which kind of reduces our profitability. So , does this kind of help with maintaining the tax-exempt status for this trust hospital?

Yogesh Sareen : Kunal, first of all, it's not before EBITDA, after EBITDA. It's in EBITDA only. It's basically from one society to the other society. The other societies are not having

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operating income, so we don't show them as a separate column , right? We put that under the elimination column. Then you see that the overall number matches. So , it's not a major movement. It's the only movement of one society donating to the other one , and it has nothing to do with tax status.

It is b

basically, under tax status, you are supposed to use the money that you have for the objectives of the society and that part of the objective is also to help other societies . If one society has surplus cash, then it can obviously donate to the other societies. It's within the ambit of their 'objective ' clause and within the ambit of the approvals that they have in terms of exemptions from the Income Tax department to donate money to other societies for furthering the objectives of the society.

Moderator: Thank you. As there are no further questions, I now hand the conference over to the management for their closing comments.

Abhay Soi: Thank you everyone , for taking time out to join us for third quarter results. We look forward to interacting with you again next quarter.

Disclaimer: This is a transcription and may contain errors. While the transcript has been edited for clarity and efforts have been made to ensure a high level of accuracy, the Company assumes no responsibility for any errors.



## **Call: May 2025**

*(no extractable text — likely a scanned image PDF)*

## Call: Aug 2025

August 21, 2025

Listing Department ,  
National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,  
BSE Limited  
Phiroze Jeejeebhoy Towers,  
Dalal Street,  
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on August 14 , 2025

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on August 14 , 2025, on financial results of the Company for the quarter ended June 30 , 2025.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/financials#earnings-call](http://www.maxhealthcare.in/financials#earnings-call).

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Aroraa  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited  
Q1 FY26 Earnings Conference Call Transcript  
August 14 , 2025

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited's Earnings Conference Call.

Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj from CDR India. Thank you, and over to you, sir.

Suraj Digawalekar: Thank you, Nirav. Good afternoon, everyone . And thank you for joining us on Max Healthcare's Q1 FY '26 Earnings Conference Call.

We have with us Mr. Abhay Soi – Chairman and Managing Director; Mr. Yogesh Sareen – Senior Director and Chief Financial Officer; and Mr. Keshav Gupta – Senior Director, Growth, M&A and Business Planning.

We will begin the call with opening remarks from the management , following which we will have the forum open for an interactive Q&A session.

Before we begin, I would like to point out that some statements made in today's discussion may be forward-looking in nature, and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks. Thank you, and over to you, Abhay.

Abhay Soi: Good morning, everyone, and thank you for joining us on Max Healthcare's Q1 FY '26 Earnings Call.

We are pleased to report a strong start to the financial year, with year-on-year growth of 27% in revenue and 23% in operating EBITDA for the Network. This marks the 19th consecutive quarter of year-on-year growth, underscoring the success of our strategy and strong execution capabilities of the team. This performance is driven, in part, by the successful integration of the hospital acquisitions made in recent years, which have significantly bolstered both our top and bottom-line results.

Looking ahead, our focus remains on commissioning the new bed capacities scheduled to come on stream during the course of the year. We expect to add approximately 1,000 brownfield and 500 greenfield beds. Trial runs have already been initiated at the new 160-bed brownfield tower at Max Mohali.

In addition, our Board has recently approved the execution of an agreement-to-lease for a "built-to-suit" 130-bed hospital in Dehradun. The site is located approximately

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100 meters from our existing 220-bed Max Dehradun facility, which reported over 80% occupancy in Q1 FY '26. The new facility is expected to be commissioned by the end of 2028 and will primarily focus on advanced oncology services, including radiation therapy, which is presently missing at our Max Dehradun facility.

As part of our strategy to focus on super-specialty care in selected geographies, we have executed a binding term sheet to divest Chitta and Anoopshahr hospitals for Rs. 40 crores, with completion expected by September '25. Of the two, only Chitta hospital was operational and reported approximately Rs. 5 crores in revenue and Rs. 1 crore of EBITDA loss in Q1 FY '26. These assets were part of the overall Jaypee Healthcare acquisition and had limited strategic alignment.

Now coming to the performance highlights of the first quarter. Please note that the term "Existing Units" hereafter refers to the Network facilities that were operational prior to Q1 FY '25, while Max Noida and Max Dwarka are categorized as the "New Units".

1) Average occupancy for the Network stood at 76% compared to 75% in both Q1 last year and the trailing quarter. However, Existing Units achieved occupancy levels of over 78%.

2) Occupied bed days increased by 26% year-on-year and 4% quarter-on-quarter.

3) Average revenue per occupied bed for the quarter was Rs. 78,000, growing nominally by 1% year-on-year as well as quarter-on-quarter. For the Existing Units, like-for-like ARPOB grew by 5% year-on-year and 2% quarter-on-quarter.

4) Network gross revenue was Rs. 2,574 crores, up from Rs. 2,028 crores in Q1 last year and Rs. 2,429 crores in the previous quarter. This reflects an increase of 27% year-on-year and 6% versus the trailing quarter. Of this, New Units reported a gross revenue of Rs. 231 crores. Like-for-like revenue growth in Existing Units was 16%, driven by an increase of 10% in occupied bed days and 5% in ARPOB.

5) Digital revenue from online marketing activities, web-based appointments and digital lead management was Rs. 744 crores, accounting for approximately 29% of the overall revenue. Website traffic witnessed over 69 lakh sessions during the quarter, growing by 61% year-on-year and 7% quarter-on-quarter.

6) International patient revenue reached Rs. 208 crores, registering a growth of 32% year-on-year and 3% quarter-on-quarter despite airspace restrictions and geopolitical volatility in certain regions.

7) Network operating EBITDA stood at Rs. 613 crores, reflecting a growth of 23% year-on-year and a 3% dip quarter-on-quarter, primarily due to the impact of annual increments and additional manpower hired for new capacities. New Units contributed Rs. 27 crores to the Network EBITDA, representing a growth of 19% compared to trailing quarter.

8) Network operating EBITDA margin was 24.9% for the quarter. Existing Units reported an EBITDA margin of 26.2%, which is 44 basis points higher than Q1 last year. Also, adjusted for the one-time donation of Rs. 12 crores, the margin increased to 26.7% for Existing Units.

9) Annualized EBITDA per bed for the Network stood at Rs. 68 lakhs. Like -for-like EBITDA per bed for Existing Units was Rs. 75 lakhs, reflecting a 7% growth year -on-year.

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10) Profit after tax for the Network was Rs. 345 crores versus Rs. 295 crores in Q1 last year and Rs. 376 crores in the previous quarter, reflecting a growth of 17% year-on-year.

11) Free cash flows for the quarter were Rs. 389 crores . We deployed Rs. 435 crores towards ongoing capacity expansion projects and facility upgrades at newer units, while Rs. 131 crore was spent towards land purchase for brownfield expansion at Max Vaishali. As a result, net debt for the Network stood at Rs. 1,755 crores compared to Rs. 1,576 crores at the end of March 2025.

12) Continuing our efforts

to support the local communities, we provided free treatment to approximately 40,000 patients from economically weaker sections of the society , worth Rs. 62 crores at hospital tariff.

13) Both our strategic business units continued to report steady growth in the revenue and profitability.

- Max@Home reported a top line of Rs. 60 crores , reflecting a robust growth of 22% year -on-year. It offers 15 specialized service lines across 15 cities, with over 50% repeat transactions.
- On the other hand, Max Lab reported a revenue of Rs. 48 crores , reflecting a growth of 19% year -on-year. It provides services in over 55 cities through its network of more than 1,300 collection centres and active partners.

14) Now coming to the status of our expansion projects :

- 268 beds at Nanavati in Phase 1 – We are in the advanced stages of commissioning three basements, ground to third floors, along with the seventh floor, over the next few weeks.
- 400 beds of Max Smart at Saket Complex – Interior and MEP fit -out works are progressing as planned, with phased commissioning expected to start towards the end of Q2 FY '26.
- Max Lucknow – The current capacity of the hospital stands at 413 beds, and we expect this to increase to 520 beds by end of the financial year. Of the 107 additional beds, 32 beds are ready for commissioning. The oncology bunkers are also in advanced stages of completion , with LINAC installation expected to start in early September of this year.
- 500 beds at Sector 56 Gurgaon – Structural MEP work is currently in progress , with high side equipment under installation . We expect to commission the facility by the end of this financial year.
- 100 beds at Max Nagpur – We are currently awaiting formal environmental clearance , while the project has been otherwise approved . Civil contract has been awarded and we expect to complete the project within 24 months.
- 397 beds at Patparganj – Post receipt of environmental clearance, barricading and tendering work is in progress. The D-wall designs, etc., have been done and work is now being awarded accordingly. This is also in line with what we had projected.
- 550 beds at Max Vikrant at Saket Complex – We are still awaiting clearance from the Forest Department for tree transplantation. A fresh application has been submitted in line with discussions with the department. In any case, this project is to start upon commissioning of the 400 beds at Max Smart by the end of this year.

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- 400 beds at Zirakpur, Mohali – Partner has received approvals for the drawing and construction, and site work is progressing at a very fast pace. The project is expected to be completed within the next 24 months.
- 140 beds at land adjoining Max Vaishali – Demolition of the existing structure is underway, and the building plans are being filed with the authorities. We expect to complete this project within the next 30 months.
- 500 beds at Thane – Partner is in the process of finalizing the master plan for

the larger site . Detailed drawings for the hospital are being prepared, and we expect submissions to begin by the end of this quarter.

- 250 beds at Pitampura, Delhi – Partner has submitted drawings for approval, while site clean -up and barricading works have been completed . Construction work to commence upon receipt of necessary approvals.

With this, we open the floor for any questions you may have.

Moderator: Thank you very much . The first question is from the line of Tushar Manudhane from Motilal Oswal Financial Services Ltd .

Tushar Manudhane: Sir, just in terms of the operational beds have increased by almost 25%, and that is how the revenue growth it seems to be given the stable ARPOB at the headline level.

So, if I exclude these additional operational beds , which would have happened over last one year, then the remaining beds performance, how it has been, if

you could  
throw some light.

Abhay Soi: Like we mentioned, Dwarka opened in July '24, and Jaypee Noida was acquired in Oct-November '24. If we just take these two facilities out, then the growth in revenue has been 16%.

Tushar Manudhane: And that is driven again by further sort of optimizing payor mix? Or is it a combination of --?

Yogesh Sareen: Basically, for the Existing Units , that means including the Lucknow and Nagpur hospitals , the growth has been 16% and 18% in revenue and operating EBITDA , as Abhay mentioned . If we take out the se two ne w hospitals, i.e., Lucknow and Nagpur, the acquisition s of which happened in the Q4 FY '24 , in that case, the growth would be around 13% in revenue and around 15% in operating EBITDA , excluding one -time donation. So that means even the base leve l of existing hospital s, after taking out Lucknow and Nagpur, has seen quite a robust growth.

Also, I want to mention that in that case, the growth in ARPOB will be 7% instead of 5% that Abhay mentioned over the Existing Units . The base level existing hospitals growth is 7% in ARPOB, 5% in the OBDs and overall growth in the revenue is 13%.

Tushar Ma nudhane: And of course, the occupancy would be, any which ways , going sort of 80% plus, right? I mean , because the headline occupancy itself is at 75%, 76%.

Yogesh Sareen: Occupancy in the base level hospital s will be around 79.9% in this quarter.

Abhay So i: Almost 80% in the quarter for the base hospitals. But if you add Lucknow and Nagpur to it, then it comes down to 78%. And then if you add the other two (Dwarka and Jaypee) which we did last year, then it comes down to that 76%, which is similar to what it was last year.

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Tushar Manudhane: Got it. Yes, fair. I mean , the further extension to this is that if I exclude this new whatever that got added over last year, then the base hospitals , given that we have already grown in this year as well, and we are already at 80% occupancy --

Abhay Soi: That's right. But I just want to tell you that we are calling the two hospitals that were added in the last 12 months, as existing hospital s, right? But if we extend thi s argument to 15 months, then it becomes four hospitals. So therefore, we are just telling you f or the existing hospitals that we are only counting the 12 months. In spite of that, the occupancy has gone up from 75% to 78%. But if we take out the two, which were added in last March, then it goes up to about 80%.

Tushar Manudhane: Got it. So subsequently, new growth because of these new additions is very much intact as we for FY '26 to '28 or '29. Just trying to understand if I exclude all these and only on the base hospitals, probably now the base, it's been dynamic in nature. So, I cannot do much about it. But let's say, if I exclude last 12 months additions, then subsequently whatever there's a recent comment and we are reaching at 80% occupancy , in the next two to three years is there still scope to grow and further increase the occupanc y? Or that becomes more or less the base, and so the addition of beds is what is going to drive the growth going forward?

Abhay Soi: No. See, for your base hospital s itself, the nomenclature changes. You see whatever has been added really over the last 12 months or the 15 months has been acquisition s and greenfield , right?

Tushar Manudhane: Yes.

Abhay Soi: For example, what we are going to be adding in the next 30 -60 days are the three brownfield s that are coming through. So, they add on to the existing facilit ies. Then it's difficult to say that, look, in a brownfield , the earlier beds are growing by this much and the newer beds are growing by that much. And since it's brownfield , the take up should be very quick.

Tushar Manu dhane: Understood. And sir, just secondly on this international patient flow, which all other geographies we have been sort of engaged with

to get this kind of high growth?

Abhay Soi: So, I do not want to go into details of specific countries. There have been newer geographies, including some of the developed countries as well. But of course, some time back, we had guided you to the fact that we were opening direct -to-fly offices in other countries. There was a focus towards international marketing, and that business is something we had called out much earlier even a few years back. And I think what this has led to is that we have got increased traction through that strategy. And we have been doing more of that and being able to drive more patients.

Tushar Manudhane: Got it. And just lastly from my side in terms of net debt, given the projects at hand, without considering any inorganic opportunity at this point of time because that will be as and when it comes, how should one sort of think of net debt at the end of FY '26?

Yogesh Sareen: Yes, the net debt will go up to some extent because we have tied up some funds for the projects underway. By the end of FY '26, we may probably have addition in the net debt to the extent of around Rs. 400-500 crores.

Abhay Soi: Rs. 400-500 crores. It will translate into Net debt -to-EBITDA of still less than 1.0x I believe.

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Moderator: Thank you. Next question is from the line of Damayanti Kerai from HSBC Securities and Capital Markets (India) Private Limited.

Damayanti Kerai: My first question is on your focus on the oncology segment. So, this is largest for you, and year-on-year also I think it has gone up. So, it's already 25%, 26% of your total hospital revenue. And I understand in newer hospitals also you are trying to offer these services. So, in terms of further growth from current level, what kind of headroom do you think is available here?

Abhay Soi: First and foremost, some of the newer hospitals, particularly Dwarka and Lucknow, do not offer radiation oncology at this point of time, because the bunkers are not there. But in the third quarter, we are expecting the bunkers to come through in both of these hospitals. Therefore, you will see a larger share of oncology certainly in these hospitals, and that will impact the overall share as well. We do not believe that there is any reason for the growth in oncology to abate. These are structural factors and we are essentially catering to the market. Essentially, the market is growing at what it is, and that is what is leading to increase in our share of oncology.

Damayanti Kerai: Okay. And do you think the kind of demand out there in the market and your leading position here, this therapy could be upwards of 30% in, say a few years from now. And why I am asking because I guess what we are hearing from your peers also, they are focusing a lot more on oncology compared to a few years back, from that perspective?

Abhay Soi: I think to a large extent we can say we are focusing on it, but we are also all reacting to what is happening in the market. And yes, we have seen growth in oncology numbers, perhaps more than other specialties. And if you compound it at that, then certainly you will start increasing the share of oncology, and it will be a higher market share. It has been growing, and it's not necessarily that the new facilities that we acquired that has led to this growth in oncology alone. Because the newer facilities, like I mentioned, have a lower component of oncology at this point of time since the radiation -oncology is not there yet.

For example, Nagpur does not have radiation -oncology. Lucknow did not have radiation -oncology. Dwarka does not have a radiation -oncology. Jaypee does have, so that's really only one out of the four new hospitals. And like I said, when we are able to expand our suite of services, which is in Q3, in at least two out of these four hospitals, then the share should go up.

Damayanti Kerai: Got it. My second

question is on your newer hospitals, which will open soon. Say, Nanavati will start in next few weeks or so, and then Smart is also coming up by end of this current quarter. So, what is the status right now on the doctor hiring part? Like when do you onboard most of the doctors, like before start of the unit or you will add on gradually as you offer more and more services? And what kind of numbers you are looking here?

Abhay Soi: All the brownfield expansions largely offer the entire suite of services. Like I mentioned earlier, some amount of hiring that was required for augmentation of these newer hospitals has already happened, particularly at Nanavati, Mohali and Smart (Saket). For instance, when we open 400 beds, we are not opening 400 beds on day one. As and when you get the floors, you open it up but we have more than adequate clinicians already onboarded to be able to support the operations. Then incrementally you will keep growing thereafter as well. But that will only be incremental, so you would not feel it. So, whatever has been absorbed is absorbed and you will not see any impact because of that.

Damayanti Kerai: Got it. That's helpful. Thank you and I will get back in the queue.

Moderator: Thank you. Next question is from the line of Shaleen from UBS Securities India Private Ltd .

Shaleen Kumar: Hi, guys. Thank you so much for the opportunity. So, one question which I want to understand, you made a comment Abhay also that direct cost has gone up, and it's largely because of the New Units . But is this going to be stable from here? For example, I can see that your direct cost is Rs. 1,000 crores right now from Rs. 917 crores to Rs. 1,015 crores . So now the growth of this unit would be inflationary or there will be more additions will be that? And the subsequent impact on the margin. So, from Q4 to Q1, I understand this is seasonally weak. But if direct cost shouldn't go up, then one should expect margin to expand from here onwards meaningfully?

Abhay Soi: As far as direct cost is concerned, the significant majority of it is manpower cost. And on 1st of April we have increased salaries , so you get that bump up from 31st March to 1st April. We have always maintained that brownfield s do not really have an impact , but 30 days before or a few weeks before when you are doing trial runs, etc., you have staffed the hospital . It may not necessarily only be clinicians, but also people down the line. You staff for the hospital before fully commissioning and you are kind of carrying that cost. So yes, I think that little bit of cost may not be generating extra revenues, but it will do so over the next few weeks, once these brownfield s kick in. Coming to your point, theoretically, it should increase.

Keshav Gupta : On the direct cost specifically going to about Rs. 1,000 crores. Yes, a bunch of doctors have joined, which are part of the system that was basically a prebuilt up for the new capacities coming in. But a lot of them will also join alongside when the capacities get commissioned phase by phase. They will go in tandem to the revenues kicking in as well.

Abhay Soi: Yes. But there's no negative impact of that.

Yogesh Sareen : Shaleen, also as we progress into the coming quarters, the share of the institutional beds will come down as far as these hospitals are concerned . Because you know that our strategy is to fill up the beds and then distil the payor mix. So, this ratio should improve . The institutional share has gone up because of the new beds, etc. and as we speak, the occupancies in hospital like Dwarka, which started only 12 months back, is already 81-82%. It's because we said let's fill it up with whatever demand we have . Once the payor mix ratio improves , the direct cost percentage will also come down.

Shaleen Kumar: Sure, sir. Because typically when we move from Q1 to Q2,

there is a sequential reasonable jump of 5%, 6% happens on the revenue. It may be more or less depending upon ramp -up of the new facility. I hope it's more. But we should not expect direct cost to also move up in the same line, right? It may not move up, right .

Yogesh Sareen: Yes, as a percentage, this should be moderate.

Abhay Soi: You are right, your hypothesis is correct.

Shaleen Kumar: Correct. All right, sir. So, we should expect a decent margin expansion then. This Rs. 12-crore donation , is it part of your CSR expense? Or is it outside of that?

Yogesh Sareen: No. CSR expense is generally incurred by the companies and not by trusts . This is a spend, which has been incurred by the Devki Devi Foundation (DDF) and done on

the recommendation of its governing body . Nevertheless, it's a one -time donation that has been made from the DDF society.

Shaleen Kumar: So, this is a part of our indirect cost?

Yogesh Sareen: It's part of the indirect overhead s, yes, at this point of time.

Abhay Soi: It's one -time.

Shaleen Kumar: One-time. So, net-net, yes, so I just wanted to ask because it is CSR then it's not one-time. But it's ex of CSR, then it's one time .

Yogesh Sareen: In a charitable society, there is no CSR. CSR only applies to companies . That money has been spent by the DDF society. So that's the reason why it is a donation and not CSR.

Shaleen Kumar: Basically, when I think about it, let's say, everything remains same next quarter, then I should add back this Rs. 12 crores to the revenue, right, to the EBITDA.

Abhay Soi: That's right. Rs. 12 crores will never happen again, yes.

Shaleen Kumar: Yes. So yes, so that's my Rs. 12 crore , which I can add back to my EBITDA, right, next quarter?

Abhay Soi: That's right.

Shaleen Kumar: And the last bit, any colour if you can give us on the 2Q? How has it been going for

you guys, right? How is the season on the vector -borne, etc? Typically, it's a strong season for us.

Abhay Soi: I am going to avoid giving any forward-looking statements. I will just tell you that normally the vector season happens towards the end of monsoon. This year the monsoon has been long. So only at the end of monsoon, we will start seeing it. There has to be stagnation of water for vector-borne diseases to kind of set in. It normally happens when the monsoon starts receding. But right now, the monsoon has been sort of continuing.

I think the important thing, to look at, is that at the existing hospitals, excluding any additions that we did over the last 15 months, there's a 7% growth in ARPOB and there is a significant increase in revenue and EBITDA. And occupancy has certainly gone up to 80% for the base hospitals.

Second thing to look at is the two hospitals (Lucknow and Nagpur), which were added 15 months ago, both these will have a better sense of maturity about them because they have been with us for longer. Both of these have also contributed to increase in the occupancies. And in spite of both of them, you still see ARPOB move up by maybe 5% and occupancies go up by a couple of percentage points.

And then if I add the two hospitals, one we really acquired in October-November '24 (Noida), the other was a Greenfield (Dwarka) in July '24 that we started. In spite of those, we see flat occupancies on an overall basis. So, even the newer facilities haven't really dragged down our operations or our numbers significantly. And yet they provide us an opportunity as they mature.

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Shaleen Kumar: Got it. Absolutely, I can see that. Thank you so much, guys. I will get back in the queue. Thank you.

Moderator: Thank you. Next question is from the line of Rajat Shah from PGIM India Asset Management Private Limited.

Rajat Shah: Yes. So, I just wanted to know regarding the growth trajectory of the acquired hospital on revenue and EBITD

A basis. How has it been post the acquisition?

Yogesh Sareen: You are talking about Nagpur and Lucknow?

Rajat Shah: Including Lucknow, Nagpur and also Noida, Dwarka.

Yogesh Sareen: Yes. So, Lucknow has been doing very well. On a Y-o-Y basis, the revenue growth is 97% and EBITDA growth is 191%. At Nagpur, we have grown the revenues by 27% Y-o-Y and EBITDA growth is also in the same range. Noida has been a bit subdued, with around 14% Y-o-Y growth in revenue and EBITDA growth is 32% Y-o-Y. Again, this is based on the numbers that we have of the last year. These are not our numbers, but basically the numbers that we could get from the erstwhile management.

Rajat Shah: Okay, perfect. That's it from my side.

Moderator: Thank you. Next question is from the line of Neha Manpuria from Bank of America Securities India Limited.

Neha Manpuria: First question, Abhay, when does Nanavati's new block fully become operational? You said that we are currently operationalizing one of the floors and I think the basement. When can we see the unit becoming, let's say, fully operational from a bed perspective?

Abhay Soi: The basement is also used for onco services, nuclear medicine, etc. So, we will be starting over the next few weeks with a couple of floors. And then by October - November '25, we should be starting the rest of the beds, including operation theatres, etc.

Neha Manpuria: Okay. Understood. And my second question is, if I look at the New Units numbers that you have given for fourth quarter versus this quarter, it seems like it's broadly similar to what we did in the fourth quarter. I thought that Dwarka had achieved breakeven last quarter, and that should be positive. So, is it just a seasonality thing? Because I thought the seasonality isn't as stark quarter-on-quarter from fourth to first. And I would have expected that, or should I assume till the time we do not see onco come in, in Dwarka, you would not really see that big ramp-up happening?

Abhay Soi: No, on the contrary. You are right that there is seasonality. But for a new unit, there is no seasonality because your growth momentum itself takes care of the seasonality and beyond. We have seen very high ramp-up of occupancies and everything else at Dwarka. So no, I do not think we are waiting for anything to come in. In fact, we are already planning brownfield expansion at Dwarka because we are running out of capacity over there.

Yogesh Sareen: Dwarka's revenue growth is 24% quarter-on-quarter, which means that we have grown the revenues from the Q4 FY '25 to Q1 FY '26 by 24%. EBITDA is also up by Rs. 7-8 crores from Q4.



Abhay Soi: Yes. And I think we have passed the breakeven and any seasonality questions over there. We are already at capacity run out status.

Neha Manpuria: Okay. And so, in which case, then I am wondering if Noida could have done better this quarter, but because I would have assumed that you would see a much larger improvement given the ramp-up in Dwarka and also Noida starting to improve performance. So just trying to get a sense, is Noida on track with what we are expecting? Or should I expect an acceleration as we go through the rest of this year?

Abhay Soi: You will certainly see acceleration in Noida as well. Like I had mentioned, we acquired it in October-November '24. The first couple of quarters are always the most crucial because you are kind of integrating it into your IT systems, etc. We changed the name of the company when we bought it from the NCLT. In this particular case, all the licenses, including transplant licenses, blood bank licenses, etc., had to be reapplied.

So those have been coming through gradually.

So, literally, we have been operating a little bit with our hands tied behind our back.

And then again, this was a company which has been in liquidation for many years.

The equipment

is out of life. We have ordered all the equipment, which have been coming in phases. All the building blocks have been put in place.

So typically, in any acquisition that you do of this nature, the second year is better than the first year. The first quarter will always be the weaker quarter, right? It will be the weakest quarter because that's perhaps the time you have been able to implement the least number of changes that you wish to because whether it's equipment, management, doctors, etc., the onboarding does take 3 to 6 months. And like I said, in this particular case, even though it's an acquisition that we did, but somewhere it was a greenfield because we lost all the licenses. And then we had to get them all renewed in the new company. But yes, most certainly, what's going to happen is that going forward, you are going to have a snowballing effect. In the quarters to come, you should see significant upside there.

Neha Manpuria: So, it's fair to assume that all of the ground work that needed to be done for Noida in terms of licensing, equipment, etc., will be done. I mean you should start seeing that from the subsequent quarters.

Abhay Soi: It is already done. But when you order the equipment, the supply chain itself comes from overseas. There is a 6-month process from the day you indent it for you to get some of these equipment in place.

Neha Manpuria: Understood. Yogesh, sir, what did you mention the Noida EBITDA at? Sorry, I missed that number.

Yogesh Sareen: For Noida, Y-o-Y growth in operating EBITDA is 32% and in revenue is 14%.

Neha Manpuria: And would you have the absolute number, sir, for the EBITDA, sorry?

Yogesh Sareen: It will be around Rs. 24 crore for the quarter.

Neha Manpuria: Okay. Thank you very much. That's helpful, sir. Thank you.

Moderator: Thank you. Next question is from the line of Kunal Dhamesha from Macquarie Capital Securities (India) Pvt. Ltd.

Kunal Dhamesha: Just continuing on the Dwarka and Noida thing. I think the total EBITDA contribution is around Rs. 23 crore or something?

Yogesh Sareen: It is Rs. 37 crore.

Kunal Dhamesha: Total contribution for Dwarka and Noida, which we have mentioned, the new hospitals, it is Rs. 27 crore, of which we are saying Rs. 24 crore is Noida, right?

Yogesh Sareen: No. You will also need to consider that the unit EBITDA will be 2.5% higher because we also cross-charge the HO cost when we report numbers to the investors. But when you see a unit performance, then it will be 2.5% higher in terms of EBITDA margin. 2.5% of the revenue is allocated to units as HO cost.

Kunal Dhamesha: So, this Rs. 27 crore is unallocated?

Yogesh Sareen: Rs. 24 crore that I mentioned is the unit EBITDA.

Kunal Dhamesha: And which would come down to what, around Rs. 20 crores, because Dwarka is...

Yogesh Sareen: It will come down by Rs. 2 crores.

Kunal Dhamesha: Because Dwarka, you said Rs. 7-8 crore, right?

Yogesh Sareen: That's right. Rs. 27 crore EBITDA is after 2.5% of Rs. 231 crore being allocated to these units. If you take out this HO cost, you will add Rs. 5 crore to this number. So, Rs. 32 crore is the combined unit EBITDA for Dwarka and Noida.

Kunal Dhamesha: Sure. Second question on the ARPOB. You suggested that including Lucknow and Nagpur, the ARPOB is around, what, 4.9% growth, right, excluding Noida and Dwarka. And then you are saying that the 3,500 bed that we originally had at 7%, that

translates to around 7% ARPOB degrowth for Lucknow and Nagpur. Is that a correct math?

Yogesh Sareen: No. I think there's some confusion. So first of all, let's take the existing hospital s, excluding even Nagpur and Lucknow, the ARPOB growth is 7% . These are the hospital s which were operation al in December '23.

Kunal Dhamesha: Correct.

Yogesh Sareen: So, factoring in Nagpur and Lucknow, which are the hospital s t

hat we acquired in

February '24 and March '24, then the ARPOB growth is 5% . If I factor in Dwarka as

well as Noida, then the ARPOB growth is 1.2%.

Kunal Dhamesha: Correct. So, I am saying that for 3,500 -bed to d o 7% ARPOB growth, the ARPOB should have been degrown in Lucknow and Nagpur, right? Then only 7% comes down to 5%.

Yogesh Sareen: No, I am not sure what you are saying because there's no Y -o-Y number for the Dwarka . There's no number for last quarter in Dwarka.

Kunal Dhamesha: No, no. Just exclude Dwarka and this thing .

Abhay Soi: There is no degrowth in ARPOB in any of th e hospitals . There's only increase in ARPOB.

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Kunal Dhamesha: But then numbers are not basically connecting. How was that 7% ? So, 3,500 beds, you are saying 7% ARPOB, right? We include Lucknow and Nagpur, which is around 600 beds, our presentation says that 4.9% growth or 5%, let's say, right? So then 7% is coming down to 5% by addition of Lucknow and Nagpur in the mix, which means -

Abhay Soi: It is a verage revenue per "occupied bed ".

Yogesh Sareen: You are getting confused. Let me just give you the numbers. In the hospitals, which were operating until December '23, the occupied beds were 2,73 2. If we factor in the two hospitals, which is Nagpur and Lucknow, the occupied beds were 3,117 . And if we take Dwarka and No ida, the occupied beds were 3,556 . Now you can do the math .

Kunal Dhamesha: I will maybe connect offline on this.

Yogesh Sareen: The math will work out.

Abhay Soi: Please get your formula right. I have seen your formula and in spite of us saying that occup ied bed days have gone up and ALOS has gone down, I believe th at statements have been made in your formula that footfalls have come down for IPD. Theoretically, that's not even possible.

Kunal Dhamesha: Sure. I will do that, sir. Second question is, we have seen significant increase in institutional business, right? Even, let's say, because last year same quarter, we would not have Noida or Dwarka, right? But there is a significant increase. And I assume that Lucknow and Nagpur are not the centre s for a lot of institutional patients, right?

Abhay Soi: No, we started institution al in Bombay also in view of the new beds coming in. We started it in Nagpur as well , where it was not there prior to our takeover. It was started last year , and so it is in Lucknow.

Kunal Dhamesha: Okay. But then existing beds, on the 3,500 beds, what would be your view? Is it more or less stable in terms of bed days from institutional or ?

Yogesh Sareen: For example, Mohali, we are adding 160 beds. You cannot take institutional at that point of time . And so, we have taken institutional in the last quarter and the occupancy has already gone up . We are basically preparing for the new beds coming in .

Abhay Soi : So, it i s in Mumbai, because it takes you six months to empanel yourself . Mumbai, for example, erstwhile there was no institutional business. Now we have e mpanelled for institutional.

Kunal Dhamesha: So, wherever the new expansion is coming, it is more strategic?

Abhay Soi: That's right.

Kunal Dhamesha: Great, sir. Thank you and all the best.

Moderator: Thank you. Next question is from the line of Mohammed Patel from Edelweiss Financial Services Ltd .

Mohammed Patel: I have a question on ARPOB growth. It has been flat overall. So how should we think of overall ARPOB in the near term?

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Abhay Soi: Well, it has grown . For existing hospitals , it has grown by 7%. If you add the two hospitals that we acquired 15 months ago, then it has grown by 5%. If you look at all of the hospitals, then it looks flat. If you are going to start a new hospital or you acquire a new hospital, the ARPOB of that particular hospital is going to be lower . Therefore, it is going to b ring the average down. Otherwise, for the existing hospital s, ARPOB

and occupancy have gone up significantly .

Mohammed Patel: So, the overall ARPOB growth will take some time to reflect, right?

Abhay Soi: Yes. When you acquire a new hospital, you acquire it because it has lower ARPOB, and you acquire it at that price. And the whole idea is to increase the occupancy and the ARPOB of that hospital. Going forward, you will see ARPOB move up.

Mohammed Patel: Okay. So eventually, after a few quarters, the overall ARPOB growth will again start inching towards the high single digit?

Abhay Soi: That's right.

Mohammed Patel: Okay. And my second question is should we...

Abhay Soi: Having said that, it also depends on where you are acquiring. We buy for ROCE. We may acquire something in a tier-2 city, where the ARPOB is, let's say, half of the present ARPOB . The ARPOB of that place may never go up to our existing cluster ARPOB. The fact is , we have to figure out what ROCE are we buying it at. If we are buying it at a 20 -25% ROCE, or if we believe we can get it to a 20-25% ROCE in spite of the hospital having a lower impact on our overall ARPOB, we will still acquire it, right? I think the cadence you are looking at is not correct. Looking at ARPOB by itself or margin by itself, of any capacity that you are acquiring or you are starting today, is not correct. What we have to see is do we have superior return on capital profile over there or not.

Let's say, if we get an opportunity to buy a chain of hospitals, which operates at less than half of the overall ARPOB we are operating at, and we are able to buy it at 25% ROCE, should we or shouldn't we acquire it ? We should, right? What it would do is bring down our overall ARPOB and then we may never be able to climb up to the same present levels of ARPOB, so be it.

Mohammed Patel: Yes, yes, I get your point. My second question is, so should we expect EBITDA margin improvements in FY '26?

Abhay Soi: We do expect improvement. But having said that, EBITDA margin is not the correct metric to assess . Our international business is growing by 32% . It is supposedly higher margin business in value terms, but it's a lower margin business in percentage terms . When you do robotics , transplants or any of the high -complexity surgical work , it gives you less margin s in percentage terms, but it gives you more in value terms. Please consider EBITDA per bed as the right cadence and not EBITDA margin. On one end, because of better occupancy, higher utilization , etc. your margins will increase. On the other end, because you are going for a superior payor mix and a superior clinical mix, your margins should decrease . But your EBITDA per bed and ROCE will increase.

Moderator: Thank you. Next question is from the line of Sujit Shah from SK Enterprises.

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Sujit Shah: Yes. Firstly, I want to appreciate the entire Max Healthcare team. At this scale expanding so rapidly while maintaining profitability is not easy task. And my all question is already asked and answered. So not any question I have to ask.

Abhay Soi: Thank you.

Moderator: Thank you very much. Ladies and gentlemen, next question is from the line of Rajesh Gajra from Informist Media Pvt. Ltd. Please go ahead.

Rajesh Gajra: Yes. Earlier in the call, you mentioned that at the end of FY '26, the net debt will be and you gave some figure for that. I was not able to catch that number. Can you please repeat that, the net debt level at the end of FY '26?

Abhay Soi: What Yogesh mentioned is that between now and end of the year, your total debt for project purposes may go up by another Rs. 400-500 crores.

Rajesh Gajra: Okay. Alright. That's it. Thank you very much.

Moderator: Thank you. As there are no further questions, I will now hand the conference over to the management for closing comments.

Abhay Soi: Thank you once again. We appreciate all your time and look forward to connecting

with you next quarter. Thank you so much.

Disclaimer: This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

## Call: Nov 2025

November 21, 2025

Listing Department ,  
National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,  
BSE Limited  
Phiroze Jeejeebhoy Towers,  
Dalal Street,  
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on November 17, 2025

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on November 17, 2025, on financial results of the Company for the quarter and half year ended September 30, 2025.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/financials#earnings-call](http://www.maxhealthcare.in/financials#earnings-call).

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Aroraa  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Health care Institute Limited  
Q2 & H1 FY'26 Earnings Conference Call  
November 17, 2025

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited's Earnings Conference Call. Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you, and over to you, Suraj.

Suraj Digawalekar : Thank you, Michelle. Good morning, everyone, and thank you for joining us on Max Healthcare's Q2 and H1 FY'26 Earnings Conference Call. We have with us Mr.

Abhay Soi, Chairman and Managing Director ; Mr. Yogesh Sareen, Senior Director and Chief Financial Officer; and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning. We will begin the call with opening remarks from the management, following which, we will have the forum open for an interactive Q&A session.

Before we begin, I would like to point out that some statements made in today's discussion may be forward-looking in nature and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay to make his opening remarks. Thank you, and over to you, Abhay.

Abhay Soi : Good morning, everyone, and thank you for joining us on Max Healthcare's Q2 and H1 FY' 26 Earnings Call.

We are pleased to report that the Network has maintained its strong growth momentum through the first half of FY'26 . For the second quarter, revenue grew by 21% year -on-year, while operating EBITDA increased by 23%, thereby extending our track record to 20 consecutive quarters of consistent growth. Importantly, this performance reflects the continued strength of our core operations , with Existing Units achieving revenue growth of 14% and EBITDA growth of 19%.

Building on this strong performance, we continue d to enhance our capacity and service offerings through brownfield expansions. At Max Mohali, the new 160 -bed

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brownfield tower has been commissioned, including the additional radiation oncology program. At Nanavati -Max, the new 268 -bed brownfield tower is set to be commissioned this week , and at Max Smart, the new 400 -bed brownfield tower will be ready for commissioning within the next 30 days.

Alongside our expansion efforts, we also streamlined our corporate structure. Divestment of hospitals in Chittha and Anoopshahr was completed in September 2025. Further, the merger of our two wholly owned subsidiaries was approved by the Hon'ble NCLT, Chandigarh Bench with an appointed date of October 5, 2024, which has resulted in a one-time benefit of INR 149 crore during the quarter.

After long last, CGHS has revised the prices effective October 13th. While some of it is yet to kick in, we expect a favourable impact of over INR 200 crore once fully implemented. Further, with regard to insurance renewals, there was an impasse with certain players leading to temporary stoppage of cashless facilities. The same has been resolved.

Now coming to the Q2 performance highlights. Please note that the term "Existing Units" hereafter refers to the Network facilities that were operational prior to Q3 FY'25 , while Jaypee Hospital s are categorized as "New Units".

1. Average occupancy for the Network stood at 77% compared to 79% in Q2 last year and 76% in the trailing quarter. Existing Units reported occupancy levels of over 79% .
2. Occupied bed days (OBDs) were up by 19% year -on-year and 4% quarter -on-quarter.
3. Average Revenue Per Occupied Bed (ARPOB) for the quarter was INR 77,300, registering 1% growth year -on-year. For the Existing Units, like -for-like ARPOB grew by 3% year -on-year and remained stable sequentially.
4. Network gross revenue stood at INR 2,692 crore compared to INR 2,228 crore in Q2 last year and INR 2,574 crore in the previous quarter. This reflects an increase of 21% year -on-year and 5% versus trailing quarter. Of this, New Units contributed INR 144 crore , while the Existing Units delivered a 14% like -for-like revenue growth, driven by an increase of 11% in OBDs and 3% in ARPOB.
5. Digital revenue from online marketing activities, web -based appointments and digital lead management rose to INR 803 crore , accounting for approximately

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30% of the overall revenue. Website traffic crossed 79 lakh sessions during the quarter, growing by 53% year -on-year and 15% quarter -on-quarter.

6. International patient revenue reached INR 231 crore , registering a growth of 25% year-on-year and 11% quarter -on-quarter.
7. Network operating EBITDA stood at INR 694 crore , reflecting a growth of 23% year-on-year and 13% quarter -on-quarter. Of this , New Units contributed INR 23 crore , while the Existing Units delivered 19% like -for-like growth in EBITDA.
8. Network operating EBITDA margin was 26.9% for the quarter. Existing Units reported an EBITDA margin of 27.5%, higher by 90 basis points year -on-year and 200 basis points sequentially.
9. Annualized EBITDA per bed for the Network stood at INR 73 lakhs . Like-for-like EBITDA per bed for Existing Units was INR 76 lakhs, reflecting a 7% year -on-year growth.
10. Profit after tax (PAT) for the Network was INR 554 crore , against INR 349 crore in Q2 last year and INR 345 crore in the previous quarter. This includes one-time favourable tax impact of INR 149 crore pursuant to the merger of Crosslay Remedies Ltd. and Jaypee Healthcare Ltd. Excluding this impact, PAT stood at INR 406 crore , reflecting 16% year -on-year growth.
11. The Network generated free cash flows of INR 291 crore during the quarter.

There was a buildup of accounts receivable in the institutional segment, which impacted the cash flow from operations. INR 456 crore was deployed towards ongoing capacity expansion projects and facility upgrades at new units, while INR 146 crore was distributed

as dividend. As a result, Net Debt for the Network stood at INR 2,067 crore compared to INR 1,755 crore at the end of June 2025.

12. Continuing our effort to support the local communities, we provided free treatment to approximately 44,000 patients from economically weaker sections of the society, worth INR 61 crore at hospital tariff.

13. Both our strategic business units continued to deliver steady growth in revenue and profitability:

- Max@Home reported revenue of INR 63 crore, reflecting a robust 20% year-on-year growth. It offers 15 specialized service lines across 15 cities with over 60% repeat transactions.

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- Max Lab reported a revenue of INR 54 crore, reflecting a 16% year-on-year growth. It provides services in over 60 cities and served more than 6 lakh patients during the quarter.

14. Now coming to the status of our expansion projects.

- Max Lucknow – The current capacity of the hospital stands at 413 beds, and we expect this to increase to 550 beds by end of this financial year. The onco radiation program, including PET-CT, will be launched in the next two weeks.
- 501 beds at Sector 56, Gurgaon – The project is progressing well at site, while GRAP had some impact on the pace of work.
- 100 beds at Max Nagpur – We have received environmental clearance and are awaiting the Consent-to-Establish (CTE). We expect to complete the project within 24 months, post receipt of the CTE.
- 397 beds at Patparganj – All approvals have been received and barricading work is complete. The project is expected to be completed by FY '28.
- 550 beds at Max Vikrant (Saket) – The project work will start immediately upon commissioning of the 400 beds at Max Smart. We expect to complete this project within 40 months.
- 400 beds at Zirakpur, Mohali – Project is continuing faster than anticipated and is now expected to be completed by end of calendar year 2027.
- 140 beds at Max Vaishali – Demolition of the existing structure is complete and approvals are in process. We expect to complete this project in 24 months, post receipt of the requisite approvals.
- 500 beds at Thane - Master plans for the site have been finalized by the partner. The project is expected to be delivered in 42 months.
- 250 beds at Pitampura, Delhi – The partner is expecting the building plan approvals during the quarter, post which the project is expected to take 36 months.

And finally, moving on to the overview of the company's performance for the first half of financial year 2026:

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15. Network gross revenue stood at INR 5,266 crore, reflecting a growth of 24% year-on-year. Of this, New Units contributed INR 278 crore, while the Existing Units delivered a 17% like-for-like revenue growth.

16. Overall Network operating EBITDA grew by 23% year-on-year to INR 1,308 crore, translating to a margin of 25.9% and EBITDA per bed of INR 71 lakhs. While Existing Units delivered an EBITDA margin of 26.5% with EBITDA per bed of INR 74 lakhs.

17. In the first half, we generated INR 679 crore of free cash flow from operations, after interest, tax, working capital changes and routine capex. INR 891 crore was deployed towards ongoing expansion projects and facility upgrades at new units, INR 131 crore was spent towards land purchase for brownfield expansion at Max Vaishali, and INR 146 crore was distributed as dividend.

With this, we open the floor for any questions that you may have. Thank you.

Moderator

Moderator: Thank you very much sir. We will now begin the question-and-answer session. The

first question is from the line of Damayanti Kerai from HSBC and Capital Markets (India) Private Limited .

Damayanti Kerai: Hi. Thank you for the opportunity. My first question is to Abhay. So what we have heard, there has been some senior doctor departure from your team in NCR hospitals. So first, have you rehired

the required doctors? And what kind of impact was there, if any?

Abhay Soi: Yes, that level of iteration is normal. You have certain teams always sort of move out, and new teams come in almost immediately. And that is what has happened.

So, we do not expect any impact.

Damayanti Kerai: So approximately, like what is the attrition rate in your doctor's team, which you said is in line with the normal trend?

Abhay Soi: Less than 1% of the doctors.

Keshav Gupta : When the new facility comes , another 0.5% happens.

Abhay Soi: Yes, so you could say, between 1% to 1.5% at best. I mean anecdotally, if two or three doctors' leave or certain doctors leave, it does not have any major impact . We have got close to 6,000 doctors , right?

Damayanti Kerai: Okay. Okay. So no impact as such and then it is a normal trend. Okay.

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Abhay Soi: Absolutely.

Damayanti Kerai: Okay. My second question is on your settlement with issues on the insurance company. So has like all the matters now been put to rest and do you have any further contracts which are coming up for negotiations and there might be some updates there?

Yogesh Sareen: So Damayanti, first of all, whatever issues we had in terms of these three insurance companies , that we have already sorted out. And not only sorted out for now, but we are also sorted out for future . In the sense that when we concluded this arrangement , we also agreed the next revision with them . There are other companies , which are also up for renewal this time, including GIPSA . Those negotiations are underway, I would say.

Abhay Soi: But this sort of impasse happens once every few years, and we do not expect a repeat of this.

Damayanti Kerai: Okay. So -- but we have not heard such instances in recent past. So that is what I was a bit curious whether things are now put in place for permanently and then we might not see similar situations coming up.

Abhay Soi: We have had suspension of cashless facilities in the past too. It happened a few years ago as well.

Yogesh Sareen: We have had issues with GIPSA also, at one point of time.

Keshav Gupta : And everyone has it. These are resolved and cashless is active.

Damayanti Kerai: Okay. Okay. And one last question. You said CGHS revision when it happens fully like for your portfolio exposure, that will be additional INR 200 crore kind of revenue which will come in?

Abhay Soi: That is right.

Damayanti Kerai: Okay. And approximately, like in your portfolio, what has been the tariff increase after this division comes into effect very broadly?

Yogesh Sareen: So Damayanti, we cannot give you the numbers of what the increase is because obviously, it is a negotiation with the insurance companies. But I think the CGHS part is clear, which is that around 10% of the CGHS revenue will increase . We have around INR 2,000 crore of revenue coming in from CGHS, and so we will have a 10% jump there with this additional revenue.

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Abhay Soi: But this is a net increase because for certain medicines, etc. they have also capped the rates.

Damayanti Kerai: Thank you. I will get back in the queue.

Moderator: Thank you. The next question is from the line of Tushar Manudhane from Motilal Oswal.

Tushar Manudhane: So just on the international patient side where the growth has been quite robust even this quarter, almost 25%. So if you could just sort of help break down into volume and realization growth?

Yogesh Sareen: Most of it was volume growth. I do not think there is any increase in the ARPOB in

that segment. So, you can assume that 25% is entirely volume growth.

Tushar Manudhane: And more so, sort of -- if you could also elaborate in terms of the geographies, like in terms of diversification.

Abhay Soi: Broadly, the geographies remain the same. Like we mentioned in the past, it is Middle East, Eastern Europe, African cluster, etc.

We do not want to give any specific countries because it is privileged information.

Tushar Manudhane: Got it, sir. And lastly on this clarity, the MSSH Saket 400 bed hospital in terms of the time line for the commissioning? Or is -- I missed -- I probably missed it on the opening remarks, if you have already highlighted.

Abhay Soi: Currently, there is deep cleaning happening at Nanavati -Max, and we are expecting it to be commissioned within this week. Max Smart will be commissioned within the next 30 days.

Tushar Manudhane: Got it, sir. And so how do you see the scale up now that post commissioning, not literally in terms of numbers, but in terms of the time line, in terms of ramp-up of occupancy or maybe the EBITDA breakeven?

Abhay Soi: EBITDA breakeven is almost immediate. We have also stated that in the past, for brownfields, we do not have any EBITDA drags necessarily.

Tushar Manudhane: Understood. Thanks, that is it from my side.

Moderator: Thank you. We take the next question from the line of Andrey Purushottam from Cogito Advisors LLP.

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Andrey Purushottam: Hi. Congratulations for the consistent set of good results. I had just 2 questions. One is that your profit growth after correcting is 16% year-on-year which lags a little behind EBITDA and revenue growth. Is there any insight you can provide to us on this?

Yogesh Sareen: You would have seen that the finance costs have gone up, and depreciation has gone up as well because we have added new hospitals in Q3 last year. And their EBITDA margins are lower. We have also taken borrowings to fund these acquisitions. As their EBITDA growth comes in level with the other hospitals, you will find that the PAT growth number will also start to follow the same pattern.

Abhay Soi: Yes. Last year, Jaypee was not there, right? So that is about INR 2,000 crore of acquisition that we have done.

Andrey Purushottam: Okay. My second question was, I wanted your view on ARPOB. Are you getting a feeling on what is the internal discussion in the company that the ARPOB is kind of -- the rate of growth of ARPOB is becoming slower, at least that is what optically it looks like? I first want to know whether this assessment from our side is something that you share. And if so, is there any discussion on what to do about ARPOB going forward?

Yogesh Sareen: ARPOB growth is 3% y-o-y. But it also includes the impact of the new hospitals that we acquired and started last year. If we take that out and consider all the existing hospitals -- existing hospitals mean the one that were operating before Q4 FY24 -- then the ARPOB growth is around 7% y-o-y. It is basically the impact of the mix that is bringing this number down since the share of the new hospitals (Lucknow, Nagpur and Dwarka) has gone up in this quarter compared to Q2 last year.

Andrey Purushottam: So, thank you once again and congratulations for a good set of results. And I also like the disclosures that you make in your presentation, they are very comprehensive. So, we appreciate that as investors.

Abhay Soi: Thank you.

Moderator: Thank you. The next question is from the line of Amit Thawani from Clear Blue Capital Advisors LLP.

Amit Thawani: My first question is on insurance. I think you have spoken about the issues in insurance that we had this quarter. And because of that, our growth in insurance has actually been one of the lowest amongst all our peers. And then you have mentioned that the cashless has been reinstated. Can you confirm that for December quarter,

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like we have already -- we have done with half of November and October, are we back on track? I mean, with industry level growth in insurance business?

Abhay Soi: I think you need to look at it on an overall basis. Insurance growth may have come down, but your self-pay growth has gone up. If you are not pr



avoiding cashless, it is

not necessary that you lose the patient. The patient gets converted to self-pay, because he can get it reimbursed later. He does not necessarily change the hospital. That's why you need to look at what is the overall occupancy between cash and insurance, and what was the impact? You will not see any major impact on the occupancy of the hospital.

Amit Thawani: Right. Just by -- I mean, my only worry is that are we doing more institutional business because the insurance patient is not coming, but that does not seem to be the case.

Abhay Soi: Yes, that's not the case. You have seen that the self-pay business share has moved up, and you will see that the occupancy has moved between cash and insurance, because that is fungible, right?

Amit Thawani: Right. That is fungible.

Yogesh Sareen: If you come to me as a doctor, you will use your insurance while you can. If you cannot, then you will just sort of pay cash.

Amit Thawani: Correct.

Abhay Soi: And if you do believe there was an impact, then the point is just that going forward, you will see the benefit of it coming back now that it is all back to normal.

Amit Thawani: Got it. Got it. Our cash flows were weak. And as you rightly mentioned, it is because of our institution al business. Just wondering because now we are coming up with Nanavati and Saket, do we expect some kind of further deterioration because of this?

Abhay Soi: No, no. Institutional payments have historically been lumpy. They do not necessarily come on time. There are certain months when there is a build-up because various PSUs, whether it is ECHS or CGHS, run out of budgets. Then they write to the ministries, get their budget replenished and pay. They have been lumpy in the past as well. So, there is nothing to be concerned about.

Amit Thawani: So we do not expect further deterioration in the cash flows?

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Abhay Soi: No, not at all. I mean, even this is not a deterioration. It is a one-time thing. In the next quarter, you will probably see a large amount of cash flows coming through.

Amit Thawani: Got it.

Abhay Soi: If you put a line through the year, it will pretty much be the same number of days. At a given point of time, lumpiness happens in institutional payments.

Amit Thawani: Got it. I understood. My last question is on Mohali. So we added 160 beds, but the presentation shows an operational increase of 53 beds only. So is it that the next 107 beds will move into operational in Q3? And the Mohali is an O&M agreement with the government. So -- I mean, because in O&M, the entire revenue does not get recognized. So does that mean that these beds really do not have a material impact on our P&L?

Abhay Soi: No, no, two things. One is, for Mohali, it is a revenue share arrangement. So, let us say, 5% of our revenues are given to the government.

Amit Thawani: Okay.

Abhay Soi: Second is, as far as the beds are concerned, the entire tower has 160 beds. Now you do not wait for the last bed and the last light to be put to commission the whole facility. For example, of 160 beds, if the first 4 floors are ready, then we start the first 4 floors. And the rest of the floors as and when they come online, you start them.

Amit Thawani: So in Q3, how much will get operational in Mohali?

Yogesh Sareen: As we speak, 90 beds are operational. Out of the 160 beds, 53 were made operational from August 1st. That means the impact in the quarter is around 35 beds because you will have to consider the weighted average. And that is the reason why the operational capacity will be lower. We are obviously not short of beds there. As we see the occupancy ramp up, we will open more beds. But in quarter 3, we will have all the beds ready. We will open them as we see the demand there.

Amit Thawani: Okay, got it. Got it. And in Lucknow and Noida, we are

seeing a lot of interest from a

lot of our peers as well. They are coming out with some big capacities. Any comments on the competitive intensity out there?

Abhay Soi: Not really. The new Noida capacity, I mean, it will not come overnight. It has been under construction for the last 4 years. Any competitor coming up with any capacity has been constructing for the last 4 to 5 years. In fact, we purchased Jaypee later.

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That was like it fell from the sky and we purchased it, right? So, I mean that was a cold start.

Amit Thawani: Okay. My last question, on the ARPOB growth, Yogesh ji mentioned we are trending at about 6% now this quarter. Is that a fair comment for our existing units?

Yogesh Sareen: We are around 7% ARPOB increase if I take the existing units. When I say existing units, I mean units which were operating in Dec'24 or before.

Amit Thawani: So this December quarter is clocking a 7% on our pre-existing units?

Yogesh Sareen: Not December quarter, I am talking September quarter.

Abhay Soi: We have not given any guidance for Q3.

Yogesh Sareen: In the September quarter, the ARPOB increase was 3.3%. If I take out the impact of the new hospitals that we added in Q4 FY'24, then the ARPOB increase in the existing units is 6.8%.

Amit Thawani: Understood. Thank you so much.

Moderator: Thank you. The next question is from the line of Kritika Damani from Prospera Financial Solutions.

Kritika Damani: Thank you. Congratulations on a strong quarter. My first question is that you have an occupancy of over 75% and a consistent trend on the ARPOB because of the strategic push in your high-acuity care like the new oncology-focused hospital. Since the case mix naturally increases the average length of stay, how are you balancing its trade-off to maintain patient throughput at your already busy hospitals?

Abhay Soi: You do it by opening new capacities, improving patient services, reducing discharge times and such. Having said that, you have to keep in mind one thing. Unlike a hotel, when a hospital is full and there are people sitting in the emergency, I can not send them away by saying, my hotel is fully booked, please go somewhere else, right?

There will be a point of diminishing returns where patients will wait in emergency, wait for a bed sometimes for 6 hours or even 1-1.5 days. Then the complaints increase. You try to do your best, but we are also in the job of serving every patient who is coming over there. So, I think that is a very important differentiation.

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Kritika Damani: Okay. My second question is that for the first half of your FY '26, can you quantify what percentage of your total hospital revenue came from the international patients?

And what is the yield profile of this business? Like how does the ARPOB and the average length of stay for an international patient compared to that of the domestic patient portfolio?

Abhay Soi: It is about 9% of the revenues. And I think typical ARPOB is about 30% higher for international patients compared to our average.

Yogesh Sareen: ALOS is 1.2x.

Abhay Soi: 1.2x, because people who come for international visiting are coming from more acute diseases, right?

Kritika Damani: Right. And last question is that your asset light divisions like the Max Lab and Max@Home are posting excellent revenue growth. So my question is about the profitability and capital efficiency. For the first half FY '26, what is the consolidated EBITDA margin? And what is the long-term margin target for these segments as it scales?

Yogesh Sareen: That is already there in the investor release, right? So Max Lab reports an EBITDA margin of around 16%, right? And for the Max@Home business, the arm's length EBITDA would be around 20%. But if I take the global EBITDA, that will be probably more than the hospital EBITDA.

Abhay Soi: These are also sub-

dued right now because, as you mentioned, we are growing at a fast pace. It is because we are investing for the growth.

Kritika Damani: All right. Thank you. I will get back in the queue.

Moderator: Thank you. The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Yes. Thanks for taking my question. Abhay, last call, you had mentioned that in probably 6 months' time, we should start seeing traction in Noida with insurance empanelment. So is that on track? Should we start seeing that improvement in Noida, let us say, in the December, March quarter? And at what point do you see Noida getting to, let us say, mid 25% margins?

Yogesh Sareen: Noida is currently around 18%.

Abhay Soi: Insurance empanelment has already happened as far as Noida is concerned.

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Yogesh Sareen: Well, on Q-on-Q, you see that there is 8% growth in Noida, right? And the EBITDA margin has also grown compared to last quarter. We do expect that to continue. And if we pick up the revenue of this hospital since January of this year, every month has been higher than the previous month. So, we are seeing the traction. We hope that, you know, this will also come up to Network level.

Abhay Soi: I think it was not insurance empanelment. You know, it was licenses for various clinical programs, which had to come through.

Neha Manpuria: Yes, yes.

Abhay Soi: Such as transplant and so on and so forth. All that has come through. So now we are seeing traction with that.

Neha Manpuria: What is the occupancy in Noida currently then?

YS So it is around 64%.

Neha Manpuria: Okay. So this, therefore, should improve as we see these, I mean, as you have seen these new clinical programs getting added, right?

Abhay Soi: That is right.

Neha Manpuria: Okay. Understood. My second question is on Dwarka. What is the occupancy in Dwarka on our operational bed and what margins are Dwarka at currently?

Yogesh Sareen: Dwarka is 81% plus occupancy in the quarter. We have 285 beds operational now. It is a 300-bedded hospital, once we get the oncology block, then we will be able to vacate some onco day-care beds and move them to the onco block and have this hospital's capacity restored to 300 beds. EBITDA (Unit) margin will be around 15%, currently.

Neha Manpuria: Okay. Okay. Okay. Understood. So, I was just -- sorry.

Abhay Soi: The typical trajectory for a greenfield would be that you sort of ramp up the occupancy first, right?

Neha Manpuria: That is correct.

Abhay Soi: And then after that, you see expansion of margins.

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Neha Manpuria: Yes. So it would then -- in that case, given that we are at 80% occupancy already for Dwarka, Dwarka should -- I mean by next year, it should be in line with corporate average in terms of margins. Would that be a fair assumption?

Abhay Soi: Well, hopefully, that should be the case, yes. But do keep in mind, even oncology bunker is not operational over there right now, right?

Neha Manpuria: Okay. Okay.

Yogesh Sareen: Yes. It also has a very high level of institutional occupancy. So, we have to bring that down. Basically, the idea was first to fill up the hospital, and then start to distill the payor mix. I think we are at a stage where we have started to distill that now.

Abhay Soi: Yes. So, you are going to see higher ARPOBs and expansion of margins. You are going to see that happening through both patient mix and clinical mix, including higher share of oncology. Once the radiation oncology starts over there, you are also going to see us planning and implementing another 200 beds on that site. I think the idea is to do the brownfield. Like I said, we have already run out of capacity there.

Neha Manpuria: Yes. Understood. Okay. Because I was just wondering about the ARPOB growth of 3%, including all hospitals, except for Noida, right? That is your

existing ARPOB

growth that you mentioned. Given that, you know, I am assuming Lucknow and Nagpur should have also seen very strong ARPOB growth. The 3% number and the 7% that you mentioned for existing network, how should I read those two numbers?

Yogesh Sareen: By the way, Dwarka's ARPOB has come down. When we started it, we did not have any institutional business, right? The institutional business started in Q3 FY'25. So actually, the ARPOB has come down.

Neha Manpuria: All right. Okay. Understood. Okay, okay. That is helpful.

Abhay Soi: Because typically, when you do a greenfield, for you to get any institutional business, it takes you about 6 months to get empanelled.

Neha Manpuria: Okay. Yes, understood.

Abhay Soi: You start off immediately with high ARPOB, low occupancy since it is only self-pay business. You do not even have any insurance contracts in the first six months.

Neha Manpuria: Okay. I would assume that we will probably do more emergency cases, which then move to higher acuity?

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Abhay Soi : That is right. We started in July last year , so till the end of the year, it was high ARPOB simply because we did not have institutional or insurance empanelment .

The self-pay segment is high ARPOB but low occupancy, and the payor mix starts changing thereafter which drags your ARPOB down eventually .

Neha Manpuria: Okay. And the CGHS, you said INR 200 crore upon completion -- so once fully completed. So what is the duration by which this will fully get reflected? Would it be a phased manner in which we will see the realization of these price increases?

Abhay Soi: They have introduced a new category called super specialty hospitals. On the portal, they have not completed the codes for that. We are expecting that in maybe the next 15-20 days .

Neha Manpuria: Okay. Okay. So that is the only thing that is pending for this to be implemented?

Yogesh Sareen : Yes, it is partially implemented -- the super specialty will be implemented once they update their portal.

Neha Manpuria: Got it. Okay. Thank you so much.

Moderator: Thank you. The next question is from the line of Sumit Gupta from Centrum Broking.

Sumit Gupta: Hi, good afternoon. Thanks for taking my question. The first is like what was the growth on the ARPOB growth of the segments, be it international or institutional -- for the ARPOB growth of the segment basically be it cash or institutional mix, what was the ARPOB growth?

Yogesh Sareen : The walk-in business, both insurance and the self -pay, grew by around 8% to 10%, on a like -to-like basis. It is basically the impact of the new hospital s that got added into the existing hospital s, which has brought down the overall growth to 3 %.

Sumit Gupta: Yes. So, no, so with respect to institutional mix like have we witnessed any ARPOB growth?

Yogesh Sareen : No, ARPOB growth is flattish in the institutional.

Abhay Soi : We will have growth coming in , going forward, right?

Yogesh Sareen : And because the rate got revised only in October, so in Q2, there is no growth in the institutional ARPOB. But in the self -pay and insurance , the growth is in the range of 8% to 10% .

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Sumit Gupta: Understood. And sir, second question is like what was the performance of Lucknow and Nagpur unit in this quarter?

Yogesh Sareen : We do not share the hospital -wise performance, but suffice to say that Lucknow did grow over the previous quarter. On a quarter -on-quarter, we reported 17-18% growth in revenue and the EBITDA has also grown by more than 30%.

Sumit Gupta: Okay. Okay. So you are maintaining the 30% plus margins in Lucknow. So we can expect this to sustain?

Yogesh Sareen : Yes.

Sumit Gupta: Okay. And for Nagpur?

Abhay Soi: Again, we do not give specific hospital by hospital numbers.

Sumit Gupta: Okay, understood. Thank you, sir. All the best.

Moderator: Thank you

. We will take the next question from the line of Prashant Kshirsagar from

Unived Corporate Research Private Limited.

Prashant Kshirsagar: My question is on the Noida hospital facility. In the -- one of the earlier presentations, you had mentioned you have land parcels with further bed potentials in Greater Noida, 400 beds and Sector 128 Noida, 700 beds. I just wanted to ask you which of the -- is the land adjacent to the existing hospital facility or it is at a dif ferent location?

Abhay Soi: The present one (Sector 128) we have in Noida is 18 acres of land . You can imagine we can keep building over there. It is a contiguous land parcel. Similarly, in Lucknow, it is 27 acres of land. So we can go for another 2,000 - 2,500 beds there . We also have another land parcel in Lucknow. At Noida , in addition to that, we have land in Greater Noida.

Prashant Kshirsagar: So that is what I am seeing. So the location is different from the existing...

Keshav Gupta : Greater Noida and Noida are separate locations. So , in your question, the 700 -bed site, which is a very faraway project , is adjacent site to the recently acquired Noida Hospital, the Jaypee Noida Hospital. And what you mentioned as a 500 -bed site, that is a separate site in Greater Noida.

Prashant Kshirsagar: So this sector 128 is adjacent...

Yogesh Sareen : It is the same complex, yes. It is the 18-acre complex that we mentioned .

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Prashant Kshirsagar: Yes, that is the 18 -acre complex. Yes. Okay. That is what I wanted to clarify. Thanks a lot, sir.

Moderator: Thank you. The next question is from the line of Kunal Dhamesha from Macquarie.

Kunal Dhamesha: Hi. Thank you for the opportunity. One clarification. Can you please share the international patient bed share for this quarter?

Abhay Soi: Yes, so similar to what it used to be earlier -- yes, it would be around 5-5.5%.

Kunal Dhamesha: And sir, for the other channels, for self-pay and TPA? Can you also share for those?

Yogesh Sareen: Self-pay is around 26 -27%, insurance is around 34 -35%. And international, as we mentioned, is 5.5%. The rest is institutional.

Kunal Dhamesha: Right. Okay. Second one for Mr. Soi on the sorting of issues with the insurance. So sir, how should we think now with whatever negotiation which would have gone between you and the insurance companies for the next 1 or 2 years, for us, the procedure rates, do we keep getting similar hikes with what we had taken in historically in the last 2, 3 years? Is it going to be higher, lower? If you can share, some colour would be great.

Abhay Soi: Well, I am not going to give you specifics, but yes, it is in line with what has been happening historically -- within the band.

Kunal Dhamesha: And then, sir, let us say, the 4 insurance companies, which at least media suggested, had suspended cashless, all 4 of them are now on like where they are doing cashless?

Abhay Soi: That is right.

Kunal Dhamesha: And then for us, are they material in terms of, let us say, out of a 35% insurance revenue, are they material in terms of contribution to our top line?

Abhay Soi: Yes. I mean they are material. But like I said, you need to look at cash and TPA as a homogeneous body, right? I mean when you cannot do cashless for patients, the question is do the patients not come or do the patients come and pay out of pocket and then claim later from the insurance companies. Their cashless facility gets suspended, not their insurance policy.

Kunal Dhamesha: Sure. So patient did come. It is just the change of the channel for now. And then now it will be went back...

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Abhay Soi: Yes, we saw a spike in our cash patients.

Yogesh Sareen: So Kunal, the share of the insurance segment went down and the share of self-pay went up in terms of bed share, right? And the overall occupied bed days also grew quarter-on-quarter. So, th

is is not a major impact, right?

Kunal Dhamesha: Sure, sure. And sir, this 7% ARPOB growth on a like-to-like basis, I know you shared that Dwarka has gone down a bit and hence, the overall is 3%. But would not the specialty mix also helped us given the medical patients are down quite a bit in this quarter on a year-on-year basis?

Abhay Soi: Yes, it takes into account that, right?

Kunal Dhamesha: Right? But then with Medical going down, should not the growth be higher for our existing hospital is like my question because oncology has also gone up in terms of the contribution. So shall we continue to maintain this around 7% plus or minus 1% going forward for existing hospitals?

Abhay Soi: But that has been historical as well, right? I do not see any changes there.

Kunal Dhamesha: Sure, sir. Sure.

Yogesh Sareen: So, if you think that internal medicine is basically the impact that you generally used to have in quarter 2. But for the new hospitals, the share of internal medicine is low. So, while there is some impact, and you can see that in the earnings update, but it is not that big of an impact.

Kunal Dhamesha: You are saying new hospital it grew and existing hospital it was not that?

Abhay Soi: Yes.

Kunal Dhamesha: And lastly, sir, on the operationalized bed, because we are opening the capacity at Nanavati and Max Saket, how should we look at operational beds in Q3 and Q4 like what the potential number from where we are in Q2?

Yogesh Sareen: It is tough to really give you the exact number because we will also have to see for how many days we open those beds, when do we get the approvals, NOC, etc.

Abhay Soi: And I want to avoid any forward-looking statements on that.

Kunal Dhamesha: Okay, but let us say, is the trajectory should be in line with more like Dwarka? Is it something like in one year we are almost up with all beds?

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Abhay Soi: I mean, its greenfield (Dwarka) versus a brown field (Nanavati & Max Smart) , right? It is very different .

Kunal Dhamesha: Sure.

Abhay Soi: Because the EBITDA ramp up for these two segments is very different. Normally it is immediately positive in a brownfield . I am going to avoid giving any forward -looking statements.

Kunal Dhamesha: Sure, sir. Thank you for all the clarity and all the best.

Moderator: Thank you. The next question is from the line of Neha Manpuria from Bank of America.

Neha Manpuria: Yes, thanks so much, sir. Just a quick question. What was the network capex intensity in the first half?

Yogesh Sareen : We spent around INR 900 crore.

Neha Manpuria: Okay. And second half would be a similar number?

Yogesh Sareen : A bit higher probably. Hopefully around INR 1,100 crore .

Neha Manpuria: All right. Okay. Got it. Thank you so much.

Moderator: Thank you. We will take the next question from the line of Gourav Bhamra from JM Financial.

Gourav Bhamra: Yes. Hi, sir. Good morning. First of all, congratulations on a good set of numbers. I just wanted clarification on two fronts. The first being that when we say that ARPOB of this quarter was subdued, that was largely due to the insurance issues and the impact of the new hospitals, right?

Abhay Soi: Not so much the insurance, I would say. Like I said, any reduction in insurance perhaps was made up because we had that many people shifting to self-pay. But I think, it was perhaps because of the new hospitals.

Gourav Bhamra: Understood. And the second question regarding the impact of CGHS, the INR 200 crore positive impact that we are expecting. I know you answered it before, but there were some disturbances. Just wanted to clarify again, what is the estimated timeline we are expecting for that to reflect in our numbers?

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Yogesh Sareen : I think some of it has already started from 13th October. But I think there is a super speciality rate that will also kick in , I think

by the end of this month. I mean, we do not know how much time it will take, but I think the expectation is that by the end of November, that rate should also be up and working .

Gourav Bhamra: Understood. Thank you, sir. That is all from my end. Thank you for the opportunity.

Moderator: Thank you. Ladies and gentlemen, we will take the next question from the line of Nitin Agarwal from DAM Capital.

Nitin Agarwal: Thanks for taking the question. Just following up on the CGHS question, sir, the CGHS impact you talked about, it is largely for CGHS or you have also included possible, I mean, the expectation is similar rate revisions will happen for all other central government agencies also. So, is that over and above this?

Abhay Soi: All central government agencies are linked to CGHS. So, this number we mentioned is for CGHS and CGHS -like accounts. So, new rates , including super speciality rates, etc. are also expected on ECHS & others accounts because they are normally in-line with CGHS . However, they have to go through the paperwork.

Nitin Agarwal: Okay. So, that essentially, our estimate pretty much includes that inflation also?

Abhay Soi: That is right. So, this is CGHS and CGHS -linked accounts.

Nitin Agarwal: Okay. Thank you so much.

Moderator: Thank you. The next question is from the line of Shaleen Kumar from UBS.

Shaleen Kumar : Yes. Hi. So, just continuing regarding the past participant. Is it fair to assume that the benefits start kicking in from FY27? What are the benefits we should talk about of around INR 200 crore?

Abhay Soi: Of what?

Shaleen Kumar : The CGHS pricing increased benefits. You know, you said in next 15 -20 days we should resolve it. And then beyond CGHS also the similar CGHS -like institutions will follow it. So, FY27 is the year where we can expect the total benefit to come in?

Abhay Soi: Yes.

Shaleen Kumar : So, then what kind of a flow -through will it have on your operating profit EBITDA?

Abhay Soi: Pretty much 85 %.

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Shaleen Kumar : So, if you are looking at roughly INR 200 crore of top line, something like INR 150 crore, 160 crore of EBITDA addition can happen through this?

Abhay Soi: We are looking at INR 200 crore and hopefully good numbers should flow through.

Shaleen Kumar : Got it, sir. Got it. Regarding Smart and Nanavati, I understand both are Brownfield, but there is a bit of a cost. Is it fair to assume that a large part of cost is already in our P&L reflecting in 2Q or can we expect some more increase in 3Q as well?

Abhay Soi: I mean, you are talking about fixed cost or variable cost?

Shaleen Kumar : Variable cost.

Abhay Soi: Variable cost, of course. Variable cost would be related to the...

Shaleen Kumar : Sorry, sorry, sorry fixed cost. Fixed cost sorry, I made a mistake.

Abhay Soi: No, I mean, these are Brownfields, right? So, your fixed cost is largely absorbed by the existing hospitals.

Shaleen Kumar : Yes. So, my point is right, because we have been in a process of keep on adding new beds, right? So, our top line goes, then our EBITDA, and then our PAT. And I think the point, we will be there where, you know, most of our hospitals reach a stage where our EBITDA start growing faster. I think we are reaching that stage. So, is it a fair assumption that even in 3Q, we should see the same kind of phenomena, that margin expansion despite adding to or despite starting to Brownfield?

Keshav Gupta: You are right. There would be a bit of operating leverage that will kick in once those brownfield s start getting full up, that may be 15 days here or there. But, yes, operating leverage should come in. That is the whole concept of brownfields .

Shaleen Kumar : Yes, yes.

Abhay Soi: So, you should have expansion of margins with brownfield s because they give you higher margins than existing hospital beds.

Shaleen Kumar : Yes, there are two thi

ngs happening, right, in our growth journey, right? One, our new hospitals, which we have added in the past, they are significantly improving in their profitability. But at the same time, also, w e are adding new hospitals, right, which are coming in at a lower margin. So, they are two balancing acts. So, given that we have added a substantial number of beds over the past 18 months, where

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the flow -through of the profitability can be higher and will negate any negative impact of these new beds. That is what I am trying to understand?

Abhay Soi: That is right. My point is something even better, right? As a new bed, because largely what is coming is b rownfield, which does not necess arily have a drag on EBITDA that you are trying to subsidize via your previous expansion.

Shaleen Kumar : For sure .

Abhay Soi: It would be not only will you have expansion or, you know, benefits coming out of what you expanded over capacity addition over the last 15 -17 months , that is about 30% capacity, but majority of the capacity coming up now, another 30 %, is brownfield . And that brownfield capacity should , soon after commissioning, get into higher EBITDA margin business to start with.

Shaleen Kumar : Got it. That is it from my side and thank you so much.

Abhay Soi: Thank you.

Moderator: Thank you. As there are no further questions, I would now like to hand the conference over to the management for closing comments. Thank you and over to you, sir.

Abhay Soi: Thank you everyone for joining us today. We appreciate all your time and look forward to interacting with you again next quarter. Thank you.

Disclaimer: Thi s is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

## Call: Feb 2026

February 12, 2026

Listing Department ,  
National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,  
BSE Limited  
Phiroze Jeejeebhoy Towers,  
Dalal Street,  
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on February 6, 2026

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on February 6, 2026, on financial results of the Company for the quarter and nine months ended December 31, 2025.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/financials#earnings-call](http://www.maxhealthcare.in/financials#earnings-call).

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Arora  
SVP - Company Secretary and Compliance Officer

Encl.: As above

Page 1 of 22

Max Healthcare Institute Limited  
Q3 & 9M FY '26 Earnings Conference Call  
February 06, 2026

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded. I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you, and over to you, Mr. Suraj.

Suraj Digawalekar : Thank you. Good morning, everyone, and thank you for joining us on Max Healthcare's Q3 and 9M FY '26 Earnings Conference Call. We have with us Mr. Abhay Soi, Chairman and Managing Director; Mr. Yogesh Sareen, Senior Director and Chief Financial Officer; and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning. We will begin the call with opening remarks from the management, following which, we will have the forum open for an interactive Q&A session. Before we begin, I would like to point out that some statements made in today's discussion may be forward-looking in nature and a disclaimer to this effect has included in the earnings presentation shared with you earlier. I would now like to invite Abhay to make his opening remarks. Thank you, and over to you, Abhay.



Abhay Soi: Good morning, everyone, and thank you for joining us on Max Healthcare Earnings Call for the third quarter and nine months ended December 2025.

We are pleased to share that the Network delivered its 21st consecutive quarter of year-on-year growth in Q3, despite excessive unanticipated seasonal softness due to lack of vector-borne diseases and transitory external factors. Revenue increased by 10% year-on-year, while operating EBITDA grew by 4%. Overall occupancies remained strong, and ramp-up of the new brownfield beds progressed in line with our expectations.

During the quarter, we commissioned 63 brownfield beds at Nanavati Max, of which 45 beds are currently occupied. At Max Mohali, 53 brownfield beds were commissioned in second quarter, of which 46 beds are currently occupied. The remaining beds at both hospitals are expected to be commissioned during the Q4

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FY '26. The incremental bed capacity at both these locations is already EBITDA and margin accretive. At Max Smart, infrastructure for around 200 beds, along with operation theaters and OPDs is ready for commissioning, and we are currently awaiting the occupancy certificate, which is expected by the end of February. We also took an important step to expand our geographic presence in Western India to develop a 450-bed hospital on a prime piece of land in Pune by 2030. In addition, driven by the exceptional ramp-up in operations at Max Dwarka, the Board has approved the addition of another 260 beds at the existing site, taking the hospital's total capacity to 560 beds.

On a sequential basis, revenue and EBITDA were impacted prima

rily due to a

temporary shift towards institutional patients following the disruption in cashless services with stand-alone health insurers, which was fully restored towards the end of the quarter. Performance was also affected by the discontinuation of select high-value patented chemotherapy drugs in light of the revised CGHS pricing guidelines and by the reduction in GST on drugs and consumables. Further, the results reflected pre-commissioning expenses related to brownfield bed additions and other non-recurring costs.

Looking ahead, with cashless services now fully restored, upward revision in CGHS tariffs expected to fully kick in by April 2026 and margin accretive incremental capacity coming on stream, we believe the Network is well positioned to continue delivering sustained growth.

Now coming to the Q3 performance highlights :

1. Average occupancy for the Network stood at 74%, compared to 75% in Q3 last year and 77% in the trailing quarter, despite an 8% year-on-year increase in operational bed capacity.
2. Occupied bed days were up by 7% year-on-year, but dipped by 4% quarter-on-quarter due to seasonality.
3. Average Revenue Per Occupied Bed (ARPOB) for the quarter was INR 77,900, registering a 3% growth year-on-year and 1% sequentially.
4. Network gross revenue stood at INR 2,608 crore, compared to INR 2,381 crore in Q3 last year and INR 2,692 crore in the previous quarter. This reflects an increase of 10% year-on-year.

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5. Digital revenue from online marketing activities, web-based appointments and digital lead management was INR 803 crore, accounting for approximately 31% of overall revenue. Website traffic crossed 71 lakhs sessions during the quarter, growing by 44% year-on-year.

6. International patient revenue was INR 230 crore, registering a growth of 14% year-on-year and accounting for 9% of the revenue from hospitals.

7. Network operating EBITDA stood at INR 648 crore, reflecting a growth of 4% year-on-year.

8. Network operating EBITDA margin was 26.1% for the quarter, compared to 27.3% in the third quarter FY '25 and 26.9% in the trailing quarter. Margin was largely impacted by payor mix change, pre-commissioning expenses for brownfield beds and GST rate changes.

9. Annualized EBITDA per bed for the network stood at INR 71 lakhs versus INR 73 lakhs in both Q3 FY '25 and the previous quarter.

10. Profit after tax (PAT) for the Network, after exceptional items, was INR 344 crore against INR 316 crore in Q3 last year and INR 554 crore in the previous quarter. During the quarter, there were exceptional items aggregating to INR

55 crore relating to impact of the Code on Wages, 2019 and provision for stamp duty on the merger of two of our subsidiaries.

11. The Network generated free cash flows of INR 281 crore during the quarter. INR 408 crore was deployed towards ongoing capacity expansion projects, facility upgrades at newer units.

12. Net debt for the Network stood at INR 2,166 crore compared to INR 2,067 crore at the end of September 2025, while the net debt-to-EBITDA ratio continued to be less than 1.

13. Continuing our efforts to support the local communities, we provided free treatment to approximately 40,000 patients from economically weaker sections of the society, worth INR 61 crore at hospital tariff.

14. Both our strategic business units continue to deliver steady growth in revenue and profitability:

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- Max@Home reported a revenue of INR 68 crore, reflecting a robust 23% year-on-year growth. It offers 16 specialized service lines across 15 cities with over 56% repeat transactions.

- Max Lab reported a revenue of INR 47 crore, reflecting 13% year-on-year growth. It provides services in over 60 cities and served more than 5 lakh patients during the quarter.

15. Now moving on to the status of our expansion projects coming

on stream in the

next 2 to 3 years:

- Max Lucknow – The current capacity of the hospital stands at 413 beds, and we expect this to increase to around 500 beds by the end of this financial year. Radiation bunker and nuclear medicine services have now commenced at the hospital.

- 500 beds at Sector -56, Gurgaon – The pace of work at site has picked up post GRAP-related disruptions. We now expect to commission the first phase by end of H1 FY '27.

- 100 beds at Nagpur – We have received Consent-to-Establish (CTE) and civil work has started. We expect to complete this project within 24 months, as communicated earlier.

- 400 beds at Zirakpur (Mohali) – Project work continues to be on track, and we are scheduled to commission the hospital in FY '28.

- 200 beds at Max Vaishali – We are awaiting environmental clearance and approval of building plans to commence the work on site. We expect to complete this project in 24 months post receipt of these approvals.

- 397 beds at Patparganj – All approvals have been received and barricading work is complete. New design for D-wall has been firmed up and the project is now expected to be completed by FY '29.

And finally, coming to the overview of the company performance for the nine months ended December 2025:

16. Network gross revenue stood at INR 7,874 crore, reflecting a strong growth of 19% year-on-year.

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17. Overall Network operating EBITDA grew by 16% year-on-year to INR 1,956 crore, translating to a margin of 26% and EBITDA per bed of INR 71 lakhs.

18. In the nine months, we generated INR 960 crore of free cash flow from operations, after interest, tax, working capital changes and routine capex. Further, INR 1,299 crore was deployed towards ongoing expansion projects and facility upgrades at newer units, INR 131 crore towards land purchase at Vaishali and INR 146 crore distributed as dividend.

With this, we open the floor for any questions you may have.

Moderator: Thank you. We will now begin the Q&A session.

The first question comes from the line of Damayanti Kerai from HSBC Securities and Capital Market (India) Private Limited.

Damayanti Kerai: Hi. Thank you for the opportunity. My first question is on your oncology contribution. So during the quarter, obviously, we saw some softness, which you attributed to discontinuation of patented chemo drugs for institutional patients. So do you think you can go back to the prior level of contribution from oncology? And what could drive it back?

Abhay Soi: Well, when you say contribution, this is high-value drugs. So, the pricing was high in terms of the revenue, but the margins were not substantive, compared to the rest

of the business. Do you mean in terms of revenue, or do you mean in terms of margins?

Damayanti Kerai: In terms of revenue. Yes, when we look at the contribution from oncology, I think which is part of the presentation, it is around 24% or so compared to 26%, 27% last year. So, I was referring from that context?

Abhay Soi: This impacts the institutional patients only. These are high-value drugs, which were low-margin, and were being used actually for the institutional patients. What they have asked now is to sell them below your purchase cost. So obviously, everybody has discontinued it.

Keshav Gupta: And oncology's contribution to the revenue mix may increase, but it is a future addition. It's just that it was increasing in the past as well for last 4-5 years as an outcome of what was happening in the society. So directionally, will that number increase? It should follow the same trajectory but let's see.

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Damayanti Kerai: Okay, sir. So we see this as like a one-time adjustment and then going ahead, the growth will be...

Yogesh Sareen: So we continue to talk to CGHS, right? There is a lot of noise among the institution

al patients. For example, CGHS has their own dispensaries to supply these medicines. They said that they will supply these medicines directly to the patients. That means when the doctor writes the prescription, the patient pays to the CGHS dispensary and brings the medicine from there.

Similar tariff has been applied to ECHS patients also, but ECHS does not have any dispensaries. So obviously, there is a lot of noise out there. And let's see how the things shape out, but we are in continued discussion with CGHS. Obviously, they prefer to supply it themselves.

Secondly, they are saying otherwise, we will give it to you at 70% of the MRP, but our margins are less than 20% in these drugs. So, there is no question about supplying these drugs. We cannot be cashed out of these drugs while supplying these drugs. I think that is where the discussion is on and we are asking CGHS to give us some top-up on cost-plus basis. So, if our cost is, let's say, INR 80, then we are saying give us some margin, say 10%. We will be okay to supply at INR 88 rather than supply at INR 70, because we are buying at INR 80.

Abhay Soi: We believe it is the error on their part when they revised the CGHS rates. This is something that does not make sense because these are branded drugs and if your margins are already, say less than 20%, and you are being asked to supply them at a 30% discount to MRP, then obviously, nobody is going to supply and we are going to have a problem. So, we have been talking to them.

Damayanti Kerai: Sure. And in oncology, again, I guess, we heard about some doctors' team departure, etc. So has team fully back in strength? And what kind of further pickup we can see in the oncology space, leaving aside the CGHS issue, which will, I think, be cleared in some quarters to come?

Abhay Soi: Typically, what happens is that for organizations like us, if there is a departure of a certain clinician, then there are almost immediate replacements from equivalent institutions. In this particular case, while you may have heard the noise of departure, there has already been an addition and there were big advertisements in the papers about it. You may have seen it. We hired a very large team who has just joined us from one of our peers, who sort of replaced that particular team that has gone to that peer. It has been a swap effectively.

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Damayanti Kerai: Yes, okay. My last question is on your regulation with the insurance partners. So have you like done renewing all the insurance contract for this cycle and you have nothing like pending on that part?

Abhay Soi: I mean, of course, this was disruptive in some manner to us in this quarter. It did not happen in the previous quarter. And you may have also read in the papers today that number of complaints of some insurance companies actually moved up by over 40% in this quarter. So obviously, there was a lot of noise because of this sort of disruption.

But all of it has been restored. We have got an increment and a mechanism has also been put in place that there will be annual increments rather than having the sunset periods. Typically, your insurance contracts expire and those negotiations take time. So, now, a process has been put in place where at least with these insurance companies, there is automatic renewal on pre-agreed sort of increments.

Damayanti Kerai: So these annual increments, it is already like pre-agreed or how this mechanism

will work out?

Abhay Soi: That is right, pre-agreed.

Damayanti Kerai: It is annual revision for all the contracts now instead of, say, 2 to 3 years cycle earlier?

Yogesh Sareen: So, you know that this whole issue started from the tariff revision. We were asking for price increase, they were asking for price reduction and that is where this whole stalemate started. So eventually, we

got a price increase. Also, while we got the price increase this time, we also sorted that out for the next year. There is a mechanism in place now. So hopefully, they will live up to it and we should not have the same kind of stalemate coming up. But this is only with the companies that we had a problem, in fact, four companies basically.

Damayanti Kerai: Okay. Thank you.

Abhay Soi: So now with those four, like Yogesh has said, this was the issue at the end of the cycle. Now mechanism has been put in place that you have automatic renewal of the contracts. So, we should not have these issues coming into the next cycle or anything like that now.

Damayanti Kerai: Got it. Thank you, Abhay. Thank you.

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Moderator: Thank you. Next question comes from the Shaleen Kumar with UBS Securities India Private Ltd.

Shaleen Kumar: Hi, sir. Continuing from some of the question asked by the previous participant, possible to get the quantum of increment -- and like -- or at least can we compare like what kind of increment will be getting compared to the past year for insurance company?

Yogesh Sareen: Shaleen, tough to really give actual numbers. We got an increment, that is for sure. It is a moderate one, but then we will not be able to give you a number.

Shaleen Kumar: Okay, but is it in the ballpark of kind of increment?

Abhay Soi: Yes. I would say it is not adverse.

Shaleen Kumar: It is not adverse, right.

Abhay Soi: It is in the ballpark of what we were seeing earlier. It is an upward revision and it is in the same ballpark.

Shaleen Kumar: Got it. Sir, one concern which we kind of -- there is a debate this happened with these four companies, can it happen with others as well? So do you think that can happen? Second, do you think that this kind of mechanism can smoothen out? Are you trying to do this?

Abhay Soi: I think clearly there are learnings when something like this happens, right? It has also led to a lot of noise both ways and inconvenience to patients, not only our patients, but also to insurance companies. And you would have read the article in ET today about the number of complaints, which have increased for these insurance companies. So hopefully, everybody has learned from it.

By the end of the day, and I keep saying this, that medical inflation is in very low single digits. If your ARPOB growth has only been 8% or 9% historically, which has included growth in oncology by around 25%, growth in robotics by around 40%, growth in international patients, etc. and you actually back it all off, what is the real growth in medical costs, apples-to-apples?

And then you apply that to 70% of the billing, because 30% of the billing is for drugs, which are at MRP, in any case. I do not think there is much sort of play there. You keep hearing anecdotally of various issues. But the fact of the matter is, that the apples-to-apples inflation has only been so much, right?

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Shaleen Kumar: Sure. Fair enough. Moving on, I heard partly of your initial commentary maybe I missed something. I heard that you talked about the EBITDA contribution from new facilities in Nanavati and Saket has already accretive. So is it possible to understand that? Obviously, when you started these things there will be some incremental cost which would have hit you in the quarter? Is it possible to quantify that kind of cost?

Abhay Soi: I think what happens is that, and this is something we have guided to in the past as well when people have had concerns on capacity expansion, etc., essentially what you are doing is that you are moving cash from your balance sheet and you are creating an asset.

Because of the operating leverage, you do not have any suppression of margins. In fact, the breakeven is almost immediate and is accretive very quickly. This is

s what

we have seen in the past and this is what we have demonstrated by both of these. Even with whatever little additional cost there is for every incremental bed, which gets commissioned now, your margins will only expand.

And we have had 39% and 30%, respectively, margins from both these units, where we have only started about 60 to 70 beds, right? As we keep adding beds, and all of those beds are coming through now as we speak, and will be all through before end of March, so you should see us continue on this accretive journey.

Now we did have GRAP-related delays but we did build close to 2.5 million square feet over the last 3 to 3.5 years. But the fact of the matter is that there has been a delay of 3 to 4 months, collectively, if you look at it. And nothing has actually moved out more than 6 months from a time line standpoint.

But at one point of time, yes, this is what we had guided to in terms of time lines, etc. On the other standpoint, if you look at it, 3-year projects will take another 4 to 5 months, but it is perhaps faster than what most developers deliver from that standpoint. And we, of course, continue to ask more of ourselves and there has been a lot of learnings in this.

Yes, there are pre-commissioning costs sitting, but it is not as if they are only sitting and not contributing. If you bring doctors along, they perhaps do not contribute as much, but they do contribute to the existing facility. So, it is a little difficult to dissect the costs from that standpoint because there is no management cost as I already explained.

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Largely, the clinician cost is the same. So, you bring more housekeeping people, more nurses, more front-end staff and so on. But they are also training in the current facilities. You're overstaffed in the current facility and then you are going to start to move them into the new one. So, it is a little difficult to thread that cost out because it is not like a typical greenfield.

Shaleen Kumar: Sure. Basically, the reason to ask you a lot of things have happened in the third quarter and that has kind of hurt our profitability. We understand that there is going to be a step jump from third quarter or fourth quarter in terms of profitability because a lot of things have been corrected. And even new facilities are coming, it will just help us to understand what kind of a step jump because one you are ...

Abhay Soi: So, I think these are the smaller factors that have affected the profitability. If you ask me, the two big factors, perhaps, which have affected it would be, first and foremost, the seasonality. Last year, right up till Diwali, we had a very strong vector-borne season, and high occupancies because of dengue, which has happened historically as well.

This year, the rainy period continued straight into winter. There was really no stagnation of water from that standpoint. Normally, what happens is that rainy season gets over, there is a humid period in between and then winter starts. So, in this humid period is when you have the vector-borne diseases. But this year, the rains continued right into the foot of winter.

So, we had very bad seasonality. You may have seen results of other companies as well, particularly North-based companies, and you will see this effect. And the second was, of course, the insurance disruption. Now when the disruption happened with the SAHI companies, we replaced all of that with institutional patients because that is the easier part to replace it with. So, you see our institutional business has moved up. As a result, occupancy did not get impacted, but our quality of revenues got impacted and therefore, our profitability.

Shaleen Kumar: Got it, sir. Got it. So, Abhay, any sense on this quarter, like fourth quarter fiscal like almost end of February, right?

Abhay Soi

i: We are unable to give you any forward-looking guidance. We have never given guidance on forward-looking numbers. We typically only give it in terms of the new capacities that will come in and what the current run rate is, etc.

Shaleen Kumar: Sure. It is okay. Last question from my side if I can. And this is more on the industry level. So a lot of debate again on this as well, it is like too much capacities

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are coming, too many hospitals are coming, and in certain micro markets for example Gurgaon right? And the hospital is, you know, it is not just the local market, it is also the intercity market, where I also come from north, right? So I know that.

But what is your take on it, right? Do you think that a lot of hospitals coming in Gurgaon can impact Saket or customer types are different? Do you think there will

be enough demand because there is enough intercity travel happening . So I just want to hear your thoughts ?

Abhay Soi : I think there are two or three things over here. If you look at it on a holistic level, I think over the next 4 to 5 years, there are about 20,000 beds , which are coming up across the country . It does not really move things . Over 5 years, the capacity increment CAGR is, I think, 5% per year.

Let us look at Gurgaon specifically . Now if a couple of hospitals are coming out of Gurgaon and we happen to be actually one of them . So, I think we are less likely to take a hit at Saket or anywhere else because if any of our doctors want to move to Gurgaon, then they will choose our hospital, right? That is sort of easier , if that location advantage is valuable to somebody.

Shaleen Kumar : Sure .

Abhay Soi : What happens after these 2 or 3 hospitals have come up? There is no visibility of another hospital coming up in the next 5 years , because if there was one coming up, then some plans would have already been passed and something would have been built, which is not the situation, right?

Shaleen Kumar : Yes.

Abhay Soi : Now when that happens, there may be some temporary disruption of costs . So, your clinician cost at that point of time may go up because, one, you are attracting in your new hospital, say, when we put up Gurgaon , so we are going to be approaching doctors and bring them, and paying them more than what they are perhaps earning at the peers.

If somebody is trying to take your doctors again, you know, they are going to offer something more. So temporarily for some time, the cost goes up. But that is also a temporary factor, right? Eventually, if there is any cost inflation, it does get passed off to the customer, and that is what we have seen historically.

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It is not the first time that you are seeing all of this capacity come up . If you look at the numbers, now we are building another 400 beds in Saket, which is the center of the city. Now it is very different from Gurgaon. Gurgaon typically does not affect Delhi but Gurgaon will affect other hospitals in Gurgaon.

Normally, it would have affected our small hospital in Gurgaon, but we are coming up with a big hospital in Gurgaon . If there were hospitals coming up in Mumbai , they may affect our hospital, but a brownfield will always be de-risked.

Keshav Gupta : And directionally what you were asking earlier, right from our own sites, right? So , you can see that the capacity we created in Mohali is already occupied, Nanavati is already occupied. Gurgaon converges entire Haryana and Rajasthan also directionally comes here. So, there is a significantly large pool of demand that gets absorbed by Gurgaon.

Moderator: We will take our next and is from the line of Karan Vora with Goldman Sachs. Please go ahead.

Karan Vora : Yes. Thank you for taking my question. The first question is with respect to insurance. So just wanted to get a sense what would

be the rough share of, say,

top 5 insurers for us as a percentage of hospitals revenue? And when we set up a new hospital, two ways. One is a greenfield and the other one is when we do a new tower in the existing setup, how easy or difficult is the empanelment ? So do we get the same rates as our other hospital in the same city or we have to negotiate from scratch? How does that work ? That is my first question.

Abhay Soi : So, on a brownfield, you do not need to negotiate because the same hospital license continues into the new . So, in case of brownfield, there is no re-discussion or re-empanelment . In case of a new hospital, that means a new hospital license, right? If you do not have the empanelment terms already agreed, then you need to empanel . That means you will make those applications for a new hospital.

Karan Vora : Okay. Got it.

Yogesh Sareen : Yes. Karan, basically most of the insurance companies, we have agreed a category of the hospital. It is CAT 1, CAT 2, CAT 3, CAT 4, right? So , whenever a new hospital comes up, the discussion is always around which category it will fall in. For example, Dwarka now is in CAT 2, Saket is CAT 1 . It is basically that discussion, and then the rates automatically apply. We don't need to get into a negotiation for each hospital separately.

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We already have a set of rates. And obviously, we agree, say, for CAT 1, we know CAT 1 to CAT 2 the difference is so much , and so on . That is how the tariff gets

drive n to the insurance company. Typically, it is not that we have to get into discussion with each hospital , but the discussion is which CAT it will fall in.

Keshav Gupta : And the top 4 insurance companies would contribute about 24 %-25% of the gro up revenue coming from insurance segment .

Karan Vora: And just one clarific ation here. So when you say we have categories determined so how long does it take? So when , say, for example, Gurgaon, whenever it comes online in the next 1 or 2 quarters, what is the expectation of the full empanelment across insurer? Like does it take 3 months, 6 mon ths, 12 months? Any r ough sense there?

Yogesh Sareen : Within 6 months.

Abhay Soi: Yes, because typically, you require NABH and to get NABH, you typically require 6 months of data. Even the institutional patients, take about 6 months to onboard.

And you have differe nt rates for NABH and non -NABH.

So, I think the first gating item for a new hospital would be to get NABH. Also , concurrently you need various licenses in a greenf ield, such a s transplant , blood bank , etc. So, it is not as if on a brownfield, where all existing licenses continue. In a greenfield, you need to apply for e ach one of those license s. You can not just start doing liver transplant or kidney transpla nt or whatever it is. So , the full range of services does not start.

Karan Vora: So basically, broadly 6 to 9 months is where you can get the NABH accreditation as we ll as the insurance empanelment . That i s the rough sense we sho uld have?

Abhay Soi: That's right. But I mean, it depends on hospital to hospital. Now of course, this was the same in Dwarka as well for us. But as you are aware, we started Dwarka last July. By now, we have already been fully occupied on the 300 beds for a while .

And we are alread y planning a brownfield for another 2 60.

We had a breakeven within 6 months ove r there, because it was cash patients, et c.

So, it is about how you also sequence it . I think the very important part over here is how do you preserve cash flow and how do you sequence your bed s. Now we could have started all 250 beds over there . But we have to forecast the number of beds, we have to plan acco rdingly and we have to staff ac cordingly.

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If you are going to staff all the beds, then you are going to lose mon ey, be i t with a brownfield or a greenfield . I

think that somewhere you need to be a l little more tactful ab out it. Our total loss for Dwarka in 6 months till breakeven was about INR 30 crore – in a greenfield in a brand -new micro market, where we d id not have any presence.

Karan Vora: And my second questi on is with respect to -- I think, Abhay, you menti oned on the - in the opening commentary that we should be back with respe ct to growth from Q4 onwards. So just wanted to get a sense. So do we foresee a step -- like a stepped or phase d manner of recovery growth that Q4, you might be partially back and Q1, you should be fully back? Or from Q4, it is a c omplete clean quarter and there should be no one -offs or no deterrent from Q4 itself?

Abhay Soi: I think the big de terrent on our growth has been capacity. I mean, essentially, if we go beyond the seasonality of it and whatever one-time disruptions were , which are back to normal . I mean everything that we have acquired over the last couple of months or the new capacitie s that we set up have b een ramping up very w ell. Two years back, the issue w as where is growth going to come from because the fact is that we were operating at very high capacity and the growth was going to be coming in essent ially from the capacity a ddition. And there has been a delay of a quarter or two. So now that it is online, then we should be back to trajectory.

Karan Vora: And the last question is with respec t to Gurgaon. Sorry, if I missed in the ope ning remarks, like do we expect it to commissio n like by Q4 end or Q1? And w hat would be the impact like maybe a quarter or 2 impact of losses from Gurgaon since it i s like a large Greenfield? Any color there will be helpful ? Thank you.

Abhay Soi: I think towards the end of H1 FY '27 is when we should be able to sort of commis sion the first phase over there. We are not going to give you a ny guidance on the losses there, but you have the history in front of you.

And I will tell you why we are not doing that . Because it is a function of the clinicians yo u are able to get and what sort of programs you are able to start. We are happy to make a bigger loss o ver a shorter pe riod of time and to have a deeper trough . That essential ly means that we have been able to get the clinicians on day 1 in a greenfield .

Karan Vora: Got it. Thanks. That i s helpful.

Moderator: Thank you. Next question comes f rom the line o f Vivek Agrawal with Citigroup.

Please go ahead.

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Vivek Agrawal: So one question on CGHS, ECHS rate revision. So you earlier talked about approximately INR 200 crore kind of a positive impact on revenues, but that is including the impact of discontinuation of some patented drugs, etc., right? So is it possible for you to split it like what is the absolute impact of rate revision? And how much of that is likely to be netted off from a discontinuation of patented drugs etc.?

Yogesh Sareen : Vivek, we already said that the net impact is INR 200 crore . We have netted off the onco impact , because onco is part of that MoU when the price got revised. So, I would say it will be probably INR 280 crore minus INR 80 crore. But I must also mention to you that the whole price increase has not happened in Q3. ECHS has revised the prices only in December. Some of the PSU are asking for new budgets, etc., to increase the prices to the CGHS levels and so that is also yet to come in . Also, the super specialty rates within that category will be available from 1st of April. So, to my mind, the full impact of this will start to come from Q1 FY27. But I think a large part will start to flow from Q4 FY26 .

Vivek, I must also mention that there is a negative impact of GST both on the revenue side and the margin side . Then you have to net off another INR 60 crore out of INR 200 crore number. So, it will be INR 140 crore sustained positive impact in the margin.

margin.

Vivek Agrawal: So actually, is it right to understand like the impact of the discontinuation of patented drug, etcetera, or the negative impact as well as including the GST, etcetera. So that is largely in this particular quarter. Basically, the GST plus discontinuation of patented drug is the net in the quarter and that the positive impact, let us say, around INR 280 crore only on the rate revision that is likely to come from...

Yogesh Sareen: No, the INR 200 crore is sustained impact of CGHS prices. So, the onco effect will continue because under their MOU, they are saying you will have to give discounts on the chemotherapy drugs of 30%. So, some of the drugs where the margin was less than 30%, we discontinued to supply and where the margin is more than 30%, we still continue to supply, but still at a lower revenue .

So, there is impact -- there are two main impacts on the CGHS price increase of the oncology drugs. One is that we discontinued some drugs and second is that some we are giving 30% discounts. So, total of both impacts will be, as I said, INR 80 crore .

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INR 80 crore is sustained impact , so that means net is INR 200 crore . And then you have to reduce out of that the GST impact. So INR 140 crore is the net impact if you ask me on an ongoing basis. So, it is not one time, I am saying sustained impact, that means you will have both going forward.

Vivek Agrawal: Understood. Thanks. This is helpful. And just one more question again on insurance side. It looks like that it has been targeted towards Max, while we are not seeing this kind of impact and for the other -- or some of your other peers, etc., in Delhi NCR. So, any specific reason for this, basically why it has been targeted towards Max? Separately, what kind of the safeguards that you have that the other insurance companies, let us say, does not do it again in future?

Abhay Soi: I think, hopefully, everybody has learned from it. Because I do not think the disruption is only for Max. I think the disruption would also be for other insurance companies. And there is a reason that other insurance companies did not join in. I think it just made some more media this time. And it has been disruptive to patients, and these patients are also customers of the insurance companies. And therefore, you have seen higher number of complaints .

Vivek Agrawal: Understood. And last question on institutional patient share . So, it has gone up quite a bit, 36% and part of this disruption. So, any color how to look at this number, let us say, 1 year down the line or 2 years down the line?

Abhay Soi: Well, I think the important thing is that when you are coming up with capacity, the more capacity you come up with, you will have institutional, exactly to what extent is another matter . But the fact of the matter is still the EBITDA per bed is higher -- even with the lower rates for the brownfield capacity additions. That is what we are kind of demonstrating right now.

Vivek Agrawal: Understood, sir. Thanks, that is it from my side.

Moderator: Thank you. Next question comes from the line of Bansi Desai with J.P. Morgan India Private Limited . Please go ahead.



Bansi Desai : So, my first question is on the growth of our existing hospital beds. So traditionally, if we see barring any seasonality impact, you still managed to see good low teens kind of growth for our existing beds despite the fact that they have been operating at like 75% plus occupancy levels, partially, it could be because of higher onco share or robotics, etc., which you mentioned?

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But as we go forward, do you think theoretically, this has to normalize at some point in time? And for us, therefore, what could be the levers which can keep the growth momentum high over the next 2, 3 years?

Abhay Soi: Look, I think seasonality, if

you look at the past 10 years has been a reality. The big difference is that last year we had big seasonality, and this year, we had actually no seasonality. So, it is a little bit of a double whammy. If you look at the numbers last year, we had some 30% -35% growth as a group year-on-year basis in Q3. So, it was a very high growth. A lot of it was also due to a little higher part on the seasonality. And whereas this time, it is lower. Now having said that, the big jump in the current year was going to be coming through capacity additions. And clearly, there has been a quarter or two of delay as far as that is concerned. So that is something which should have contributed to increase in revenue. And I think as and when we start getting that on stream, we are going to see that.

Bansi Desai : Yes. So Abhay, actually, I was just trying to say that if I actually look at this quarter and if I map it over a 2-year CAGR, this number still suggests a 20% CAGR over 2 years. So, if I remove that seasonality impact, and this is beside the fact that we have not added as many beds?

So, what I was trying to understand is that it does mean that our existing beds are still growing well in that low-teens rate. And we have seen that peers that most mature beds beyond a point once you reached your optimal occupancy levels, will probably come down to high single-digit kind of growth rates, etc. So, I was just trying to understand that from our side, if seasonality were to remain favorable, then do we continue to see the kind of growth that we have seen?

Abhay Soi: There are two aspects to it. One is we are not looking at peaks and troughs as far as seasonality is concerned, and we are just looking at it on a static basis, saying that if you were to put a line straight through to it, then what would the mean be. So, we are not looking at extensive seasonality or it happening or not happening. I think secondly, in terms of where the existing beds are concerned, we have always sort of guided that there are levers. Some of those levers get obfuscated, because we are opening new beds.

But the fact of the matter is the real higher institutional business, which will be getting distilled through it. But it gets obfuscated by the fact that we are opening new beds. So, there are levers in the current capacity, it is not as if they are not. I mean it depends how you look at it, how you bifurcate this.

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Yogesh Sareen: I think also as management our job is to make sure that even the existing hospitals grow. So, on one hand, they can grow on the ARPOB side, the case mix side, etc. and on the other we are also adding more beds in these hospitals. We are trying to do brownfield expansion, where in the same campus, when you add more beds, then the existing hospital will also grow. So, there is no existing bed versus new; it is the same one hospital that we are trying to grow.

For example, wherever we have higher occupancies, we try to get brownfield beds there. So, you know we are now adding 260 beds to the existing Dwarka hospital. That will make the existing hospital grow.

Abhay Soi: In Dwarka, while we are putting the additional beds over there, our current mix has almost got 50% institutional patients over there. And in the first year, as you are going to ramp up capacity, you are going to take all sorts of business. And as you go along, you are going to see that distilling.

Bansi Desai: And my second question is more clarificatory in nature. You mentioned that the GST rate has also had a bearing on our margin. So just wanted to understand this would have not impacted our absolute EBITDA, right? It was just impacted our margins because your realizations go down?

Yogesh Sareen: No, it does because when you bill to the patients, the bill is at MRP, but you do not pay the

GST on the margin because these goods are used for delivery of medical services. So, there is an impact on the margin as well.

Abhay Soi: So that is how Yogesh led to INR 200 crore net of onco drug impact and then minus another INR 60 crore for GST. You are coming to about INR 140 crore as a result of this entire CGHS revision as well as GST.

Bansi Desai: Because I would assume that GST realization or EBITDA would have been net of that tax amount, right?

Yogesh Sareen: Probably we will have to get online separately. I will walk you through as to why it impacts the margins.

Bansi Desai: Okay, okay, got it. Thank you.

Moderator: Thank you. Next question comes from the line of Tushar Manudhane, Motilal Oswal Financial Services Limited. Please go ahead.

Tushar Manudhane: Thanks for the opportunity. Sir, first, clarification, the top 4-5 insurance companies from the 25% of the insurance business, right?

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Abhay Soi: Yes.

Tushar Manudhane: Yes, secondly, we have been doing roughly INR 400 crore -- INR 420 crore plus/minus sort of a capex per quarter. But FY '26, I guess, the target is up to INR 1,900 crore. So are we on track to do that kind of capex?

Yogesh Sareen: Yes.

Abhay Soi: What typically happens is that a lot of the capex is back ended. You don't necessarily relate work done to capex. I do not think it works in tandem necessarily.

Yogesh Sareen: Also, when you do a cash flow projection, you will always do a conservative projection, right? That is how we plan for it. We do not want the project to suffer because of financial closure, etc. So, you will always find that we will be spending less than what we are projecting because of the fact that we project a conservative number.

Tushar Manudhane: Even if I take roughly INR 400 - 450 crore per quarter sort of a capex, subsequently for FY '27, also, it will be higher capex, right? I am not referring to bed addition. I am not even connecting that to bed addition. I am just referring to the amount that would spend for the new hospitals effectively?

Yogesh Sareen: I think, you have the numbers already in the slide. We will revisit the numbers at the end of March '25 in any case, and then probably we will float the new numbers, but as of now you have the numbers available in our presentation.

Abhay Soi: They are fairly conservative numbers. So, we would certainly not be doing more than that.

Tushar Manudhane: And given the current sort of cash flow, which is like roughly INR 300 crore + cash flow from operations, I am referring to. So does it mean that you still have some more debt coming on balance sheet?

Yogesh Sareen: I think the incremental beds that we are getting on stream now, they will start to give us operating margin. And we mentioned that all these beds are EBITDA as well as margin accretive. So, we obviously expect better cash flows from these operations now that the new beds are getting operationalized.

Tushar Manudhane: So, which means effectively INR 2,100 crore net debt is that the number in FY '27 as well. Does that -- I mean is that the safe assumption?

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Yogesh Sareen: I think last time also the question was asked, it will go up by around INR 500-600 crore in terms of net debt, but it will be still less than 1 unless we do any M&A, etc.

Tushar Manudhane: And just lastly, Smart, there has been like almost 5, 6 months delay. So -- and still like the regulatory approval is to come through, so February '26, is that sort of now largely certain or that might get pushed for this?

Abhay Soi: So as far as Max Smart is concerned, the original time given was FY '28. This project has taken us about 24 to 25 months, in its entirety. We expect approvals by end of Feb '25. It has just got ten ready now, and we have just applied for approval. So, there will be no delay in ap

provals. I want to put that also in place. So, it is not as if the project has been lying ready and sort of approvals is what is delaying it. The project has been delivered now and approvals have been applied for and we are expecting it by end of Feb '25. In fact, whatever attendant approvals are, they have been coming through very quickly and very smoothly. Actually, in this one, we have been ahead of schedule. This one and Zirakpur, we seem to be ahead of schedule.

Moderator: Thank you. The last question comes from the line of Nitin Agarwal with DAM Capital. Please go ahead.

Nitin Agarwal: Thanks for taking my question. Abhay, you have talked a couple of times about the

Dwarka hospital and the fact that we have seen encouraging progress on it to go in for expansion. If you can just give us some more colour on what's been the progress -- financial progress of the hospital in terms of where it is reached right now, which prompted you to go for expansion at this early stage?

Abhay Soi: We are already operating at close to 75% capacity, although almost half of the business continues to be institutional. Like I said, early -- the life cycle of a hospital, you would first think about ramping up occupancy. And you ramp up occupancy with all sorts of business, and then you start distilling it. So we are already doing 20% margins at Dwarka while 50% of the business is institutional. When we put up capacity going forward, it will take us at least 2 years to put up that capacity. That means in the better part of two years, I would not have enough beds, easy to surmise that from 75%, we will probably go to 85%, so we will be distilling our beds. We are already seeing month-on-month, there are more cash and insurance patients coming through and the institutional patients are reducing. And therefore, you will see the margins move up. That is the encouraging part.

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Nitin Agarwal: And secondly, on the Jaypee Hospital, can you give us any update on the progress on that?

Yogesh Sareen: So Jaypee Hospital is doing well now. I think the occupancy has improved over the period in the hospital. So, are you looking for some specific numbers for that?

Nitin Agarwal: I am just curious in terms of where -- from where you acquired to where it is come to, what the distance we have covered in terms of performance improvement in the business?

Yogesh Sareen: So I think if I consider year-on-year, it will be more than 30% growth. We acquired this in same quarter last year. The revenue is up by 30%-35% and EBITDA has also improved. Obviously, we cannot give you specific numbers in terms of revenue but I think that is the status. And when I say 30% increase in revenue, that obviously means that the revenue has come down first because when we acquired the hospital, we stopped all the referrals, etc., the revenue tanked a bit and then we brought it back.

So, this 30% growth Y-o-Y same quarter is basically after that dip, which happened in the first quarter after acquisition and then we built it up. The real growth would actually be around 40%.

Nitin Agarwal: And in terms of profitability, is it now closer to your network profitability? How far is it from there?

Yogesh Sareen: No, it is not. It is less. So, I think the first endeavour was to stabilize the operations. We had a lot of complaints in that hospital, so we were working on that. I think the margin is probably 3-4% lower than the overall margin that we have in the Network.

Nitin Agarwal: And secondly, when we look at our business for the next couple of years, the EBITDA per bed for us has been a pretty dramatic journey, which we have had over the years, around 7.5 million to 8 million is where we are at. I mean, does the network EBITDA stabilize around that? Or do you see opportunities for us to significantly increment

it from these levels? And how should we think about EBITDA per bed take a 2-3 year view from here?

Keshav Gupta: No. We do not go towards the direction for chasing EBITDA per bed also as a base. Our trajectory is to deploy capital efficiently to have yields on the capital.

Abhay Soi: We may have a lower EBITDA per bed, but you have to focus on ROCE over there. I mean we are not focusing on making EBITDA per bed or ARPOB accretive or whatever. We are focusing on ROCE.

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Nitin Agarwal: And last one. Over the next 2 years, from where do we see our operational beds sort of fitting out, we are about 4600 operational beds right now. Where do we end up in the next 2 years?

Abhay Soi: We are around 4,800 right now. And we are adding another 1,200 beds, including Gurgaon. So that is around 6,000 and by FY '28, we should be hitting 8,000. It is available in the presentation.

Nitin Agarwal: About 8,500 beds should we look at that...

Abhay Soi: Please look at the presentation for the exact year-wise listing. The presentation has beds mentioned location wise and how many brownfields, how many greenfields, etc.

Nitin Agarwal: With respect to the delays and all that you foresee by the time we finish FY28, you still brought more or less in the same ballpark that we had in the presentation, is what I meant.

Abhay Soi: That is right. So, I mean we have given it out in the presentation, updated numbers. The current quarter numbers will be updated for any delays.

Nitin Agarwal: Okay, perfect. Thank you so much.

Moderator: Thank you. Ladies and gentlemen, we have reached the end of question-and-answer session. I would now like to hand the conference over to the management for closing comments.

Abhay Soi: Thank you, everyone, for joining us today. We appreciate all your time. We look forward to interacting with you again next quarter. Thank you very much.

Disclaimer : This is a transcription and may contain transcription errors. The transcript has been edited for clarity. The Company takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy .

## Call: May 2026

May 29, 2026

Listing Department ,  
National Stock Exchange of India Limited  
Exchange Plaza, Plot C -1, Block G,  
Bandra Kurla Complex, Bandra (E),  
Mumbai – 400 051

Symbol: MAXHEALTH Listing Department,  
BSE Limited  
Phiroze Jeejeebhoy Towers,  
Dalal Street,  
Mumbai – 400 001

Scrip Code: 543220

Sub.: Transcript of Earnings Call held on May 22, 2026

Ref.: Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015

Dear Sir / Madam,

Please find enclosed copy of transcript of earnings conference call, organised on May 22, 2026, on financial results of the Company for the quarter and financial year ended March 31, 2026.

The said transcript is also available on the website of the Company at [www.maxhealthcare.in/financials#earnings-call](http://www.maxhealthcare.in/financials#earnings-call).

Kindly take the same on record.

Thanking you

Yours truly,  
For Max Healthcare Institute Limited

Dhiraj Arora  
SVP - Company Secretary and Compliance Officer

Encl.: As above

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Max Healthcare Institute Limited  
Q4 & FY '26 Earnings Conference Call  
May 22, 2026

Moderator: Ladies and gentlemen, good day, and welcome to Max Healthcare Institute Limited Earnings Conference Call. Please note that this conference is being recorded.

I now hand the conference over to Mr. Suraj Digawalekar from CDR India. Thank you, and over to you, sir.

Suraj Digawalekar : Thank you, Neeerav. Good morning, everyone, and thank you for joining us on Max Healthcare's Q4 and FY '26 Earnings Conference Call. We have with us Mr. Abhay Soi, Chairman and Managing Director; Mr. Yogesh Sareen, Senior Director and Chief Financial Officer; and Mr. Keshav Gupta, Senior Director – Growth, M&A and Business Planning. We will begin the call with opening remarks from the management, following which we will have the forum open for an interactive Q&A session.

Before we begin, I would like to point out that statements made today may be forward-looking in nature, and a disclaimer to this effect has been included in the earnings presentation shared with you earlier.

I would now like to invite Abhay, to make his opening remarks. Thank you, and

over to you, Abhay.

Abhay Soi : Good morning, everyone, and thank you for joining us on Max Healthcare's earnings call for fourth quarter and full year ended March 2026.

Let me begin by highlighting that over the last 2 quarters, we have rolled out phased commissioning of more than 20% additional brownfield capacity across our hospitals in Mohali, Nanavati in Mumbai and Max Smart in Delhi . All the beds will be ready to be operationalized over the next 2 to 3 months. Further, we expect to add another 10% capacity once our 500 -bed greenfield hospital in Gurgaon is commissioned during the year. We have already onboarded clinical and non - clinical talent for these capacities and expect significant operating leverage to come through as operations progressively ramp up.

We are also pleased to share that we have completed the acquisition of a controlling stake in Kalinga Hospital Limited this month. Kalinga owns and operates a 250 -bed hospital on a prime 10 -acre land parcel in the heart of Bhubaneswar . The acquisition marks our entry into Eastern India and provides us a strong platform with established clinical programs and significant potential for future expansion at the existing site. We have firmed up plans to revamp and expand the facility.

Page 2 of 16 Further, the Board has approved an investment of INR 1,400 crore for the construction of a 700 -bed greenfield hospital at Shaheed Path, Lucknow. This investment reflects our continued confidence in the region where we have seen encouraging momentum since the acquisition of our existing facility. The proposed hospital will add meaningful bed capacity and position us to serve the growing demand for high -quality health care services

in one of North India's important health care markets.

With respect to the Q4 performance, the Network delivered its 22nd consecutive quarter of year-on-year growth , with the revenue increasing by 10% and operating EBITDA by 8%.

As we move into FY '27, our priorities remain focused on scaling the recently commissioned capacities, integrating Kalinga Hospital into the Network , and progressing our outlined expansion projects, including the Sector -56 Gurgaon hospital . At the same time, our existing hospital operations continue to provide a steady foundation , supported by strong clinical capabilities and consistent execution across the Network. This positions us well to deliver sustained growth while maintaining capital discipline.

Now , coming to the Q4 performance highlights :

- Average occupancy for the Network continued to be more than 75% despite increase in operational bed capacity , with most of the units operating at near optimal capacity.
- Occupied bed days (OBDs) were up by 8% year -on-year and 4% quarter -on-quarter.
- Average Length of Stay (ALOS) was temporarily higher by 9% compared to Q4 last year, characteristic spurt due to multi -location capacity rollout simultaneously.
- Average Revenue Per Occupied Bed (ARPOB) for the quarter stood at INR 77,900. This was after absorbing the impact of higher ALOS and discontinuation of select high -value chemotherapy drugs for institutional patients.
- Network gross revenue stood at INR 2,664 crore compared to INR 2,429 crore in Q4 last year and INR 2,608 crore in the previous quarter. This reflects an increase of 10% year -on-year and 2% quarter -on-quarter. Due to discontinuation of select high -value chemotherapy drugs for institutional patients, share of oncology in in-patient revenues dropped to 21% from 26% in Q4 FY '25 and 24% in Q3 FY '26. Excluding oncology, gross revenue grew by 15% year -on-year and 5% quarter -on-quarter.
- International patient revenue was INR 227 crore , registering a growth of 12% year-on-year and accounting for 9% of the revenue from hospitals.
- Digital revenue from online marketing activities, web -based appointments and digital lead management was INR 838 crore, accounting for approximately

Page 3 of 16 31% of the overall revenue. Website traffic crossed 90 lakh sessions during the quarter, growing by 39% year -on-year.

- Network operating EBITDA stood at INR 682 crore, reflecting a growth of 8% year-on-year and 5% quarter -on-quarter.

- Network operating EBITDA margin was 26.8% for the quarter, compared to 27.2% in Q4 FY '25 and 26.1% in the trailing quarter.
- Annualized EBITDA per bed for the Network stood at INR 73 lakh, versus INR 74 lakh in Q4 FY '25 and INR 71 lakh in the previous quarter. This was also reflective of the higher ALOS.
- Profit after Tax (PAT) for the Network was INR 387 crore, against INR 376 crore in Q4 last year and INR 344 crore in the previous quarter.
- The Network generated free cash flows of INR 581 crore during the quarter. INR 328 crore was deployed towards ongoing capacity expansion projects and facility upgrades at newer facilities.
- Net debt for the Network stood at INR 1,908 crore compared to INR 2,166 crore at the end of December '25, and the Net debt-to-EBITDA ratio continued to be less than 1.
- Continuing our efforts to support the local communities, we provided free treatment to approximately 42,000 patients from economically weaker section of society, worth INR 59 crore at hospital tariff.
- Both our strategic business units continue to deliver steady growth in revenue and profitability :
  - o Max@Home reported revenues of INR 73 crore, reflecting a 30% year-on-year growth. It offers 16 specialized service lines across 15 cities, with over 56% repeat transactions.
  - o Max Lab reported a revenue of INR 52 crore, reflecting 14% year-on-year

growth. It provides services in over 60 cities and served nearly 6 lakh patients during the quarter.

- Now, moving on to the status of our expansion projects coming on stream in the next 2 to 3 years :

- Max Lucknow : The current capacity of the hospital stands at 426 beds, and we expect this to increase to 570 beds over the next 2 quarters.
- 500 beds at Sector 56 Gurgaon : Interior and facade works have started. We are targeting to commission this facility by the end of this year.
- 100 beds at Max Nagpur : Project work continues to be on track, and we expect commissioning by FY28.
- 400 beds at Zirakpur, Mohali : Structural work is ongoing, and we are on schedule to commission the hospitals in FY28.

Page 4 of 16 v. 260 beds at Max Dwarka : Building plan submission is underway and the project is expected to take 24 months to complete.

- 200 beds at Max Vaishali : We are waiting building plan approvals, while all other clearances in place. Project is expected to take 24 months post receipt of approvals.

- 400 beds at Max Patparganj : D-wall construction has started, and we expect commissioning by FY29.

And finally, coming to the overview of the company's performance for the full year ended March 2026 :

- During the year, we have initiated phased commissioning of nearly 20% additional brownfield capacity across the Network.
- Network gross revenue stood at INR 10,538 crore, reflecting a growth of 16% year-on-year.
- Overall Network operating EBITDA grew by 14% year-on-year to INR 2,638 crore, translating to a margin of 26.2% and EBITDA per bed of INR 72 lakhs.
- Profit after tax (PAT) for the Network increased to INR 1,631 crore compared to INR 1,336 crore in FY25, registering a growth of 22%.
- During the year, we generated INR 1,541 crore of free cash from operations after interest, tax, working capital changes and routine capex. Further, INR 1,627 crore was deployed towards ongoing expansion projects and facility upgrades at newer units, INR 131 crore towards land purchases at Vaishali, and INR 146 crore was distributed as dividend.

With this, we open the floor for any questions you may have.

Moderator: Thank you very much. The first question is from the line of Neha Manpuria from Bank of America Securities.

Neha Manpuria : My first question is on the brownfield beds that we have added. When do we start seeing them contributing to EBITDA more meaningfully? Did I hear it correctly that all of these brownfield beds will be commissioned in the next two quarters. So, second quarter, third quarter is when they should start showing up more meaningfully on EBITDA?

Abhay Soi : So they are already contributing to EBITDA and it is not any form a negative contribution . But what happens is that you sort of get the better end of operating leverage as you go along because right now, for the total 1,000 -odd beds , we have initiated a phased rollout of less er beds.

Say, if you have 400 beds at Max Smart, which are being rolled o ut in a phased commissioning, you would have started with about 100 beds over there. So , as you open up the balance beds over the next couple of quarters, you will see the ent ire operating leverage as the balance beds get occupied because your costs related to even the brow nfield are not linear effectively.

Page 5 of 16 Neha Manpuria : Okay. And is it fair to assume that the occupancy and these ramping up should not be a problem ? We should g et to a fairly good level of occupancy as soon as we start these beds. That should not be a problem , right? That would be a fair assumption.

Abhay Soi : Yes. So , I mean it is a two-way thing. I mean , we do not open beds if we do not have occupancy, but what we have seen is a very good ramp -up of that occupancy. And therefore, you have seen in spite of ne w beds opening up, occupancy remains high. But having said that,

I must also point out what is embedded within it is also higher ALOS. So , what tends to happ en is you are just a little more efficient when you do not have the beds.

When you open up th e new beds, there is a tendency for the ALOS to increase. It is slightly temporary in nature. We have tightened it again, but you have seen th at ALOS h as gone up by 9%. So, the impact of that is that shows up in both your ARPOB and occ upancy – while the occupancy seems a little higher, your ARPOB comes through a little lower.

Neha Manpuria : Understood. My second question is on Gurug ram. Did I hear you correctly tha t we are now expecting Gurugram commissioning by the end of th is fiscal year? I mean I am not sur e if I picked that up correctly?

Abhay Soi : Yes, we are expecting commissioning by the end of the year.

Neha Manpuria : Okay. And we should not be expecting any further delay on that because that is been pushed out a few times now.

Abhay Soi : That is right.

Neha Manpuria : Okay. And sorry, one last question, if I may. On the Bhubaneswar asset that we have acquired, this will start integrating from first quarter itself? Or is there any approval, etc , that we requi re before closing this?

Abhay Soi : Yes, from t he first quarter . We have already acquired the majority stake so we will be consolidating it.

Neha Manpuria : Okay. I have a f ew more questions, but I will get back in the queue. Thank you.

Abhay Soi : Yes.

Moderator: Thank you. Next question is from the line of Banshi De sai from JP Morgan.

Banshi Desai: Yes, thanks for the opportunity. So just again on Gurugram, how should we th ink about the operationalization of beds, assuming we commission towards the end of fiscal '27? What wi ll be the Phase 1 operationalization? And what is the count that we should expect in fiscal '28?

Abhay Soi : So, I think in fiscal '28, we will be looking at breaking even within the year . I mean , it is a greenfield, as you are aware. Having said that, our experience with the Dwarka greenfield, we operationalized it , we guided to a 1 -year breakeven , but we actually broke even in 6 months. We had an operating loss, consolidated EBITDA loss in the first 6 months of about INR 35 crore. However, by th e end of the 12 - month period , it was less than INR 10 crore . Even if that number is more or less in

Page 6 of 16 this case, it is not a meaningful change to perhaps what the projections are going to be.

Banshi D esai: But in terms o f beds, are we expecting phase -wise manne r of operationalization here, because it's a 500 bed facility ...

Abhay Soi : Yes. You always do it , even tactically, in that manner . So physically, the beds come out in phases, but even so tactica lly because if you h ave 500 beds, day one, you are not going to have occupancy of 500 beds. So , you do not operationalize or staff all the 500 beds. So , if I take the example of Dwar ka, we had 300 b eds, we started with 140 beds. We ramped up occupancy, bro ke even with the 140 . So, the balance beds start to yield as you go along.

You are seeing a similar sort of story play out in our brownfield s right now. And with respect to even the green field at Gurgaon, you are going to start with, let's say, about 200 -odd beds. And once y ou kind of break even within that, then you start rolling out the balance beds.



Bansi Desai: Understood. That is clear. And my second question is on the onco share decline that we have seen in Q4. While clearly, the reason highlighted is a discontinuation of chemo drugs, but it still feels a bit sharp, given we had quantified onco drug impact to be about INR80 -odd crore. So, if you could help us understand what has happened here? And by when do we expect this to reverse?

Abhay Soi: There are two things. You have the onco drugs, that is day care, the margins on which were coming out a little perverse to us. So, discontinu

ation of these high -

value drugs not only impacts your top line, but also impacts your OBDs (occupied bed days), which are related to it because some of the patients, which are coming for day care, are also admitted in the night. So, it has a knock-on effect on that as well. Our OBDs have sort of come down by about 5% to 6% in oncology, which is related to this.

Bansi Desai: So, I mean, do we have a plan in place, how do we replace it? What alternative protocols would you have?

Yogesh Sareen: Basically, some of this is permanent because we know that we will not be able to do this kind of a business on the minus-margin basis. As you have also seen in this quarter, although the OBDs have degrown by 6% in oncology, but we have overall OBDs have grown. That means the other specialties have been able to compensate for it. So, I think that is the plan even going forward, that we do not expect the share of oncology come back to 25 -26% as it was earlier. It will continue to hover around 21 -22%, and we will then have the other specialties fill up that vacuum.

Bansi Desai: Got it. And the CGHS rate revision benefit, has that started to flow through in Q4?

Yogesh Sareen: Yes. All except the super specialty rates. A very large part is already in, but there is a small part, which is left out, which is around INR 25-30 crore per annum. That will be phased out over this year. It has started to come in two hospitals, but balance is still pending. But that is all in this quarter.

Abhay Soi: We are expecting that to come in the next few months.

Bansi Desai: Thank you.

Page 7 of 16 Moderator: Thank you. Next question is from the line of Damayanti Kerai from H SBC Securities and Capital Markets (India) Private Limited.

Damayanti Kerai: My first question is clarification. Abhay, you mentioned you are rolling out beds in a phased manner, even for say facility like Smart. So, help me to understand this better. In the past, whenever you have opened or commissioned brownfield facility, I understand the ramp-up happened much faster than what we are seeing right now. So according to you, anything has changed since then? That is why you are going for more gradual phased way of prescribing beds.

Abhay Soi: No, we have always opened brownfield in a phased manner. Whether brownfield or greenfield, it has always been opened in a phased manner. And it is opened in a phased manner because as soon as any part of a new facility, any floors are ready, there is always a tearing need for those floors. And you have seen that play out in the occupancy as well. So, we try to put it to work as soon as possible. So whichever floors are ready, and it is the same at Nanavati, it is the same at Mohali, it is the same at Smart. It has been same in the past at Shalimar Bagh or Vaishali or every facility, which we have rolled out has been rolled out in this manner.

Keshav Gupta: We mentioned in the speech also, our available capacity of operational beds have gone up by 8%. So, the OBD has gone up 8%. As and when the beds are ready, they are being taken up. In context, the Smart beds have become live within 26-27 months of the project starting. So typically, a project of this size takes about 36 months. But because we are taking our projects to operations within a shorter time, floor by floor, hence, they are also being taken up floor by floor.

Damayanti Kerai: Sure. And when it comes to ramp-up of some of the newer facilities. You mentioned about Dwaraka. Similarly, can you update on the status of the Noida unit, how it is ramping up in terms of occupancy, etc.? Last quarter, if I remember correctly, you mentioned there are some issues which you are trying to resolve. So, if some update you can share on Noida unit?

Yogesh Sareen: So, the Noida has ramped up well now in this quarter. So the occupancies have

been more than around 64 -65%. There is further room to grow. But on the revenue side, it has done well compared to last quarter. And we have also hired doctors in that hospital. I would say we are happy with where we are with respect Noida when it comes to quarter-on-quarter growth.

Abhay Soi: Yes, I think it has pulled around very well. And in terms of EBITDA, also, it is a substantial growth that we have seen over there. It is well on its way.

Damayanti Kerai : It is in line with our expectations, right?

Abhay Soi: Yes, absolutely. We are very encouraged by the last quarter and more.

Damayanti Kerai : Okay. And it can very well go to the Network level occupancy of say 75% or so ?

Abhay Soi: Absolutely. I think maybe shortly, we will need to do a brownfield over there and add more beds as well.

Damayanti Kerai : Okay. Good to hear. And my second question is a clarification on discontinuation of oncology drugs. You mentioned 5 -6% OBD got knocked off because of it. Just wanted to understand, these drugs are like high in terms of ticket size, etc. but were they meaningful contributor at the EBITDA level also?

Page 8 of 16 Yogesh Sareen : Yes, earlier, they were given at MRP, and we had 15-16% margins on those. So that used to flow to the EBITDA .

Abhay Soi: It was a 16% margin . But because now we have to give a 30% discount to MRP , so we sort of discontinued it .

Yogesh Sareen : Earlier they were contributing to EBITDA . So, that is the reason when we gave to the price impact of the CGHS, we netted off that. To that extent it has been absorbed through the price increase .

Abhay Soi: We suggested I think it is a INR 200 crore net benefit...

Yogesh Sareen : INR140 crore after GST.

Abhay Soi: After GST , it was INR140 crore . With the CGHS rate revision, while we have got higher rates, but we got capped out on this high -value drugs. So , from a CGHS standpoint, the net benefit was INR 200 crore . There was also an impact of GST, so net of it the total benefit was INR 140 crore .

Damayanti Kerai : INR200 crore, you mentioned it is a gross number and then when you include the GST impact, etc., INR 140 crore is the number, which was coming from it...

Yogesh Sareen : That is right. Of this INR 30 crore, as I mentioned is super specialty rates, which has not flowed in yet.

Abhay Soi: So that is about INR 100-110 crore has flowed in. About INR30 -40 crore is yet to flow.

Yogesh Sareen: This is all annual numbers .

Damayanti Kerai : Sorry, just to process , another INR30 crore flow out from here?

Abhay Soi: Yes. So out of INR 140 crore, INR 30-40 crore is yet to flow in. About INR100 - 110 crore has already flown in on an annualized basis.

Damayanti Kerai : And I think my last question is on the pipeline projects, which you indicated, which are coming up in '28 or so. Is any facility there where we are seeing some delays, etc., in terms of approvals or it is just like completion of the facility, which should be done as per your indicated timeline? So, any regulatory clearance or any other clearances which are due?

Abhay Soi: No, nothing . No regulatory clearance is pending over and above what has been anticipated. In the past, there have been delays because of GRAP 3. We have had delays because of shortage of manpower due to the LPG crisis .

Forest approval was delayed because there was an issue with respect to Delhi tree transplantation where the Supreme Court had taken cognizance of . There was a matter between the Lieutenant Governor versus the Supreme Court - contempt of court matter because of which it got stalled.

So, these are not typically regulatory approvals, which have been sort of delayed .

But I think we had issues and incidents with respect to pollution and shutdowns of construction or tree transplantation or Iran war causing shortage of LPG and

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herefore, manpower not showing up at site.

Page 9 of 16 Damayanti Kerai : Okay, that is helpful. Thank you. I will get back into the queue .

Mod erator : Thank you. Next question is from the line of Karan Vora from Goldman Sachs.

Karan Vora : The first question is with respect to doctor costs. So , we see that our doctor costs have gone up and we have hired in advance. But just wanted to get a sense on for which all expansions have we hired and like which are the hospitals where further doctor additions are still pending, which might hit the cost line item in the next one to two quarters?

Abhay Soi : That is a very good question. So , there are two things. End of 2024, we essentially added close to 25% to 30% more capacity. This was whether through Dwarka , whether through the Jaypee acquisition, Sahara done some time before that or Alexis in Nagpur. This year we have already started a phased rollout of 20% plus more capacity, which includes Mohali (Punjab ), Nanavati (Mumbai), Smart (Saket ) as well as we added more beds in Lucknow as well.

So, somebody asked me a question before this about Noida. And we have seen meaningful improvement over there because we have expanded our doctor base.

We have seen the same in Luck now where we are ramping up . We have seen the same in Smart. We have seen the same in Nanavati, Mumbai. We have seen the same in Mohali.

So, all of these new ones which are coming, we have added doctors. Even Dwarka, now it is operating at 80-85% capacity. But through the last quarter, we had added some more people over there as well. So yes, so I think it has been pretty much quite secular across the portfolio because it has been a multi-location addition of capacity . You are going to see the same thing in Bhubaneswar now because we will add manpower over there. Although it should not move the overall needle simply because there is just one hospital. But the minute you are doing it in 4 or 5 multi-locations and at the same point of time, you are going to see a little bit of lumpiness.

Karan Vora : Okay. Got it.

Abhay Soi : The reason I mentioned in end of '24 is that when you do sudden acquisitions, it is not as if you are able to do all your hiring in the next 6 -12 months. It takes time and moves in a phased manner , particularly if there is competition in those areas.

Karan Vora : Okay. Got it. And sir, just to better understand, so this INR 435 crore for doctor fees in Q4, that should not materially change going forward, at least for the next few quarters. Is that the fair way to think about it?

Abhay Soi : Yes, it should actually start getting operating leverage in fact. My belief is that marginally all of this percentage starts coming down

Karan Vora : Got it. And this Kalinga Hospital. So , any start-up losses or like what about breakeven timelines? How should we think about that?

Abhay Soi : It is already profitable. I think there is about INR 10-odd crore of EBITDA. So, you are not starting with a negative .

Karan Vora : INR 10 crore per quarter ?

Page 10 of 16 Abhay Soi : Annual.

Karan Vora : Annual. Okay. Got it. And so , we have like unlike some other places where we had to discontinue businesses, right, because we wanted to streamline practices. All those things have been taken care of. And even after that, we should be able to maintain broadly that EBITDA run rate. So , no chance of it going into negative, that is what I am trying to.

Abhay Soi : Let me just correct you . Even in the other places, where we corrected the business behavior, so to say, it did not go into negative, we just made less profit. None of these acquisitions have been loss-making or even after taking over when we did whatever actions we took, it did not go into a negative territory.

Karan Vora: Okay. Got it. Thanks for the clarification.

Abhay Soi

: Even in this case, it is INR 10 crore positive and we have just done the acquisition. So, you are going to see it ramp up and in case we are taking some actions, you still would not see meaningful negative numbers .

Karan Vora : Got it. And the last question would be with respect to the new units or whatever we have operationalized in the last 12 to 15 months, how have their overall revenue and margin trajectory look like, any color there will be helpful?

Abhay Soi : Basically before rolling out this 20-25% capacity rollout that we have recently done over the last 3 to 5 months , the previous generation for about 12 -13 months, we did not really add any capacity. We added it before that, again, which was a little lumpy. We did Lucknow, Nagpur, Dwarka , Jaypee ; so are those the four you are referring to?

Karan Vora : Yes.

Abhay Soi : Okay. I think all four have done significantly well. Lucknow, for example, is doing pretty much 5x of EBITDA of what we acquired it for. The meaningful addition in Nagpur, and in Noida also we have seen that . Dwarka is not only operating at 80 - 85% capacity utilization, but we are already planning a brownfield over there of another 200 -plus beds. In fact, oncology bunker over there and the oncology center over there is in Dwarka yet to start, which is going to start by next month. So that should reap even further benefits. So, we are seeing those benefits come through.

Karan Vora: Got it. Helpful. Thanks.

Abhay Soi: Yes. But you are going to see a bit of a hockey stick there because you have the benefit. When you move from like, let us say, in Noida, when you move from the 65% to that 75 -80%. You see at 50 -55%, you break even right, most units. At 60 - 65%, you start doing very well, but the real juice starts coming from that 60 -65% up to that 80 %-odd occupancy that we do.

Moderator: Thank you. Next question is from the line of Tushar Manudhane from Motilal Oswal Financial Services Ltd.

Tushar Manudhane : Thanks for the opportunity. This was more on the Lucknow side, while we have a very decent land bank as far as Sahara hospital is concerned. And, of course, in the history you see Shaheed Path land was also acquired more or less at the similar timeline. So how are we evaluating in terms of which land bank to sort of set up? I

Page 11 of 16 understand that the Board has given sort of approval for Shaheed Path. But if you could help understand how are we going to sort of utilize both the land banks and build up Lucknow sort of hospital network.

Abhay Soi : So, I think it is a very good question. We have a land bank of about 27 acres of land at Gomti Nagar, which is right in the heart of Lucknow, and 5-plus acres, which is on Shaheed Path. We are expanding our capacity over the next few months, you will see our capacity of Gomti Nagar go up to about 570 beds. What we intend to do is we intend to start another hospital. We want to have a multi-location strategy of about 700 beds in Shaheed Path, which we will be operating in a phased manner. It should take about 3 years to build it.

Simultaneously, we believe we will be running out of capacity at Lucknow at Gomti Nagar as well. So, we will be adding another 200-300 beds over there. So, we are going to have a multi-pronged strategy. I think one does not have much to do with the other because these are 2 locations, which means we will have two sets of doctors and clinicians. It allows you to do that.

You see, if we have one, in terms of the strength of senior clinicians, you have one Chairman cardiology, one Chairman oncology, etc., so it becomes a choke point for other senior personnel coming in. When you have multiple locations, it allows you to do that.

And we have seen that advantage play out for us in Delhi, in other places, etc. so, we like the whole cluster approach. I mean, you have benefits of

brownfield, but it

is also beneficial to have a cluster approach and then have brownfields emerging at multiple locations.

Tushar Manudhane : Understood. So, this is not so much more sort of a cannibalizing given that it is hardly a 14 kilometers distance, it will be more like a multi-location playing out?

Abhay Soi : Absolutely.

Yogesh Sareen : Yes. Also, on this Shaheed Path, the capacity can go up to roughly 900 beds. So, I mean, there is more we can build there even after these 700 beds.

Tushar Manudhane : So, I mean, it reflects the kind of confidence you have on this Lucknow as a location, building up such a strong almost it will be about 1,000, in fact, more than 1,000 plus sort of a bed size eventually and not like immediately, but over a period of time?

Abhay Soi : Absolutely. Let me put it this way, over the next decade, I see even probably Gomti Nagar going close to perhaps 2,000 beds, in just that one location. We will do it in a phased manner. We have the land bank over there. And the kind of ramp-up we are seeing is because our clinical programs are very strong over there.

Tushar Manudhane : Understood, second, sir, are you seeing the risk of these medicines being taken directly by the, let us say, health scheme or CGHS in other therapies?

Abhay Soi : Firstly, because of being in Delhi, we have a larger amount of institutional business, which was coming to our Delhi hospitals and this impacts that largely.

What they basically said is that we have to provide these drugs at 30% discount to MRP.

Page 12 of 16 And so, they have not selected the drugs. They said all medicines at a 30% discount to MRP or alternatively CGHS, patients can get the medicine from CGHS. So, from a hospital standpoint, if your margins are less than 30%, then you probably discontinue it because it becomes loss-making, but the patient has the option of buying it from the CGHS dispensary.

But our present margins, on a drug-on-drug basis also is reflective of that action. It is not a government action, it is a CGHS price revision. So, you get some and you lose some, I guess. The net benefit is INR 200 crore. After GST impact, which is a separate hit that we got, it becomes INR 140 crore net benefit.

Tushar Manudhane : I got you. Sir, just that clarification, this is onco therapy specific or this is more between CGHS doing it across the therapies?

Abhay Soi : All chemotherapies.

Tushar Manudhane : Yes, this is why I was trying to ask that, is this getting expanded to other therapies where the patient...

Abhay Soi : No. This is the first time they made any changes on the price in 14 years.

Tushar Manudhane : Understood. And practically, if CGHS is able to provide the medicines on a timely basis? Or do you think the patient will come back to the hospital pharmacy or the

hospital ?

Yogesh Sa ree n : I think there i s a lot of noise among the CGH S pensioners, etc due to delays .

Abhay Soi : We have been given to understand that they are reco nsidering it. But again, one does not know . There has been a lot of representation made even by patients on this.

Tush ar Manudhane : Got it, sir.

Abhay Soi : One of the problem s is that this also applies to ECHS, for example and other P SUs.

But while CGHS has dispensar ies to dispense th ese dr ugs, ECHS and other PSUs do not have the ir dispensar ies and they cannot access CGHS dispensar ies. So, the problem with these drugs are that they are not accessible to them.

Tushar Manudhane : Got it. So effectively , this should sort of start getting reflected back probably in maybe some time in future ?

Abhay Soi : Sorry, it should get reversed you are saying?

Tushar Manudhan e: Yes.

Abhay Soi : I mean it stands to reason.

Tushar Manudhane : Understood. Got it.

Abhay Soi : Otherwise, we will have service men who are not able to access these, which is presently the case.

Page 13 of 16 Tushar Manudhane : Got it. And just lastly, with Gurugram, we h ad to sort of start second half. So , is it like now we are indicating in for a few mo nths, sort of taking some time? Is that the way to think about?

Abhay Soi : No, that is right. We were saying second half and we mean t middle of second half , and now it is end of th e second half. But there have been two issues over here.

One was a lot of labour went back during elections , particularly Bengal elections. It was a bit of a festive season for them from that standpoint. So yes, we had a lot of reduction in manpo wer at the s ites. Second is also the LPG issue. Most labour , they cook their own food , so we had disruption over there because the labour did not have LPG . We have started serving meals for all labour at our sites now , in order t o surpass that .

The labour is not on our books , they are the contract or's labour . So, there is a particular method that you have to follow to even initiate that because literally, we have got about 1,100 -1,200 people working on the Gurugram site . So, our kitchen is supplying meals to the 1,100 -1,200 p eople . So yes, these are the issues which have caused a delay o f a couple of months.

Tushar Manudhane : Okay. Thanks f or that clarification. And just lastly, if I may, just one more, if I may squeeze in . With respect to doctor talent cos t, give n the kin d of bed addition and starting of hospitals by multiple corporates probably over the last , maybe couple of years and subs equently over the next 4 to 5 years. Are you seeing this doctor talent cost sort of the negotiating power moving to doc tors from corpor ate?

Abhay Soi : There are two things. Firstly, it is not a new phenomenon that hospitals come up in locations where there are existing hospitals. And it is not as if in any micro market, it is like you have got five hospitals and three more have come up or four more have come up. It is pretty much been one here and one there .

But when that happens, you know your own clinicians tend to negotiate their compensations and it does go up from that standpoint. But having said tha t, it does even ou t and it is transfer red over a period of time to the patient because eventually, it's stil l a 10-12% PAT margin business , it is capital intensive, and we are reinvesting . It is a natural phen omenon. It gets a little lumpy sometimes. But it is not as if it is not passed throu gh - you will see this normalize across the board. I am not just saying it for Max, but I believe for the entire industry.

Tushar Manudhane : Got it sir this is helpful. Got it. Thanks a lot.

Moderator: Thank you. Next question is from th e line of Aditya Ch heda from InCred Asset Management.

Aditya Chheda : Hi, thank you for the opportunity. It seems like confusion regarding the discontinuation of c hemo drugs due to MoU conditionalities, this is specific to Max Healthcare in a region or th is is industry -wide pan-India. And if I understood it correct, it had a knock of negative impact on revenue to the tune of INR130 crore, and they had around 16 -17% EBITDA margins, if you can help clarify that?

Abhay Soi: Firstly, this is an industry -wide p henomenon. Secondly , as a proportion of its business, Max perhaps does the maximum amount of oncology business. And then within that, if you actua lly see, we probably do the maximum amount of institutional business as well , which is CGHS , ECHS , etc. So, al though it is an

Page 14 of 16 industry -wide phenomenon, the impact would be felt maximum by us for these reasons.

Yogesh Sareen : The overall impact of the discontinuation of these drugs is IN R 200 crore in the drug billing, i.e. top line.

Aditya Chheda: Is this the quarterly impact ?

Yogesh Sareen : This is the annual number. Annual billing of chemotherapy drugs will come down by INR 200 crore because of the discontinuation.

Aditya Chheda: Got it. Thank you. Thanks for the clarification.

Moderator: Thank you. Next question is from the line of Abdulkader Puranwala from I CICI Securities.

Abdulkader Puranwala: Just a follow-up on this CGHS part. So, if I see the contribution from CGHS, it is barely moved the needle. And where our onco revenues are seeing a sizable dip.

And within that, if you could help us understand that what portion of your CGHS revenues actually comes from oncology and how should we look ahead as well?

Yogesh Sareen: Obviously, there's a lesser impact on the beds for this. These chemotherapies are billed in the day care. When you discontinue these drug billings or providing these drugs to the patient, there's an impact on the revenue but there's no impact on the beds. Nevertheless, there are some of these patients who also then avail surgeries in the network and some of these patients are also then admitted in the hospital. To that end, the Occupied Bed Days (OBDs) for oncology patients have come down by 6% YoY. Now your question is that within the overall CGHS business, how much is oncology? That'll be probably around 35-40%.

Keshav Gupta: It used to be 50% as it is reduced to 40% now.

Abdulkader Puranwala: Understood, sir. And sir, then on your existing Network that is prior to any bed additions that you have done, in terms of the steady state revenue and EBITDA growth, sir, how should we look at in terms of your, say, bed network, what you had developed for '25 or for '26. And the levers of growth, would it be more ARPOB driven? There is some element of case mix as well, which can help to at least post a single digit kind of a growth.

Yogesh Sareen : Basically, there are two elements which make the revenue go up. One is the ARPOB and other is the Occupied Bed Days (OBDs). You know that even at the existing hospitals, we are adding more beds. There is brownfield expansion being done in Mohali, Nanavati, Lucknow and Smart. Our role is to make sure that where we have 80-85% occupancies, then we add more beds there and try to ensure that there is OBD growth also.

So, if we do not have OBD growth, then the only growth would be the ARPOB growth rate, which will be, let us say, 6-7% of the revenues. But our role is to ensure that wherever we find this high-occupancy situation, then we add more beds.

Abdulkader Puranwala: Okay understood. And sir, last one from my end. Would it be possible to quantify the EBITDA drag with the new hospitals would have had in Q4 and for the full year '26?

Page 15 of 16 Yogesh Sareen: No, it is very tough to do it now. You can do it in the probably first month or two, but then the patients start to mingle. So, when we put another Brownfield tower, we will not be able to separate the revenues or EBITDA of Tower 1 and Tower 2.

Abhay Soi: Because what you also do, for example, is move certain specialties into A-Tower versus B-Tower.

So, when you move those specialties, then wherever you move it from, those beds get occupied by other specialties and where you move it to, that gets occupied, so, both will have its own trajectory. Let's say, if I move oncology into a new tower, then that becomes an onco-tower. So, in the previous place where it was occupying those beds, those beds get occupied by other specialties. Whereas in the new tower, you will have largely oncology, so you will have different ARPOBs and different EBITDA per bed, etc.

Abdulkader Puranwala: Okay, understood.

Abhay Soi: But on a consolidated basis, what we have seen is that there is no real pressure on margins.

Abdulkader Puranwala: Okay, got it.

Moderator: Thank you. Next question is from the line of Lavanya from UBS Securities India Private Ltd.

Lavanya: Hello. Thank you for the opportunity, sir. Just a clarification on chemo drugs, again, sorry, here, we are losing out on OBD

also. So, if it is only the dispensary

and the drugs to who are -- I mean, to whom we are losing the OBD patients in general, the 5-6% impact on OBD, clarification on that will be great? And Q1 so

far -- I mean until now, you are seeing a full impact of this chemo drugs, at least, right?

Yogesh Sareen: That is correct. Quarter 4 has already had the full impact. We started to discontinue these drugs in Oct '25. So, to that extent, quarter 4 has the full impact of discontinuation of all these chemo drugs. What we have not gotten yet is the full impact of the price gain on the CGHS side, which will come starting this quarter. So, as I said, 2 of the hospitals have already got the super specialty rates. And during the course of the year, we will have more hospital also getting those rates.

Abhay Soi: So, on the other question that you have on the OBDs, whom are we losing to? Well, we do not exactly know. They go, perhaps, to the disorganized sectors or smaller nursing homes.

Lavanya: Okay. Got it. And here do we expect any resolution or this is going to be there for the next couple of quarters and until the base of Q3, like it will start to...

Abhay Soi: Like I said earlier, it does not seem to stand to reason, because it is not as if our margins are any different from the price at which perhaps the government is buying it. These are medicines like Keytruda, which is the largest and the fastest-growing oncology medicine in the world. And they do not make any exceptions for anybody in terms of what the margins are, for example, one is that.

Page 16 of 16 Secondly, the CGHS and CGHS rate applied to the other public sector undertakings and other panels like ECHS, which is ex-servicemen and so on. While CGHS patients have an option of procuring the drugs directly from the CGHS dispensary or their pharmacy. The ECHS and PSUs do not have that option because they do not have those pharmacies.

So of course, a lot of patients are complaining because they are unable to access these drugs, at least Keytruda, and they have to move to other medicines. And it is not a necessity, since every patient is different. For some patients, it is necessary. I guess they are finding it difficult to source this, so we are facing a lot of complaints, and I am sure so is the CGHS. And we are hoping that they will reconsider this. But asking for timelines, it is difficult to pin that down.

Lavanya: Okay. Got it. So just a clarification, ECHS on the PSU patients are seeing disruption to the treatment for the last 6 months or they are moving out to other medicines, whatever is alternate available?

Abhay Soi: Same thing. They are going to behave the same way as the CGHS patients.

Yogesh Sareen: Yes, and there are other alternatives to some of these medicines.

Lavanya: Okay, got it.

Abhay Soi: It is not that there are no alternatives to it, there are alternatives, okay? But they may not be preferred alternatives from a clinical standpoint. The question is the efficacy. If a doctor believes that this particular branded drug has more efficacy for a particular patient may not be for B patient, but for A particular patient. I mean, very simply, each one of us reacts differently to different paracetamol. Reacts differently to even a different antibiotic, let alone chemo.

Moderator: Thank you very much. Ladies and gentlemen, we will take that as the last question. I will now hand the conference over to the management for closing comments.

Abhay Soi: Thank you, everyone, for joining us today. We appreciate all your time and look forward to interacting with you again next quarter.

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